

Detail Report

CR Number: 2025290000		Implementation Date: 24-Apr-26	Related CR: 2025290000		MedDRA Code #
Change Requested	Add a New HLGT	Final Disposition	Final Placement		
Proposed HLGT	<i>Developmental and epileptic encephalopathies</i>	Rejected After Suspension			
To SOC	<i>Congenital, familial and genetic disorders</i>				

MSSO Comment: The proposal to add a new HLGT Developmental and epileptic encephalopathies to SOC Congenital, familial and genetic disorders is not approved after suspension. The proposed term is aligned with the International League Against Epilepsy (ILAE) classification system. While the ILAE classification has been developed by experts in the field of epilepsy, it is still evolving and is not yet widely adopted in clinical practice. In addition, the classification is highly specialized and includes rare epilepsy disorders. MedDRA is focused on and utilized by a broad user community who may not need these rare highly specific disorders organized in their own hierarchy structure. Some of the proposed LLT/PT concepts in this batch 10121839 were added but were accommodated in the existing MedDRA HLGT/HLT hierarchy. For analysis, customized search strategies may be used.

CR Number: 2025290002		Implementation Date: 24-Apr-26	Related CR: 2025290002		MedDRA Code #
Change Requested	Add a New HLT	Final Disposition	Final Placement		
Proposed HLT	<i>Neonate/infant onset developmental and epileptic encephalopathy syndromes</i>	Rejected After Suspension			
To HLGT	<i>Developmental and epileptic encephalopathies</i>				

MSSO Comment: The proposal to add a new HLT Neonate/infant onset developmental and epileptic encephalopathy syndromes to HLGT Developmental and epileptic encephalopathies is not approved after suspension. The proposed term is aligned with the International League Against Epilepsy (ILAE) classification system. While the ILAE classification has been developed by experts in the field of epilepsy, it is still evolving and is not yet widely adopted in clinical practice. In addition, the classification is highly specialized and includes rare epilepsy disorders. MedDRA is focused on and utilized by a broad user community who may not need these rare highly specific disorders organized in their own hierarchy structure. Some of the proposed LLT/PT concepts in this batch 10121839 were added but were accommodated in the existing MedDRA HLGT/HLT hierarchy. For analysis, customized search strategies may be used.

CR Number: 2025290003		Implementation Date: 24-Apr-26	Related CR: 2025290003		MedDRA Code #
Change Requested	Add a New HLT	Final Disposition	Final Placement		
Proposed HLT	<i>Childhood-onset developmental and epileptic encephalopathy syndromes</i>	Rejected After Suspension			
To HLGT	<i>Developmental and epileptic encephalopathies</i>				

MSSO Comment: The proposal to add a new HLT Childhood-onset developmental and epileptic encephalopathy syndromes to HLGT Developmental and epileptic encephalopathies is not approved after suspension. The proposed term is aligned with the International League Against Epilepsy (ILAE) classification system. While the ILAE classification has been developed by experts in the field of epilepsy, it is still evolving and is not yet widely adopted in clinical practice. In addition, the classification is highly specialized and includes rare epilepsy disorders. MedDRA is focused on and utilized by a broad user community who may not need these rare highly specific disorders organized in their own hierarchy structure. Some of the proposed LLT/PT concepts in this batch 10121839 were added but were accommodated in the existing MedDRA HLGT/HLT hierarchy. For analysis, customized search strategies may be used.

Detail Report

CR Number: 2025307001	Implementation Date: 26-Mar-26	Related CR: 2025307001	MedDRA Code #
Change Requested	Add a new SMQ	Final Disposition	Final Placement
Proposed SMQ	<i>Neurocognitive Impairment</i>	Rejected After Suspension	

MSSO Comment: The proposal to add a new SMQ Neurocognitive Impairment is not approved after suspension. The ICH M1 Points to Consider (PtC) Working group (an international team of regulatory and industry representatives) evaluated this request and does not recommend the development of this SMQ. Neurocognitive impairment as well as the proposed reworded SMQ name Neural impairment is a broad construct, and its classification varies across academic and clinical frameworks. A clear and consistently applied definition is essential for establishing appropriate inclusion and exclusion criteria for the SMQ, but the diversity of classification approaches and differing etiological criteria across groups makes this challenging. Additionally, there is a vast number of terms that could potentially be included and that vary from one product to another. Accordingly, a tailored approach to adverse event monitoring and risk assessment for these drug-specific effects is recommended, rather than reliance on a single, uniform framework. In conclusion, the combinations of conditions and their severity that represent neurocognitive/neural impairment and those that preclude safe driving are too variable to develop a standardized list of terms. Therefore, an SMQ is not considered an adequate methodology to address this safety topic.

CR Number: 2025336000	Implementation Date: 08-Jan-26	Related CR: 2025336000	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	<i>McMahon score risk of renal failure</i>	Approved Not as Requested	Proposed PT
			<i>McMahon score for renal failure</i>
			10093404
			HLT primary
			<i>Renal function analyses</i>
			10038454
			SOC primary
			<i>Investigations</i>
			10022891

MSSO Comment: The proposal to add a new PT McMahon score risk of renal failure is approved but not as requested. The McMahon score was developed to predict the risk of severe acute kidney injury or mortality in patients presenting to the hospital with rhabdomyolysis. The proposed term will be modified for MedDRA capitalization and clarity and added as PT McMahon score for renal failure to HLT Renal function analyses.

Detail Report

CR Number: 2025336002		Implementation Date: 08-Jan-26	Related CR: 2025336002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Intranodal haemangioma</i>	Approved as Requested	Proposed PT <i>Intranodal haemangioma</i>	10093402
			HLT primary <i>Haematologic neoplasms NEC</i>	10018845
			SOC primary <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary <i>Haematologic neoplasms NEC</i>	10018845
			SOC secondary <i>Blood and lymphatic system disorders</i>	10005329
			HLT secondary <i>Vascular neoplasms</i>	10057189
			SOC secondary <i>Vascular disorders</i>	10047065

MSSO Comment: The proposal to add a new term Intranodal haemangioma is approved as requested. Intranodal hemangiomas are rare benign vascular tumors of the lymph nodes, often misdiagnosed as malignant lymphadenopathies due to their clinical and radiological features. Intranodal haemangioma will be added as PT to bi-axial HLT Haematologic neoplasms NEC (primary SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps), secondary SOC Blood and lymphatic system disorders) and secondary HLT Vascular neoplasms. In a related change, the American English spelled counterpart LLT Intranodal hemangioma will also be added to new PT Intranodal haemangioma.

CR Number: 2026008001		Implementation Date: 08-Jan-26	Related CR: 2025336002	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Intranodal hemangioma</i>	Approved as Requested	Proposed LLT <i>Intranodal hemangioma</i>	10093403 Current
To PT	<i>Intranodal haemangioma</i>		To PT <i>Intranodal haemangioma</i>	10093402

MSSO Comment:

Detail Report

CR Number: 2025336003		Implementation Date: 08-Jan-26	Related CR: 2025336003	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Administration incorrect</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product administration error</i>		<i>Administration incorrect</i>	10001314 Non-Current
To PT	<i>Product administration issue</i>		From PT	
			<i>Product administration error</i>	10081576
			To PT	
			<i>Product administration issue</i>	10092935

MSSO Comment: The proposal to move the LLT Administration incorrect from PT Product administration error to PT Product administration issue is approved as requested for better alignment.

CR Number: 2025337000		Implementation Date: 08-Jan-26	Related CR: 2025337000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hyperglycemic glycosuria</i>	Rejected		

MSSO Comment: The proposal to add a new term Hyperglycemic glycosuria is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. In addition, MedDRA generally does not qualify the most common etiology in the term name.

CR Number: 2025337001		Implementation Date: 08-Jan-26	Related CR: 2025337001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Uterine transposition</i>	Approved Not as Requested	Proposed PT	
HLT primary	<i>Fertility and fertilisation interventions female</i>		<i>Uterine transposition procedure</i>	10093405
SOC primary	<i>Surgical and medical procedures</i>		HLT primary	
			<i>Fertility and fertilisation interventions female</i>	10016471
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new PT Uterine transposition to primary HLT Fertility and fertilisation interventions female in SOC Surgical and medical procedures is approved but not as requested. Uterine transposition is a procedure for preserving gonadal and uterine function in patients requiring pelvic radiotherapy for non-gynecological cancer, with the potential for achieving spontaneous pregnancy and successful delivery. The proposed term will be modified for clarity and added as PT Uterine transposition procedure to HLT Fertility and fertilisation interventions female.

Detail Report

CR Number: 2025337002		Implementation Date: 08-Jan-26	Related CR: 2025337002	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	Cystic thyroid nodules	Approved Not as Requested	Proposed LLT Cystic thyroid nodule	10093406 Current
			To PT Thyroid mass	10058900
MSSO Comment: The proposal to add a new term Cystic thyroid nodules is approved but not as requested. Thyroid nodules are common, and a large proportion have mixed cystic and solid components. In some studies, a nodule is called a cyst only if it is predominantly cystic on ultrasonography; however, in others, the term is applied to nodules that have any areas of cystic degeneration, which may include up to 50 percent of thyroid nodules. The proposed term will be modified into the singular form and added as LLT Cystic thyroid nodule to PT Thyroid mass.				

CR Number: 2025337005		Implementation Date: 13-Mar-26	Related CR: 2025337005	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Vascular embolic device unintended vessel occlusion	Approved After Suspension	Proposed PT Vascular embolic device unintended vessel occlusion	10093570
			HLT primary Vascular complications associated with device	10069789
			SOC primary General disorders and administration site conditions	10018065
			HLT secondary Non-site specific necrosis and vascular insufficiency NEC	10029558
			SOC secondary Vascular disorders	10047065
MSSO Comment: The proposal to add a new PT Vascular embolic device unintended vessel occlusion is approved as requested after suspension. Vascular embolic devices, used to block blood flow for conditions like aneurysms, carry risks of unintended vessel occlusion, where they block the wrong vessel (non-target embolization), migrate distally, or cause blockage due to clumping. Vascular embolic device unintended vessel occlusion will be added as PT to primary HLT Vascular complications associated with device, and to secondary HLT Non-site specific necrosis and vascular insufficiency NEC.				

Detail Report

CR Number: 2025337006		Implementation Date: 13-Mar-26	Related CR: 2025337006	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device inadequate penetration</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device inadequate penetration</i>	10093573 Current
			To PT <i>Vascular embolic device complication during placement</i>	10093571

MSSO Comment: The proposal to add a new PT Vascular embolic device inadequate penetration is approved but not as requested after suspension. Inadequate penetration of a vascular embolic device occurs when the embolic material fails to reach the target distal vasculature, often leading to premature proximal occlusion, inadequate treatment of the vascular lesion (such as in AVMs or tumors), or unintended non-target embolization. Vascular embolic device inadequate penetration will be added as sub-concept LLT to new PT Vascular embolic device complication during placement. In a related change, new PT Vascular embolic device complication during placement will be added to primary HLT Vascular complications associated with device, to secondary HLT Device issues NEC and to secondary HLT Non-site specific vascular disorders NEC.

CR Number: 2026072005		Implementation Date: 13-Mar-26	Related CR: 2025337006	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device complication during placement</i>	Approved as Requested	Proposed PT <i>Vascular embolic device complication during placement</i>	10093571
HLT primary	<i>Vascular complications associated with device</i>		HLT primary <i>Vascular complications associated with device</i>	10069789
SOC primary	<i>General disorders and administration site conditions</i>		SOC primary <i>General disorders and administration site conditions</i>	10018065
			HLT secondary <i>Device issues NEC</i>	10069794
			SOC secondary <i>Product issues</i>	10077536
			HLT secondary <i>Non-site specific vascular disorders NEC</i>	10047067
			SOC secondary <i>Vascular disorders</i>	10047065

MSSO Comment:

Detail Report

CR Number: 2025337007		Implementation Date: 13-Mar-26	Related CR: 2025337007	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device excessive penetration</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device excessive penetration</i> To PT <i>Vascular embolic device complication during placement</i>	10093582 Current 10093571
MSSO Comment: The proposal to add a new PT Vascular embolic device excessive penetration is approved but not as requested after suspension. Excessive penetration of vascular embolic devices refers to the unintended, deep migration of embolic agents (liquids, beads, or coils) beyond the target area, or into non-target vessels. Vascular embolic device excessive penetration will be added as sub-concept LLT to new PT Vascular embolic device complication during placement added in change request 2025337006.				

CR Number: 2025337008		Implementation Date: 18-May-26	Related CR: 2025337008	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device poor visualization</i>	Rejected After Suspension		
MSSO Comment: The proposal to add a new PT Vascular embolic device poor visualization is not approved after suspension. The proposed concept is ambiguous and may be interpreted in numerous ways such as a procedural problem, complication associated with device or even an intrinsic device issue.				

CR Number: 2025337009		Implementation Date: 13-Mar-26	Related CR: 2025337009	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device premature polymerization</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device premature polymerization</i> To PT <i>Vascular embolic device complication during placement</i>	10093575 Current 10093571
MSSO Comment: The proposal to add a new PT Vascular embolic device premature polymerization is approved but not as requested after suspension. Premature polymerization in vascular embolic devices, particularly liquid embolic agents like N-butyl cyanoacrylate (NBCA) or ethylene vinyl alcohol copolymer (EVOH/Onyx), is a significant complication where the agent solidifies inside the delivery catheter or before reaching the target vessel. Vascular embolic device premature polymerization will be added as sub-concept LLT to new PT Vascular embolic device complication during placement added in change request 2025337006. In a related change, the British English counterpart LLT Vascular embolic device premature polymerisation will also be added to PT Vascular embolic device complication during placement.				

Detail Report

CR Number: 2026072006		Implementation Date: 13-Mar-26	Related CR: 2025337009		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Vascular embolic device premature polymerisation</i>	Approved as Requested	Proposed LLT		
To PT	<i>Vascular embolic device complication during placement</i>		<i>Vascular embolic device premature polymerisation</i>	10093585	Current
			To PT		
			<i>Vascular embolic device complication during placement</i>	10093571	

MSSO
Comment:

CR Number: 2025337010		Implementation Date: 13-Mar-26	Related CR: 2025337010		
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Vascular embolic device prolonged polymerization</i>	Approved Not as Requested After Suspension	Proposed LLT		
			<i>Vascular embolic device prolonged polymerization</i>	10093577	Current
			To PT		
			<i>Vascular embolic device complication during placement</i>	10093571	

MSSO
Comment: The proposal to add a new PT Vascular embolic device prolonged polymerization is approved but not as requested after suspension. Vascular embolic device prolonged polymerization refers to delayed solidification, risking non-target spread of the embolic device. Vascular embolic device prolonged polymerization will be added as sub-concept LLT to new PT Vascular embolic device complication during placement added in change request 2025337006. In a related change, the British English counterpart LLT Vascular embolic device prolonged polymerisation will also be added to PT Vascular embolic device complication during placement.

CR Number: 2026072007		Implementation Date: 13-Mar-26	Related CR: 2025337010		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Vascular embolic device prolonged polymerisation</i>	Approved as Requested	Proposed LLT		
To PT	<i>Vascular embolic device complication during placement</i>		<i>Vascular embolic device prolonged polymerisation</i>	10093584	Current
			To PT		
			<i>Vascular embolic device complication during placement</i>	10093571	

MSSO
Comment:

Detail Report

CR Number: 2025337011		Implementation Date: 13-Mar-26	Related CR: 2025337011	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device catheter entrapment</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device catheter entrapment</i> To PT <i>Vascular embolic device complication during placement</i>	10093580 Current 10093571
MSSO Comment: The proposal to add a new PT Vascular embolic device catheter entrapment is approved but not as requested after suspension. Vascular embolic device catheter entrapment occurs when the catheter becomes stuck in the embolic material. Vascular embolic device catheter entrapment will be added as sub-concept LLT to new PT Vascular embolic device complication during placement added in change request 2025337006.				

CR Number: 2025337012		Implementation Date: 13-Mar-26	Related CR: 2025337012	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device cast degradation</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device cast degradation</i> To PT <i>Vascular embolic device cast complication</i>	10093574 Current 10093572
MSSO Comment: The proposal to add a new PT Vascular embolic device cast degradation is approved but not as requested after suspension. Vascular embolic device cast degradation occurs when there is breakdown of the solidified embolic cast over time. Vascular embolic device cast degradation will be added as sub-concept LLT to new PT Vascular embolic device cast complication which will be added, in a related change to primary HLT Vascular complications associated with device, to secondary HLT Non-site specific vascular disorders NEC and to secondary HLT Device issues NEC.				

Detail Report

CR Number: 2026072004

Implementation Date: 13-Mar-26

Related CR: 2025337012

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Vascular embolic device cast complication</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Vascular complications associated with device</i>		<i>Vascular embolic device cast complication</i>	10093572
SOC primary	<i>General disorders and administration site conditions</i>		HLT primary	
			<i>Vascular complications associated with device</i>	10069789
			SOC primary	
			<i>General disorders and administration site conditions</i>	10018065
			HLT secondary	
			<i>Non-site specific vascular disorders NEC</i>	10047067
			SOC secondary	
			<i>Vascular disorders</i>	10047065
			HLT secondary	
			<i>Device issues NEC</i>	10069794
			SOC secondary	
			<i>Product issues</i>	10077536

MSSO
Comment:

CR Number: 2025337013

Implementation Date: 13-Mar-26

Related CR: 2025337013

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Vascular embolic device cast fragmentation</i>	Approved Not as Requested After Suspension	Proposed LLT	
			<i>Vascular embolic device cast fragmentation</i>	10093578 Current
			To PT	
			<i>Vascular embolic device cast complication</i>	10093572

MSSO
Comment: The proposal to add a new PT Vascular embolic device cast fragmentation is approved but not as requested after suspension. Vascular embolic device cast fragmentation occurs when the cast breaks into smaller pieces. Vascular embolic device cast fragmentation will be added as sub-concept LLT to new PT Vascular embolic device cast complication (added in change request 2025337012).

Detail Report

CR Number: 2025337014		Implementation Date: 13-Mar-26	Related CR: 2025337014	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device cast extrusion</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device cast extrusion</i>	10093576 Current
			To PT <i>Vascular embolic device cast complication</i>	10093572
MSSO Comment: The proposal to add a new PT Vascular embolic device cast extrusion is approved but not as requested after suspension. Vascular embolic device cast extrusion occurs when the cast protrudes outside the intended vessel or tissue. Vascular embolic device cast extrusion will be added as sub-concept LLT to new PT Vascular embolic device cast complication added in change request 2025337012.				
CR Number: 2025337015		Implementation Date: 13-Mar-26	Related CR: 2025337015	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device cast expansion</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device cast expansion</i>	10093583 Current
			To PT <i>Vascular embolic device cast complication</i>	10093572
MSSO Comment: The proposal to add a new PT Vascular embolic device cast expansion is approved but not as requested after suspension. Vascular embolic device cast expansion occurs when the cast enlarges after deployment, affecting adjacent structures. Vascular embolic device cast expansion will be added as sub-concept LLT to new PT Vascular embolic device cast complication added in change request 2025337012.				
CR Number: 2025337016		Implementation Date: 13-Mar-26	Related CR: 2025337016	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device cast migration</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device cast migration</i>	10093579 Current
			To PT <i>Vascular embolic device cast complication</i>	10093572
MSSO Comment: The proposal to add a new PT Vascular embolic device cast migration is approved but not as requested after suspension. Vascular embolic device cast migration occurs when there is displacement of the cast from its original position. Vascular embolic device cast migration will be added as sub-concept LLT to new PT Vascular embolic device cast complication added in change request 2025337012.				

Detail Report

CR Number: 2025337017		Implementation Date: 13-Mar-26	Related CR: 2025337017	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vascular embolic device cast toxicity</i>	Approved Not as Requested After Suspension	Proposed LLT <i>Vascular embolic device cast toxicity</i> To PT <i>Vascular embolic device cast complication</i>	10093581 Current 10093572
MSSO Comment: The proposal to add a new PT Vascular embolic device cast toxicity is approved but not as requested after suspension. Vascular embolic device cast toxicity refers to local or systemic toxic effects from the embolic material. Vascular embolic device cast toxicity will be added as sub-concept LLT to new PT Vascular embolic device cast complication added in change request 2025337012.				

CR Number: 2025338000		Implementation Date: 08-Jan-26	Related CR: 2025338000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Statin induced myalgia</i>	Rejected		
MSSO Comment: The proposal to add a new LLT Statin induced myalgia is not approved. The MSSO considers this proposed term to be of a "due to" nature. Terms that state or imply a "due to" concept are generally not added to MedDRA because causality is not inherent in the coded term, but is instead determined by the reporter/investigator or by the sponsor.				

CR Number: 2025338001		Implementation Date: 08-Jan-26	Related CR: 2025338001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Neutropenic ulcerative mucositis</i>	Rejected		
MSSO Comment: The proposal to add a new LLT Neutropenic ulcerative mucositis is not approved. Neutropenic ulcerative mucositis is a severe inflammatory and ulcerative injury of the oral and gastrointestinal mucosa that occurs in the setting of profound neutropenia, most commonly following intensive chemotherapy or hematopoietic stem-cell transplantation. It results from impaired mucosal regeneration combined with an inadequate neutrophil-mediated immune response, leading to painful ulcerations and risk of secondary infection. MedDRA cannot accommodate a specific term for every etiology and/or pathophysiology relationship. The concept can be represented by existing MedDRA terms. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.				

Detail Report

CR Number: 2025338002		Implementation Date: 08-Jan-26	Related CR: 2025338002		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Drug associated acute kidney injury</i>	Rejected			

MSSO Comment: The proposal to add a new term Drug associated acute kidney injury is not approved. The MSSO considers this proposed term to be of a “due to” nature. Terms that state or imply a “due to” concept are generally not added to MedDRA because causality is not inherent in the coded term, but is instead determined by the reporter/investigator or by the sponsor. As kidney injury is an important drug-induced concept, it is available in MedDRA as LLT Drug-induced kidney injury. Further qualifiers will not be added, though. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.

CR Number: 2025338009		Implementation Date: 08-Jan-26	Related CR: 2025338009		MedDRA Code #
Change Requested	Promote an LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Fibroadenosis of breast</i>	Approved Not as Requested	Proposed LLT		
From PT	<i>Fibroadenoma of breast</i>		<i>Fibroadenosis of breast</i>	10016617	Current
			From PT		
			<i>Fibroadenoma of breast</i>	10016613	
			To PT		
			<i>Fibrocystic breast disease</i>	10016621	

MSSO Comment: The proposal to promote the LLT Fibroadenosis of breast from PT Fibroadenoma of breast is approved but not as requested. Fibroadenosis is a form of diffuse benign fibrocystic change rather than a discrete tumor. It fits better under the umbrella PT of Fibrocystic breast disease because that PT encompasses a range of benign, non-neoplastic changes in the breast tissue, including things like fibrosis, breast adenosis, small cysts, and other related changes. LLT Fibroadenosis of breast will be moved from PT Fibroadenoma of breast to PT Fibrocystic breast disease. In a related change, LLT Breast fibroadenosis, LLT Fibroadenosis breast, LLT Fibroadenosis of breast (benign) will all be moved from PT Fibroadenoma of breast to PT Fibrocystic breast disease. Furthermore, PT Fibroadenomatoid mastopathy will be demoted under PT Fibrocystic breast disease, because it represents a non-standardized, informal descriptor for mixed fibrocystic and fibroadenomatoid changes rather than a formally recognized category in breast pathology.

Detail Report

CR Number: 2026008003		Implementation Date: 08-Jan-26	Related CR: 2025338009		
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Fibroadenosis breast</i>	Approved as Requested	Proposed LLT		
From PT	<i>Fibroadenoma of breast</i>		<i>Fibroadenosis breast</i>	10016616 Current	
To PT	<i>Fibrocystic breast disease</i>		From PT		
			<i>Fibroadenoma of breast</i>	10016613	
			To PT		
			<i>Fibrocystic breast disease</i>	10016621	

MSSO
Comment:

CR Number: 2026008004		Implementation Date: 08-Jan-26	Related CR: 2025338009	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Fibroadenosis of breast (benign)</i>	Approved as Requested	Proposed LLT	
From PT	<i>Fibroadenoma of breast</i>		<i>Fibroadenosis of breast (benign)</i>	10016618 Current
To PT	<i>Fibrocystic breast disease</i>		From PT	
			<i>Fibroadenoma of breast</i>	10016613
			To PT	
			<i>Fibrocystic breast disease</i>	10016621

MSSO
Comment:

CR Number: 2026008005		Implementation Date: 08-Jan-26	Related CR: 2025338009	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Fibroadenomatoid mastopathy</i>	Approved as Requested	PT to merge	
To PT	<i>Fibrocystic breast disease</i>		<i>Fibroadenomatoid mastopathy</i>	10085280 Current
			To PT	
			<i>Fibrocystic breast disease</i>	10016621

MSSO
Comment:

Detail Report

CR Number: 2026008002		Implementation Date: 08-Jan-26	Related CR: 2025338009	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Breast fibroadenosis	Approved as Requested	Proposed LLT	10006249 Current
From PT	Fibroadenoma of breast		Breast fibroadenosis	
To PT	Fibrocystic breast disease		From PT	10016613
			Fibroadenoma of breast	
			To PT	10016621
			Fibrocystic breast disease	

MSSO
Comment:

CR Number: 2025339000		Implementation Date: 08-Jan-26	Related CR: 2025339000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Venous recanalization procedure	Approved as Requested	Proposed LLT	10093408 Current
			Venous recanalization procedure	
			To PT	10093407
			Venous recanalisation procedure	

MSSO
Comment: The proposal to add a new LLT Venous recanalization procedure is approved as requested. A venous recanalization procedure is a minimally invasive intervention that restores blood flow in veins that are blocked or narrowed, often due to thrombosis or chronic occlusion. It typically involves techniques such as angioplasty, stenting, or catheter-directed thrombolysis to reopen the affected vein and improve circulation. Venous recanalization procedure will be added as an LLT to a new PT Venous recanalisation procedure. In a related change, PT Venous recanalisation procedure will be added to HLT Venous therapeutic procedures.

CR Number: 2026008006		Implementation Date: 08-Jan-26	Related CR: 2025339000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Venous recanalisation procedure	Approved as Requested	Proposed PT	10093407
HLT primary	Venous therapeutic procedures		Venous recanalisation procedure	
SOC primary	Surgical and medical procedures		HLT primary	10047231
			Venous therapeutic procedures	
			SOC primary	10042613
			Surgical and medical procedures	

MSSO
Comment:

Detail Report

CR Number: 2025339001		Implementation Date: 08-Jan-26	Related CR: 2025339001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Chest wall hernia	Approved Not as Requested	Proposed PT	
			Chest wall hernia	10093409
			HLT primary	
			Thoracic musculoskeletal disorders	10043475
			SOC primary	
			Respiratory, thoracic and mediastinal disorders	10038738
			HLT secondary	
			Musculoskeletal and connective tissue conditions NEC	10080711
			SOC secondary	
			Musculoskeletal and connective tissue disorders	10028395

MSSO Comment: The proposal to add a new LLT Chest wall hernia is approved but not as requested. A chest wall hernia occurs when lung tissue or other contents push out through a weak spot or opening in the chest wall, often appearing as a bulge, and can be caused by trauma, surgery (iatrogenic) or severe coughing (spontaneous). Chest wall hernia will be added as PT with a primary link to HLT Thoracic musculoskeletal disorders and a secondary link to HLT Musculoskeletal and connective tissue conditions NEC.

CR Number: 2025339007		Implementation Date: 08-Jan-26	Related CR: 2025339007	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Neuropathy	Rejected		
From PT	Neuropathy peripheral			
To PT	Nervous system disorder			

MSSO Comment: The proposal to move the LLT Neuropathy from PT Neuropathy peripheral to PT Nervous system disorder is not approved. Although neuropathy, in a broader sense, can also be used to describe any disorder of the nerves, including less common or non-peripheral situations, it is used mainly to refer to disorders of the peripheral nervous system. Similar dysfunction within the CNS is termed myelopathy (spinal cord) or encephalopathy (brain). As well, moving the term would greatly impact legacy data.

Detail Report

CR Number: 2025339008		Implementation Date: 08-Jan-26	Related CR: 2025339008		MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement		
Proposed LLT	<i>Typhlitis</i>	Rejected			
From PT	<i>Colitis</i>				
To PT	<i>Neutropenic colitis</i>				
MSSO Comment:	The proposal to move the LLT Typhlitis from PT Colitis to PT Neutropenic colitis is not approved. Typhlitis is etymologically defined as inflammation of the cecum (cectitis) and is not intrinsically linked to neutropenia. Standard medical dictionaries define typhlitis and cecitis as synonymous. Preserving typhlitis under colitis therefore maintains terminological accuracy. To ensure clinical specificity where needed, the existing LLT Neutropenic typhlitis can be used.				

CR Number: 2025339009		Implementation Date: 08-Jan-26	Related CR: 2025339009		MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement		
Proposed LLT	<i>Typhlitis</i>	Rejected			
From PT	<i>Colitis</i>				
To PT	<i>Neutropenic colitis</i>				
MSSO Comment:	The proposal to move the LLT Typhlitis from PT Colitis to PT Neutropenic colitis is not approved. The proposed move has already been rejected several times, last time in CR 2025339008.				

CR Number: 2025341000		Implementation Date: 08-Jan-26	Related CR: 2025341000		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Periocular tinea</i>	Rejected			
MSSO Comment:	The proposal to add a new term Periocular tinea is not approved. Periocular tinea is not a distinct or formally recognized anatomical location of dermatophytosis. It is best understood as one possible localization of tinea faciei, and occasionally an extension of tinea capitis. MedDRA cannot accommodate a specific term for every condition or disorder with a respective anatomical location. The concept can be represented with less anatomic specificity by terms such as PT Tinea faciei.				

Detail Report

CR Number: 2025341001		Implementation Date: 15-Jan-26	Related CR: 2025341001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Right carotid adventitia denudation</i>	Approved Not as Requested After Suspension	Proposed PT <i>Carotid adventitial denudation procedure</i>	10093468
			HLT primary <i>Autonomic nerve therapeutic procedures</i>	10003838
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Right carotid adventitia denudation is approved but not as requested after suspension. Carotid adventitial denudation is a surgical procedure in which the adventitial layer of the carotid artery is stripped to disrupt perivascular sympathetic nerve fibers. It produces localized sympathetic denervation of the carotid artery without removal of the cervical sympathetic ganglia. The proposed term will be modified for clarity and removal of laterality and added as PT Carotid adventitial denudation procedure to HLT Autonomic nerve therapeutic procedures.

CR Number: 2025341002		Implementation Date: 08-Jan-26	Related CR: 2025341002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Photobiomodulation</i>	Rejected		

MSSO Comment: The proposal to add a new term Photobiomodulation is not approved. The proposed concept has recently been added to MedDRA as LLT Photobiomodulation therapy in change request 2025300001 and will be available in Version 29.0.

CR Number: 2025341003		Implementation Date: 08-Jan-26	Related CR: 2025341003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Gastrohelcosis</i>	Rejected		

MSSO Comment: The proposal to add a new term Gastrohelcosis is not approved. The suffix -helcosis is Greek for ulceration. Although the term can be understood despite its non-standard construction, it is not formally adopted in clinical practice. Instead, clinicians use "gastric ulcer" or "peptic ulcer" in medical English, which are existing MedDRA concepts.

Detail Report

CR Number: 2025341004	Implementation Date: 08-Jan-26	Related CR: 2025341004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Nodules of arantius</i>	Rejected	

MSSO Comment: The proposal to add a new term Nodules of arantius is not approved. The nodules of Arantius (semilunar valve nodules) are normal anatomical structures, not pathological findings. If you are considering a specific pathology, please resubmit with clarification of term name, the specific need for the term and relevant scientific literature.

CR Number: 2025341005	Implementation Date: 08-Jan-26	Related CR: 2025341005	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Non-contrast computed tomography of the paranasal sinuses</i>	Rejected	

MSSO Comment: The proposal to add a new term Non-contrast computed tomography of the paranasal sinuses is not approved. Paranasal sinus CT scans are typically performed without contrast to evaluate the anatomy of the sinuses, detect sinus opacification, mucosal thickening, polyps, or bony changes. MedDRA generally does not qualify the most common presentation of a concept, i.e. non-contrast, in the term name. Please consider LLT CT scan of paranasal sinus under PT Computerised tomogram head depending on your clinical scenario.

CR Number: 2025341006	Implementation Date: 08-Jan-26	Related CR: 2025341006	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Ct of sinuses</i>	Rejected	

MSSO Comment: The proposal to add a new term Ct of sinuses is not approved. The proposed concept is anatomical vague and ambiguous. If referring to the paranasal sinuses, please consider existing LLT CT scan of paranasal sinus.

Detail Report

CR Number: 2025341007	Implementation Date: 12-Jan-26	Related CR: 2025341007	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Periorificial dermatitis.	Rejected After Suspension	

MSSO Comment: The proposal to add a new term Periorificial dermatitis. is not approved after suspension. Periorificial dermatitis refers to a dermatitis localized around any orifice, e.g. eyes, nostrils, mouth, and occasionally the genitals. As such it is an anatomically imprecise and unspecific term.

CR Number: 2025342000	Implementation Date: 09-Jan-26	Related CR: 2025342000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Low-grade fibromyxoid sarcoma (LGFMS)	Approved Not as Requested	Proposed PT
To PT	Soft tissue sarcoma		Low-grade fibromyxoid sarcoma
			10093410
			HLT primary
			Fibrosarcomas malignant
			10016634
			SOC primary
			Neoplasms benign, malignant and unspecified (incl cysts and polyps)
			10029104
			HLT secondary
			Malignant musculoskeletal and connective tissue neoplasms
			10025681
			SOC secondary
			Musculoskeletal and connective tissue disorders
			10028395

MSSO Comment: The proposal to add a new LLT Low-grade fibromyxoid sarcoma (LGFMS) to PT Soft tissue sarcoma is approved but not as requested. Low-grade fibromyxoid sarcoma is a malignant fibroblastic neoplasm characterized by alternating collagenous and myxoid areas, deceptively bland spindle cells with a whorling growth pattern, and arcades of small blood vessels. These tumours consistently have either FUS-CREB3L2 or FUS-CREB3L1 gene fusions. The proposed term will be modified by exclusion of the acronym and added as PT Low-grade fibromyxoid sarcoma to primary HLT Fibrosarcomas malignant to secondary HLT Malignant musculoskeletal and connective tissue neoplasms.

Detail Report

CR Number: 2025342001		Implementation Date: 09-Jan-26	Related CR: 2025342001		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Combined hamartoma of the retina and retinal pigment epithelium (CHRRPE)	Approved Not as Requested	Proposed LLT	Combined hamartoma of retina and retinal pigment epithelium	10093411 Current
To PT	Benign neoplasm of retina		To PT	Retinal hamartoma	10092433
MSSO Comment: The proposal to add a new LLT Combined hamartoma of the retina and retinal pigment epithelium (CHRRPE) to PT Benign neoplasm of retina is approved but not as requested. Simple and combined hamartomas of the retinal pigment epithelium (RPE) are hamartomas composed of normal retinal elements but lacking the distribution and organization of the normal retina and RPE. Simple hamartomas of the RPE involve only the RPE, whereas combined hamartomas of the retina and RPE involve cellular elements of both the retina and the RPE. The proposed term will be modified by exclusion of the acronym and added as LLT Combined hamartoma of retina and retinal pigment epithelium to PT Retinal hamartoma.					

CR Number: 2025342007		Implementation Date:	08-Jan-26	Related CR:	2025342007
Change Requested	Link (move) an LLT to another PT	Final Disposition		Final Placement	MedDRA Code #
Proposed LLT	<i>Needle cap not removed for injection</i>	Rejected			
From PT	<i>Device use error</i>				
To PT	<i>Device use issue</i>				
MSSO Comment:	The proposal to move the LLT Needle cap not removed for injection from PT Device use error to PT Device use issue is not approved. Needle cap not removed for injection would always be considered an error as it is always an inadvertent action and any other scenario is not plausible.				

CR Number: 2025342008		Implementation Date: 08-Jan-26	Related CR: 2025342008		
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Frozen solvent used in drug reconstitution	Approved as Requested	Proposed LLT	Frozen solvent used in drug reconstitution	10092454 Current
From PT	Product preparation error		From PT	Product preparation error	10076869
To PT	Product preparation issue		To PT	Product preparation issue	10081301
MSSO Comment: The proposal to move the LLT Frozen solvent used in drug reconstitution from PT Product preparation error to PT Product preparation issue is approved as requested. Frozen solvent used in drug reconstitution may not always be an error and will be moved for better alignment.					

Detail Report

CR Number: 2025342009		Implementation Date: 09-Jan-26	Related CR: 2025342009	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Typhlitis</i>	Rejected		
From PT	<i>Colitis</i>			
To PT	<i>Neutropenic colitis</i>			
MSSO Comment:	The proposal to move the LLT Typhlitis from PT Colitis to PT Neutropenic colitis is not approved. The proposed move has already been rejected several times, last time in change request 2025339008.			

CR Number: 2025343000		Implementation Date: 09-Jan-26	Related CR: 2025343000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Bacterial pcr test</i>	Rejected		
MSSO Comment:	The proposal to add a new term Bacterial pcr test is not approved. A bacterial PCR test is not a single, well-defined diagnostic entity but a broad methodological category. Its specificity is not inherent to PCR itself, but depends on the exact assay design: the genetic target, the organisms included, the type of sample, and the clinical purpose. Some PCRs are highly specific (species- or gene-targeted), while others are intentionally broad (e.g., 16S or multiplex panels). Therefore, without further specification, the term is imprecise and heterogeneous. On the other hand, bacterial PCR tests are methodologically subsumed within multipathogen assays when these include bacteria, which can be represented by existing PT Multipathogen PCR test.			

Detail Report

CR Number: 2025343003		Implementation Date: 13-Feb-26	Related CR: 2025343003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Persistent spinal pain syndrome</i>	Approved After Suspension	Proposed PT <i>Persistent spinal pain syndrome</i>	10093520
			HLT primary <i>Musculoskeletal and connective tissue pain and discomfort</i>	10068757
			SOC primary <i>Musculoskeletal and connective tissue disorders</i>	10028395
			HLT secondary <i>Musculoskeletal procedural complications</i>	10028392
			SOC secondary <i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary <i>Lumbar spinal cord and nerve root disorders</i>	10052772
			SOC secondary <i>Nervous system disorders</i>	10029205

MSSO
Comment: The proposal to add a new term Persistent spinal pain syndrome is approved as requested after suspension. Persistent spinal pain syndrome (PSPS) is a condition characterized by chronic pain of spinal origin. PSPS has two types: type 1 refers to pain without associated surgery, while type 2 refers to pain resulting from or continuing after spinal surgery. PSPS type 2 was formerly known as failed back surgery syndrome. Persistent spinal pain syndrome will be added as PT, with a primary link to HLT Musculoskeletal and connective tissue pain and discomfort and to secondary HLT Musculoskeletal procedural complications and to secondary HLT Lumbar spinal cord and nerve root disorders. In a related change, PT Failed back surgery syndrome will be demoted under this new PT Persistent spinal pain syndrome, as it is now more commonly referred to as PSPS type 2.

CR Number: 2026044007		Implementation Date: 13-Feb-26	Related CR: 2025343003	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Failed back surgery syndrome</i>	Approved as Requested	PT to merge <i>Failed back surgery syndrome</i>	10067487 Current
To PT	<i>Persistent spinal pain syndrome</i>		To PT <i>Persistent spinal pain syndrome</i>	10093520

MSSO
Comment:

Detail Report

CR Number: 2025344000		Implementation Date: 09-Jan-26	Related CR: 2025344000		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	Adenotonsillar hypertrophy	Rejected			
HLT primary	Pharyngeal disorders (excl infections and neoplasms)				
SOC primary	Respiratory, thoracic and mediastinal disorders				
MSSO Comment:	The proposal to add a new PT Adenotonsillar hypertrophy to primary HLT Pharyngeal disorders (excl infections and neoplasms) in SOC Respiratory, thoracic and mediastinal disorders is not approved. The proposed term describes a condition at multiple anatomical locations. This concept may be represented in MedDRA by coding with more than one term, for example, LLT Adenoidal hypertrophy and LLT Tonsillar hypertrophy. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.				

CR Number: 2025344001		Implementation Date: 09-Jan-26	Related CR: 2025344001		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Vesicocolic fistula	Rejected			

MSSO Comment: The proposal to add a new LLT Vesicocolic fistula is not approved. The proposed concept exists as LLT Colovesical fistula.

CR Number: 2025344002		Implementation Date: 09-Jan-26	Related CR: 2025344002		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Parvovirus B19 IgM antibody test	Approved as Requested	Proposed LLT		
To PT	Parvovirus B19 test		Parvovirus B19 IgM antibody test	10093412	Current
			To PT		
			Parvovirus B19 test	10070366	

MSSO Comment: The proposal to add a new LLT Parvovirus B19 IgM antibody test to PT Parvovirus B19 test is approved as requested. Parvovirus B19 IgM antibodies indicate a recent or acute infection; a positive result appears early (approximately 7–10 days after infection), reflecting active or very recent infection, and usually becomes negative within weeks to months.

Detail Report

CR Number: 2025344003		Implementation Date: 09-Jan-26	Related CR: 2025344003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Parvovirus B19 IgG antibody test</i>	Approved as Requested	Proposed LLT	
To PT	<i>Parvovirus B19 test</i>		<i>Parvovirus B19 IgG antibody test</i>	10093413 Current
			To PT	
			<i>Parvovirus B19 test</i>	10070366

MSSO Comment: The proposal to add a new LLT Parvovirus B19 IgG antibody test to PT Parvovirus B19 test is approved as requested. Parvovirus B19 IgG antibodies indicate past exposure to Parvovirus B19; a positive result appears later after infection and usually persists for life, reflecting previous infection and long-term immunity rather than active disease.

CR Number: 2025344004		Implementation Date: 09-Jan-26	Related CR: 2025344004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Lymphocyte subset panel abnormal</i>	Approved as Requested	Proposed PT	
			<i>Lymphocyte subset panel abnormal</i>	10093414
			HLT primary	
			<i>White blood cell analyses</i>	10047938
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Lymphocyte subset panel abnormal is approved as requested. A laboratory result indicating that one or more lymphocyte subpopulations fall outside established reference ranges (such as reduced CD4+ T cells, an inverted CD4/CD8 ratio, expanded B-cell populations, or decreased NK cells) reflecting an underlying disturbance of immune homeostasis. Lymphocyte subset panel abnormal will be added as PT to HLT White blood cell analyses.

Detail Report

CR Number: 2025344005		Implementation Date: 09-Jan-26	Related CR: 2025344005	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Hallucinogen persisting perception disorder</i>	Approved as Requested	Proposed PT	
			<i>Hallucinogen persisting perception disorder</i>	10093415
			HLT primary	
			<i>Perception disturbances NEC</i>	10082213
			SOC primary	
			<i>Psychiatric disorders</i>	10037175

MSSO Comment: The proposal to add a new PT Hallucinogen persisting perception disorder is approved as requested. Hallucinogen persisting perception disorder involves the return, during sobriety, of sensory disturbances previously experienced while intoxicated with a hallucinogen. Although any sensory modality can be affected, visual phenomena are most common, including geometric hallucinations, peripheral motion illusions, flashes or intensification of color, image trails, visual snow, afterimages, halos, and distortions in object size. These disturbances may be intermittent or nearly constant, can endure for extended periods, and must generate meaningful distress or functional impairment. The diagnosis requires exclusion of alternative medical or psychiatric causes. Hallucinogen persisting perception disorder will be added as PT to primary HLT Perception disturbances NEC.

CR Number: 2025345000		Implementation Date: 09-Jan-26	Related CR: 2025345000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hyperimmune plasma transfusion</i>	Approved as Requested	Proposed PT	
			<i>Hyperimmune plasma transfusion</i>	10093416
			HLT primary	
			<i>Blood and blood product treatment</i>	10019028
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Hyperimmune plasma transfusion is approved as requested. Hyperimmune plasma transfusion is the therapeutic administration of plasma obtained from donors with deliberately high and standardized antibody titers against a specific pathogen, achieved through selection and/or targeted immunization, with the aim of providing potent passive immunity to the recipient. Hyperimmune plasma transfusion will be added as a PT to HLT Blood and blood product treatment.

Detail Report

CR Number: 2025346000		Implementation Date: 09-Jan-26	Related CR: 2025346000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Pneumothorax repair</i>	Approved Not as Requested	Proposed PT	
To PT	<i>Lung operation</i>		<i>Pneumothorax repair</i>	10093417
			HLT primary	
			<i>Pleural therapeutic procedures</i>	10035612
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new LLT Pneumothorax repair to PT Lung operation is approved but not as requested. Pneumothorax repair is a medical or surgical intervention aimed at re-expanding a collapsed lung and preventing recurrence by removing air from the pleural space and sealing the source of the air leak. Pneumothorax repair will be added as a PT to HLT Pleural therapeutic procedures.

CR Number: 2025349000		Implementation Date: 09-Jan-26	Related CR: 2025349000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement	
Proposed Term	<i>Desquamative interstitial pneumonia</i>	Approved as Requested	Proposed LLT	
			<i>Desquamative interstitial pneumonia</i>	10093418 Current
			To PT	
			<i>Idiopathic interstitial pneumonia</i>	10078268

MSSO Comment: The proposal to add a new term Desquamative interstitial pneumonia is approved as requested. Idiopathic interstitial pneumonias (IIPs) are a subset of diffuse parenchymal lung diseases of unknown etiology characterized by expansion of the interstitial compartment with an infiltrate of inflammatory cells. Desquamative interstitial pneumonia (DIP) is an important histologic sub-group of IIPs. DIP and respiratory bronchiolitis-associated interstitial lung disease are part of a histologic spectrum of macrophage accumulation, with the distinction dependent on the extent and distribution of this process. Desquamative interstitial pneumonia will be added as LLT to PT Idiopathic interstitial pneumonia.

CR Number: 2025349001		Implementation Date: 09-Jan-26	Related CR: 2025349001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement	
Proposed Term	<i>Food coloring allergy</i>	Rejected		

MSSO Comment: The proposal to add a new term Food coloring allergy is not approved. The concept exists in MedDRA as LLT Reaction to food coloring or alternative terms like LLT/PT Reaction to food additive or LLT/PT Allergic reaction to excipient. Please consider these for your specific needs.

Detail Report

CR Number: 2025349002		Implementation Date: 09-Jan-26	Related CR: 2025349002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Transcervical fracture of femoral neck</i>	Approved as Requested	Proposed LLT <i>Transcervical fracture of femoral neck</i>	10093419 Current
			To PT <i>Femoral neck fracture</i>	10016450
MSSO Comment: The proposal to add a new term Transcervical fracture of femoral neck is approved as requested. Transcervical fracture of femoral neck is a fracture in which the break runs across the mid-portion of the femoral neck, between the femoral head and the intertrochanteric region. Transcervical fracture of femoral neck will be added as LLT to PT Femoral neck fracture.				

CR Number: 2025349003		Implementation Date: 12-Jan-26	Related CR: 2025349003	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Prescription drug used from another patient</i>	Rejected		
From PT	<i>Prescription drug used without a prescription</i>			
MSSO Comment: The proposal to promote the LLT Prescription drug used from another patient from PT Prescription drug used without a prescription is not approved. The proposed unique scenario description is generally not the intended interpretation of the LLT Prescription drug used from another patient. Prescription is understood to mean a valid prescription for a person's own individual use.				

CR Number: 2025350000		Implementation Date: 09-Jan-26	Related CR: 2025350000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Small bowel feces sign (SBFS)</i>	Approved Not as Requested	Proposed LLT <i>Small bowel feces sign</i>	10093420 Current
			To PT <i>Small intestinal obstruction</i>	10041101
MSSO Comment: The proposal to add a new LLT Small bowel feces sign (SBFS) is approved but not as requested. The small bowel feces sign can be observed on abdominal CT and has been described as a finding specific for gradual small bowel obstruction or another severe small bowel abnormality. The proposed term will be modified by removing the acronym and will be added as an LLT Small bowel feces sign to PT Small intestinal obstruction. In a related change, the British English spelled counterpart LLT Small bowel faeces sign will also be added to PT Small intestinal obstruction.				

Detail Report

CR Number: 2026009005		Implementation Date: 09-Jan-26	Related CR: 2025350000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Small bowel faeces sign</i>	Approved as Requested	Proposed LLT	
To PT	<i>Small intestinal obstruction</i>		<i>Small bowel faeces sign</i>	10093421 Current
			To PT	
			<i>Small intestinal obstruction</i>	10041101

MSSO
Comment:

CR Number: 2025350001		Implementation Date: 12-Jan-26	Related CR: 2025350001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Singleton Merten syndrome</i>	Approved Not as Requested	Proposed PT	
			<i>Singleton-Merten syndrome</i>	10093453
			HLT primary	
			<i>Dental disorders congenital</i>	10044036
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Congenital cardiovascular disorders NEC</i>	10010402
			SOC secondary	
			<i>Cardiac disorders</i>	10007541
			HLT secondary	
			<i>Musculoskeletal and connective tissue conditions NEC</i>	10080711
			SOC secondary	
			<i>Musculoskeletal and connective tissue disorders</i>	10028395

MSSO
Comment: The proposal to add a new LLT Singleton Merten syndrome is approved but not as requested. Singleton-Merten dysplasia (or Singleton-Merten syndrome) is characterized by dental dysplasia, progressive calcification of the thoracic aorta with stenosis, osteoporosis and expansion of the marrow cavities in hand bones. Additional features include generalized muscle weakness and atrophy, and chronic psoriasiform skin eruptions. The proposed term will be modified and added including a hyphen as PT Singleton-Merten syndrome to primary HLT Dental disorders congenital, to secondary HLT Congenital cardiovascular disorders NEC, to secondary HLT Musculoskeletal and connective tissue conditions NEC, to secondary HLT Dental developmental disorders and anomalies and to secondary HLT Aortic necrosis and vascular insufficiency.

Detail Report

CR Number: 2026012002		Implementation Date: 12-Jan-26	Related CR: 2025350001	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Singleton-Merten syndrome</i>	Approved as Requested	Proposed PT	
To HLT	<i>Aortic necrosis and vascular insufficiency</i>		<i>Singleton-Merten syndrome</i>	10093453
			To HLT	
			<i>Aortic necrosis and vascular insufficiency</i>	10002901

MSSO
Comment:

CR Number: 2026012003		Implementation Date: 12-Jan-26	Related CR: 2025350001	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Singleton-Merten syndrome</i>	Approved as Requested	Proposed PT	
To HLT	<i>Dental developmental disorders and anomalies</i>		<i>Singleton-Merten syndrome</i>	10093453
			To HLT	
			<i>Dental developmental disorders and anomalies</i>	10012321

MSSO
Comment:

Detail Report

CR Number: 2025351000 Implementation Date: 12-Jan-26 Related CR: 2025351000

Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Urinary fungal mass	Approved Not as Requested	Proposed PT	
To PT	Urinary tract infection fungal		Urinary tract fungal bezoar	10093454
			HLT primary	
			Fungal infections NEC	10017536
			SOC primary	
			Infections and infestations	10021881
			HLT secondary	
			Renal obstructive disorders	10038488
			SOC secondary	
			Renal and urinary disorders	10038359

MSSO Comment: The proposal to add a new LLT Urinary fungal mass to PT Urinary tract infection fungal is approved but not as requested. Urinary tract fungus ball is a tumor-like mass formed by colonization of a fungus in a body cavity, usually a bronchus or pulmonary cavity but occasionally the nasal cavity or urinary tract. It may also be called a fungal bezoar. Proposed term will be modified to represent the most common medically referenced term and added as PT Urinary tract fungal bezoar to primary HLT Fungal infections NEC and to secondary HLT Renal obstructive disorders.

CR Number: 2025352000 Implementation Date: 09-Jan-26 Related CR: 2025352000

Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Renal vein transposition	Approved as Requested	Proposed PT	
			Renal vein transposition	10093427
			HLT primary	
			Venous therapeutic procedures	10047231
			SOC primary	
			Surgical and medical procedures	10042613

MSSO Comment: The proposal to add a new term Renal vein transposition is approved as requested. Renal vein transposition is a surgical procedure, often used for Nutcracker syndrome, where the left renal vein is moved and reattached to the inferior vena cava at a different, lower spot to relieve compression between the aorta and superior mesenteric artery. Renal vein transposition will be added as PT to HLT Venous therapeutic procedures.

Detail Report

CR Number: 2025352001		Implementation Date: 09-Jan-26	Related CR: 2025352001		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Seroperitoneum drainage</i>	Rejected			

MSSO Comment: The proposal to add a new term Seroperitoneum drainage is not approved. Seroperitoneum refers specifically to free fluid of serous nature. MedDRA cannot accommodate procedure terms of such granularity. Please consider existing LLTs under PT Abdominal cavity drainage for your specific coding needs.

CR Number: 2025352002		Implementation Date: 09-Jan-26	Related CR: 2025352002		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Iridocyclectomy</i>	Approved as Requested	Proposed PT <i>Iridocyclectomy</i>		10093432
			HLT primary <i>Iris and ciliary body therapeutic procedures</i>		10022947
			SOC primary <i>Surgical and medical procedures</i>		10042613

MSSO Comment: The proposal to add a new term Iridocyclectomy is approved as requested. Iridocyclectomy is a surgical procedure that involves the removal of a portion of the iris and the ciliary body. It is typically performed primarily for managing tumors of the anterior uveal tract, such as small melanomas of the iris or ciliary body, or occasionally for other localized anterior segment lesions. Iridocyclectomy will be added as PT to HLT Iris and ciliary body therapeutic procedures.

CR Number: 2025352003		Implementation Date: 09-Jan-26	Related CR: 2025352003		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Weaver-dunn procedure</i>	Approved Not as Requested	Proposed LLT <i>Weaver-Dunn procedure</i>		10093451 Current
			To PT <i>Shoulder operation</i>		10062242

MSSO Comment: The proposal to add a new term Weaver-dunn procedure is approved but not as requested. The Weaver-Dunn procedure is a surgical technique used to treat chronic acromioclavicular joint dislocations by transferring the coracoacromial ligament to the distal end of the clavicle to stabilize the joint. It often includes resection of the distal clavicle to reduce pain and improve shoulder function. The proposed term will be modified for MedDRA capitalization rules and added as LLT Weaver-Dunn procedure to PT Shoulder operation.

Detail Report

CR Number: 2025352004	Implementation Date: 09-Jan-26	Related CR: 2025352004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement

Proposed Term	<i>Single anastomosis duodenolleal bypass with sleeve gastrectomy</i>	Rejected
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MSSO Comment: The proposal to add a new term Single anastomosis duodenolleal bypass with sleeve gastrectomy is not approved. Please consider recently added LLT Single anastomosis sleeve-ileal bypass to PT Sleeve gastrectomy (change request 2025294004) for your coding needs.

CR Number: 2025352005	Implementation Date: 09-Jan-26	Related CR: 2025352005	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement

Proposed Term	<i>Clitoroplasty</i>	Approved as Requested	Proposed PT <i>Clitoroplasty</i>	10093433
			HLT primary <i>External female genital therapeutic procedures</i>	10015738
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Clitoroplasty is approved as requested. Clitoroplasty is a surgical procedure involving reduction, reconstruction, or creation of clitoral tissue with the goal of preserving sensation and function. It is used in contexts such as intersex care, gender-affirming surgery, reconstruction after trauma or female genital cutting, and—rarely and controversially—for elective cosmetic reasons. Clitoroplasty will be added as PT to HLT External female genital therapeutic procedures.

CR Number: 2025352006	Implementation Date: 09-Jan-26	Related CR: 2025352006	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement

Proposed Term	<i>Clitoropexy</i>	Approved as Requested	Proposed LLT <i>Clitoropexy</i>	10093440	Current
			To PT <i>Clitoroplasty</i>	10093433	

MSSO Comment: The proposal to add a new term Clitoropexy is approved as requested. Clitoropexy is a surgical procedure in which the clitoris is repositioned or anchored, rather than reduced or removed, to achieve a more typical anatomical position while preserving clitoral tissue and sensation. It is most commonly used in the management of certain differences of sex development (such as congenital adrenal hyperplasia) as a nerve-sparing alternative, and occasionally in reconstructive contexts following trauma or prior surgery. Clitoropexy will be added as a sub-concept LLT to new PT Clitoroplasty added in change request 2025352005.

Detail Report

CR Number: 2025352007		Implementation Date: 09-Jan-26	Related CR: 2025352007	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Laser hair removal</i>	Approved as Requested	Proposed LLT <i>Laser hair removal</i>	10093452 Current
			To PT <i>Depilation</i>	10012364

MSSO Comment: The proposal to add a new term Laser hair removal is approved as requested. Although laser hair removal is primarily used for cosmetic reasons, clinical uses include conditions where excess or problematic hair causes inflammation, infection, scarring, or hormonal/medical issues, such as hirsutism, pseudofolliculitis barbae, folliculitis, pilonidal disease, and hypertrichosis. It is also applied in gender-affirming care, wound management, and to prevent skin trauma or pigmentation changes from repeated hair removal. Laser hair removal will be added as LLT to PT Depilation.

CR Number: 2025352008		Implementation Date: 09-Jan-26	Related CR: 2025352008	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Phonomicrosurgery</i>	Approved as Requested	Proposed LLT <i>Phonomicrosurgery</i>	10093443 Current
			To PT <i>Vocal cord operation</i>	10077452

MSSO Comment: The proposal to add a new term Phonomicrosurgery is approved as requested. Phonomicrosurgery is a specialized form of microsurgery aimed at preserving or improving voice quality by precisely removing lesions or correcting structural abnormalities of the vocal folds. Phonomicrosurgery will be added as an LLT to PT Vocal cord operation.

CR Number: 2025352009		Implementation Date: 09-Jan-26	Related CR: 2025352009	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Metacarpal aplasia</i>	Approved as Requested	Proposed LLT <i>Metacarpal aplasia</i>	10093441 Current
			To PT <i>Congenital hand malformation</i>	10062335

MSSO Comment: The proposal to add a new term Metacarpal aplasia is approved as requested. Metacarpal aplasia is a rare congenital, developmental defect characterized by the complete absence of one or more metacarpal bones in the hand, often resulting in associated digit malformations. Metacarpal aplasia will be added as an LLT to PT Congenital hand malformation.

Detail Report

CR Number: 2025352010		Implementation Date: 09-Jan-26	Related CR: 2025352010	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Levatorplasty	Approved Not as Requested	Proposed PT Anal levatorplasty	10093428
			HLT primary Anorectal therapeutic procedures	10002580
			SOC primary Surgical and medical procedures	10042613
MSSO Comment: The proposal to add a new term Levatorplasty is approved but not as requested. Anal levatorplasty is a surgical procedure involving the levator ani muscle (pelvic floor muscles) to treat conditions like fecal incontinence, or rectal prolapse. The proposed term will be modified for clarity and added as PT Anal levatorplasty to HLT Anorectal therapeutic procedures.				

CR Number: 2025352011		Implementation Date: 09-Jan-26	Related CR: 2025352011	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Quadricepsplasty	Approved as Requested	Proposed PT Quadricepsplasty	10093437
			HLT primary Muscle therapeutic procedures	10028321
			SOC primary Surgical and medical procedures	10042613
MSSO Comment: The proposal to add a new term Quadricepsplasty is approved as requested. Quadricepsplasty is a muscle-tendon surgical procedure to release, lengthen, or repair the quadriceps muscle or tendon to improve knee function, usually in cases where there is knee stiffness or limited range of motion. Its type is chosen based on the severity and location of stiffness, balancing knee flexion gain with preservation of quadriceps strength. Quadricepsplasty will be added as PT to HLT Muscle therapeutic procedures.				

CR Number: 2025352012		Implementation Date: 09-Jan-26	Related CR: 2025352012	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Velopharyngoplasty	Approved as Requested	Proposed LLT Velopharyngoplasty	10093449 Current
			To PT Pharyngeal reconstruction	10034833

MSSO Comment: The proposal to add a new term Velopharyngoplasty is approved as requested. Velopharyngoplasty is a surgical procedure aimed at improving the function of the velopharyngeal mechanism and to correct velopharyngeal insufficiency. Velopharyngoplasty will be added as LLT to PT Pharyngeal reconstruction.

Detail Report

CR Number: 2025352013	Implementation Date: 09-Jan-26	Related CR: 2025352013	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Robot assisted hysterectomy</i>	Rejected	

MSSO Comment: The proposal to add a new term Robot assisted hysterectomy is not approved. The proposed concept exists as LLT Robotic-assisted hysterectomy to PT Hysterectomy.

CR Number: 2025352014	Implementation Date: 09-Jan-26	Related CR: 2025352014	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Emergent mania/hypomania</i>	Rejected	

MSSO Comment: The proposal to add a new term Emergent mania/hypomania is not approved. MedDRA usually does not include terms describing the course of a disease or episodes of changes.

CR Number: 2025352015	Implementation Date: 09-Jan-26	Related CR: 2025352015	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Sterno</i> plasty	Approved as Requested	Proposed PT <i>Sterno</i> plasty 10093423 HLT primary <i>Chest wall and mediastinal therapeutic procedures</i> 10008494 SOC primary <i>Surgical and medical procedures</i> 10042613

MSSO Comment: The proposal to add a new term Sterno

plasty is approved as requested. Sterno

plasty refers to a specialized surgical procedure primarily used to treat deep sternal wound infection or sternal dehiscence following cardiac surgery. Sterno

plasty will be added as new PT to HLT Chest wall and mediastinal therapeutic procedures.

Detail Report

CR Number: 2025352016		Implementation Date: 09-Jan-26	Related CR: 2025352016	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Ejaculatory duct obstruction</i>	Approved as Requested	Proposed PT <i>Ejaculatory duct obstruction</i>	10093430
			HLT primary <i>Reproductive tract disorders NEC (excl neoplasms)</i>	10027696
			SOC primary <i>Reproductive system and breast disorders</i>	10038604

MSSO Comment: The proposal to add a new term Ejaculatory duct obstruction is approved as requested. Ejaculatory duct obstruction is a pathological condition which is characterized by the obstruction of one or both ejaculatory ducts. It is a cause of male infertility and/or pelvic pain. Ejaculatory duct obstruction will be added as a PT to HLT Reproductive tract disorders NEC (excl neoplasms).

CR Number: 2025352017		Implementation Date: 09-Jan-26	Related CR: 2025352017	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Transurethral resection of ejaculatory duct</i>	Approved as Requested	Proposed PT <i>Transurethral resection of ejaculatory duct</i>	10093429
			HLT primary <i>Male genital tract therapeutic procedures NEC</i>	10038591
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Transurethral resection of ejaculatory duct is approved as requested. Transurethral resection of ejaculatory duct is a surgical procedure to treat ejaculatory duct obstruction by unblocking the ducts, allowing sperm and seminal fluid to mix, improving fertility, and relieving symptoms like painful or low-volume ejaculation by removing cysts or scar tissue. Transurethral resection of ejaculatory duct will be added as PT to HLT Male genital tract therapeutic procedures NEC.

Detail Report

CR Number: 2025352018		Implementation Date: 09-Jan-26	Related CR: 2025352018	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	<i>Laryngeal tuberculosis</i>	Approved as Requested	Proposed PT	
			<i>Laryngeal tuberculosis</i>	10093438
			HLT primary	
			<i>Tuberculous infections</i>	10044756
			SOC primary	
			<i>Infections and infestations</i>	10021881
			HLT secondary	
			<i>Bacterial upper respiratory tract infections</i>	10004054
			SOC secondary	
			<i>Respiratory, thoracic and mediastinal disorders</i>	10038738

MSSO
Comment: The proposal to add a new term Laryngeal tuberculosis is approved as requested. Laryngeal involvement is rare in tuberculosis, representing around 1% of all cases of this infection worldwide. Given the larynx' location in the airway, this form of tuberculosis is of particular importance because it is highly contagious and presents as tuberculous laryngitis. Laryngeal tuberculosis will be added as a PT to primary HLT Tuberculous infections and to secondary HLT Bacterial upper respiratory tract infections. In a related change, PT Tuberculous laryngitis will be demoted under new PT Laryngeal tuberculosis.

CR Number: 2026009006		Implementation Date: 09-Jan-26	Related CR: 2025352018	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Tuberculous laryngitis</i>	Approved as Requested	PT to merge	
To PT	<i>Laryngeal tuberculosis</i>		<i>Tuberculous laryngitis</i>	10045072 Current
			To PT	
			<i>Laryngeal tuberculosis</i>	10093438

MSSO
Comment:

Detail Report

CR Number: 2025352019		Implementation Date: 09-Jan-26	Related CR: 2025352019	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Oropharynx pailloma</i>	Approved Not as Requested	Proposed PT <i>Oropharynx papilloma</i>	10093424
			HLT primary <i>Oropharyngeal, nasopharyngeal and tonsillar neoplasms benign</i>	10031119
			SOC primary <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary <i>Upper respiratory tract neoplasms</i>	10046311
			SOC secondary <i>Respiratory, thoracic and mediastinal disorders</i>	10038738
			HLT secondary <i>Benign oral cavity neoplasms</i>	10030968
			SOC secondary <i>Gastrointestinal disorders</i>	10017947

MSSO Comment: The proposal to add a new term Oropharynx pailloma is approved but not as requested. Oropharynx papilloma is a benign, HPV-induced (often types 6 and 11) wart-like growths found on the soft palate, tongue, or tonsils, often treated via simple surgical excision. The proposed term will be modified with corrected spelling and added as PT Oropharynx papilloma to primary HLT Oropharyngeal, nasopharyngeal and tonsillar neoplasms benign, to secondary HLT Upper respiratory tract neoplasms and to secondary HLT Benign oral cavity neoplasms.

CR Number: 2025352020		Implementation Date: 09-Jan-26	Related CR: 2025352020	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Aquablation</i>	Rejected		

MSSO Comment: The proposal to add a new term Aquablation is not approved. Proposed term is a trademark name for a treatment of benign prostatic hyperplasia. MedDRA does not include trademark names and as such the proposed concept is represented by the existing non-trademark concept LLT Prostate waterjet ablation.

Detail Report

CR Number: 2025352021		Implementation Date: 09-Jan-26	Related CR: 2025352021	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Postoperative hyperthyroidism</i>	Approved as Requested	Proposed LLT <i>Postoperative hyperthyroidism</i>	10093450 Current
			To PT <i>Hyperthyroidism</i>	10020850

MSSO Comment: The proposal to add a new term Postoperative hyperthyroidism is approved as requested. Postoperative hyperthyroidism is a transient condition following parathyroidectomy (31-42% of cases) or neck surgery, caused by "palpation thyroiditis"—the release of thyroid hormone from trauma-injured follicles. Postoperative hyperthyroidism will be added as an LLT to PT Hyperthyroidism.

CR Number: 2025352022		Implementation Date: 09-Jan-26	Related CR: 2025352022	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Sclerosis of breast</i>	Rejected		

MSSO Comment: The proposal to add a new term Sclerosis of breast is not approved. Sclerosis of breast is a benign area of hardened breast tissue. The most common types of sclerosing lesions of the breast are sclerosing adenosis and breast radial scar. Both LLT Adenosis sclerosing and LLT Breast radial scar already exist in MedDRA.

CR Number: 2025352023		Implementation Date: 09-Jan-26	Related CR: 2025352023	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Contracted gallbladder</i>	Approved as Requested	Proposed PT <i>Contracted gallbladder</i>	10093425
			HLT primary <i>Gallbladder disorders NEC</i>	10017629
			SOC primary <i>Hepatobiliary disorders</i>	10019805

MSSO Comment: The proposal to add a new term Contracted gallbladder is approved as requested. Pathologic gallbladder contraction refers to a persistently small, non-distensible gallbladder due to chronic inflammation, fibrosis, or long-standing cystic duct obstruction, often from gallstones. Unlike normal postprandial contraction, it occurs even in a fasting state and is commonly associated with chronic cholecystitis and impaired gallbladder function. Contracted gallbladder will be added as a PT to HLT Gallbladder disorders NEC.

Detail Report

CR Number: 2025352024		Implementation Date: 09-Jan-26	Related CR: 2025352024	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Elmslie-trillat procedure</i>	Approved Not as Requested	Proposed LLT <i>Elmslie-Trillat procedure</i>	10093445 Current
			To PT <i>Knee operation</i>	10049548
MSSO Comment: The proposal to add a new term Elmslie-trillat procedure is approved but not as requested. The Elmslie-Trillat procedure is a procedure for correction of patello-femoral malalignment in acute and recurrent dislocations or subluxation of the patella. Proposed term will be modified for MedDRA capitalization conventions and added as an LLT Elmslie-Trillat procedure to PT Knee operation.				

CR Number: 2025352025		Implementation Date: 09-Jan-26	Related CR: 2025352025	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Angina follicularis</i>	Rejected		
MSSO Comment: The proposal to add a new term Angina follicularis is not approved. The proposed term is rarely found in literature and is represented with the more commonly used synonym term, LLT Lacunar tonsillitis.				

CR Number: 2025352026		Implementation Date: 09-Jan-26	Related CR: 2025352026	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Alveolar bone reconstruction</i>	Approved as Requested	Proposed PT <i>Alveolar bone reconstruction</i>	10093431
			HLT primary <i>Dental and gingival therapeutic procedures</i>	10012317
			SOC primary <i>Surgical and medical procedures</i>	10042613
MSSO Comment: The proposal to add a new term Alveolar bone reconstruction is approved as requested. Alveolar bone reconstruction refers to surgical techniques aimed at restoring lost alveolar bone (the part of the jaw that supports teeth), most commonly to enable dental implant placement, but also to improve function and aesthetics. Alveolar bone reconstruction will be added as PT to HLT Dental and gingival therapeutic procedures. In a related change, PT Dental bone graft, which represent an essential step in alveolar bone reconstruction, will be demoted under the new PT Alveolar bone reconstruction.				

Detail Report

CR Number: 2026009007		Implementation Date: 09-Jan-26	Related CR: 2025352026	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Dental bone graft</i>	Approved as Requested	PT to merge	
To PT	<i>Alveolar bone reconstruction</i>		<i>Dental bone graft</i>	10090365 Current
			To PT	
			<i>Alveolar bone reconstruction</i>	10093431

MSSO
Comment:

CR Number: 2025352027		Implementation Date: 09-Jan-26	Related CR: 2025352027	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Intravertebral disc decompression</i>	Rejected		

MSSO
Comment:

The proposal to add a new term Intravertebral disc decompression is not approved. Intervertebral disc decompression refers to the intended outcome of different non-surgical or surgical medical procedures, intended to relieve pressure on spinal nerves caused by herniated, bulging, or degenerative intervertebral discs. The proposed concept can be represented with existing MedDRA terms (e.g. Spinal decompression and Intervertebral disc compression). Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.

CR Number: 2025352028		Implementation Date: 09-Jan-26	Related CR: 2025352028	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Family history of obesity</i>	Approved as Requested	Proposed LLT	
			<i>Family history of obesity</i>	10093442 Current
			To PT	
			<i>Familial risk factor</i>	10064117

MSSO
Comment:

The proposal to add a new term Family history of obesity is approved as requested. Even if common obesity is strongly influenced by environmental, cultural and lifestyle factors, BMI is still highly heritable within any given environment, as it has been proven by classical adoption studies and extensive twin and family research, which consistently show heritability estimates of roughly 40–75%. Family history of obesity will be added as LLT to PT Familial risk factor.

Detail Report

CR Number: 2025352029		Implementation Date: 09-Jan-26	Related CR: 2025352029	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Tau protein positive</i>	Approved Not as Requested	Proposed PT	
			<i>Tau protein increased</i>	10093434
			HLT primary	
			<i>Neurologic diagnostic procedures</i>	10029285
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Tau protein positive is approved but not as requested. Tau protein is one of the three biomarkers used for the diagnosis of Alzheimer's disease, together with A β 42. The diagnostic use of biomarkers in cerebrospinal fluid has some limitations because their invasive acquisition by lumbar puncture may cause adverse effects. The most common practice in clinical centers is measurement in plasma or serum, as this approach is minimally invasive and therefore easier to obtain and process. The two main proteins involved in the pathological process, A β and Tau, can also be visualized using neuroimaging techniques such as PET. Tau exists along a continuous concentration range, and even normal individuals have measurable tau. Therefore, rather than the threshold-dependent meaning conferred by "positive", the proposed term will be modified to a the more convenient PT Tau protein increased, which will be added to the HLT Neurologic diagnostic procedures.

CR Number: 2025352030		Implementation Date: 09-Jan-26	Related CR: 2025352030	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Tau protein negative</i>	Approved Not as Requested	Proposed PT	
			<i>Tau protein decreased</i>	10093439
			HLT primary	
			<i>Neurologic diagnostic procedures</i>	10029285
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Tau protein negative is approved but not as requested. Tau protein is one of the three biomarkers used for the diagnosis of Alzheimer's disease, together with A β 42. The diagnostic use of biomarkers in cerebrospinal fluid has some limitations because their invasive acquisition by lumbar puncture may cause adverse effects. The most common practice in clinical centers is measurement in plasma or serum, as this approach is minimally invasive and therefore easier to obtain and process. The two main proteins involved in the pathological process, A β and Tau, can also be visualized using neuroimaging techniques such as PET. Tau exists along a continuous concentration range, and even normal individuals have measurable tau. Therefore, rather than the threshold-dependent meaning conferred by "negative", the proposed term will be modified to a the more convenient PT Tau protein decreased, which will be added to the HLT Neurologic diagnostic procedures.

Detail Report

CR Number: 2025352031		Implementation Date: 09-Jan-26	Related CR: 2025352031	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Foraminoplasty</i>	Approved as Requested	Proposed LLT <i>Foraminoplasty</i>	10093446 Current
			To PT <i>Foraminotomy</i>	10066924
MSSO Comment:	The proposal to add a new term Foraminoplasty is approved as requested. Foraminoplasty is a surgical procedure that enlarges the neural foramen through targeted remodeling of bone and/or soft tissues with the aim of relieving nerve root compression while preserving surrounding structures. Foraminoplasty represents a more specific variant of Foraminotomy that emphasizes controlled remodeling of osseous and soft-tissue structures, often using minimally invasive techniques. Foraminoplasty will be added as LLT to PT Foraminotomy.			

CR Number: 2025352032		Implementation Date: 09-Jan-26	Related CR: 2025352032	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cochleostomy</i>	Approved as Requested	Proposed PT <i>Cochleostomy</i>	10093426
			HLT primary <i>Inner ear therapeutic procedures</i>	10022397
			SOC primary <i>Surgical and medical procedures</i>	10042613
MSSO Comment:	The proposal to add a new term Cochleostomy is approved as requested. Cochleostomy is most frequently one of the surgical access routes to the cochlea within the context of cochlear implant surgery. It differs from insertion through the round window, in that cochleostomy involves creating a new opening instead of using a natural anatomical structure. Cochleostomy will be added as a PT to the HLT Inner ear therapeutic procedures.			

CR Number: 2025352033		Implementation Date: 09-Jan-26	Related CR: 2025352033	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cryoballoon ablation</i>	Approved Not as Requested	Proposed LLT <i>Cardiac cryoballoon ablation</i>	10093444 Current
			To PT <i>Cardiac ablation</i>	10059864
MSSO Comment:	The proposal to add a new term Cryoballoon ablation is approved but not as requested. Cryoballoon ablation is a minimally invasive cardiac procedure using a special balloon catheter to freeze and scar problematic heart tissue, usually in the pulmonary veins. It is an effective and well-established treatment for paroxysmal atrial fibrillation, with outcomes comparable to radiofrequency ablation. The proposed term will be modified for anatomic clarity and added as LLT Cardiac cryoballoon ablation to PT Cardiac ablation aligned with LLT Pulmonary vein isolation.			

Detail Report

CR Number: 2025352034		Implementation Date: 09-Jan-26	Related CR: 2025352034	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Anacusis</i>	Approved as Requested	Proposed LLT <i>Anacusis</i>	10093447 Current
			To PT <i>Deafness</i>	10011878
MSSO Comment: The proposal to add a new term Anacusis is approved as requested. Anacusis is the technical, audiological term for total hearing loss—that is, no measurable auditory perception at any frequency. Anacusis will be added as LLT to PT Deafness, aligned with its quasi-synonym LLT Complete hearing loss.				

CR Number: 2025352035		Implementation Date: 09-Jan-26	Related CR: 2025352035	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Endoscopic gastric mucosal ablation</i>	Approved as Requested	Proposed LLT <i>Endoscopic gastric mucosal ablation</i>	10093448 Current
			To PT <i>Gastrointestinal endoscopic therapy</i>	10065100
MSSO Comment: The proposal to add a new term Endoscopic gastric mucosal ablation is approved as requested. Endoscopic gastric mucosal ablation is a minimally invasive endoscopic procedure that uses thermal energy to ablate the gastric mucosa, primarily to control bleeding or eradicate abnormal mucosal tissue, with effects limited to the mucosal layer and a generally low risk of complications. Endoscopic gastric mucosal ablation will be added as an LLT to PT Gastrointestinal endoscopic therapy.				

CR Number: 2025352036		Implementation Date: 09-Jan-26	Related CR: 2025352036	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hip circumference reduced</i>	Approved Not as Requested	Proposed PT <i>Hip circumference decreased</i>	10093435
			HLT primary <i>Physical examination procedures and organ system status</i>	10071941
			SOC primary <i>Investigations</i>	10022891
MSSO Comment: The proposal to add a new term Hip circumference reduced is approved but not as requested. A smaller hip circumference — especially when paired with a high waist circumference — is associated with greater risk of cardiovascular disease, metabolic syndrome, and higher mortality. Hip circumference is measured in a simple, standardized way using a flexible measuring tape, with special attention to measure the maximum circumference over the hips and buttocks. The proposed term will be modified by using the qualifier "decreased" which is the usual qualification of investigation terms in MedDRA. Hip circumference decreased will be added as PT to HLT Physical examination procedures and organ system status.				

Detail Report

CR Number: 2025352037		Implementation Date: 09-Jan-26	Related CR: 2025352037	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Protein z deficiency</i>	Approved Not as Requested	Proposed PT <i>Protein Z deficiency</i>	10093436
			HLT primary <i>Coagulation factor deficiencies</i>	10009737
			SOC primary <i>Blood and lymphatic system disorders</i>	10005329

MSSO Comment: The proposal to add a new term Protein z deficiency is approved but not as requested. Protein Z, a vitamin K–dependent protein, functions as a cofactor to downregulate coagulation by forming a complex with the plasma protein Z-dependent protease inhibitor. The complex predominantly inactivates factor Xa on phospholipid surfaces. Its deficiency or dysfunction predisposes to venous thrombosis (mainly in patients who also have other clotting abnormalities). The proposed term will be modified following MedDRA capitalization rules. Since the congenital form is extremely rare and the acquired form is clearly more common, the proposed concept will be added as the PT Protein Z deficiency to the HLT Coagulation factor deficiencies.

CR Number: 2025352038		Implementation Date: 09-Jan-26	Related CR: 2025352038	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Gingivoplasty</i>	Approved as Requested	Proposed PT <i>Gingivoplasty</i>	10093422
			HLT primary <i>Dental and gingival therapeutic procedures</i>	10012317
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Gingivoplasty is approved as requested. Gingivoplasty is a dental surgery that reshapes and contours gum tissue for cosmetic or functional reasons. Gingivoplasty will be added as PT to HLT Dental and gingival therapeutic procedures.

CR Number: 2025352039		Implementation Date: 09-Jan-26	Related CR: 2025352039	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Benign ventricular tumor or neoplasm</i>	Rejected		

MSSO Comment: The proposal to add a new term Benign ventricular tumor or neoplasm is not approved. The proposed concept does not correspond to a recognized clinicopathological entity. The intraventricular space hosts heterogeneous tumors (e.g. subependymoma, central neurocytoma, choroid plexus papilloma...) with different histogenesis, age distribution, imaging features, and clinical behavior; grouping them under the proposed umbrella term adds little medical precision. In addition, ventricular is an anatomically imprecise term as it can refer to brain or heart.

Detail Report

CR Number: 2025356000	Implementation Date: 09-Jan-26	Related CR: 2025356000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Sacral cyst</i>	Rejected	

MSSO Comment: The proposal to add a new term Sacral cyst is not approved. The term sacral cyst is highly nonspecific and broad, as it encompasses a heterogeneous group of cystic lesions arising in or around the sacrum. In the vast majority of cases, however, a sacral cyst identified on imaging corresponds to a Tarlov (perineural) cyst, which is typically benign and often an incidental finding and it is already represented by LLTs Perineural cyst and Tarlov's cyst.

CR Number: 2025356001	Implementation Date: 24-Feb-26	Related CR: 2025356001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Blood 25-hydroxy vitamin D decreased</i>	Rejected After Suspension	

MSSO Comment: The proposal to add a new term Blood 25-hydroxy vitamin D decreased is not approved after suspension. For less specific investigation results, consider using LLTs linked to PT Vitamin D decreased. In a related change, PT Blood 1,25-dihydroxycholecalciferol and PT Blood 25-hydroxycholecalciferol will both be demoted under PT Vitamin D. Furthermore, PT Blood 1,25-dihydroxycholecalciferol decreased and PT Blood 25-hydroxycholecalciferol decreased will both be demoted under PT Vitamin D decreased. In addition, PT Blood 1,25-dihydroxycholecalciferol increased and PT Blood 25-hydroxycholecalciferol increased will both be demoted under PT Vitamin D increased.

CR Number: 2026055005	Implementation Date: 24-Feb-26	Related CR: 2025356001	MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement
PT to merge	<i>Blood 1,25-dihydroxycholecalciferol</i>	Approved as Requested	PT to merge
To PT	<i>Vitamin D</i>		<i>Blood 1,25-dihydroxycholecalciferol</i> 10059897 Current
			To PT
			<i>Vitamin D</i> 10050713

MSSO Comment:

Detail Report

CR Number: 2026055007		Implementation Date: 24-Feb-26	Related CR: 2025356001		
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
PT to merge	<i>Blood 1,25-dihydroxycholecalciferol decreased</i>	Approved as Requested	PT to merge		
To PT	<i>Vitamin D decreased</i>		<i>Blood 1,25-dihydroxycholecalciferol decreased</i>	10053811	Current
			To PT		
			<i>Vitamin D decreased</i>	10062189	

MSSO
Comment:

CR Number: 2026055010		Implementation Date: 24-Feb-26	Related CR: 2025356001		
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
PT to merge	<i>Blood 25-hydroxycholecalciferol increased</i>	Approved as Requested	PT to merge		
To PT	<i>Vitamin D increased</i>		<i>Blood 25-hydroxycholecalciferol increased</i>	10053812	Current
			To PT		
			<i>Vitamin D increased</i>	10062134	

MSSO
Comment:

CR Number: 2026055009		Implementation Date: 24-Feb-26	Related CR: 2025356001		
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
PT to merge	<i>Blood 1,25-dihydroxycholecalciferol increased</i>	Approved as Requested	PT to merge		
To PT	<i>Vitamin D increased</i>		<i>Blood 1,25-dihydroxycholecalciferol increased</i>	10053810	Current
			To PT		
			<i>Vitamin D increased</i>	10062134	

MSSO
Comment:

Detail Report

CR Number: 2026055008		Implementation Date: 24-Feb-26	Related CR: 2025356001	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Blood 25-hydroxycholecalciferol decreased</i>	Approved as Requested	PT to merge	
To PT	<i>Vitamin D decreased</i>		<i>Blood 25-hydroxycholecalciferol decreased</i>	10053813 Current
			To PT	
			<i>Vitamin D decreased</i>	10062189

MSSO
Comment:

CR Number: 2026055006		Implementation Date: 24-Feb-26	Related CR: 2025356001	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Blood 25-hydroxycholecalciferol</i>	Approved as Requested	PT to merge	
To PT	<i>Vitamin D</i>		<i>Blood 25-hydroxycholecalciferol</i>	10059936 Current
			To PT	
			<i>Vitamin D</i>	10050713

MSSO
Comment:

CR Number: 2025356002		Implementation Date: 12-Jan-26	Related CR: 2025356002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Creatine kinase isoenzyme increased</i>	Rejected		

MSSO
Comment: The proposal to add a new term Creatine kinase isoenzyme increased is not approved. The proposed concept is represented by LLT/PT Creatine kinase increased.

Detail Report

CR Number: 2025357000		Implementation Date: 12-Jan-26	Related CR: 2025357000	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Inappropriate solution used in product reconstitution issue</i>	Approved Not as Requested	Proposed LLT <i>Inappropriate solution used in product reconstitution</i>	10093457 Current
To PT	<i>Product preparation issue</i>		To PT <i>Product preparation issue</i>	10081301
MSSO Comment: The proposal to add a new LLT Inappropriate solution used in product reconstitution issue to PT Product preparation issue is approved but not as requested. The proposed term will be modified for coherence with other similar terms and added as LLT Inappropriate solution used in product reconstitution to PT Product preparation issue.				

CR Number: 2025357001		Implementation Date: 12-Jan-26	Related CR: 2025357001	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Product reconstitution not performed</i>	Approved as Requested	Proposed LLT <i>Product reconstitution not performed</i>	10093456 Current
To PT	<i>Product preparation issue</i>		To PT <i>Product preparation issue</i>	10081301
MSSO Comment: The proposal to add a new LLT Product reconstitution not performed to PT Product preparation issue is approved as requested. The proposed term will be added for scenarios where reconstitution of a powdered or concentrated form of a substance is not performed.				

CR Number: 2025357002		Implementation Date: 12-Jan-26	Related CR: 2025357002	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Product reconstitution omission error</i>	Approved as Requested	Proposed LLT <i>Product reconstitution omission error</i>	10093455 Current
To PT	<i>Product preparation error</i>		To PT <i>Product preparation error</i>	10076869
MSSO Comment: The proposal to add a new LLT Product reconstitution omission error to PT Product preparation error is approved as requested. The proposed term will be added for scenarios where reconstitution of a powdered or concentrated form of a substance is not performed/omitted in error.				

Detail Report

CR Number: 2025358003		Implementation Date: 12-Jan-26	Related CR: 2025358003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	FACET ARTHRITIS	Approved Not as Requested	Proposed LLT Facet arthritis	10093459 Current
			To PT Facet joint syndrome	10054813
MSSO The proposal to add a new LLT FACET ARTHRITIS is approved but not as requested. Facet arthritis is a facet joint disease with an inflammatory and symptomatic aspect.				
Comment: The proposed term will be modified for MedDRA capitalization rules and added as LLT Facet arthritis to PT Facet joint syndrome.				

CR Number: 2025358004		Implementation Date: 12-Jan-26	Related CR: 2025358004	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Increased viscosity of lower respiratory secretion	Rejected		
MSSO The proposal to add a new PT Increased viscosity of lower respiratory secretion is not approved. Please consider existing LLT/PT Increased viscosity of bronchial				
Comment: secretion for your coding needs.				

CR Number: 2025358005		Implementation Date: 12-Jan-26	Related CR: 2025358005	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Epsilon wave	Approved Not as Requested	Proposed PT Electrocardiogram epsilon wave present	10093458
			HLT primary ECG investigations	10053104
			SOC primary Investigations	10022891
MSSO The proposal to add a new PT Epsilon wave is approved but not as requested. The epsilon wave is a marker of delayed activation of the right ventricular free wall and				
Comment: outflow tract and is considered a major diagnostic criterion for arrhythmogenic right ventricular dysplasia. The epsilon wave has the appearance of a reproducible distinct wave (low-amplitude small deflections) between the end of the QRS complex to the onset of the T wave in the right precordial leads, particularly in lead V1. The proposed term will be modified for clarity and added as PT Electrocardiogram epsilon wave present to HLT ECG investigations.				

Detail Report

CR Number: 2025363000		Implementation Date: 13-Jan-26	Related CR: 2025363000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Tuberculosis immune response test</i>	Approved as Requested	Proposed LLT	
From PT	<i>Interferon gamma release assay positive</i>		<i>Tuberculosis immune response test</i>	10092991 Current
To PT	<i>Interferon gamma release assay</i>		From PT	
			<i>Interferon gamma release assay positive</i>	10072866
			To PT	
			<i>Interferon gamma release assay</i>	10073542
MSSO Comment: The proposal to move the LLT Tuberculosis immune response test from PT Interferon gamma release assay positive to PT Interferon gamma release assay is approved as requested. The unqualified LLT Tuberculosis immune response test will be moved from qualified PT Interferon gamma release assay positive to unqualified PT Interferon gamma release assay.				

CR Number: 2025363001		Implementation Date: 13-Jan-26	Related CR: 2025363001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Tuberculosis immune response test positive</i>	Approved as Requested	Proposed LLT	
To PT	<i>Interferon gamma release assay positive</i>		<i>Tuberculosis immune response test positive</i>	10093465 Current
			To PT	
			<i>Interferon gamma release assay positive</i>	10072866
MSSO Comment: The proposal to add a new LLT Tuberculosis immune response test positive to PT Interferon gamma release assay positive is approved as requested, to represent the qualified counterpart of LLT Tuberculosis immune response test.				

CR Number: 2025363002		Implementation Date: 12-Jan-26	Related CR: 2025363002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Squamous cell carcinoma in situ of the cervix</i>	Rejected		
MSSO Comment: The proposal to add a new term Squamous cell carcinoma in situ of the cervix is not approved. The proposed concept combines several qualifiers by adding histologic subtype and staging information. The concept is best represented by coding the histologic subtype and the stage with separate terms, which already exist in MedDRA. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.				

Detail Report

CR Number: 2025363003 Implementation Date: 14-Jan-26

Related CR: 2025363003

Change Requested

Link (move) a PT to another
HLT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT *Fasciitis* Rejected
From HLT *Musculoskeletal and connective tissue infections
and inflammations NEC*
To HLT *Connective tissue disorders NEC*

MSSO
Comment: The proposal to move the PT Fasciitis from HLT Musculoskeletal and connective tissue infections and inflammations NEC to HLT Connective tissue disorders NEC is not approved. Fasciitis is defined as inflammation of fascia. The proposed term is adequately placed under HLT Musculoskeletal and connective tissue infections and inflammations NEC, since this HLT includes both infectious conditions as well as inflammatory conditions.

CR Number: 2025363004 Implementation Date: 14-Jan-26

Related CR: 2025363004

Change Requested

Link (move) a PT to another
HLT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT *Myofascitis* Approved as Requested Proposed PT
From HLT *Connective tissue disorders NEC* *Myofascitis* 10052443
To HLT *Musculoskeletal and connective tissue infections
and inflammations NEC* From HLT
Connective tissue disorders NEC 10074472
To HLT
*Musculoskeletal and connective tissue
infections and inflammations NEC* 10052776

MSSO
Comment: The proposal to move the PT Myofascitis from HLT Connective tissue disorders NEC to HLT Musculoskeletal and connective tissue infections and inflammations NEC is approved as requested. Myofascitis it is defined as inflammation of a muscle and thus will be placed more appropriately under HLT Musculoskeletal and connective tissue infections and inflammations NEC.

CR Number: 2025364000 Implementation Date: 12-Jan-26

Related CR: 2025364000

Change Requested

Link (move) an LLT to another
PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed LLT *Peripheral sensory neurotoxicity* Rejected
From PT *Neurotoxicity*
To PT *Peripheral sensory neuropathy*

MSSO
Comment: The proposal to move the LLT Peripheral sensory neurotoxicity from PT Neurotoxicity to PT Peripheral sensory neuropathy is not approved. Peripheral sensory neuropathy is a broader concept and describes the condition of nerve damage affecting sensory nerves, while peripheral sensory neurotoxicity describes a cause, specifically when toxins damage those sensory nerves.

Detail Report

CR Number: 2025364001	Implementation Date: 30-Jan-26	Related CR: 2025364001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	High fluorescent reticulocyte count increased	Rejected After Suspension	

MSSO Comment: The proposal to add a new term High fluorescent reticulocyte count increased is not approved after suspension. Assessment of fluorescence reticulocyte counting is usually provided as a ratio/fraction or percentage as test results. One may use the existing unqualified test term names for count/percentages in laboratory database fields. The clinical assessment of an increase or decrease is difficult, as the low and high-fluorescent reticulocytes move in different directions. To provide a means of coding a shift towards more immature reticulocytes, a new PT Immature reticulocyte fraction increased was added in change request 2025364004.

CR Number: 2025364002	Implementation Date: 30-Jan-26	Related CR: 2025364002	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Medium fluorescent reticulocyte count increased	Rejected After Suspension	

MSSO Comment: The proposal to add a new LLT Medium fluorescent reticulocyte count increased is not approved after suspension. Assessment of fluorescence reticulocyte counting is usually provided as a ratio/fraction or percentage as test results. One may use the existing unqualified test term names for count/percentages in laboratory database fields. The clinical assessment of an increase or decrease is difficult, as the low and high-fluorescent reticulocytes move in different directions. To provide a means of coding a shift towards more immature reticulocytes, a new PT Immature reticulocyte fraction increased was added in change request 2025364004.

CR Number: 2025364003	Implementation Date: 30-Jan-26	Related CR: 2025364003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Low fluorescent reticulocyte count decreased	Rejected After Suspension	

MSSO Comment: The proposal to add a new term Low fluorescent reticulocyte count decreased is not approved after suspension. Assessment of fluorescence reticulocyte counting is usually provided as a ratio/fraction or percentage as test results. One may use the existing unqualified test term names for count/percentages in laboratory database fields. The clinical assessment of an increase or decrease is difficult, as the low and high-fluorescent reticulocytes move in different directions. To provide a means of coding a shift towards more immature reticulocytes, a new PT Immature reticulocyte fraction increased was added in change request 2025364004.

Detail Report

CR Number: 2025364004		Implementation Date: 30-Jan-26	Related CR: 2025364004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Immature reticulocyte fraction increased</i>	Approved After Suspension	Proposed PT <i>Immature reticulocyte fraction increased</i>	10093498
			HLT primary <i>Red blood cell analyses</i>	10038148
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Immature reticulocyte fraction increased is approved as requested after suspension. The immature reticulocyte fraction is a key indicator of erythropoietic activity, reflecting the bone marrow's response to red blood cell demand. The immature reticulocyte fraction is the sum of the fraction of high-fluorescence intensity regions plus the fraction of middle-fluorescence intensity regions. Proposed term will be added as PT Immature reticulocyte fraction increased to HLT Red blood cell analyses.

CR Number: 2026004000		Implementation Date: 12-Jan-26	Related CR: 2026004000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Vaginal lump</i>	Rejected		

MSSO Comment: The proposal to add a new term Vaginal lump is not approved. Lump is an imprecise word and terms containing this word will generally not be added into MedDRA when a more precise "disorder" terms exist. Please consider specific existing terms or e.g. PT Vaginal lesion depending on your specific coding scenario.

CR Number: 2026005001		Implementation Date: 13-Feb-26	Related CR: 2026005001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Plasma dopamine decreased</i>	Approved After Suspension	Proposed LLT <i>Plasma dopamine decreased</i>	10093522 Current
			To PT <i>Dopamine decreased</i>	10093521

MSSO Comment: The proposal to add a new LLT Plasma dopamine decreased is approved as requested after suspension. Plasma dopamine levels may aid in the biochemical diagnosis and monitoring of neuroblastomas as well as may be useful in evaluating dopamine-secreting pheochromocytomas and paragangliomas. Plasma dopamine decreased will be added as LLT to new PT Dopamine decreased. In a related change, Dopamine decreased will be added as PT to HLT Adrenal medulla tests.

Detail Report

CR Number: 2026044008		Implementation Date: 13-Feb-26	Related CR: 2026005001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Dopamine decreased</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Adrenal medulla tests</i>		<i>Dopamine decreased</i>	10093521
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Adrenal medulla tests</i>	10001373
			SOC primary	
			<i>Investigations</i>	10022891

MSSO
Comment:

CR Number: 2026005002		Implementation Date: 12-Jan-26	Related CR: 2026005002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Bronchitis aggravated</i>	Rejected		

MSSO
Comment: The proposal to add a new term Bronchitis aggravated is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept.

CR Number: 2026005017		Implementation Date: 12-Jan-26	Related CR: 2026005017	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Postprandial fullness</i>	Rejected		

MSSO
Comment: The proposal to add a new LLT Postprandial fullness is not approved. Please use existing LLT Postprandial distress syndrome which is a syndrome characterized by a bothersome sensation of postprandial fullness and/or early satiety, occurring after usual meals and not explained by organic disease.

Detail Report

CR Number: 2026005018		Implementation Date: 12-Jan-26	Related CR: 2026005018	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Bulging duodenal papilla</i>	Approved as Requested	Proposed PT <i>Bulging duodenal papilla</i>	10093460
			HLT primary <i>Structural and other bile duct disorders</i>	10042257
			SOC primary <i>Hepatobiliary disorders</i>	10019805
			HLT secondary <i>Pancreatic disorders NEC</i>	10027692
			SOC secondary <i>Gastrointestinal disorders</i>	10017947

MSSO Comment: The proposal to add a new term Bulging duodenal papilla is approved as requested. A bulging duodenal papilla is a finding on imaging and endoscopy. Normally, the duodenal papilla cannot be differentiated from duodenal mucosal folds. Bulging duodenal papilla will be added as PT to primary HLT Structural and other bile duct disorders and to secondary HLT Pancreatic disorders NEC.

CR Number: 2026006000		Implementation Date: 12-Jan-26	Related CR: 2026006000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Mesenteric lipodystrophy</i>	Approved as Requested	Proposed LLT <i>Mesenteric lipodystrophy</i>	10093461 Current
			To PT <i>Mesenteric panniculitis</i>	10063031

MSSO Comment: The proposal to add a new term Mesenteric lipodystrophy is approved as requested. Mesenteric lipodystrophy is an inflammatory condition with variable fibrosis of mesenteric fat, usually of the small intestine, causing a solid mass that may displace or obstruct the intestine. It is generally considered an earlier or milder phase of the same disease process as sclerosing mesenteritis or mesenteric panniculitis. Mesenteric lipodystrophy will be added as LLT to PT Mesenteric panniculitis.

Detail Report

CR Number: 2026006001		Implementation Date: 13-Jan-26	Related CR: 2026006001	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Lichtenstein repair</i>	Approved as Requested	Proposed LLT	
To PT	<i>Inguinal hernia repair</i>		<i>Lichtenstein repair</i>	10093462 Current
			To PT	
			<i>Inguinal hernia repair</i>	10022020

MSSO Comment: The proposal to add a new LLT Lichtenstein repair to PT Inguinal hernia repair is approved as requested. The Lichtenstein repair is considered the gold standard of inguinal hernia repair in which the repair is accomplished by covering the opening of the hernia with a patch of mesh, instead of sewing the edge of the hole together.

CR Number: 2026006002		Implementation Date: 13-Jan-26	Related CR: 2026006002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement	
Proposed Term	<i>Odontohypophosphatasia</i>	Approved as Requested	Proposed LLT	
			<i>Odontohypophosphatasia</i>	10093463 Current
			To PT	
			<i>Hypophosphatasia</i>	10049933

MSSO Comment: The proposal to add a new term Odontohypophosphatasia is approved as requested. Odontohypophosphatasia (odonto-HPP) is the mildest form of hypophosphatasia, where symptoms are limited to the teeth with no skeletal issues, though some odonto-HPP patients may later develop systemic hypophosphatasia. It typically presents as premature loss of primary teeth, without clinical or radiological evidence of bone disease at diagnosis. Odontohypophosphatasia will be added as sub-concept LLT to PT Hypophosphatasia.

CR Number: 2026006003		Implementation Date: 13-Jan-26	Related CR: 2026006003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement	
Proposed Term	<i>Sudoscan</i>	Rejected		

MSSO Comment: The proposal to add a new term Sudoscan is not approved. The Sudoscan is a trademark name for a device which provides a quantitative measure of chloride conductance (measured in microsiemens) and its results serve as a biomarker to assess sweat gland function in relation to sweat gland innervation. MedDRA does not add trademark names and as such, the existing LLTs under PT Sweat test and LLT/PT Quantitative sudomotor axon reflex test may be considered for use depending on case scenario.

Detail Report

CR Number: 2026006004		Implementation Date: 13-Jan-26	Related CR: 2026006004		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Inner ear pain	Rejected			

MSSO Comment: The proposal to add a new term Inner ear pain is not approved. The proposed concept is available as LLT Pain in ear under PT Ear pain, whereas the LLT/PT External ear pain is also available to distinguish external ear pain.

CR Number: 2026007000		Implementation Date: 13-Jan-26	Related CR: 2026007000		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Tanning bed	Rejected			

MSSO Comment: The proposal to add a new term Tanning bed is not approved. Tanning bed is a device not a procedure name. Please consider existing term options under HLT Phototherapies for your coding needs related to UV light therapies.

Detail Report

CR Number: 2026007001		Implementation Date: 13-Jan-26	Related CR: 2026007001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	ReNU syndrome	Approved as Requested	Proposed PT ReNU syndrome	10093464
			HLT primary Central nervous system disorders congenital NEC	10009713
			SOC primary Congenital, familial and genetic disorders	10010331
			HLT secondary Pervasive developmental disorders NEC	10034740
			SOC secondary Psychiatric disorders	10037175
			HLT secondary Seizures and seizure disorders NEC	10039912
			SOC secondary Nervous system disorders	10029205

MSSO Comment: The proposal to add a new term ReNU syndrome is approved as requested. ReNU syndrome is characterized by hypotonia, global developmental delay, intellectual disability with poor or absent speech, delayed motor development, feeding difficulties, short stature, seizures, and dysmorphic features. Neuroradiological abnormalities, including ventriculomegaly, hypoplasia of the corpus callosum, and a decreased white matter volume, are observed in many individuals. Most individuals have the same highly recurrent single base insertion (n.64_65insT) in RNU4-2. ReNU syndrome will be added as PT to primary HLT Central nervous system disorders congenital NEC, to secondary HLT Pervasive developmental disorders NEC and to secondary HLT Seizures and seizure disorders NEC.

CR Number: 2026008007		Implementation Date: 14-Jan-26	Related CR: 2026008007	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Postprandial somnolence	Approved as Requested	Proposed LLT Postprandial somnolence	10093466 Current
			To PT Somnolence	10041349

MSSO Comment: The proposal to add a new term Postprandial somnolence is approved as requested. Postprandial somnolence is considered pathologic when the degree of sleepiness is excessive, recurrent, or disproportionate to meal size and interferes with normal functioning or safety. It may indicate an underlying disorder such as diabetes mellitus with postprandial dysglycemia, autonomic dysfunction, sleep disorders, or other metabolic or neurologic conditions requiring clinical evaluation. Postprandial somnolence will be added as LLT to PT Somnolence.

Detail Report

CR Number: 2026008008	Implementation Date: 14-Jan-26	Related CR: 2026008008	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Allergy laryngopharyngitis	Rejected	

MSSO Comment: The proposal to add a new LLT Allergy laryngopharyngitis is not approved. MedDRA usually separates disorders of the larynx and pharynx in different HLTs. A term combining both anatomical locations is considered a combination term. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.

CR Number: 2026009000	Implementation Date: 13-Mar-26	Related CR: 2026009000	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Infection associated with childbirth	Rejected After Suspension	

MSSO Comment: The proposal to add a new PT Infection associated with childbirth is not approved after suspension. The proposed term is ambiguous as it may refer to the infection in the mother or child. Please consider existing terms under HLT Puerperal infections or LLT/PT Vertical infection transmission for your coding needs depending on the clinical scenario.

CR Number: 2026009001	Implementation Date: 13-Mar-26	Related CR: 2026009001	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Intrapartum infection	Rejected After Suspension	
To PT	Infection associated with childbirth		

MSSO Comment: The proposal to add a new LLT Intrapartum infection to PT Infection associated with childbirth is not approved after suspension. The proposed term is ambiguous as it may refer to the infection in the mother or child. Please consider existing terms under HLT Puerperal infections or LLT/PT Vertical infection transmission for your coding needs depending on the clinical scenario.

Detail Report

CR Number: 2026009002 **Implementation Date:** 13-Mar-26 **Related CR:** 2026009002 [MedDRA Code #](#)

[Change Requested](#) Add a New LLT [Final Disposition](#) [Final Placement](#)

Proposed LLT *Postpartum infection* Rejected After Suspension

To PT *Infection associated with childbirth*

MSSO Comment: The proposal to add a new LLT Postpartum infection to PT Infection associated with childbirth is not approved after suspension. The proposed term is ambiguous as it may refer to the infection in the mother or child. Please consider existing terms under HLT Puerperal infections or LLT/PT Vertical infection transmission for your coding needs depending on the clinical scenario.

CR Number: 2026009003 **Implementation Date:** 14-Jan-26 **Related CR:** 2026009003 [MedDRA Code #](#)

[Change Requested](#) Add a New Term [Final Disposition](#) [Final Placement](#)

Proposed Term *Immune-mediated fracture* Rejected

MSSO Comment: The proposal to add a new term Immune-mediated fracture is not approved. Immune-mediated fracture represents a final outcome secondary to the osteoporosis induced by a checkpoint inhibitor, rather than a direct immune-related adverse event.

CR Number: 2026009004 **Implementation Date:** 14-Jan-26 **Related CR:** 2026009004 [MedDRA Code #](#)

[Change Requested](#) Reassign Primary SOC [Final Disposition](#) [Final Placement](#)

Proposed PT *Tattoo* Rejected

Old Primary SOC *Social circumstances*

New Primary SOC *Surgical and Medical Procedures*

MSSO Comment: The proposal to reassign the primary SOC of PT Tattoo from current SOC Social circumstances to SOC Surgical and Medical Procedures is not approved. In order to capture tattooing for micropigmentation procedures in medical contexts please consider existing LLT/PT Permanent cosmetic dermapigmentation, depending on your scenario. Additionally, LLT/PT Fiducial marker placement can also be considered for small, permanent cutaneous marks applied for medical purposes to serve as a fixed reference point for anatomical localization, imaging alignment, or treatment planning (e.g., radiotherapy).

Detail Report

CR Number: 2026010000		Implementation Date: 26-Mar-26	Related CR: 2026010000		MedDRA Code #
Change Requested	Add a new SMQ	Final Disposition	Final Placement		
Proposed SMQ	<i>Stevens-Johnson syndrome and Toxic epidermal necrolysis</i>	Rejected After Suspension			

MSSO Comment: The proposal to add a new SMQ Stevens-Johnson syndrome and Toxic epidermal necrolysis is not approved after suspension. The ICH M1 Points to Consider (PtC) Working group (an international team of regulatory and industry representatives) evaluated this request and does not recommend the development of SMQ Stevens-Johnson syndrome and Toxic epidermal necrolysis. Given the mortality and morbidity of severe cutaneous reactions, failing to detect a true case (false negative) carries severe consequences, as might happen with overly restrictive case definition. The sensitivity of the search needs to be appropriately balanced. As such SMQ Severe cutaneous adverse reactions is designed to adequately identify potential cases and relies on downstream clinical review to refine specificity rather than building excessive restrictions into initial case identification. The existing SMQ Severe cutaneous adverse reactions addresses common clinical and pharmacovigilance needs of the broad MedDRA user community and can be modified to create a Customised MedDRA Query Based on an SMQ, if needed. A review of terms in the existing SMQ Severe cutaneous adverse reactions is being undertaken and potential changes may be considered.

CR Number: 2026012000		Implementation Date: 14-Jan-26	Related CR: 2026012000		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Antiresorptive agent-related osteonecrosis of the jaw</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Antiresorptive agent-related osteonecrosis of the jaw is not approved. Please consider existing LLT Medication-related osteonecrosis of jaw and LLT Bisphosphonate-related osteonecrosis of the jaw for your coding needs, depending on case scenario.

CR Number: 2026012010		Implementation Date: 15-Jan-26	Related CR: 2026012010		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	CHRONIC LYMPHOCYTIC LEUKEMIA/SMALL LYMPHOCYTIC LYMPHOMA -	Rejected			

MSSO Comment: The proposal to add a new term CHRONIC LYMPHOCYTIC LEUKEMIA/SMALL LYMPHOCYTIC LYMPHOMA - is not approved. Chronic lymphocytic leukemia (CLL) is considered to be the same biological entity (i.e., one disease with different manifestations) to the mature (peripheral) B cell neoplasm small lymphocytic lymphoma (SLL), one of the indolent non-Hodgkin lymphomas. The term CLL is used when the disease manifests primarily in the blood, whereas the term SLL is used when involvement is primarily in lymph nodes. The term CLL/SLL is used in classification systems to indicate the disease, while in clinical practice the distinction is made based on disease presentation. Please consider existing terms for CLL and SLL for your specific coding needs.

Detail Report

CR Number: 2026013000		Implementation Date: 15-Jan-26	Related CR: 2026013000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Peripheral nerve revision</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Peripheral nerve operation</i>		<i>Peripheral nerve revision surgery</i>	10093467 Current
			To PT	
			<i>Peripheral nerve operation</i>	10061341
MSSO Comment: The proposal to add a new LLT Peripheral nerve revision to PT Peripheral nerve operation is approved but not as requested. Peripheral nerve revision surgery can provide benefits to patients experiencing symptoms after a failed primary procedure. Proposed term will be modified for clarity and added as LLT Peripheral nerve revision surgery to PT Peripheral nerve operation.				

CR Number: 2026014000		Implementation Date: 20-Jan-26	Related CR: 2026014000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Obstructive cholecystitis</i>	Rejected		
MSSO Comment: The proposal to add a new term Obstructive cholecystitis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. Please consider existing terms such as LLT/PT Cholecystitis and LLT/PT Gallbladder obstruction, depending on the case specific scenario.				

CR Number: 2026014001		Implementation Date: 20-Jan-26	Related CR: 2026014001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Porphyrin plasma</i>	Approved Not as Requested	Proposed PT	
			<i>Porphyrins plasma</i>	10093471
			HLT primary	
			<i>Haem metabolism analyses</i>	10018812
			SOC primary	
			<i>Investigations</i>	10022891
MSSO Comment: The proposal to add a new term Porphyrin plasma is approved but not as requested. Plasma porphyrins are measured to diagnose and monitor porphyrias, conditions where these light-sensitive molecules build up due to enzyme defects in heme production, causing skin issues (like blisters) or neurological symptoms, with increased levels indicating active disease. It is used for monitoring Porphyria Cutanea Tarda or ruling out porphyria in kidney failure patients with skin problems. Proposed term will be modified and added in plural form as PT Porphyrins plasma to HLT Haem metabolism analyses.				

Detail Report

CR Number: 2026014002		Implementation Date: 20-Jan-26	Related CR: 2026014002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Porphyrin plasma increased</i>	Approved Not as Requested	Proposed PT	
			<i>Porphyrins plasma increased</i>	10093470
			HLT primary	
			<i>Haem metabolism analyses</i>	10018812
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Porphyrin plasma increased is approved but not as requested. The proposed term will be modified and added in the plural form as PT Porphyrins plasma increased to HLT Haem metabolism analyses, as the qualified counterpart of PT Porphyrins plasma added in change request 2026014001.

CR Number: 2026014003		Implementation Date: 20-Jan-26	Related CR: 2026014003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Intravitreal gass injection</i>	Approved Not as Requested	Proposed LLT	
			<i>Intravitreal gas injection</i>	10093473 Current
			To PT	
			<i>Ophthalmic fluid-air exchange procedure</i>	10067701

MSSO Comment: The proposal to add a new term Intravitreal gass injection is approved but not as requested. Intravitreal gas injection is a procedure mostly performed on patients with a retinal detachment. The gas bubble covers the site of detachment sealing it off to prevent further damage to the retina. Proposed term will be modified for spelling and added as LLT Intravitreal gas injection to PT Ophthalmic fluid-air exchange procedure.

CR Number: 2026014004		Implementation Date: 20-Jan-26	Related CR: 2026014004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Acquired hypermobility</i>	Approved Not as Requested	Proposed PT	
			<i>Acquired joint hypermobility</i>	10093469
			HLT primary	
			<i>Joint related disorders NEC</i>	10027685
			SOC primary	
			<i>Musculoskeletal and connective tissue disorders</i>	10028395

MSSO Comment: The proposal to add a new term Acquired hypermobility is approved but not as requested. Acquired joint hypermobility can be due to the shape of bones (such as shallow hip sockets), reduced muscle tone or due to an injury or trauma. It can also be acquired through training and exercise. Proposed term will be modified for clarity and added as PT Acquired joint hypermobility to HLT Joint related disorders NEC.

Detail Report

CR Number: 2026014005		Implementation Date: 20-Jan-26	Related CR: 2026014005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Porphyrin blood decreased</i>	Approved Not as Requested	Proposed PT	
			<i>Porphyrins plasma decreased</i>	10093472
			HLT primary	
			<i>Haem metabolism analyses</i>	10018812
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Porphyrin blood decreased is approved but not as requested. The proposed term will be modified for alignment and added in the plural form as PT Porphyrins plasma decreased to HLT Haem metabolism analyses, as the qualified counterpart of PT Porphyrins plasma added in change request 2026014001.

CR Number: 2026014006		Implementation Date: 20-Jan-26	Related CR: 2026014006	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>HMG CoA reductase antibody test positive</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Autoimmunity analyses</i>		<i>HMG CoA reductase antibody test positive</i>	10093474
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Autoimmunity analyses</i>	10003828
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new PT HMG CoA reductase antibody test positive to primary HLT Autoimmunity analyses in SOC Investigations is approved as requested. A positive HMG-CoA reductase antibody test indicates the presence of an immune-mediated necrotizing myopathy, most often associated with prior exposure to statins. It supports an autoimmune mechanism rather than a toxic statin-induced myopathy, as muscle weakness and elevated creatine kinase levels typically persist despite discontinuation of the statin. Test positivity has important therapeutic implications, as the condition usually requires immunosuppressive treatment rather than simple withdrawal of the drug. The proposed term will be added as the qualified test term counterpart of PT HMG CoA reductase antibody test.

CR Number: 2026015000		Implementation Date: 20-Jan-26	Related CR: 2026015000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Spitting out medication</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Spitting out medication is not approved. The proposed LLT is describing a situation. The term is neither describing the reason for spitting out the medication, which could be a helpful indicator for product quality considerations, nor does it address whether the correct medication dose was taken in the end. The term does not address intentionality (does the child understand it needs to take the medication) to distinguish between medication errors or intentional use issues. Therefore, the term is considered imprecise and ambiguous and will not be added to MedDRA.

Detail Report

CR Number: 2026015002		Implementation Date: 20-Jan-26	Related CR: 2026015002		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	<i>Post procedural rash</i>	Rejected			
HLT primary	<i>Non-site specific procedural complications</i>				
SOC primary	<i>Injury, poisoning and procedural complications</i>				
MSSO Comment:	The proposal to add a new PT Post procedural rash to primary HLT Non-site specific procedural complications in SOC Injury, poisoning and procedural complications and secondary HLT Rashes, eruptions and exanthems NEC in SOC Skin and subcutaneous tissue disorders is not approved. MedDRA refrains from adding post-procedural terms for every nonspecific dermatological finding. The existence of other post-procedural dermatological terms in MedDRA does not establish a requirement to continue adding similar terms by analogy. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.				

CR Number: 2026015003		Implementation Date: 20-Jan-26	Related CR: 2026015003		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	<i>Urticaria drug induced</i>	Rejected			
MSSO Comment:	The proposal to add a new PT Urticaria drug induced is not approved. The proposed term exists as LLT Urticaria drug-induced and is appropriately placed under PT Urticaria.				

CR Number: 2026019000		Implementation Date: 20-Jan-26	Related CR: 2026019000		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	<i>Buffer base</i>	Rejected			
HLT primary	<i>Blood gas and acid base analyses</i>				
SOC primary	<i>Investigations</i>				
MSSO Comment:	The proposal to add a new PT Buffer base to primary HLT Blood gas and acid base analyses in SOC Investigations is not approved. Buffer base as the sum of all buffer anions in blood gas analysis was introduced in 1958. The parameter has been superseded by base excess (BE) and standard base excess (SBE) in standard laboratory analyses, which provide the same clinical insight in a more standardized and interpretable way. Please consider existing terms related to Base excess for your specific coding needs.				

Detail Report

CR Number: 2026019001		Implementation Date: 27-Jan-26	Related CR: 2026019001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Radiocephalic fistula</i>	Approved as Requested	Proposed LLT <i>Radiocephalic fistula</i>	10093475 Current
			To PT <i>Arteriovenous fistula operation</i>	10003190
MSSO Comment: The proposal to add a new term Radiocephalic fistula is approved as requested. Since initially described in 1966, radiocephalic fistula or Brescia–Cimino fistula is one of the most commonly performed fistulas in the world of arteriovenous access. It connects the radial artery and the cephalic vein. The indications for insertion include, but are not limited to, hemodialysis. Radiocephalic fistula will be added as LLT to PT Arteriovenous fistula operation aligned with its eponym expression LLT Brescia-Cimino fistula.				

CR Number: 2026020000		Implementation Date: 27-Jan-26	Related CR: 2026020000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Acupoint embedding</i>	Approved as Requested	Proposed LLT <i>Acupoint embedding</i>	10093476 Current
			To PT <i>Acupoint application therapy</i>	10083706
MSSO Comment: The proposal to add a new term Acupoint embedding is approved as requested. Acupoint embedding refers to any technique where a material is implanted into an acupoint for sustained stimulation. The embedded material produces continuous acupoint stimulation working as a semi-permanent acupuncture needle that results in decreasing the total time of the treatment and increasing the treatment efficacy. Acupoint embedding will be added as LLT to the PT Acupoint application therapy. In a related change, LLT Acupoint catgut embedding will be moved from PT Acupuncture to PT Acupoint application therapy, for better alignment.				

CR Number: 2026027003		Implementation Date: 27-Jan-26	Related CR: 2026020000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Acupoint catgut embedding</i>	Approved as Requested	Proposed LLT <i>Acupoint catgut embedding</i>	10090149 Current
From PT	<i>Acupuncture</i>		From PT <i>Acupuncture</i>	10000646
To PT	<i>Acupoint application therapy</i>		To PT <i>Acupoint application therapy</i>	10083706
MSSO Comment:				

Detail Report

CR Number: 2026020001		Implementation Date: 27-Jan-26	Related CR: 2026020001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Genital tract infection</i>	Approved as Requested	Proposed LLT <i>Genital tract infection</i>	10093477 Current
			To PT <i>Genital infection</i>	10048461

MSSO Comment: The proposal to add a new term Genital tract infection is approved as requested. A genital tract infection is an infectious process affecting the mucosal structures and internal organs of the genital tract, and it constitutes a conceptually subordinate and more specific category within the broader concept of genital infection, which encompasses infectious conditions of the genital region as a whole, including external genitalia. Genital tract infection will be added as an LLT to PT Genital infection.

CR Number: 2026020003		Implementation Date: 27-Jan-26	Related CR: 2026020003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Chemical-induced migraines</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Chemical-induced migraines is not approved. Although chemical or odor exposure are well recognized triggers for migraine, these triggers do not create a separate diagnostic category. Chemical-induced migraine is not a formal diagnosis, it does not appear as a distinct entity in international classifications, and it lacks agreed diagnostic criteria. The term chemical-induced migraines combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

CR Number: 2026020004		Implementation Date: 27-Jan-26	Related CR: 2026020004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Functional mitral insufficiency</i>	Approved as Requested	Proposed LLT <i>Functional mitral insufficiency</i>	10093483 Current
			To PT <i>Mitral valve incompetence</i>	10027727

MSSO Comment: The proposal to add a new LLT Functional mitral insufficiency is approved as requested. Functional mitral insufficiency describes secondary mitral regurgitation due to ventricular pathology rather than primary valve disease. Functional mitral insufficiency will be added as LLT to PT Mitral valve incompetence.

Detail Report

CR Number: 2026020005		Implementation Date: 27-Jan-26	Related CR: 2026020005		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Food-dependent exercise-induced anaphylaxis</i>	Approved Not as Requested	Proposed LLT <i>Cofactor-enhanced anaphylactic reaction to food</i>	10093484	Current
			To PT <i>Anaphylactic reaction</i>	10002198	

MSSO Comment: The proposal to add a new LLT Food-dependent exercise-induced anaphylaxis is approved but not as requested. Food-dependent exercise-induced anaphylaxis is a rare but potentially severe allergic reaction that can occur upon eating a specific food allergen followed by exercise as a triggering cofactor. Cofactors are external factors that can lower the threshold dose of an allergen and amplify the severity of allergic reactions, turning mild or moderate responses into severe anaphylaxis. These include exercise, nonsteroidal anti-inflammatory drugs, alcohol, menstruation, and infections. The proposed term will be modified and added as a more general LLT Cofactor-enhanced anaphylactic reaction to food to PT Anaphylactic reaction.

CR Number: 2026020006		Implementation Date: 27-Jan-26	Related CR: 2026020006		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Completion hysterectomy</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Completion hysterectomy is not approved. A completion hysterectomy refers to a hysterectomy performed after a prior gynecologic surgery that did not include removal of the uterus (or did not fully remove it), with the intent of completing definitive surgical management. MedDRA does not usually add specific terms for reoperations. The hysterectomy can be coded with existing terms describing the most specific surgery type.

CR Number: 2026020007		Implementation Date: 27-Jan-26	Related CR: 2026020007		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Bronchiectasis-COPD Overlap Syndrome</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Bronchiectasis-COPD Overlap Syndrome is not approved. Bronchiectasis-COPD Overlap Syndrome starts to be recognized as a clinical phenotype or overlap syndrome in research and specialty literature, but it has not yet reached the status of an internationally distinguished clinical diagnostic entity. The co-existence of COPD and bronchiectasis is well recognized, but usually only described as comorbidities. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.

Detail Report

CR Number: 2026020008		Implementation Date: 27-Jan-26	Related CR: 2026020008	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Double aortic arch</i>	Approved Not as Requested	Proposed PT <i>Double aortic arch</i>	10093479
			HLT primary <i>Great vessel disorders congenital</i>	10018717
			SOC primary <i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary <i>Vascular malformations and acquired anomalies</i>	10047091
			SOC secondary <i>Vascular disorders</i>	10047065

MSSO Comment: The proposal to add a new LLT Double aortic arch is approved but not as requested. Double aortic arch is the commonest form of a group of defects that affect the development of the aorta. This defect causes an abnormal circular formation of blood vessels, called a vascular ring, that can result in life-threatening airway obstruction. Double aortic arch will be added as PT to primary HLT Great vessel disorders congenital and secondary HLT Vascular malformations and acquired anomalies.

CR Number: 2026020009		Implementation Date: 27-Jan-26	Related CR: 2026020009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Onychoplasty</i>	Approved Not as Requested	Proposed PT <i>Onychoplasty</i>	10093481
			HLT primary <i>Nail therapeutic procedures</i>	10028701
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new LLT Onychoplasty is approved but not as requested. Onychoplasty is a specialized procedure to repair and reconstruct damaged nails, often using resin or gel to rebuild the nail plate for protection and improved appearance. Onychoplasty will be added as a PT to HLT Nail therapeutic procedures.

Detail Report

CR Number: 2026020010		Implementation Date: 27-Jan-26	Related CR: 2026020010	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Intravascular ultrasound</i>	Approved Not as Requested	Proposed PT <i>Intravascular ultrasound</i>	10093482
			HLT primary <i>Vascular imaging procedures NEC</i>	10047078
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT Intravascular ultrasound is approved but not as requested. Intravascular ultrasound is an intravascular imaging modality primarily used in interventional cardiology to characterize lesion morphology, quantify plaque burden, guide stent sizing, assess stent expansion, and identify procedural complications. Intravascular ultrasound will be added as PT to HLT Vascular imaging procedures NEC.

CR Number: 2026020011		Implementation Date: 27-Jan-26	Related CR: 2026020011	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Tubotitis</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Tubotitis is not approved. The proposed term is rarely used in modern medicine and there are other more widely used terms which represent the concept such as LLT/PT Otolpalpingitis.

CR Number: 2026020012		Implementation Date: 27-Jan-26	Related CR: 2026020012	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Hypertensive dyscirculatory encephalopathy</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Hypertensive dyscirculatory encephalopathy is not approved. Hypertensive dyscirculatory encephalopathy" is a legacy / regional term, mostly used in older Eastern European and post-Soviet literature, to describe chronic cerebral dysfunction attributed to long-standing hypertension. It lacks clear, standardized diagnostic criteria and does not map cleanly onto contemporary classifications. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. Please consider LLT Hypertensive encephalopathy and LLT Dyscirculatory encephalopathy for your coding needs, depending on specific case scenario.

Detail Report

CR Number: 2026020013	Implementation Date: 27-Jan-26	Related CR: 2026020013	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Inflammation of internal acoustic meatus</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Inflammation of internal acoustic meatus is not approved. Inflammation of internal acoustic meatus is a radiological (MRI) descriptor. It indicates an inflammation affecting the structures passing through the passage between inner ear and brain, potentially affecting nerves and meninges through pressure injury. The term describes an anatomical region, rather than to a concrete pathology.

CR Number: 2026020014	Implementation Date: 27-Jan-26	Related CR: 2026020014	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Waist fracture</i>	Approved Not as Requested	Proposed LLT <i>Scaphoid waist fracture</i> 10093485 Current To PT <i>Wrist fracture</i> 10048049

MSSO Comment: The proposal to add a new LLT Waist fracture is approved but not as requested. A waist fracture is almost always referring to a fracture of the "waist" (the mid-portion) of the scaphoid, the carpal bone that is typically injured after a fall with the wrist in extension. It is the most common location of scaphoid fracture and carries a risk of delayed union or avascular necrosis if not properly diagnosed and treated. The proposed term will be modified for clarity and added as LLT Scaphoid waist fracture to PT Wrist fracture, aligned with existing LLT Fractured scaphoid.

CR Number: 2026020015	Implementation Date: 27-Jan-26	Related CR: 2026020015	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Charcot foot reconstruction</i>	Approved Not as Requested	Proposed PT <i>Charcot foot reconstruction</i> 10093478 HLT primary <i>Limb therapeutic procedures</i> 10024502 SOC primary <i>Surgical and medical procedures</i> 10042613

MSSO Comment: The proposal to add a new LLT Charcot foot reconstruction is approved but not as requested. Charcot reconstruction is a complex orthopedic procedure used to treat severe foot deformities caused by Charcot neuroarthropathy, most commonly associated with diabetic neuropathy. The surgery aims to repair, realign, and stabilize a collapsed foot rather than restore normal anatomy. Its main goal is to achieve a stable, plantigrade foot capable of bearing weight in regular footwear. Charcot foot reconstruction will be added as PT to HLT Limb therapeutic procedures.

Detail Report

CR Number: 2026020016		Implementation Date: 27-Jan-26	Related CR: 2026020016		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Volar plate release</i>	Approved as Requested	Proposed LLT <i>Volar plate release</i>	10093486	Current
			To PT <i>Finger repair operation</i>	10016687	

MSSO Comment: The proposal to add a new LLT Volar plate release is approved as requested. A volar plate release is a surgical technique used to restore motion in a stiff finger joint by partially or completely dividing the volar plate, a dense fibrocartilaginous structure on the palm side of the joint which stabilizes the proximal and distal interphalangeal joints. It is aimed to prevent hyperextension or adhesions after injuries such as hyperextension trauma, avulsion fractures, or in congenital/acquired contractures like camptodactyly. Volar plate release will be added as an LLT to PT Finger repair operation.

CR Number: 2026020017		Implementation Date: 27-Jan-26	Related CR: 2026020017		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Percutaneous vertebral artery intervention</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Percutaneous vertebral artery intervention is not approved. Percutaneous vertebral artery intervention provides detailed anatomical description and the route of access, but no specific type of intervention making it an unspecific term.

CR Number: 2026020018		Implementation Date: 27-Jan-26	Related CR: 2026020018		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Epicondyle release surgery</i>	Approved Not as Requested	Proposed PT <i>Epicondyle release surgery</i>	10093480	
			HLT primary <i>Joint therapeutic procedures</i>	10023221	
			SOC primary <i>Surgical and medical procedures</i>	10042613	

MSSO Comment: The proposal to add a new LLT Epicondyle release surgery is approved but not as requested. Epicondyle release surgery is a surgical procedure used to treat chronic epicondylitis that has not responded to conservative management. It involves releasing and debriding the diseased tendon fibers at their attachment to the medial or lateral epicondyle of the humerus. Epicondyle release surgery will be added as a PT to HLT Joint therapeutic procedures.

Detail Report

CR Number: 2026020019		Implementation Date: 27-Jan-26	Related CR: 2026020019	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Gigantomastia</i>	Approved as Requested	Proposed LLT <i>Gigantomastia</i>	10093487 Current
			To PT <i>Breast enlargement</i>	10006242
MSSO Comment: The proposal to add a new LLT Gigantomastia is approved as requested. Gigantomastia is a condition characterized by excessive breast tissue resulting in abnormally large breasts, which may be associated with physical discomfort, functional limitation, or psychosocial distress. Gigantomastia will be added as LLT to PT Breast enlargement.				

CR Number: 2026020020		Implementation Date: 27-Jan-26	Related CR: 2026020020	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Product lost</i>	Rejected		
MSSO Comment: The proposal to add a new LLT Product lost is not approved. While the PTC documents do advise to capture the cause or contributing factors for the dose omission to help determine if the omission is a medication error or not, it is not possible to add terms for every possible scenario.				

Detail Report

CR Number: 2026021000		Implementation Date: 27-Jan-26	Related CR: 2026021000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Vestibular syndrome</i>	Approved as Requested	Proposed PT	
			<i>Vestibular syndrome</i>	10093488
			HLT primary	
			<i>Inner ear disorders NEC</i>	10023566
			SOC primary	
			<i>Ear and labyrinth disorders</i>	10013993
			HLT secondary	
			<i>Nausea and vomiting symptoms</i>	10028817
			SOC secondary	
			<i>Gastrointestinal disorders</i>	10017947
			HLT secondary	
			<i>Vertigos NEC</i>	10052769
			SOC secondary	
			<i>Nervous system disorders</i>	10029205

MSSO
Comment: The proposal to add a new term Vestibular syndrome is approved as requested. A vestibular syndrome is a clinical syndrome characterized by symptoms resulting from dysfunction of the vestibular system, including vertigo or dizziness, imbalance, postural instability, spatial disorientation, and associated autonomic symptoms, irrespective of onset, duration, or underlying cause. Vestibular syndrome will be added as PT with a primary link to HLT Inner ear disorders NEC and to secondary HLT Nausea and vomiting symptoms and to secondary HLT Vertigos NEC. In a related change, PT Acute vestibular syndrome will be demoted under this new PT Vestibular syndrome.

CR Number: 2026027004		Implementation Date: 27-Jan-26	Related CR: 2026021000	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Acute vestibular syndrome</i>	Approved as Requested	PT to merge	
To PT	<i>Vestibular syndrome</i>		<i>Acute vestibular syndrome</i>	10063559 Current
			To PT	
			<i>Vestibular syndrome</i>	10093488

MSSO
Comment:

Detail Report

CR Number: 2026021010		Implementation Date: 27-Jan-26	Related CR: 2026021010		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Hand ulceration	Approved as Requested	Proposed LLT		
To PT	Skin ulcer		Hand ulceration	10093489	Current
			To PT		
			Skin ulcer	10040943	

MSSO Comment: The proposal to add a new LLT Hand ulceration to PT Skin ulcer is approved as requested. Skin ulcers develop as a result of poor circulation, infections, or prolonged pressure and may occur on hands and feet or elsewhere on the body.

CR Number: 2026021011		Implementation Date: 27-Jan-26	Related CR: 2026021011		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Genitourinary inflammation	Rejected			

MSSO Comment: The proposal to add a new LLT Genitourinary inflammation is not approved. If no solid reasons are provided to justify them, MedDRA does not usually support the creation of generic inflammation terms conceived as a counterpart to equivalent infection-related terms, such as Genitourinary tract infection in this case.

CR Number: 2026021012		Implementation Date: 27-Jan-26	Related CR: 2026021012		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Allergy laryngopharyngitis	Rejected			

MSSO Comment: The proposal to add a new LLT Allergy laryngopharyngitis is not approved. The proposed term has recently been rejected in change request 2026008008 as it combines several anatomical regions. Such terms are usually only added to MedDRA if the combination term is distinct with respect to treatment, prognosis, and clinical approaches. Please refer to Section 3.5 and Section 3.7 of the MedDRA Term Selection: Points to Consider Document regarding coding approaches/considerations.

Detail Report

CR Number: 2026022000		Implementation Date: 27-Jan-26	Related CR: 2026022000	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Juvenile parkinsonism	Approved as Requested	Proposed PT	
			Juvenile parkinsonism	10093490
			HLT primary	
			Central nervous system disorders congenital NEC	10009713
			SOC primary	
			Congenital, familial and genetic disorders	10010331
			HLT secondary	
			Structural change, deposit and degeneration of eye NEC	10042260
			SOC secondary	
			Eye disorders	10015919
			HLT secondary	
			Parkinson's disease and parkinsonism	10034005
			SOC secondary	
			Nervous system disorders	10029205

MSSO Comment: The proposal to add a new PT Juvenile parkinsonism is approved as requested. Juvenile Parkinson's disease is a rare movement disorder that presents before the age of 21 years. Juvenile parkinsonism will be added as PT to primary HLT Central nervous system disorders congenital NEC, to secondary HLT Structural change, deposit and degeneration of eye NEC and to secondary HLT Parkinson's disease and parkinsonism.

CR Number: 2026022001		Implementation Date: 27-Jan-26	Related CR: 2026022001	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	Kufor-Rakeb syndrome	Approved as Requested	PT to merge	
To PT	Juvenile parkinsonism		Kufor-Rakeb syndrome	10091529 Current
			To PT	
			Juvenile parkinsonism	10093490

MSSO Comment: The proposal to demote the PT Kufor-Rakeb syndrome under PT Juvenile parkinsonism is approved as requested. Kufor-Rakeb syndrome is one of the distinct types of Juvenile parkinsonism caused by the ATP13A2 mutation and inherited as an autosomal recessive. Juvenile parkinsonism was added as a PT in change request 2026022000.

Detail Report

CR Number: 2026023000	Implementation Date: 27-Jan-26	Related CR: 2026023000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Dependent lung changes</i>	Rejected	

MSSO Comment: The proposal to add a new term Dependent lung changes is not approved. As noted in subscriber's justification, dependent lung changes are physiologic phenomena, in which gravity causes preferential perfusion and mild compression of alveoli in dependent regions, leading to transient increased attenuation or atelectasis in supine or decubitus positions. The proposed concept is mainly a radiologic descriptor and not a pathological entity.

CR Number: 2026023002	Implementation Date: 30-Jan-26	Related CR: 2026023002	MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement
Proposed LLT	<i>Fracture plated</i>	Rejected	
From PT	<i>Internal fixation of fracture</i>		
To PT	<i>External fixation of fracture</i>		

MSSO Comment: The proposal to move the LLT Fracture plated from PT Internal fixation of fracture to PT External fixation of fracture is not approved. An orthopedic plate is a form of internal fixation used in orthopedic surgery to hold fractures in place. It is a procedure of fracture stabilization with a plate and screws applied surgically. On the contrary, external fixation uses pins or wires connected to a frame outside the body, not plates.

CR Number: 2026026000	Implementation Date: 27-Jan-26	Related CR: 2026026000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Sliding syndrome</i>	Rejected	

MSSO Comment: The proposal to add a new term Sliding syndrome is not approved. Syndrome de glissement - a rapid, global functional decline in a frail older adult following an acute stressor, consistent with terminal frailty / failure to thrive - does not have a recognized or standardized English equivalent. Its components are acknowledged, but the unified syndrome is not available as an internationally recognized clinical entity in English or other European languages. Even reviews in French geriatric journals explicitly note that "syndrome de glissement" is controversial and poorly defined.

Detail Report

CR Number: 2026026042		Implementation Date: 30-Jan-26	Related CR: 2026026042	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Hypokinetic hypertrophic cardiomyopathy</i>	Approved Not as Requested	Proposed LLT <i>Hypertrophic cardiomyopathy with reduced ventricular function</i> To PT <i>Hypertrophic cardiomyopathy</i>	10093492 Current 10020871
MSSO Comment:	The proposal to add a new LLT Hypokinetic hypertrophic cardiomyopathy is approved but not as requested. Hypertrophic cardiomyopathy with reduced ventricular function (also called Hypokinetic hypertrophic cardiomyopathy) is an end-stage, adverse phenotype of hypertrophic cardiomyopathy in which persistent myocardial hypertrophy is accompanied by reduced systolic function. It involves myocyte loss, replacement fibrosis, and microvascular ischemia, leading to a hypokinetic left ventricle and reduced ejection fraction. Although uncommon, it carries a worse prognosis. The proposed term will be modified to the term most commonly found in literature and added as LLT Hypertrophic cardiomyopathy with reduced ventricular function to PT Hypertrophic cardiomyopathy.			

CR Number: 2026026043		Implementation Date: 30-Jan-26	Related CR: 2026026043	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Clenched fist syndrome</i>	Approved Not as Requested	Proposed LLT <i>Clenched fist syndrome</i> To PT <i>Conversion disorder</i>	10093491 Current 10010893
MSSO Comment:	The proposal to add a new PT Clenched fist syndrome is approved but not as requested. The Clenched fist syndrome is a type of a psychiatric disorder, in which the patients show flexion finger contractures. Although no organic etiology can be identified, the syndrome in most cases presents with pain and paradoxical stiffness and is believed to be a conversion disorder, i.e. unconsciously motivated and produced. Clenched fist syndrome will be added as LLT to PT Conversion disorder.			

CR Number: 2026026044		Implementation Date: 30-Jan-26	Related CR: 2026026044	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Lumbar injury</i>	Rejected		
MSSO Comment:	The proposal to add a new LLT Lumbar injury is not approved. A lumbar injury is an injury affecting the structures of the lumbar spine, which consists of the five lower vertebrae associated anatomical structures such as intervertebral discs, muscles, ligaments, facet joints, spinal nerves, and supporting connective tissues. The term "lumbar injury" is often used as a broad or nonspecific descriptor and is considered unspecific and potentially ambiguous.			

Detail Report

CR Number: 2026027000		Implementation Date: 30-Jan-26	Related CR: 2026027000		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Nasolacrimal duct obstruction	Rejected			

MSSO Comment: The proposal to add a new term Nasolacrimal duct obstruction is not approved. The concept exists in MedDRA in both the acquired and congenital form with various synonym LLTs linked to either PT Dacryostenosis acquired or PT Dacryostenosis congenital including e.g. LLT Obstruction of nasolacrimal duct, neonatal.

CR Number: 2026027001		Implementation Date: 30-Jan-26	Related CR: 2026027001		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	Dysmorphic urinary red blood cells present	Approved as Requested	Proposed PT		
HLT primary	Urinalysis NEC		Dysmorphic urinary red blood cells present		10093497
SOC primary	Investigations		HLT primary		
			Urinalysis NEC		10046512
			SOC primary		
			Investigations		10022891

MSSO Comment: The proposal to add a new PT Dysmorphic urinary red blood cells present to primary HLT Urinalysis NEC in SOC Investigations is approved as requested. Dysmorphic haematuria refers to the presence of misshapen red blood cells in urine, which is a sign of a potential pathology in glomeruli. These cells have altered shapes, such as acanthocytes or stomatocytes, and their presence, especially along with red blood cell casts and proteinuria, suggests glomerular disease. Dysmorphic urinary red blood cells present will be added as qualified counterpart of PT Dysmorphic urinary red blood cells.

CR Number: 2026027005		Implementation Date: 30-Jan-26	Related CR: 2026027005		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Hepatoptosis	Approved as Requested	Proposed LLT		
			Hepatoptosis		10093495 Current
			To PT		
			Hepatic displacement		10051565

MSSO Comment: The proposal to add a new term Hepatoptosis is approved as requested. Hepatoptosis is downward displacement of the liver, which may result from laxness of suspensory ligaments, diminished tone of abdominal muscles, emphysema, right pleural effusion or empyema, subphrenic abscess, or spinal deformity. Hepatoptosis will be added as LLT to PT Hepatic displacement. In a related change, PT Liver prolapse will be demoted under PT Hepatic displacement.

Detail Report

CR Number: 2026030001		Implementation Date: 30-Jan-26	Related CR: 2026027005		
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #	
PT to merge	<i>Liver prolapse</i>	Approved as Requested	PT to merge		
To PT	<i>Hepatic displacement</i>		<i>Liver prolapse</i>	10058438	Current
			To PT		
			<i>Hepatic displacement</i>	10051565	

MSSO
Comment:

CR Number: 2026027006		Implementation Date: 30-Jan-26	Related CR: 2026027006		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Liver hypermobility</i>	Rejected			

MSSO The proposal to add a new term Liver hypermobility is not approved. The concept is adequately represented by recently added synonym LLT Hepatoptosis and LLT
Comment: Wandering liver in change requests 2026027005 and 2026027008, respectively.

CR Number: 2026027007		Implementation Date: 30-Jan-26	Related CR: 2026027007		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Floating liver</i>	Rejected			

MSSO The proposal to add a new term Floating liver is not approved. The concept is adequately represented by recently added synonym LLT Hepatoptosis and LLT Wandering
Comment: liver in change requests 2026027005 and 2026027008, respectively.

Detail Report

CR Number: 2026027008		Implementation Date: 30-Jan-26	Related CR: 2026027008	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Wandering liver</i>	Approved as Requested	Proposed LLT <i>Wandering liver</i>	10093494 Current
			To PT <i>Hepatic displacement</i>	10051565
MSSO Comment: The proposal to add a new term Wandering liver is approved as requested. Wandering Liver is a synonym for hepatoptosis and describes when the liver moves out of its normal position due to lax suspensory ligaments, sometimes leading to vascular issues or bowel obstruction. Wandering liver will be added as LLT to PT Hepatic displacement aligned with synonym LLT Hepatoptosis added in change request 2026027005.				

CR Number: 2026027009		Implementation Date: 30-Jan-26	Related CR: 2026027009	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Ectopic liver</i>	Approved as Requested	Proposed PT <i>Ectopic liver</i>	10093493
			HLT primary <i>Hepatobiliary abnormalities congenital</i>	10019803
			SOC primary <i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary <i>Hepatic and hepatobiliary disorders NEC</i>	10027681
			SOC secondary <i>Hepatobiliary disorders</i>	10019805
MSSO Comment: The proposal to add a new term Ectopic liver is approved as requested. Ectopic liver is a rare, usually asymptomatic, developmental anomaly defined as the presence of mature, autonomous hepatic tissue located outside the normal liver without any direct vascular or biliary connection to it. Ectopic liver will be added as PT to primary HLT Hepatobiliary abnormalities congenital and secondary HLT Hepatic and hepatobiliary disorders NEC.				

Detail Report

CR Number: 2026028000		Implementation Date: 30-Jan-26	Related CR: 2026028000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Invasive pulmonary aspergillosis</i>	Approved Not as Requested	Proposed PT <i>Invasive pulmonary aspergillosis</i>	10093496
			HLT primary <i>Aspergillus infections</i>	10003486
			SOC primary <i>Infections and infestations</i>	10021881
			HLT secondary <i>Fungal lower respiratory tract infections</i>	10017537
			SOC secondary <i>Respiratory, thoracic and mediastinal disorders</i>	10038738

MSSO Comment: The proposal to add a new LLT Invasive pulmonary aspergillosis is approved but not as requested. Invasive pulmonary aspergillosis is a rapidly progressive and often fatal lung fungal infection in which Aspergillus invades lung tissue, often with vascular involvement, occurring mainly in immunocompromised patients and associated with high morbidity and mortality. Invasive pulmonary aspergillosis will be added as PT with a primary link to HLT Aspergillus infections, and a secondary link to HLT Fungal lower respiratory tract infections. In a related change, LLT Invasive bronchopulmonary aspergillosis will be moved from PT Bronchopulmonary aspergillosis to new PT Invasive pulmonary aspergillosis.

CR Number: 2026030002		Implementation Date: 30-Jan-26	Related CR: 2026028000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Invasive bronchopulmonary aspergillosis</i>	Approved as Requested	Proposed LLT <i>Invasive bronchopulmonary aspergillosis</i>	10022881 Current
From PT	<i>Bronchopulmonary aspergillosis</i>		From PT <i>Bronchopulmonary aspergillosis</i>	10006473
To PT	<i>Invasive pulmonary aspergillosis</i>		To PT <i>Invasive pulmonary aspergillosis</i>	10093496

MSSO Comment:

Detail Report

CR Number: 2026029000		Implementation Date: 02-Feb-26	Related CR: 2026029000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Nasopharyngeal cyst</i>	Approved Not as Requested	Proposed LLT <i>Nasopharynx cyst</i>	10093499 Current
			To PT <i>Pharyngeal cyst</i>	10034821
MSSO Comment: The proposal to add a new term Nasopharyngeal cyst is approved but not as requested. Pharynx is divided into nasopharynx, oropharynx and laryngopharynx. Cysts in the nasopharynx are rare lesions, which are infrequently recognized on routine nasopharyngoscopy or imaging. Though these cysts are asymptomatic in most cases, they may be the source for spontaneous cerebrospinal fluid rhinorrhea, purulent rhinorrhea refractory to antibiotics and intranasal steroids, rhinosinusitis, nasal obstruction, headache, seizures, halitosis and middle ear effusion. The proposed term will be modified for clarity and added as LLT Nasopharynx cyst to PT Pharyngeal cyst.				

CR Number: 2026029001		Implementation Date: 02-Feb-26	Related CR: 2026029001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Dermal filler displacement</i>	Rejected		
MSSO Comment: The proposal to add a new term Dermal filler displacement is not approved. Dermal filler displacement and Dermal filler migration are used synonymously in context of fillers; therefore, only Dermal filler migration will be added in change request 2026029002.				

CR Number: 2026029002		Implementation Date: 02-Feb-26	Related CR: 2026029002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Dermal filler migration</i>	Approved as Requested	Proposed PT <i>Dermal filler migration</i>	10093500
			HLT primary <i>Skin procedural complications</i>	10040911
			SOC primary <i>Injury, poisoning and procedural complications</i>	10022117
MSSO Comment: The proposal to add a new term Dermal filler migration is approved as requested. Dermal filler migration is defined as the presence of filler at a location distant from the injection site. Dermal filler migration will be added as new PT to HLT Skin procedural complications.				

Detail Report

CR Number: 2026029003		Implementation Date: 13-Feb-26		Related CR: 2026029003	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Airway device insertion</i>	Approved After Suspension	Proposed PT		
HLT primary	<i>Respiratory tract therapeutic procedures NEC</i>		<i>Airway device insertion</i>	10093523	
SOC primary	<i>Surgical and medical procedures</i>		HLT primary		
			<i>Respiratory tract therapeutic procedures NEC</i>	10052727	
			SOC primary		
			<i>Surgical and medical procedures</i>	10042613	
MSSO					
Comment:	The proposal to add a new PT Airway device insertion to primary HLT Respiratory tract therapeutic procedures NEC in SOC Surgical and medical procedures is approved as requested after suspension. Airways device insertion will be added as a broad concept for airway device therapies, as not all airway devices are patency devices. In a related change, existing PT Airway patency device insertion will be demoted under new PT Airway device insertion.				

CR Number: 2026044009		Implementation Date: 13-Feb-26		Related CR: 2026029003			
<u>Change Requested</u>		<u>Final Disposition</u>		<u>Final Placement</u>			
Demote a PT				<u>MedDRA Code #</u>			
PT to merge	<i>Airway patency device insertion</i>	Approved as Requested		PT to merge			
To PT	<i>Airway device insertion</i>			<i>Airway patency device insertion</i>	10081227 Current		
				To PT			
				<i>Airway device insertion</i>	10093523		

MSSO
Comment:

Detail Report

CR Number: 2026029004		Implementation Date: 02-Feb-26	Related CR: 2026029004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Verheij Syndrome	Approved Not as Requested	Proposed PT Verheij syndrome	10093501
			HLT primary Gene mutations and other alterations NEC	10083625
			SOC primary Congenital, familial and genetic disorders	10010331
			HLT secondary Pervasive developmental disorders NEC	10034740
			SOC secondary Psychiatric disorders	10037175
			HLT secondary Nervous system disorders NEC	10057185
			SOC secondary Nervous system disorders	10029205

MSSO Comment: The proposal to add a new LLT Verheij Syndrome is approved but not as requested. Intellectual disability-cardiac anomalies-short stature-joint laxity syndrome due to mutation in PUF60 gene, also known as Verheij syndrome, is a rare genetic multiple congenital anomalies/dysmorphic syndrome characterized by intrauterine and postnatal growth restriction, global developmental delay, intellectual disability, and dysmorphic facial features. The proposed term will be modified for MedDRA capitalization rules and added as PT Verheij syndrome to primary HLT Gene mutations and other alterations NEC, secondary HTL Pervasive developmental disorders NEC, secondary HLT Nervous system disorders NEC and secondary HLT Musculoskeletal and connective tissue deformities of skull, face and buccal cavity.

CR Number: 2026033001		Implementation Date: 02-Feb-26	Related CR: 2026029004	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Verheij syndrome	Approved as Requested	Proposed PT Verheij syndrome	10093501
To HLT	Musculoskeletal and connective tissue deformities of skull, face and buccal cavity		To HLT Musculoskeletal and connective tissue deformities of skull, face and buccal cavity	10028378

MSSO Comment:

Detail Report

CR Number: 2026033002		Implementation Date: 10-Feb-26	Related CR: 2026033002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Diaphragmatic mesothelial cyst</i>	Approved as Requested	Proposed LLT <i>Diaphragmatic mesothelial cyst</i>	10093502 Current
			To PT <i>Congenital cyst</i>	10062328

MSSO Comment: The proposal to add a new term Diaphragmatic mesothelial cyst is approved as requested. Mesothelial cysts of the diaphragm are uncommon congenital lesions that arise from coelomic remnants. They often appear as cystic masses between the thoracic wall and the liver, particularly in the right posterolateral costophrenic angle. Diaphragmatic mesothelial cyst will be added as an LLT to PT Congenital cyst.

CR Number: 2026033003		Implementation Date: 10-Feb-26	Related CR: 2026033003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Tarsal cyst</i>	Rejected		

MSSO Comment: The proposal to add a new term Tarsal cyst is not approved. The proposed term is ambiguous, as it can also refer to the tarsal region of the foot. Please consider existing terms such as LLT Meibomian cyst or LLT/PT Chalazion.

CR Number: 2026035000		Implementation Date: 10-Feb-26	Related CR: 2026035000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Acquired cerebrovascular arteriovenous malformation</i>	Approved as Requested	Proposed LLT <i>Acquired cerebrovascular arteriovenous malformation</i>	10093504 Current
			To PT <i>Acquired vascular malformation cerebral</i>	10093503

MSSO Comment: The proposal to add a new term Acquired cerebrovascular arteriovenous malformation is approved as requested. Cerebral arteriovenous malformations (AVMs) are rare, complex vascular lesions of uncertain pathogenesis involving dysplastic vessel formation. Originally thought to be exclusively congenital, intracerebral AVMs are now known to also arise de novo in genetically predisposed individuals and following surgery or trauma. Acquired cerebrovascular arteriovenous malformation will be added as an LLT to new PT Acquired vascular malformation cerebral added in change request 2026035001.

Detail Report

CR Number: 2026035001		Implementation Date: 10-Feb-26	Related CR: 2026035001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Acquired vascular malformation cerebral</i>	Approved Not as Requested	Proposed PT	
			<i>Acquired vascular malformation cerebral</i>	10093503
			HLT primary	
			<i>Central nervous system vascular disorders NEC</i>	10007962
			SOC primary	
			<i>Nervous system disorders</i>	10029205
			HLT secondary	
			<i>Vascular malformations and acquired anomalies</i>	10047091
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO Comment: The proposal to add a new LLT Acquired vascular malformation cerebral is approved but not as requested. Cerebral arteriovenous malformations (AVMs) are rare, complex vascular lesions of uncertain pathogenesis involving dysplastic vessel formation. Originally thought to be exclusively congenital, intracerebral AVMs are now known to also arise de novo in genetically predisposed individuals and following surgery or trauma. Acquired vascular malformation cerebral will be added as a PT to primary HLT Central nervous system vascular disorders NEC and to secondary HLT Vascular malformations and acquired anomalies.

CR Number: 2026035003		Implementation Date: 10-Feb-26	Related CR: 2026035003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Coronary artery lesion</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Coronary artery lesion is not approved. The MedDRA Introductory Guide states "Lesion terms may be considered for inclusion in MedDRA when the word "lesion" is part of a medical concept, e.g., PT Glomerulonephritis minimal lesion or a well documented medical concept, e.g., LLT Brain lesion. However, the term will not be added when adding a broad "lesion" term only adds an additional imprecise term to existing "disorder" concepts.

Detail Report

CR Number: 2026035004		Implementation Date: 10-Feb-26	Related CR: 2026035004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Squamous cell carcinoma in situ of the cervix</i>	Approved as Requested	Proposed LLT <i>Squamous cell carcinoma in situ of the cervix</i> To PT <i>Squamous cell carcinoma of the cervix</i>	10093505 Current 10041848
MSSO Comment: The proposal to add a new term Squamous cell carcinoma in situ of the cervix is approved as requested. Although the concept may be captured with combination coding using existing terms, Squamous cell carcinoma in situ of the cervix will be added as an LLT to PT Squamous cell carcinoma of the cervix. The MSSO has reconsidered to add the concept due to its clinical significance.				

CR Number: 2026036000		Implementation Date: 10-Feb-26	Related CR: 2026036000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Thyroid sarcoidosis</i>	Approved as Requested	Proposed PT <i>Thyroid sarcoidosis</i> HLT primary <i>Thyroid disorders NEC</i> SOC primary <i>Endocrine disorders</i> HLT secondary <i>Acute and chronic sarcoidosis</i> SOC secondary <i>Immune system disorders</i>	 10093506 10043712 10014698 10039487 10021428

MSSO Comment: The proposal to add a new term Thyroid sarcoidosis is approved as requested. It is extremely rare for sarcoidosis to occur solely in the thyroid gland, but it is usually associated with the involvement of other organs, primarily the lungs and lymph nodes. Typical manifestations are diffuse goiter and solitary or multiple thyroid nodules. Thyroid function remains intact in the majority of cases, but sometimes it can result in hypothyroidism or hyperthyroidism. Thyroid sarcoidosis will be added as PT to primary HLT Thyroid disorders NEC and secondary HLT Acute and chronic sarcoidosis.

Detail Report

CR Number: 2026036001		Implementation Date: 10-Feb-26	Related CR: 2026036001		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Foreskin wart</i>	Rejected			

MSSO Comment: The proposal to add a new term Foreskin wart is not approved. It is not possible to represent every anatomically specific location of warts. The existing term LLT/PT Penile wart may be used to represent the general concept.

CR Number: 2026037001		Implementation Date: 10-Feb-26	Related CR: 2026037001		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Device not primed</i>	Approved as Requested	Proposed LLT		
To PT	<i>Wrong technique in device usage process</i>		<i>Device not primed</i>	10093507	Current
			To PT		
			<i>Wrong technique in device usage process</i>	10077040	

MSSO Comment: The proposal to add a new LLT Device not primed to PT Wrong technique in device usage process is approved as requested. Device priming refers to the crucial process of preparing a device—such as medical infusion pumps, inhalers, or mechanical pumps—for its first use or after maintenance to ensure it operates accurately and safely.

CR Number: 2026040019		Implementation Date: 13-Feb-26	Related CR: 2026040019		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Ocular amyloidosis</i>	Approved as Requested	Proposed PT		
			<i>Ocular amyloidosis</i>	10093508	
			HLT primary		
			<i>Ocular disorders NEC</i>	10030032	
			SOC primary		
			<i>Eye disorders</i>	10015919	
			HLT secondary		
			<i>Amyloidoses</i>	10002023	
			SOC secondary		
			<i>Immune system disorders</i>	10021428	

MSSO Comment: The proposal to add a new term Ocular amyloidosis is approved as requested. Ocular amyloidosis is an uncommon manifestation of amyloid deposition and represents a rare site of involvement in systemic disease; it may affect different ocular structures, including the eyelids, conjunctiva, cornea, vitreous body, orbit, lacrimal gland, and extraocular muscles. Ocular amyloidosis will be added as PT with a primary link to HLT Ocular disorders NEC and secondary link to HLT Amyloidoses.

Detail Report

CR Number: 2026040063		Implementation Date: 13-Feb-26	Related CR: 2026040063	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Gastric artery embolisation</i>	Approved as Requested	Proposed PT	
			<i>Gastric artery embolisation</i>	10093510
			HLT primary	
			<i>Arterial therapeutic procedures (excl aortic)</i>	10003167
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new PT Gastric artery embolisation is approved as requested. Gastric artery embolisation is a minimally invasive procedure that reduces blood flow to the stomach by blocking specific gastric arteries. It is used to treat conditions such as upper gastrointestinal bleeding or, experimentally, to reduce hunger by lowering ghrelin hormone production. Gastric artery embolisation will be added as PT to HLT Arterial therapeutic procedures (excl aortic). In a related change, the American English spelled counterpart LLT Gastric artery embolization will be added to new PT Gastric artery embolisation.

CR Number: 2026044001		Implementation Date: 13-Feb-26	Related CR: 2026040063	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Gastric artery embolization</i>	Approved as Requested	Proposed LLT	
To PT	<i>Gastric artery embolisation</i>		<i>Gastric artery embolization</i>	10093511 Current
			To PT	
			<i>Gastric artery embolisation</i>	10093510

MSSO Comment:

Detail Report

CR Number: 2026040064		Implementation Date: 13-Feb-26	Related CR: 2026040064	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Gustatory hyperhidrosis</i>	Approved as Requested	Proposed PT <i>Gustatory hyperhidrosis</i>	10093509
			HLT primary <i>Cranial nerve disorders NEC</i>	10011306
			SOC primary <i>Nervous system disorders</i>	10029205
			HLT secondary <i>Apocrine and eccrine gland disorders</i>	10002982
			SOC secondary <i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new PT Gustatory hyperhidrosis is approved as requested. Although sweating on lips, forehead, scalp, and nose while eating hot and spicy foods is a physiologic response via trigeminovascular reflex, pathologic gustatory sweating is usually asymmetric and intense. Gustatory hyperhidrosis will be added as a PT to primary HLT Cranial nerve disorders NEC and to secondary HLT Apocrine and eccrine gland disorders. In a related change, PT Auriculotemporal syndrome will be change from primary SOC Gastrointestinal disorders to SOC Nervous system disorders, so that the primary HLT is now Cranial nerve disorders NEC. Furthermore, PT Auriculotemporal syndrome will change its secondary HLT from HLT Salivary gland disorders NEC to HLT Apocrine and eccrine gland disorders.

CR Number: 2026044002		Implementation Date: 13-Feb-26	Related CR: 2026040064	
<u>Change Requested</u>	Reassign Primary SOC	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Auriculotemporal syndrome</i>	Approved as Requested	Proposed PT <i>Auriculotemporal syndrome</i>	10059237
Old Primary SOC	<i>Gastrointestinal disorders</i>		Old Primary SOC <i>Gastrointestinal disorders</i>	10017947
New Primary SOC	<i>Nervous system disorders</i>		New Primary SOC <i>Nervous system disorders</i>	10029205

MSSO Comment:

Detail Report

CR Number: 2026044003		Implementation Date: 13-Feb-26	Related CR: 2026040064	
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Auriculotemporal syndrome</i>	Approved as Requested	Proposed PT	
From HLT	<i>Salivary gland disorders NEC</i>		<i>Auriculotemporal syndrome</i>	10059237
To HLT	<i>Apocrine and eccrine gland disorders</i>		From HLT	
			<i>Salivary gland disorders NEC</i>	10039407
			To HLT	
			<i>Apocrine and eccrine gland disorders</i>	10002982

MSSO
Comment:

CR Number: 2026040065		Implementation Date: 13-Feb-26	Related CR: 2026040065	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Femoroplasty</i>	Rejected		

MSSO
Comment: The proposal to add a new PT Femoroplasty is not approved. Femoroplasty is an ambiguous term because it is used in orthopedic and vascular procedures. Even in the context of orthopedic surgery, the term is imprecise, as it can refer to “femoral osteoplasty” or “femoral head/neck osteochondroplasty”, depending on the exact procedure.

CR Number: 2026040066		Implementation Date: 13-Feb-26	Related CR: 2026040066	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Non-traumatic fracture</i>	Rejected		
From PT	<i>Pathological fracture</i>			
To PT	<i>Fracture</i>			

MSSO
Comment: The proposal to move the LLT Non-traumatic fracture from PT Pathological fracture to PT Fracture is not approved. The definition of “non-traumatic fracture” as one not caused by a single event is conceptually too narrow. Trauma refers to tissue injury from external mechanical forces, which may be acute or cumulative. Stress fractures result from repeated submaximal loads that produce progressive microdamage, ultimately exceeding the bone’s repair capacity; thus, the mechanism is also mechanical and of the same nature as an acute trauma. Therefore, the concept of non-traumatic fracture excludes both acute and chronic (stress) fractures, which renders it as a synonym of the PT Pathological fracture.

Detail Report

CR Number: 2026041000		Implementation Date: 13-Feb-26	Related CR: 2026041000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Mucorales PCR test</i>	Approved as Requested	Proposed LLT <i>Mucorales PCR test</i>	10093517 Current
			To PT <i>Mucorales test</i>	10093514

MSSO Comment: The proposal to add a new LLT Mucorales PCR test is approved as requested. A Mucorales PCR test detects Mucorales DNA from tissue, blood, or bronchoalveolar lavage. It is more sensitive than culture in detecting fungi belonging to the order Mucorales, which cause mucormycosis—a severe, opportunistic fungal infection. Mucorales PCR test will be added as LLT to a new PT Mucorales test. In a related change, PT Mucorales test will be added to HLT Fungus identification and serology.

CR Number: 2026044004		Implementation Date: 13-Feb-26	Related CR: 2026041000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Mucorales test</i>	Approved as Requested	Proposed PT <i>Mucorales test</i>	10093514
HLT primary	<i>Fungus identification and serology</i>		HLT primary <i>Fungus identification and serology</i>	10017545
SOC primary	<i>Investigations</i>		SOC primary <i>Investigations</i>	10022891

MSSO Comment:

CR Number: 2026041001		Implementation Date: 13-Feb-26	Related CR: 2026041001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Total marrow and lymphoid irradiation</i>	Approved as Requested	Proposed LLT <i>Total marrow and lymphoid irradiation</i>	10093516 Current
			To PT <i>Radiotherapy</i>	10037794

MSSO Comment: The proposal to add a new LLT Total marrow and lymphoid irradiation is approved as requested. Total marrow and lymphoid irradiation (TMLI) is an emerging alternative to total body irradiation, primarily adopted at tertiary centers with advanced technical capabilities, where its organ-sparing profile allows dose escalation and potentially reduced regimen-related toxicity. TMLI is an image-guided radiation therapy technique that delivers dose to the entire skeletal bone marrow and major lymphoid tissues, including lymph node chains and spleen, while sparing non-target organs. It is primarily used as a conditioning regimen before hematopoietic stem cell transplantation with lower off-target toxicity than total body irradiation. Total marrow and lymphoid irradiation will be added as LLT to PT Radiotherapy aligned with LLT Total body radiation therapy.

Detail Report

CR Number: 2026041002		Implementation Date: 13-Feb-26	Related CR: 2026041002	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Hormonal contraceptive removal</i>	Approved Not as Requested	Proposed PT <i>Hormonal contraceptive implant removal</i>	10093512
			HLT primary <i>Contraceptive methods female</i>	10010812
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new LLT Hormonal contraceptive removal is approved but not as requested. Hormonal contraceptive implant removal involves a minor outpatient procedure where a small incision is made in the skin of the upper arm to extract the implant. The proposed term will be modified for consistency with existing MedDRA terms and added as PT Hormonal contraceptive implant removal to HLT Contraceptive methods female.

CR Number: 2026041003		Implementation Date: 13-Feb-26	Related CR: 2026041003	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>FETAL ASCITES</i>	Approved Not as Requested	Proposed LLT <i>Fetal ascites</i>	10093518 Current
			To PT <i>Foetal ascites</i>	10093513

MSSO Comment: The proposal to add a new LLT FETAL ASCITES is approved but not as requested. Fetal ascites is the abnormal accumulation of fluid in the fetal abdominal cavity. It can result from immune or non-immune hydrops, congenital infections, urinary tract obstructions, chromosomal abnormalities, or heart failure. The proposed term will be modified for MedDRA capitalization rules and added as LLT Fetal ascites to the British English spelled PT Foetal ascites. In a related change, PT Foetal ascites will be added to primary HLT Foetal complications NEC and to secondary HLT Peritoneal and retroperitoneal disorders.

Detail Report

CR Number: 2026044005		Implementation Date: 13-Feb-26	Related CR: 2026041003	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Foetal ascites</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Foetal complications NEC</i>		<i>Foetal ascites</i>	10093513
SOC primary	<i>Pregnancy, puerperium and perinatal conditions</i>		HLT primary	
			<i>Foetal complications NEC</i>	10016850
			SOC primary	
			<i>Pregnancy, puerperium and perinatal conditions</i>	10036585
			HLT secondary	
			<i>Peritoneal and retroperitoneal disorders</i>	10034653
			SOC secondary	
			<i>Gastrointestinal disorders</i>	10017947

MSSO
Comment:

CR Number: 2026041004		Implementation Date: 13-Feb-26	Related CR: 2026041004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pelvic girdle muscle weakness</i>	Approved as Requested	Proposed LLT	
			<i>Pelvic girdle muscle weakness</i>	10093515 Current
			To PT	
			<i>Muscular weakness</i>	10028372

MSSO
Comment: The proposal to add a new LLT Pelvic girdle muscle weakness is approved as requested. Pelvic girdle muscle weakness refers to reduced strength in the muscles surrounding the hip and pelvis, manifesting clinically as difficulty rising from a seated position, climbing stairs, and a characteristic waddling gait. Its medical significance lies in its role as a key diagnostic indicator for progressive neuromuscular disorders such as limb-girdle muscular dystrophy and spinal muscular atrophy. Pelvic girdle muscle weakness will be added as LLT to PT Muscular weakness.

Detail Report

CR Number: 2026041005	Implementation Date: 13-Feb-26	Related CR: 2026041005	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Airplane ear	Rejected	

MSSO Comment: The proposal to add a new LLT Airplane ear is not approved. Airplane ear is a colloquial expression for painful misbalance of air pressure in the middle ear and the air pressure outside the ear. The concept is represented in MedDRA in several LLTs linked to PT Barotitis media.

CR Number: 2026041006	Implementation Date: 13-Feb-26	Related CR: 2026041006	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Palpebral Fissure	Rejected	

MSSO Comment: The proposal to add a new LLT Palpebral Fissure is not approved. The palpebral fissure is a normal anatomical structure, essential for eye protection, tear distribution, and blinking. Only significant deviations in its size or shape indicate underlying pathology.

CR Number: 2026042000	Implementation Date: 13-Feb-26	Related CR: 2026042000	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Bacterial tolerance	Approved Not as Requested	Proposed LLT
HLT primary	Therapeutic and nontherapeutic responses		Bacterial tolerance
SOC primary	General disorders and administration site conditions		To PT
			Pathogen resistance
			10093519 Current
			10034133

MSSO Comment: The proposal to add a new PT Bacterial tolerance to primary HLT Therapeutic and nontherapeutic responses in SOC General disorders and administration site conditions is approved but not as requested. Tolerance is a transient survival capacity under bactericidal antibiotics that does not involve active growth. It often precedes resistance because it is more likely to arise through mutation. Clinically, it is relevant because it can contribute to treatment failure and act as a prelude to resistance, yet it often goes unnoticed in the laboratory. Bacterial tolerance will be added as an LLT to PT Pathogen resistance. In a related change, PT Pathogen resistance will be added to secondary HLT Therapeutic and nontherapeutic responses.

Detail Report

CR Number: 2026044006		Implementation Date: 13-Feb-26	Related CR: 2026042000	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Pathogen resistance</i>	Approved as Requested	Proposed PT	
To HLT	<i>Therapeutic and nontherapeutic responses</i>		<i>Pathogen resistance</i>	10034133
			To HLT	
			<i>Therapeutic and nontherapeutic responses</i>	10043409

MSSO
Comment:

CR Number: 2026043000		Implementation Date: 17-Feb-26	Related CR: 2026043000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Medium fluorescent reticulocyte count increased</i>	Rejected		

MSSO
Comment:

The proposal to add a new LLT Medium fluorescent reticulocyte count increased is not approved. It is clinically possible to observe a higher proportion of immature reticulocytes by fluorescence together with a low total reticulocyte count, particularly in contexts of erythropoietic stress, partial bone marrow suppression, or dynamic phases of recovery. This clinical situation would lead to the selection of two opposite concepts at the PT level, since coding with the proposed "Medium fluorescent reticulocyte count increased" would map to the PT Reticulocyte count increased, whereas the total reticulocyte count in one of the clinical situations described above would map to the opposite PT Reticulocyte count decreased. It should also be taken into account that terms related to the measurement of fluorescence levels in reticulocytes represent a highly specific concept that we do not consider appropriate to represent at the PT level. Please reconsider carefully our rejection statement in change request 2025364002 as well as the addition of a new PT Immature reticulocyte fraction increased in change request 2025364004.

CR Number: 2026043001		Implementation Date: 17-Feb-26	Related CR: 2026043001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Low fluorescent reticulocyte count decreased</i>	Rejected		

MSSO
Comment:

The proposal to add a new term Low fluorescent reticulocyte count decreased is not approved. In situations of strong erythropoietic stimulation, such as active hemolysis, acute blood loss during marrow response, early recovery after treatment of iron or vitamin deficiencies, or following exogenous EPO administration, the total reticulocyte count increases as the bone marrow accelerates production, but there is a shift toward more immature, highly fluorescent reticulocytes. As a result, the fraction of low fluorescent (more mature) reticulocytes may decrease. In a situation like this, the proposed term "Low fluorescent reticulocyte count decreased", which would be added under PT Reticulocyte count decreased, would coexist in the same case with PT Reticulocyte count increase. It should also be taken into account that terms related to the measurement of fluorescence levels in reticulocytes represent a highly specific concept that we do not consider appropriate to represent at the PT level. Please reconsider carefully our rejection statement in change request 2025364003 as well as the addition of a new PT Immature reticulocyte fraction increased in change request 2025364004.

Detail Report

CR Number: 2026043002	Implementation Date: 17-Feb-26	Related CR: 2026043002	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Low fluorescence ratio reticulocyte decreased	Rejected	

MSSO Comment: The proposal to add a new LLT Low fluorescence ratio reticulocyte decreased is not approved. A decreased low fluorescence ratio reticulocyte count reflects an increase in the immature reticulocyte fraction (increase in high and medium fluorescence reticulocyte fractions) and would add complexity without offering independent clinical significance. Please consider new PT Immature reticulocyte fraction increased in change request 2025364004.

CR Number: 2026043003	Implementation Date: 17-Feb-26	Related CR: 2026043003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Uterosacral ligament suspension	Approved as Requested	Proposed LLT Uterosacral ligament suspension To PT Vaginal prolapse repair
			10093524 Current 10070519

MSSO Comment: The proposal to add a new term Uterosacral ligament suspension is approved as requested. Uterosacral ligament suspension is a surgical procedure designed to restore support of the vaginal vault or uterus in women with pelvic organ prolapse. It's classified as a "native tissue repair" because it uses the patient's own anatomical structures rather than synthetic mesh. Uterosacral ligament suspension will be added as LLT to PT Vaginal prolapse repair.

CR Number: 2026043004	Implementation Date: 17-Feb-26	Related CR: 2026043004	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Gastrointestinal mycosis	Rejected	

MSSO Comment: The proposal to add a new LLT Gastrointestinal mycosis is not approved. The concept exists in LLT/PT Gastrointestinal fungal infection as well as LLT/PT Fungal oesophagitis in the case of esophageal mycosis. Gastrointestinal mycosis uses the term mycosis, which is simply the medical word of Greek origin for fungal infection.

Detail Report

CR Number: 2026043005	Implementation Date: 17-Feb-26	Related CR: 2026043005	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Allograft vasculopathy</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Allograft vasculopathy is not approved. The term "allograft vasculopathy" is used predominantly in cardiac transplantation, a context for which the PT Cardiac allograft vasculopathy already exists in MedDRA. In case there is a need for another organ/system, please clarify and re-submit including the medical justification.

CR Number: 2026043006	Implementation Date: 17-Feb-26	Related CR: 2026043006	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Mechanical rash</i>	Rejected	

MSSO Comment: The proposal to add a new term Mechanical rash is not approved. The proposed term is ambiguous and not universally understood and may refer to multiple skin reactions such as irritant contact dermatitis, urticaria (hives), small broken blood vessels (petechiae) or other.

CR Number: 2026043007	Implementation Date: 17-Feb-26	Related CR: 2026043007	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Thoracic stomach syndrome</i>	Approved Not as Requested	Proposed LLT
To PT	<i>Postoperative thoracic procedure complication</i>		<i>Thoracic stomach syndrome</i> 10093526 Current
			To PT
			<i>Tension gastrothorax</i> 10093525

MSSO Comment: The proposal to add a new LLT Thoracic stomach syndrome to PT Postoperative thoracic procedure complication is approved but not as requested. Thoracic stomach syndrome is a postoperative complication that occurs after esophagectomy with gastric pull-up reconstruction, in which the transposed stomach located within the thoracic cavity becomes acutely distended. The resulting gastric dilatation can compress the lungs, mediastinum, and great vessels, leading to respiratory distress and hemodynamic compromise. Clinically, it may mimic tension pneumothorax, but management requires prompt gastric decompression rather than pleural intervention. Thoracic stomach syndrome will be added as LLT to new PT Tension gastrothorax. In a related change, new PT Tension gastrothorax will be added to primary HLT Mediastinal disorders, to secondary HLT Diaphragmatic hernias and to secondary HLT Respiratory tract and thoracic cavity procedural complications.

Detail Report

CR Number: 2026048002

Implementation Date: 17-Feb-26

Related CR: 2026043007

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Tension gastrothorax</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Mediastinal disorders</i>		<i>Tension gastrothorax</i>	10093525
SOC primary	<i>Respiratory, thoracic and mediastinal disorders</i>		HLT primary	
			<i>Mediastinal disorders</i>	10027072
			SOC primary	
			<i>Respiratory, thoracic and mediastinal disorders</i>	10038738
			HLT secondary	
			<i>Diaphragmatic hernias</i>	10012714
			SOC secondary	
			<i>Gastrointestinal disorders</i>	10017947
			HLT secondary	
			<i>Respiratory tract and thoracic cavity procedural complications</i>	10052823
			SOC secondary	
			<i>Injury, poisoning and procedural complications</i>	10022117

MSSO
Comment:

CR Number: 2026043008

Implementation Date: 17-Feb-26

Related CR: 2026043008

Change Requested

Reassign Primary SOC

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Paroxysmal nocturnal haemoglobinuria</i>	Approved as Requested	Proposed PT	
Old Primary SOC	<i>Renal and urinary disorders</i>		<i>Paroxysmal nocturnal haemoglobinuria</i>	10034042
New Primary SOC	<i>Blood and lymphatic system disorders</i>		Old Primary SOC	
			<i>Renal and urinary disorders</i>	10038359
			New Primary SOC	
			<i>Blood and lymphatic system disorders</i>	10005329

MSSO
Comment: The proposal to reassign the primary SOC of PT Paroxysmal nocturnal haemoglobinuria from current SOC Renal and urinary disorders to SOC Blood and lymphatic system disorders is approved as requested as the disease is multisystemic with clear hematologic etiology. Paroxysmal nocturnal hemoglobinuria (PNH) is a rare, acquired disorder in which hematopoietic stem cells and their cellular progeny have reduced or absent glycosylphosphatidylinositol (GPI)-anchored proteins on the cell surface. Loss of the GPI-linked complement inhibitors, on red blood cells leads to chronic and/or paroxysmal intravascular hemolysis and a propensity for thrombosis, organ dysfunction, and hypocellular or dysplastic bone marrow.

Detail Report

CR Number: 2026044010	Implementation Date: 17-Feb-26	Related CR: 2026044010	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Systemic aspergillosis	Rejected	

MSSO Comment: The proposal to add a new term Systemic aspergillosis is not approved. In infectious disease literature, "systemic" and "disseminated" are used interchangeably. If the intention is to represent the severity descriptor "involvement of ≥ 2 discontinuous organs", that criterion is explicitly tied to "disseminated aspergillosis," which is the more established term. The ≥ 2 discontinuous organs criterion is primarily meaningful as a marker of hematogenous dissemination and adverse prognosis and as a formal discriminator between localized invasive disease and disseminated aspergillosis. However, descriptors of severity are generally not included in the terminology, except when pertinent to the specificity of the term (e.g., severe vs. mild mental retardation). Please, consider 1.7 Exclusion Criteria of the MedDRA Introductory Guide.

CR Number: 2026047000	Implementation Date: 26-Mar-26	Related CR: 2026047000	MedDRA Code #
Change Requested	Rename SOC	Final Disposition	Final Placement
Term to modify	Social circumstances	Rejected After Suspension	
Replacement term	Social circumstances and life cycle issues		

MSSO Comment: The proposal to rename the SOC Social circumstances to Social circumstances and life cycle issues is not approved after suspension. SOC names are not intended to reflect a comprehensive description of the concepts and categories of terms which are contained within. The Social circumstances (SOC) provides a grouping for those factors that may give insight into personal issues that could have an effect on the event being reported. Essentially, SOC Social circumstances contains information about the person, not the adverse event. It reflects situational (often non-clinical) concepts which may either result from or have an impact on a clinical disorder or condition. "Life cycle issues" is just one of these situational concepts, but it is impractical to "single it out" for inclusion in the name of the SOC itself. Please reference section 6.25 in the MedDRA Introductory Guide for a comprehensive description of types of concepts included in the SOC Social circumstances.

CR Number: 2026048000	Implementation Date: 20-Feb-26	Related CR: 2026048000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Ener jet	Rejected	

MSSO Comment: The proposal to add a new LLT Ener jet is not approved. The EnerJet procedure is a non-surgical, needle-free, skin-remodeling treatment using high-pressure jet technology to deliver hyaluronic acid or other compounds into the deep dermis. It treats acne scars, stretch marks, wrinkles, and provides a "kinetic lift" by stimulating collagen production and creating controlled, deep-tissue stimulation. Proposed concept is a trademark name and based on MedDRA conventions may not be added to MedDRA.

Detail Report

CR Number: 2026048001		Implementation Date: 20-Feb-26	Related CR: 2026048001		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Ultrasound pelvis and abdomen</i>	Rejected			
To PT	<i>Ultrasound abdomen</i>				

MSSO Comment: The proposal to add a new LLT Ultrasound pelvis and abdomen to PT Ultrasound abdomen is not approved. PT Ultrasound abdomen and PT Ultrasound pelvis already exist in MedDRA. Keeping terms separate preserves conceptual clarity and hierarchical accurateness and prevents unnecessary combinations based only on anatomical contiguity. Moreover, the existence of combined terms of contiguous anatomical regions in other imaging modalities, such as computed tomography, does not imply adopting the same approach in ultrasound. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

CR Number: 2026049000		Implementation Date: 20-Feb-26	Related CR: 2026049000		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Nonreassuring foetal status</i>	Rejected			
To PT	<i>Foetal distress syndrome</i>				

MSSO Comment: The proposal to add a new LLT Nonreassuring foetal status to PT Foetal distress syndrome is not approved. The proposed term is a descriptive, non-specific expression that - despite a relative international acceptance - lacks standardized diagnostic criteria and is inherently subjective in interpretation. International clinical practice has progressively moved away from the reassuring/non-reassuring terminology in favor of more objective and structured classifications, such as categories I, II and III of fetal heart rate measured by cardiotocography. Please consider existing MedDRA terms depending upon your specific case scenario.

CR Number: 2026049001		Implementation Date: 20-Feb-26	Related CR: 2026049001		MedDRA Code #
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement		
Proposed PT	<i>Female genital mutilation</i>	Rejected			
Old Primary SOC	<i>Social circumstances</i>				
New Primary SOC	<i>Surgical and medical procedures</i>				

MSSO Comment: The proposal to reassign the primary SOC of PT Female genital mutilation from current SOC Social circumstances to SOC Surgical and medical procedures is not approved. Female genital mutilation (FGM) is, by definition non-therapeutic: it is a non-medical intervention with no proven health benefit and seen more as a social and cultural practice. Generally, most international human rights organizations condemn this practice.

Detail Report

CR Number: 2026049002		Implementation Date: 20-Feb-26	Related CR: 2026049002	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Abdominal transposition</i>	Approved as Requested	PT to merge	10000098 Current
To PT	<i>Heterotaxia</i>		<i>Abdominal transposition</i>	
			To PT	10067265
		<i>Heterotaxia</i>		
MSSO Comment:	The proposal to demote the PT Abdominal transposition under PT Heterotaxia is approved as requested. Abdominal transposition and LLT Visceral transposition under PT Heterotaxia are seen as synonyms.			

CR Number: 2026049003		Implementation Date: 20-Feb-26	Related CR: 2026049003	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Subaortic stenosis, congenital</i>	Approved Not as Requested	Proposed LLT	10042299 Current
From PT	<i>Hypertrophic cardiomyopathy</i>		<i>Subaortic stenosis, congenital</i>	
HLT primary	<i>Cardiac valve disorders congenital</i>		From PT	10020871
			<i>Hypertrophic cardiomyopathy</i>	
		To PT	10042431	
		<i>Subvalvular aortic stenosis</i>		
MSSO Comment:	The proposal to promote the LLT Subaortic stenosis, congenital from PT Hypertrophic cardiomyopathy to primary HLT Cardiac valve disorders congenital in SOC Congenital, familial and genetic disorders and secondary HLT Aortic valvular disorders in SOC Cardiac disorders is approved but not as requested. Subvalvular aortic stenosis (subaortic stenosis) is a rare congenital heart defect characterized by a fibrous membrane or muscular ridge below the aortic valve that obstructs blood flow from the left ventricle, causing it to work harder and potentially fail. LLT Subaortic stenosis, congenital will be moved from PT Hypertrophic cardiomyopathy to PT Subvalvular aortic stenosis. In a related change, PT Subvalvular aortic stenosis will be added to HLT Cardiac valve disorders congenital. Then the primary SOC for PT Subvalvular aortic stenosis will be changed from SOC Cardiac disorders to SOC Congenital, familial and genetic disorders, in order to reflect the predominantly congenital nature of this disorder.			

Detail Report

CR Number: 2026051009		Implementation Date: 20-Feb-26	Related CR: 2026049003	
<u>Change Requested</u>	Reassign Primary SOC	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Subvalvular aortic stenosis</i>	Approved as Requested	Proposed PT	
Old Primary SOC	<i>Cardiac disorders</i>		<i>Subvalvular aortic stenosis</i>	10042431
New Primary SOC	<i>Congenital, familial and genetic disorders</i>		Old Primary SOC	
			<i>Cardiac disorders</i>	10007541
			New Primary SOC	
			<i>Congenital, familial and genetic disorders</i>	10010331

MSSO
Comment:

CR Number: 2026051008		Implementation Date: 20-Feb-26	Related CR: 2026049003	
<u>Change Requested</u>	Link a PT to a HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Subvalvular aortic stenosis</i>	Approved as Requested	Proposed PT	
To HLT	<i>Cardiac valve disorders congenital</i>		<i>Subvalvular aortic stenosis</i>	10042431
			To HLT	
			<i>Cardiac valve disorders congenital</i>	10007614

MSSO
Comment:

Detail Report

CR Number: 2026049004		Implementation Date: 20-Feb-26	Related CR: 2026049004	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Congenital intrahepatic shunt</i>	Approved Not as Requested	Proposed LLT	
From PT	<i>Malformation venous</i>		<i>Congenital intrahepatic shunt</i>	10062312 Non-Current
To PT	<i>Hepatic arteriovenous malformation</i>		From PT	
			<i>Malformation venous</i>	10025532
			To PT	
			<i>Congenital intrahepatic portosystemic shunt</i>	10093530

MSSO Comment: The proposal to move the LLT Congenital intrahepatic shunt from PT Malformation venous to PT Hepatic arteriovenous malformation is approved but not as requested. Congenital intrahepatic shunt usually refers to a portosystemic shunt in which portal blood bypasses the hepatic sinusoids with reduced detoxification and metabolic processing. LLT Congenital intrahepatic shunt will be moved from PT Malformation venous to new PT Congenital intrahepatic portosystemic shunt. In a related change, new PT Congenital intrahepatic portosystemic shunt will be added to primary HLT Venous disorders congenital, to secondary HLT Hepatic vascular disorders and to secondary HLT Vascular malformations and acquired anomalies. Furthermore, LLT Congenital intrahepatic shunt will be made non-current, as it is an imprecise term.

CR Number: 2026051010		Implementation Date: 20-Feb-26	Related CR: 2026049004	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Congenital intrahepatic portosystemic shunt</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Venous disorders congenital</i>		<i>Congenital intrahepatic portosystemic shunt</i>	10093530
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary	
			<i>Venous disorders congenital</i>	10047224
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Hepatic vascular disorders</i>	10019712
			SOC secondary	
			<i>Hepatobiliary disorders</i>	10019805
			HLT secondary	
			<i>Vascular malformations and acquired anomalies</i>	10047091
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO Comment:

Detail Report

CR Number: 2026051011		Implementation Date: 20-Feb-26	Related CR: 2026049004		
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #	
LLT to change	<i>Congenital intrahepatic shunt</i>	Approved as Requested	LLT to change		
Status	<i>Non-current</i>		<i>Congenital intrahepatic shunt</i>	10062312	Non-Current
			Status		
			<i>Non-current</i>		

MSSO
Comment:

CR Number: 2026049005		Implementation Date: 20-Feb-26	Related CR: 2026049005		
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #	
LLT to change	<i>Coloboma of optic disc</i>	Approved as Requested	LLT to change		
Status	<i>current</i>		<i>Coloboma of optic disc</i>	10009936	
			Status		
			<i>current</i>		

MSSO
Comment: The proposal to change status of LLT Coloboma of optic disc to current is approved as requested. Optic disc coloboma is a congenital structural anomaly of the optic nerve resulting from incomplete closure of the embryonic fissure, with variable functional impact depending on its extent and associated macular involvement. In a related change, LLT Coloboma of optic disc will be promoted from PT Coloboma to PT level and linked to primary HLT Optic nerve and optic disc disorders congenital, to secondary HLT Optic disc abnormalities NEC and to secondary HLT Congenital and hereditary neurological disorders NEC.

CR Number: 2026051013		Implementation Date: 20-Feb-26	Related CR: 2026049005		
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Coloboma of optic disc</i>	Approved as Requested	Proposed PT		
To HLT	<i>Congenital and hereditary neurological disorders NEC</i>		<i>Coloboma of optic disc</i>	10009936	
			To HLT		
			<i>Congenital and hereditary neurological disorders NEC</i>	10010333	

MSSO
Comment:

Detail Report

CR Number: 2026051012		Implementation Date: 20-Feb-26	Related CR: 2026049005	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Coloboma of optic disc</i>	Approved as Requested	Proposed LLT	
From PT	<i>Coloboma</i>		<i>Coloboma of optic disc</i>	10009936
HLT primary	<i>Optic nerve and optic disc disorders congenital</i>		From PT	
			<i>Coloboma</i>	10009934
			HLT primary	
			<i>Optic nerve and optic disc disorders congenital</i>	10030926
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Optic disc abnormalities NEC</i>	10030913
			SOC secondary	
			<i>Eye disorders</i>	10015919

MSSO
Comment:

CR Number: 2026049006		Implementation Date: 24-Feb-26	Related CR: 2026049006	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	<i>HIV spondyloarthritis</i>	Rejected		

MSSO
Comment: The proposal to add a new term HIV spondyloarthritis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities.

Detail Report

CR Number: 2026049007		Implementation Date: 24-Feb-26	Related CR: 2026049007		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>HIV ankylosing spondylitis</i>	Rejected			

MSSO Comment: The proposal to add a new term HIV ankylosing spondylitis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities.

CR Number: 2026049008		Implementation Date: 24-Feb-26	Related CR: 2026049008		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Neonatal bacterial sepsis</i>	Approved as Requested	Proposed LLT <i>Neonatal bacterial sepsis</i>	10093531	Current
			To PT		
			<i>Sepsis neonatal</i>	10040049	

MSSO Comment: The proposal to add a new term Neonatal bacterial sepsis is approved as requested. Neonatal sepsis is an invasive infection, usually bacterial, occurring during the neonatal period. Diagnosis is based on physical examination and bacterial culture. Neonatal bacterial sepsis will be added as sub-concept LLT to PT Sepsis neonatal.

CR Number: 2026049009		Implementation Date: 20-Feb-26	Related CR: 2026049009		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Rhesus antibodies (anti E)</i>	Approved as Requested	Proposed LLT <i>Rhesus antibodies (anti E)</i>	10093527	
To PT	<i>Rhesus antibodies</i>		To PT		
			<i>Rhesus antibodies</i>	10039033	

MSSO Comment: The proposal to add a new LLT Rhesus antibodies (anti E) to PT Rhesus antibodies is approved as requested. The Rhesus E antibody (anti-E) is an IgG alloantibody directed against the E antigen of the Rh blood group system. If a person lacks E antigen (E-) and is exposed to E-positive red blood cells (via transfusion or pregnancy), they may form anti-E antibodies, which can cause hemolytic transfusion reactions or hemolytic disease of the fetus and newborn. The "E" antigen (uppercase E) and "e" antigen (lowercase e) are alternative forms encoded at the same locus (RHCE gene). Anti-e is rarer but can also cause hemolytic transfusion reactions or hemolytic disease of the fetus and newborn. In a related change, the positive form LLT Rhesus antibodies (anti E) positive will be added to PT Rhesus antibodies positive. Furthermore, the negative result will be added as LLT Rhesus antibodies (anti E) negative to PT Rhesus antibodies negative.

Detail Report

CR Number: 2026051007		Implementation Date: 20-Feb-26	Related CR: 2026049009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Rhesus antibodies (anti E) negative</i>	Approved as Requested	Proposed LLT	
To PT	<i>Rhesus antibodies negative</i>		<i>Rhesus antibodies (anti E) negative</i>	10093528
			To PT	
			<i>Rhesus antibodies negative</i>	10039035

MSSO
Comment:

CR Number: 2026051006		Implementation Date: 20-Feb-26	Related CR: 2026049009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Rhesus antibodies (anti E) positive</i>	Approved as Requested	Proposed LLT	
To PT	<i>Rhesus antibodies positive</i>		<i>Rhesus antibodies (anti E) positive</i>	10093529
			To PT	
			<i>Rhesus antibodies positive</i>	10039036

MSSO
Comment:

CR Number: 2026049010		Implementation Date: 24-Feb-26	Related CR: 2026049010	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Port occlusion</i>	Approved as Requested	Proposed LLT	
From PT	<i>Device occlusion</i>		<i>Port occlusion</i>	10090303 Current
To PT	<i>Vascular device occlusion</i>		From PT	
			<i>Device occlusion</i>	10064685
			To PT	
			<i>Vascular device occlusion</i>	10080803

MSSO
Comment: The proposal to move the LLT Port occlusion from PT Device occlusion to PT Vascular device occlusion is approved as requested for better alignment with LLT Central venous catheter occlusion.

Detail Report

CR Number: 2026050000	Implementation Date: 27-Feb-26	Related CR: 2026050000	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	<i>Dose omission outside of patient control</i>	Suspended	

MSSO Comment: The proposal to add a new PT Dose omission outside of patient control is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.

CR Number: 2026051000	Implementation Date: 26-Feb-26	Related CR: 2026051000	MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement
PT to merge	<i>Colon operation</i>	Approved as Requested	PT to merge
To PT	<i>Large intestine operation</i>		<i>Colon operation</i> 10061779 Current
			To PT
			<i>Large intestine operation</i> 10062032

MSSO Comment: The proposal to demote the PT Colon operation under PT Large intestine operation is approved as requested. MedDRA shows frequent bidirectional cross-mapping between LLTs referring to the colon and those referring to the large intestine, indicating conceptual overlap. Moreover, the term "large intestine" is anatomically broader, as it encompasses also the cecum and other proximal segments. Given that the colon is a constituent part of the large intestine, the proposed demotion will increase terminological coherence.

CR Number: 2026051001	Implementation Date: 27-Feb-26	Related CR: 2026051001	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	<i>Parathyroid nodule</i>	Approved Not as Requested	Proposed LLT
			<i>Parathyroid nodule</i> 10093532 Current
			To PT
			<i>Parathyroid disorder</i> 10033942

MSSO Comment: The proposal to add a new PT Parathyroid nodule is approved but not as requested. Although requests for "nodules" in various tissues or organs are generally rejected as non-specific, parathyroid nodule warrants an exceptional consideration. In the parathyroid context, a nodular lesion is far more likely to correspond to a clinically meaningful, most commonly a parathyroid adenoma or a hyperplastic nodular enlargement, both of which have direct diagnostic, biochemical, and therapeutic implications. Parathyroid nodule will be added as an LLT to PT Parathyroid disorder.

Detail Report

CR Number: 2026051003		Implementation Date: 27-Feb-26	Related CR: 2026051003		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Cancer therapy-related cardiac dysfunction	Rejected			
To PT	Cardiac dysfunction				

MSSO Comment: The proposal to add a new LLT Cancer therapy-related cardiac dysfunction to PT Cardiac dysfunction is not approved. The proposed concept is a "due to" concept, as well as an umbrella term, grouping a variety of different medical conditions with different biological mechanisms, treatment implications and prognosis.

CR Number: 2026051004		Implementation Date: 26-Feb-26	Related CR: 2026051004		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Erosive pustulosis	Approved as Requested	Proposed LLT		
			Erosive pustulosis	10093533	Current
			To PT		
			Erosive pustular dermatosis	10088153	

MSSO Comment: The proposal to add a new term Erosive pustulosis is approved as requested. Erosive pustular dermatosis presents with localised areas of pustules, lakes of pus, or crusts, which overlie eroded plaques or nodules and mainly affects the sun damaged, bald scalp of older individuals. Erosive pustular dermatosis is also known as erosive pustulosis and erosive pustular dermatosis of the scalp. Erosive pustulosis will be added as synonym LLT to PT Erosive pustular dermatosis.

CR Number: 2026051005		Implementation Date: 27-Feb-26	Related CR: 2026051005		MedDRA Code #
Change Requested	Change Status of LLT	Final Disposition	Final Placement		
LLT to change	Cervical stricture	Approved as Requested	LLT to change		
Status	non-current		Cervical stricture	10008318	Non-Current
			Status		
			non-current		

MSSO Comment: The proposal to change status of LLT Cervical stricture to non-current is approved as requested. LLT Cervical stricture will be non-current as it is an ambiguous term. This term which was added in MedDRA version 2.1 does not follow naming conventions based on the MedDRA introductory guide in which "cervical" without further qualifier refers to the anatomical location of neck.

Detail Report

CR Number: 2026051014	Implementation Date: 27-Feb-26	Related CR: 2026051014	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Periorbital allergy	Rejected	

MSSO Comment: The proposal to add a new PT Periorbital allergy is not approved. To avoid overpopulation of MedDRA, not every location of an allergic skin manifestation can be added to MedDRA. Please consider selection of LLTs under PT Dermatitis allergic or capture the event/reaction that occurred due to the allergy. Allergy typically refers to a mechanism (pathogenesis) and is usually informed with reference to the causative agent and/or the specific clinical manifestation, not by anatomical location alone.

CR Number: 2026052000	Implementation Date: 27-Feb-26	Related CR: 2026052000	MedDRA Code #
Change Requested	Promote an LLT	Final Disposition	Final Placement
Proposed LLT	Anterior uveitis	Approved Not as Requested	Proposed PT
From PT	Iridocyclitis		Iridocyclitis
			10022941 Current
			With LLT
			Anterior uveitis
			10002709

MSSO Comment: The proposal to promote the LLT Anterior uveitis from PT Iridocyclitis is approved but not as requested. In modern practice, anterior uveitis has replaced iridocyclitis as the preferred term in the scientific literature. Instead of promotion, the PT Iridocyclitis will be swapped with LLT Anterior uveitis to be represented at PT level.

Detail Report

CR Number: 2026052001		Implementation Date: 27-Feb-26	Related CR: 2026052001	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Intermediate uveitis</i>	Approved as Requested	Proposed LLT	
From PT	<i>Uveitis</i>		<i>Intermediate uveitis</i>	10022557
			From PT	
			<i>Uveitis</i>	10046851
			HLT primary	
			<i>Iris and uveal tract infections, irritations and inflammations</i>	10022953
			SOC primary	
			<i>Eye disorders</i>	10015919
			HLT secondary	
			<i>Immune and associated conditions NEC</i>	10027682
			SOC secondary	
			<i>Immune system disorders</i>	10021428

MSSO Comment: The proposal to promote the LLT Intermediate uveitis from PT Uveitis is approved as requested. Intermediate uveitis represents a distinct anatomical category defined by inflammation predominantly involving the vitreous, with or without peripheral retinal or pars plana involvement. It is not merely a descriptive variant of general uveitis but a well-established entity with specific clinical features, associations, and management implications. LLT Intermediate uveitis will be promoted from PT Uveitis to PT level and linked to primary HLT Iris and uveal tract infections, irritations and inflammations and to secondary HLT Immune and associated conditions NEC. In a related change, LLT Pars planitis and LLT Peripheral uveitis will both be moved from PT Uveitis to new PT Intermediate uveitis.

CR Number: 2026058005		Implementation Date: 27-Feb-26	Related CR: 2026052001	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Peripheral uveitis</i>	Approved as Requested	Proposed LLT	
From PT	<i>Uveitis</i>		<i>Peripheral uveitis</i>	10034631 Current
To PT	<i>Intermediate uveitis</i>		From PT	
			<i>Uveitis</i>	10046851
			To PT	
			<i>Intermediate uveitis</i>	10022557

MSSO Comment:

Detail Report

CR Number: 2026058004		Implementation Date: 27-Feb-26	Related CR: 2026052001	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pars planitis</i>	Approved as Requested	Proposed LLT	
From PT	<i>Uveitis</i>		<i>Pars planitis</i>	10034052 Current
To PT	<i>Intermediate uveitis</i>		From PT	
			<i>Uveitis</i>	10046851
			To PT	
			<i>Intermediate uveitis</i>	10022557

MSSO
Comment:

CR Number: 2026052002		Implementation Date: 27-Feb-26	Related CR: 2026052002	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Panuveitis</i>	Approved as Requested	Proposed LLT	
From PT	<i>Uveitis</i>		<i>Panuveitis</i>	10033687
			From PT	
			<i>Uveitis</i>	10046851
			HLT primary	
			<i>Iris and uveal tract infections, irritations and inflammations</i>	10022953
			SOC primary	
			<i>Eye disorders</i>	10015919
			HLT secondary	
			<i>Immune and associated conditions NEC</i>	10027682
			SOC secondary	
			<i>Immune system disorders</i>	10021428

MSSO
Comment: The proposal to promote the LLT Panuveitis from PT Uveitis is approved as requested. Panuveitis is a generalized inflammation of not only the whole of the uveal tract but also involves the retina and vitreous humor. Panuveitis is a major cause of blindness and visual morbidity. When the condition is unrecognized or inadequately treated, profound and irreversible vision loss can occur. LLT Panuveitis will be promoted from PT Uveitis to PT level and linked to primary HLT Iris and uveal tract infections, irritations and inflammations and to secondary HLT Immune and associated conditions NEC.

Detail Report

CR Number: 2026052003		Implementation Date: 27-Feb-26	Related CR: 2026052003	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Posterior uveitis</i>	Approved Not as Requested	Proposed PT	10008769 Current
From PT	<i>Chorioretinitis</i>		<i>Chorioretinitis</i>	
			With LLT	10036370
		<i>Posterior uveitis</i>		
MSSO Comment:	The proposal to promote the LLT Posterior uveitis from PT Chorioretinitis is approved but not as requested. Posterior uveitis is a broad anatomical category defined by primary inflammation of the retina and/or choroid. Chorioretinitis is a specific morphological subtype within the spectrum of posterior uveitis, characterized by combined choroidal and retinal involvement, often infectious in origin. Instead of promotion LLT Posterior uveitis will be swapped with PT Chorioretinitis to be represented at PT level. In a related change, new PT Posterior uveitis will also be linked to HLT Immune and associated conditions NEC.			

CR Number: 2026058006		Implementation Date: 27-Feb-26	Related CR: 2026052003	
<u>Change Requested</u>	Link a PT to a HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Posterior uveitis</i>	Approved as Requested	Proposed PT	10036370
To HLT	<i>Immune and associated conditions NEC</i>		<i>Posterior uveitis</i>	
			To HLT	10027682
			<i>Immune and associated conditions NEC</i>	

MSSO Comment:

CR Number: 2026052004		Implementation Date: 26-Feb-26	Related CR: 2026052004	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Conjunctival inflammation</i>	Rejected		
HLT primary	<i>Conjunctival infections, irritations and inflammations</i>			
SOC primary	<i>Eye disorders</i>			

MSSO Comment: The proposal to add a new PT Conjunctival inflammation to primary HLT Conjunctival infections, irritations and inflammations in SOC Eye disorders is not approved. Systematically adding separate PTs for "inflammation" alongside "-itis" terms would cause repetitive term proliferation without adding meaningful granularity, particularly when suitable non-infective terms are already available. Such expansion could ultimately complicate coding practices rather than improve them.

Detail Report

CR Number: 2026052005

Implementation Date: 06-Mar-26

Related CR: 2026052005

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Mixed squamous neuroendocrine carcinoma</i>	Approved Not as Requested After Suspension	Proposed PT	
HLT primary	<i>Endocrine neoplasms NEC</i>		<i>Mixed squamous neuroendocrine carcinoma</i>	10093552
SOC primary	<i>Endocrine disorders</i>		HLT primary	
			<i>Neoplasms malignant site unspecified NEC</i>	10029105
			SOC primary	
			<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary	
			<i>Endocrine neoplasms NEC</i>	10014714
			SOC secondary	
			<i>Endocrine disorders</i>	10014698

MSSO Comment: The proposal to add a new PT Mixed squamous neuroendocrine carcinoma to primary HLT Endocrine neoplasms NEC in SOC Endocrine disorders is approved but not as requested after suspension. Mixed squamous neuroendocrine carcinoma is composed of a squamous cell carcinoma component and a neuroendocrine carcinoma component. It is usually found in sites where squamous carcinoma is common (e.g., esophagus, lung, cervix). It is also a subtype within the Mixed neuroendocrine–non-neuroendocrine neoplasms (MiNENs). MiNENs are mixed epithelial neoplasms in which a neuroendocrine component is combined with a non-neuroendocrine one, each of which is morphologically and immunohistochemically recognizable. Mixed squamous neuroendocrine carcinoma will be added as PT to primary HLT Neoplasms malignant site unspecified NEC and secondary HLT Endocrine neoplasms NEC. In a related change, LLT Mixed neuroendocrine non-neuroendocrine neoplasm will be promoted from PT Neuroendocrine tumour to PT level and linked to HLT Neoplasms malignant site unspecified NEC and secondary to HLT Endocrine neoplasms NEC.

Detail Report

CR Number: 2026065001		Implementation Date: 06-Mar-26	Related CR: 2026052005	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	Mixed neuroendocrine non-neuroendocrine neoplasm	Approved as Requested	Proposed LLT	10092915
From PT	Neuroendocrine tumour		Mixed neuroendocrine non-neuroendocrine neoplasm	
HLT primary	Neoplasms malignant site unspecified NEC		From PT	
			Neuroendocrine tumour	10052399
			HLT primary	
			Neoplasms malignant site unspecified NEC	10029105
			SOC primary	
			Neoplasms benign, malignant and unspecified (incl cysts and polyps)	10029104
HLT secondary	Endocrine neoplasms NEC		HLT secondary	
			Endocrine neoplasms NEC	10014714
			SOC secondary	
Endocrine disorders	10014698			

MSSO
Comment:

CR Number: 2026054000		Implementation Date: 06-Mar-26	Related CR: 2026054000	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	Geographic atrophy	Rejected After Suspension		
From PT	Dry age-related macular degeneration			

MSSO
Comment: The proposal to promote the LLT Geographic atrophy from PT Dry age-related macular degeneration is not approved after suspension. To avoid the overpopulation at PT level, Geographic atrophy will remain subordinated at LLT level. For your analysis and coding needs, please consider using the LLT level.

Detail Report

CR Number: 2026054001		Implementation Date: 06-Mar-26	Related CR: 2026054001		MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement		
PT to merge	<i>Rash pruritic</i>	Rejected			
To PT	<i>Rash</i>				

MSSO Comment: The proposal to demote the PT Rash pruritic under PT Rash is not approved. The proposed term has always been at PT level since version 2.1 and based on legacy data considerations and impact, it will remain on PT level.

CR Number: 2026054002		Implementation Date: 26-Feb-26	Related CR: 2026054002		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Pulmonary micronodules</i>	Rejected			
To PT	<i>Pulmonary mass</i>				

MSSO Comment: The proposal to add a new LLT Pulmonary micronodules to PT Pulmonary mass is not approved. The MSSO generally refrains from adding "nodule" terms as these generally represent additional unspecific "mass" or "disorder" terms. Nodule terms can be added when a nodule represents a full diagnostic expression e.g., PT Milker's nodules. Furthermore, MedDRA terms are usually added in singular form.

CR Number: 2026054003		Implementation Date: 27-Feb-26	Related CR: 2026054003		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Fascicular ventricular tachycardia</i>	Approved as Requested	Proposed LLT		
			<i>Fascicular ventricular tachycardia</i>	10093534	Current
			To PT		
			<i>Ventricular tachycardia</i>	10047302	

MSSO Comment: The proposal to add a new term Fascicular ventricular tachycardia is approved as requested. Fascicular ventricular tachycardia is an important cardiac arrhythmia which is characterized by relatively narrow QRS complex and right bundle branch block pattern and is the second most common cause of idiopathic left ventricular tachycardia. Fascicular ventricular tachycardia will be added as LLT to PT Ventricular tachycardia.

Detail Report

CR Number: 2026055002		Implementation Date: 27-Feb-26	Related CR: 2026055002	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Congenital condition aggravated in adulthood</i>	Approved Not as Requested	Proposed LLT <i>Congenital condition aggravated</i> To PT <i>Condition aggravated</i>	10093535 Non-Current 10010264

MSSO Comment: The proposal to add a new LLT Congenital condition aggravated in adulthood is approved but not as requested. Aggravation of a congenital condition is a clinically meaningful worsening in the severity, frequency, extent, or functional impact of a disease that was present at birth, occurring after a period of relative stability. The proposed term will be modified and added as LLT Congenital condition aggravated to PT Condition aggravated.

CR Number: 2026055003		Implementation Date: 27-Feb-26	Related CR: 2026055003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cardiac congenital condition aggravated in adulthood</i>	Rejected		

MSSO Comment: The proposal to add a new term Cardiac congenital condition aggravated in adulthood is not approved. The proposed concept represents a combination of the congenital condition in question (for which appropriate MedDRA term should be identified) with the LLT Congenital condition aggravated added in change request 2026055002. Please, consider the Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document which defines these as "combination terms" and provides coding guidance.

CR Number: 2026055004		Implementation Date: 27-Feb-26	Related CR: 2026055004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Pulmonary congenital condition aggravated in adulthood</i>	Rejected		

MSSO Comment: The proposal to add a new term Pulmonary congenital condition aggravated in adulthood is not approved. The proposed concept represents a combination of the congenital condition in question (for which appropriate MedDRA term should be identified) with the LLT Congenital condition aggravated added in change request 2026055002. Please, consider the Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document which defines these as "combination terms" and provides coding guidance.

Detail Report

CR Number: 2026055011 Implementation Date: 27-Feb-26 Related CR: 2026055011

Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Glisson capsule resection	Approved Not as Requested	Proposed LLT	
To PT	glissonectomy		Glisson capsule resection	10093536 Current
			To PT	
			Glissonectomy	10092153

MSSO Comment: The proposal to add a new LLT Glisson capsule resection to PT glissonectomy is approved but not as requested. Glisson capsule resection will be added as a synonym to existing LLT/PT Glissonectomy. LLT Glisson capsule resection will be added to PT Glissonectomy with corrected capitalization.

CR Number: 2026056000 Implementation Date: 27-Feb-26 Related CR: 2026056000

Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Link (move) an LLT to another PT	Approved as Requested	Proposed LLT	
From PT	Pneumocystosis recurrent		Pneumocystosis recurrent	10066723 Current
To PT	Pneumocystis jirovecii infection		From PT	
	Pneumocystis jirovecii pneumonia		Pneumocystis jirovecii infection	10073756
			To PT	
			Pneumocystis jirovecii pneumonia	10073755

MSSO Comment: The proposal to move the LLT Pneumocystosis recurrent from PT Pneumocystis jirovecii infection to PT Pneumocystis jirovecii pneumonia is approved as requested for better alignment with LLT Pneumocystosis, although academically pneumocystosis refers to an infection, it clinically manifests most often as a pneumonia.

CR Number: 2026056001 Implementation Date: 03-Mar-26 Related CR: 2026056001

Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Add a New Term	Approved as Requested	Proposed LLT	
	Sinus opacification		Sinus opacification	10093537 Current
			To PT	
			Sinus disorder	10062244

MSSO Comment: The proposal to add a new term Sinus opacification is approved as requested. Sinus opacification is a nonspecific imaging finding, although it is most frequently associated with rhinosinusitis. It refers to the loss of normal air content within a paranasal sinus, resulting in partial or complete radiographic opacity on imaging studies such as CT or plain radiography. Sinus opacification will be added as an LLT to PT Sinus disorder.

Detail Report

CR Number: 2026057000		Implementation Date: 03-Mar-26	Related CR: 2026057000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Deviated pituitary stalk</i>	Approved as Requested	Proposed PT <i>Deviated pituitary stalk</i>	10093538
			HLT primary <i>Hypothalamic and pituitary disorders NEC</i>	10021109
			SOC primary <i>Endocrine disorders</i>	10014698

MSSO Comment: The proposal to add a new term Deviated pituitary stalk is approved as requested. Deviated pituitary stalk is a structural imaging finding, which can indicate a nearby mass. Deviated pituitary stalk will be added as PT to HLT Hypothalamic and pituitary disorders NEC.

CR Number: 2026057001		Implementation Date: 03-Mar-26	Related CR: 2026057001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Synovial hyperplasia</i>	Approved Not as Requested	Proposed PT <i>Synovial thickening</i>	10093539
			HLT primary <i>Synovial disorders</i>	10013360
			SOC primary <i>Musculoskeletal and connective tissue disorders</i>	10028395

MSSO Comment: The proposal to add a new term Synovial hyperplasia is approved but not as requested. Synovial hyperplasia is a microscopic finding with a true increase in the number of synovial lining cells (up to 8-10 layers). This can be visualized on imaging by surrogate parameters such as thickening of the synovia and suggestive patterns on different imaging modalities. Because the justification provided describes an imaging finding, the proposed term will be modified to a more appropriate imaging term and added as PT Synovial thickening to HLT Synovial disorders.

CR Number: 2026057003		Implementation Date: 03-Mar-26	Related CR: 2026057003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Nuclear medicine hepatobiliary imaging</i>	Approved Not as Requested	Proposed LLT <i>Hepatobiliary nuclear scan</i>	10093542 Current
			To PT <i>Hepatobiliary scan</i>	10061946

MSSO Comment: The proposal to add a new term Nuclear medicine hepatobiliary imaging is approved but not as requested. A hepatobiliary nuclear scan is a diagnostic imaging test in nuclear medicine that uses a radiotracer injected intravenously to evaluate the function of the liver, gallbladder, bile ducts, and small intestine. The proposed term will be modified for natural language word order and added as LLT Hepatobiliary nuclear scan to PT Hepatobiliary scan.

Detail Report

CR Number: 2026057004		Implementation Date: 03-Mar-26	Related CR: 2026057004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Pyrophosphate scan</i>	Approved Not as Requested	Proposed LLT <i>Myocardial pyrophosphate scan</i>	10093543 Current
			To PT <i>Scan myocardial perfusion</i>	10054082

MSSO Comment: The proposal to add a new term Pyrophosphate scan is approved but not as requested. A pyrophosphate scan—more formally called a technetium-99m pyrophosphate (Tc-99m PYP) scan—is a nuclear medicine imaging test primarily used to evaluate certain types of heart disease, especially cardiac amyloidosis. The proposed term will be modified for clarity and added as LLT Myocardial pyrophosphate scan to PT Scan myocardial perfusion.

CR Number: 2026057005		Implementation Date: 03-Mar-26	Related CR: 2026057005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Megadolichocolon</i>	Rejected		

MSSO Comment: The proposal to add a new term Megadolichocolon is not approved. Megadolichocolon is a descriptive anatomical term, referring to a colon that is both longer than normal and widened in diameter. In many countries, especially in contemporary English-language medical literature, “megadolichocolon” is uncommon and largely replaced by clearer diagnostic classifications.

CR Number: 2026057006		Implementation Date: 03-Mar-26	Related CR: 2026057006	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cheilognathopalatoschisis</i>	Approved as Requested	Proposed LLT <i>Cheilognathopalatoschisis</i>	10093545 Current
			To PT <i>Cleft lip and palate</i>	10009260

MSSO Comment: The proposal to add a new term Cheilognathopalatoschisis is approved as requested. Cheilognathopalatoschisis refers to a combined congenital anomaly consisting of a cleft lip, upper jaw, and palate. Both cleft lip and cleft palate arise from failed fusion of structures derived from the maxillary prominences and inherently includes a defect of maxillary development. Cheilognathopalatoschisis will be added as an LLT to PT Cleft lip and palate.

Detail Report

CR Number: 2026057007		Implementation Date: 03-Mar-26	Related CR: 2026057007		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Median neuropathy	Rejected			

MSSO Comment: The proposal to add a new term Median neuropathy is not approved. Median neuropathy is a disorder in which the median nerve is damaged or compressed. It can occur as a focal mononeuropathy, such as in carpal tunnel syndrome, or as part of a broader peripheral neuropathy affecting multiple nerves. MedDRA cannot accommodate every possible combination of a specific nerve and disorder. Please consider existing terms such as LLT/PT Median nerve injury or LLT/PT Median nerve compression or LLT/PT Mononeuropathy or a combination of these depending on your specific scenario applying medical judgement as described in the MedDRA term selection points to consider document section 3.7 describing considerations related to body site vs. event specificity.

CR Number: 2026057008		Implementation Date: 03-Mar-26	Related CR: 2026057008		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Cardiovascular computed tomography	Rejected			

MSSO Comment: The proposal to add a new term Cardiovascular computed tomography is not approved. The proposed concept is already represented in MedDRA. Please consider LLTs under PT Computerised tomogram heart.

CR Number: 2026057009		Implementation Date: 03-Mar-26	Related CR: 2026057009		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Magnetic resonance imaging orbit	Approved as Requested	Proposed PT Magnetic resonance imaging orbit	10093540	
			HLT primary Ophthalmic histopathology and imaging procedures	10030866	
			SOC primary Investigations	10022891	

MSSO Comment: The proposal to add a new term Magnetic resonance imaging orbit is approved as requested. The anatomical term orbit refers to the bony socket and all its contents (eye, muscles, nerves, vessels, fat, etc.) and is the standard region of interest in orbital imaging. Magnetic resonance imaging orbit will be added as PT to HLT Ophthalmic histopathology and imaging procedures.

Detail Report

CR Number: 2026057010		Implementation Date: 03-Mar-26	Related CR: 2026057010	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Colon dyskinesia</i>	Approved as Requested	Proposed LLT <i>Colon dyskinesia</i>	10093541 Current
			To PT <i>Gastrointestinal motility disorder</i>	10061173

MSSO Comment: The proposal to add a new term Colon dyskinesia is approved as requested. Colon dyskinesia is a descriptive term for a pathological pattern of colonic movement. It is primarily diagnosed through functional studies (colonic manometry, transit studies). Colon dyskinesia will be added as LLT to PT Gastrointestinal motility disorder.

CR Number: 2026057011		Implementation Date: 03-Mar-26	Related CR: 2026057011	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>RNU4-2-related neurodevelopmental disorder</i>	Approved as Requested	Proposed LLT <i>RNU4-2-related neurodevelopmental disorder</i>	10093544 Current
			To PT <i>ReNU syndrome</i>	10093464

MSSO Comment: The proposal to add a new term RNU4-2-related neurodevelopmental disorder is approved as requested. ReNU syndrome is characterized by hypotonia, global developmental delay, intellectual disability with poor or absent speech, delayed motor development, feeding difficulties, short stature, seizures, and dysmorphic features. Neuroradiological abnormalities, including ventriculomegaly, hypoplasia of the corpus callosum, and a decreased white matter volume, are observed in many individuals. Most individuals have the same highly recurrent single base insertion (n.64_65insT) in RNU4-2. RNU4-2-related neurodevelopmental disorder will be added as LLT to recently added PT ReNU syndrome (change request 2026007001).

CR Number: 2026058000		Implementation Date: 06-Mar-26	Related CR: 2026058000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Autism spectrum disorder level 1</i>	Approved as Requested	Proposed LLT <i>Autism spectrum disorder level 1</i>	10093547 Current
To PT	<i>Autism spectrum disorder</i>		To PT <i>Autism spectrum disorder</i>	10063844

MSSO Comment: The proposal to add a new LLT Autism spectrum disorder level 1 to PT Autism spectrum disorder is approved as requested. Level 1 is the mildest form of autism spectrum disorder. Generally, children at this level have more mild symptoms, but still have a hard time communicating appropriately or engaging with others. Autism spectrum disorder level 1 is also referred to as "Asperger's syndrome" or "high-functioning autism".

Detail Report

CR Number: 2026058001		Implementation Date: 06-Mar-26	Related CR: 2026058001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Autism spectrum disorder level 2</i>	Approved as Requested	Proposed LLT	
To PT	<i>Autism spectrum disorder</i>		<i>Autism spectrum disorder level 2</i>	10093548 Current
			To PT	
			<i>Autism spectrum disorder</i>	10063844
MSSO The proposal to add a new LLT Autism spectrum disorder level 2 to PT Autism spectrum disorder is approved as requested. Children with autism spectrum disorder level 2				
Comment: tend to have very narrow interests and engage in repetitive behaviors that can make it difficult for them to function or participate in certain situations.				

CR Number: 2026058002		Implementation Date: 06-Mar-26	Related CR: 2026058002	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Autism spectrum disorder level 3</i>	Approved as Requested	Proposed LLT	
To PT	<i>Autism spectrum disorder</i>		<i>Autism spectrum disorder level 3</i>	10093546 Current
			To PT	
			<i>Autism spectrum disorder</i>	10063844
MSSO The proposal to add a new LLT Autism spectrum disorder level 3 to PT Autism spectrum disorder is approved as requested. Children at autism spectrum disorder level 3				
Comment: have severe deficits in verbal and nonverbal social communication skills that cause severe impairments in functioning, very limited initiation of social interactions, and minimal response to social overtures from others.				

CR Number: 2026058003		Implementation Date: 06-Mar-26	Related CR: 2026058003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Eccrine syringofibroadenoma</i>	Approved as Requested	Proposed LLT	
			<i>Eccrine syringofibroadenoma</i>	10093549 Current
			To PT	
			<i>Sweat gland tumour</i>	10042658
MSSO The proposal to add a new LLT Eccrine syringofibroadenoma is approved as requested. Syringofibroadenoma is a proliferative lesion derived from the intraepidermal				
Comment: eccrine duct, characterized by a proliferation of cords and strands of epithelial cells with ductal differentiation. It is often regarded as a reactive eccrine syringofibroadenomatous hyperplasia. Eccrine syringofibroadenoma will be added as LLT to the PT Sweat gland tumour.				

Detail Report

CR Number: 2026058007		Implementation Date: 06-Mar-26	Related CR: 2026058007	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Neuroretinitis	Approved as Requested	Proposed LLT	
From PT	Optic neuritis		Neuroretinitis	10060701
			From PT	
			Optic neuritis	10030942
			HLT primary	
			Retinal, choroid and vitreous infections and inflammations	10038909
			SOC primary	
			Eye disorders	10015919
			HLT secondary	
			Optic nerve disorders NEC	10030933
			SOC secondary	
			Nervous system disorders	10029205

MSSO Comment: The proposal to promote the LLT Neuroretinitis from PT Optic neuritis is approved as requested. Neuroretinitis is distinct from optic neuritis because it involves not only inflammation of the anterior optic nerve but also characteristic retinal involvement. Its defining feature is the macular star pattern caused by lipid exudates in the retina, which is not seen in typical optic neuritis. Optic neuritis is limited to inflammation of the optic nerve and is often demyelinating in origin. Neuroretinitis will be promoted from PT Optic neuritis to PT level with a primary link to HLT Retinal, choroid and vitreous infections and inflammations and secondary link to HLT Optic nerve disorders NEC.

CR Number: 2026059000		Implementation Date: 06-Mar-26	Related CR: 2026059000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Poison ivy allergic reaction	Rejected		

MSSO Comment: The proposal to add a new term Poison ivy allergic reaction is not approved. Please consider LLT Poison ivy rash or LLT\PT Allergy to plants depending on your specific scenario.

Detail Report

CR Number: 2026060000		Implementation Date: 06-Mar-26	Related CR: 2026060000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Coronary hypersensitivity syndrome	Approved as Requested	Proposed LLT	
To PT	Kounis syndrome		Coronary hypersensitivity syndrome	10093550 Current
			To PT	
			Kounis syndrome	10069167
MSSO Comment: The proposal to add a new LLT Coronary hypersensitivity syndrome to PT Kounis syndrome is approved as requested. Coronary hypersensitivity syndrome is a synonym of Kounis syndrome because both terms describe the same clinical entity: an acute coronary syndrome precipitated by an allergic or hypersensitivity reaction mediated by mast cell activation and inflammatory mediators.				

CR Number: 2026060001		Implementation Date: 06-Mar-26	Related CR: 2026060001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Allergic coronary syndrome	Rejected		
To PT	Kounis syndrome			
MSSO Comment: The proposal to add a new LLT Allergic coronary syndrome to PT Kounis syndrome is not approved. Allergic coronary syndrome cannot be considered a specific synonym of type I Kounis syndrome. The term represents a broad, not fully standardized term.				

CR Number: 2026060002		Implementation Date: 06-Mar-26	Related CR: 2026060002	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Anaphylactic coronary syndrome	Rejected		
To PT	Kounis syndrome			
MSSO Comment: The proposal to add a new LLT Anaphylactic coronary syndrome to PT Kounis syndrome is not approved. Anaphylactic coronary syndrome appears occasionally in publications or case reports, but it is not established as a valid concept in literature.				

Detail Report

CR Number: 2026062000		Implementation Date: 06-Mar-26	Related CR: 2026062000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Family history of liver disease</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Disease risk factor</i>		<i>Family history of liver disease</i>	10093551 Current
			To PT	
			<i>Familial risk factor</i>	10064117
MSSO Comment:	The proposal to add a new LLT Family history of liver disease to PT Disease risk factor is approved but not as requested. A family history of liver disease significantly increases the risk for similar conditions, including genetic disorders like hemochromatosis, Wilson disease, or alpha-1 antitrypsin deficiency. Family history of liver disease will be added as LLT to PT Familial risk factor.			
CR Number: 2026062001		Implementation Date: 26-May-26	Related CR: 2026062001	
Change Requested	Add a New HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed HLT	<i>Product recommendation errors and issues</i>	Suspended		
To HLGT	<i>Medication errors and other product use errors and issues</i>			
MSSO Comment:	The proposal to add a new HLT Product recommendation errors and issues to HLGT Medication errors and other product use errors and issues is suspended. The MSSO would like to take additional time to study this proposed change. This is a 'Complex Change' to MedDRA since it affects one of the higher levels of the hierarchy. Complex Changes are only made in the X.0 release of MedDRA in March each year. As per MSSO policy for review and user consultation on all proposed Complex Changes, additional time is needed to consider this proposed change. You will be notified once a decision has been made on the disposition of your request.			
CR Number: 2026062002		Implementation Date: 13-Mar-26	Related CR: 2026062002	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Product miscommunication error</i>	Rejected After Suspension		
HLT primary	<i>Medication errors</i>			
MSSO Comment:	The proposal to add a new PT Product miscommunication error to primary HLT Medication errors is not approved after suspension. The proposed term is ambiguous and may represent a mix of cases of different scenarios. Additionally, the various scenarios described are root causes for medication errors. Consider capturing the concept with LLT/PT Circumstance or information capable of leading to medication errors depending on your specific scenarios.			

Detail Report

CR Number: 2026062003		Implementation Date: 10-Mar-26	Related CR: 2026062003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Renal papilloma</i>	Approved as Requested	Proposed PT <i>Renal papilloma</i>	10093554
			HLT primary <i>Renal neoplasms benign</i>	10038487
			SOC primary <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary <i>Renal neoplasms</i>	10038486
			SOC secondary <i>Renal and urinary disorders</i>	10038359

MSSO Comment: The proposal to add a new term Renal papilloma is approved as requested. Urothelial papilloma (renal or renal pelvic) is a true benign lesion characterized by delicate papillary architecture lined by urothelium of normal thickness without cytological atypia; it is very rare in the renal pelvis. Renal papilloma will be added as PT with a primary link to HLT Renal neoplasms benign and secondary link to HLT Renal neoplasms.

CR Number: 2026062004		Implementation Date: 10-Mar-26	Related CR: 2026062004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Spinal biopsy or vertebral biopsy</i>	Approved Not as Requested	Proposed LLT <i>Vertebral biopsy</i>	10093564 Current
			To PT <i>Biopsy bone</i>	10004735

MSSO Comment: The proposal to add a new term Spinal biopsy or vertebral biopsy is approved but not as requested. A vertebral biopsy is a diagnostic procedure in which a sample of bone tissue is obtained from a vertebra for the diagnose of different pathologies, such as infection, primary bone tumors, metastatic disease, or other infiltrative or inflammatory conditions affecting the vertebra. The proposed term will be modified to Vertebral biopsy and added as LLT to the PT Biopsy bone.

Detail Report

CR Number: 2026062005	Implementation Date: 10-Mar-26	Related CR: 2026062005	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Stool retention</i>	Rejected	

MSSO Comment: The proposal to add a new term Stool retention is not approved. The term stool retention is ambiguous and does not represent a clearly defined clinical entity. In clinical practice, it is often used as an imprecise or colloquial way of referring to constipation and could lead to inconsistent coding and variability in interpretation among users.

CR Number: 2026062006	Implementation Date: 10-Mar-26	Related CR: 2026062006	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Meniscus repair</i>	Approved as Requested	Proposed LLT <i>Meniscus repair</i> To PT <i>Meniscus operation</i>
			10093557 Current 10054948

MSSO Comment: The proposal to add a new term Meniscus repair is approved as requested. Meniscus repair refers to surgical procedures intended to preserve and restore the native meniscal tissue by repairing a meniscal tear, typically through suturing or fixation techniques, rather than removing the damaged portion of the meniscus. Meniscus repair will be added as LLT to PT Meniscus operation.

CR Number: 2026062007	Implementation Date: 10-Mar-26	Related CR: 2026062007	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Laser ablation</i>	Approved as Requested	Proposed LLT <i>Laser ablation</i> To PT <i>Laser surgery</i>
			10093556 Current 10090499

MSSO Comment: The proposal to add a new term Laser ablation is approved as requested. Laser ablation is a therapeutic technique that uses laser energy to destroy or vaporize tissue through rapid and controlled heating, producing localized elimination of tissue without conventional surgical resection. It is used to treat superficial or well-defined lesions and allows precise tissue destruction with minimal damage to the surrounding tissue. Laser ablation will be added as LLT to PT Laser surgery.

Detail Report

CR Number: 2026062008		Implementation Date: 10-Mar-26	Related CR: 2026062008	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Autotransplantation</i>	Approved as Requested	Proposed LLT <i>Autotransplantation</i>	10093561 Current
			To PT <i>Transplant</i>	10057677
MSSO Comment: The proposal to add a new term Autotransplantation is approved as requested. Autotransplantation is the transplantation of an organ, tissue, or cells from one site to another within the same individual. Because the donor and the recipient are the same person, there is no risk of immunologic rejection. Autotransplantation will be added as LLT to the PT Transplant as a more frequent synonym of existing LLT Autograft.				

CR Number: 2026062009		Implementation Date: 10-Mar-26	Related CR: 2026062009	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Splenic autotransplantation</i>	Approved as Requested	Proposed PT <i>Splenic autotransplantation</i>	10093553
			HLT primary <i>Lymphoid tissue therapeutic procedures</i>	10025308
			SOC primary <i>Surgical and medical procedures</i>	10042613
MSSO Comment: The proposal to add a new term Splenic autotransplantation is approved as requested. Splenic autotransplantation is the surgical implantation of fragments of a patient's own splenic tissue into another site in the body, usually the peritoneal cavity or the omentum, after splenectomy, with the aim of preserving or partially restoring splenic immune function. Splenic autotransplantation will be added as PT to HLT Lymphoid tissue therapeutic procedures.				

CR Number: 2026062010		Implementation Date: 10-Mar-26	Related CR: 2026062010	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Reconstructive surgery</i>	Approved as Requested	Proposed LLT <i>Reconstructive surgery</i>	10093562 Current
			To PT <i>Surgery</i>	10042609
MSSO Comment: The proposal to add a new term Reconstructive surgery is approved as requested. Reconstructive surgery is a general term referring to a branch of surgery focused on restoring the form and function of body parts that have been damaged or lost due to congenital defects, trauma, disease, infection, or previous surgery. Reconstructive surgery will be added as LLT to PT Surgery.				

Detail Report

CR Number: 2026062011		Implementation Date: 10-Mar-26	Related CR: 2026062011	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Anorectovaginoplasty</i>	Approved as Requested	Proposed LLT <i>Anorectovaginoplasty</i>	10093558 Current
			To PT <i>Vaginoperineoplasty</i>	10059349
MSSO Comment: The proposal to add a new term Anorectovaginoplasty is approved as requested. Anorectovaginoplasty is a reconstructive surgical procedure used mainly to correct congenital malformations of the female perineum, particularly anorectal anomalies such as persistent cloaca, imperforate anus with vestibular or vaginal fistula, or a common urogenital sinus. The goal of the operation is to separate and reconstruct the rectum, vagina, and urethra so that three normal and distinct openings are created. Anorectovaginoplasty will be added as LLT to the PT Vaginoperineoplasty.				
CR Number: 2026062012		Implementation Date: 13-Mar-26	Related CR: 2026062012	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Abdominoperineal reconstruction</i>	Approved After Suspension	Proposed PT <i>Abdominoperineal reconstruction</i>	10093586
			HLT primary <i>Anorectal therapeutic procedures</i>	10002580
			SOC primary <i>Surgical and medical procedures</i>	10042613
MSSO Comment: The proposal to add a new term Abdominoperineal reconstruction is approved as requested after suspension. Abdominoperineal reconstruction is a surgical procedure performed to restore anatomical structure and function in defects involving both the lower abdomen and the perineum. It is most commonly required after extensive oncologic resections, trauma, or severe infection, and typically involves the use of tissue flaps, grafts, or other reconstructive techniques to close the defect and reestablish stable soft-tissue coverage and pelvic support. Abdominoperineal reconstruction will be added as a PT to HLT Anorectal therapeutic procedures.				
CR Number: 2026062013		Implementation Date: 10-Mar-26	Related CR: 2026062013	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hearing disorder</i>	Approved as Requested	Proposed LLT <i>Hearing disorder</i>	10093559 Current
			To PT <i>Auditory disorder</i>	10003778
MSSO Comment: The proposal to add a new term Hearing disorder is approved as requested. Hearing disorders involve disruptions in the auditory system, categorized mainly as conductive (outer/middle ear blockages), sensorineural (inner ear/nerve damage), or mixed. Hearing disorder will be added as an LLT to PT Auditory disorder.				

Detail Report

CR Number: 2026062014		Implementation Date: 10-Mar-26	Related CR: 2026062014		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Viral encephalopathy</i>	Rejected			

MSSO Comment: The proposal to add a new term Viral encephalopathy is not approved. Viral encephalopathy is not a well-defined term in medical classification. In clinical practice, reference is more often made to encephalopathy associated with viral infection (for example, influenza-associated encephalopathy) or to metabolic encephalopathy secondary to infection, rather than to viral encephalopathy as a standard entity.

CR Number: 2026062015		Implementation Date: 10-Mar-26	Related CR: 2026062015		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Bacterial encephalopathy</i>	Rejected			

MSSO Comment: The proposal to add a new term Bacterial encephalopathy is not approved. In clinical usage, bacterial infections of the brain parenchyma are referred to as bacterial encephalitis or, more commonly, as brain abscess or cerebritis, conditions that involve demonstrable infection and inflammation. When diffuse brain dysfunction occurs in the context of a bacterial infection without direct invasion of the brain, it is usually described as encephalopathy secondary to systemic infection or sepsis-associated encephalopathy, rather than "bacterial encephalopathy" as a distinct clinical entity. □

CR Number: 2026062016		Implementation Date: 10-Mar-26	Related CR: 2026062016		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Hepatic hemosiderosis</i>	Approved as Requested	Proposed LLT <i>Hepatic hemosiderosis</i> To PT <i>Hepatic iron overload</i>	10093560	Current
				10090169	

MSSO Comment: The proposal to add a new term Hepatic hemosiderosis is approved as requested. Hepatic hemosiderosis is the deposit of an abnormal quantity of hemosiderin in the liver. Hepatic hemosiderosis will be added as an LLT to PT Hepatic iron overload, aligned with synonym LLT Hepatic siderosis. In a related change, the counterpart British English spelled LLT Hepatic haemosiderosis will also be added to PT Hepatic iron overload.

Detail Report

CR Number: 2026069030		Implementation Date: 10-Mar-26	Related CR: 2026062016		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Hepatic haemosiderosis</i>	Approved as Requested	Proposed LLT		
To PT	<i>Hepatic iron overload</i>		<i>Hepatic haemosiderosis</i>	10093563	Current
			To PT		
			<i>Hepatic iron overload</i>	10090169	

MSSO
Comment:

CR Number: 2026062017		Implementation Date: 10-Mar-26	Related CR: 2026062017		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Beeswax allergy</i>	Rejected			

MSSO
Comment: The proposal to add a new term Beeswax allergy is not approved. A beeswax allergy is relatively uncommon often presenting as contact dermatitis. Beeswax is usually found in lip balms and cosmetics. Please consider coding using the most specific clinical manifestation possible, such as LLT/PT Dermatitis contact, or one of its subordinate LLTs depending on specific scenario.

CR Number: 2026062018		Implementation Date: 10-Mar-26	Related CR: 2026062018		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Respiratory rehabilitation</i>	Approved Not as Requested	Proposed LLT		
			<i>Respiratory rehabilitation therapy</i>	10093555	Current
			To PT		
			<i>Respiratory therapy</i>	10059828	

MSSO
Comment: The proposal to add a new term Respiratory rehabilitation is approved but not as requested. Respiratory rehabilitation therapy is a multidisciplinary therapeutic program designed for people with chronic lung disease. It combines medical management, physical training, education, and behavioral strategies to improve breathing, exercise tolerance, and quality of life. Respiratory rehabilitation therapy will be added as LLT to PT Respiratory therapy.

Detail Report

CR Number: 2026062019	Implementation Date: 09-Mar-26	Related CR: 2026062019	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Carnitine deficiency myopathy</i>	Rejected	

MSSO Comment: The proposal to add a new term Carnitine deficiency myopathy is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. MedDRA cannot accommodate specific combination terms for rare multisystemic disease with every possible clinical manifestation.

CR Number: 2026062020	Implementation Date: 10-Mar-26	Related CR: 2026062020	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Athletes heart</i>	Rejected	

MSSO Comment: The proposal to add a new term Athletes heart is not approved. Athlete's heart is not a formal clinical diagnosis but refers to a physiological adaptation of the heart to prolonged, intensive exercise. Normal physiological expressions are usually not added to MedDRA. The pathological manifestation can be found in concept LLT/PT Athletic heart syndrome.

CR Number: 2026062021	Implementation Date: 10-Mar-26	Related CR: 2026062021	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Vulval ablation</i>	Rejected	

MSSO Comment: The proposal to add a new term Vulval ablation is not approved. Vulval ablation, often referred to as laser ablation of the vulva, is a medical procedure in which abnormal, precancerous, or diseased tissue on the vulva is removed or destroyed using a laser. In medical practice, this term is not used as an entity but rather in reference to a specific vulvar lesion, as in ablation of vulvar intraepithelial neoplasia or ablation of vulvar lesions. The proposed concept can be captured with combination coding with LLT Laser ablation (added in change request 2026062007) and the most appropriate vulvar lesion concept term.

Detail Report

CR Number: 2026063000	Implementation Date: 09-Mar-26	Related CR: 2026063000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Hymenal caruncle</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Hymenal caruncle is not approved. Hymenal caruncles are common, typically asymptomatic, and are most often normal anatomical variations and as such normal anatomical variations are not added in MedDRA.

CR Number: 2026063001	Implementation Date: 09-Mar-26	Related CR: 2026063001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Extraconal hematoma</i>	Approved Not as Requested	Proposed LLT <i>Extraconal orbital hematoma</i> To PT <i>Orbital haematoma</i>
			10093565 Current 10083565

MSSO Comment: The proposal to add a new term Extraconal hematoma is approved but not as requested. An extraconal orbital hematoma is a localized collection of blood inside the eye's bony orbit but outside the muscle cone, usually due to trauma, surgery, or vascular injury. It can cause eyelid swelling, proptosis, and rarely optic nerve compression, requiring observation or surgical intervention if vision is threatened. The proposed term will be modified for clarity to Extraconal orbital hematoma and will be added as a sub-concept LLT to PT Orbital haematoma. In a related change, the British English spelled counterpart LLT Extraconal orbital haematoma will also be added to PT Orbital haematoma.

CR Number: 2026069031	Implementation Date: 10-Mar-26	Related CR: 2026063001	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Extraconal orbital haematoma</i>	Approved as Requested	Proposed LLT <i>Extraconal orbital haematoma</i> To PT <i>Orbital haematoma</i>
To PT			10093566 Current 10083565

MSSO Comment:

Detail Report

CR Number: 2026063002	Implementation Date: 10-Mar-26	Related CR: 2026063002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Rhinoviral pneumonia</i>	Rejected	

MSSO Comment: The proposal to add a new term Rhinoviral pneumonia is not approved. The evidence that rhinovirus (HRV) can independently cause pneumonia has strengthened considerably in recent years through advances in PCR monitoring in pneumonia patients. The most compelling evidence comes from histological detection of HRV within lower lung tissue and clinical cohorts meeting criteria for simple viral pneumonia with HRV as the primary pathogen. This is supported by multiple observational and controlled studies which find HRV significantly associated with pneumonia in adults and children, above background detection in asymptomatic individuals, although co-infections remain common and a substantial proportion of cases involve multiple pathogens. The concept is not yet fully established and may be captured with combination coding.

CR Number: 2026063003	Implementation Date: 09-Mar-26	Related CR: 2026063003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Cholestatic drug-induced liver injury</i>	Rejected	

MSSO Comment: The proposal to add a new term Cholestatic drug-induced liver injury is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. Additionally, MedDRA limits the inclusion of terms with several qualifiers to the minimum.

CR Number: 2026063004	Implementation Date: 10-Mar-26	Related CR: 2026063004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Perioperative pain</i>	Rejected	

MSSO Comment: The proposal to add a new term Perioperative pain is not approved. Please consider LLTs linked to PT Procedural pain.

Detail Report

CR Number: 2026063005		Implementation Date: 10-Mar-26	Related CR: 2026063005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Palpation</i>	Approved as Requested	Proposed PT <i>Palpation</i>	10093567
			HLT primary <i>Physical examination procedures and organ system status</i>	10071941
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new term Palpation is approved as requested. Palpation is a physical examination technique in which the examiner uses the hands and fingers to feel body tissues or organs in order to assess characteristics such as size, shape, consistency, tenderness, temperature, or the presence of masses. Palpation will be added as PT to the HLT Physical examination procedures and organ system status.

CR Number: 2026064000		Implementation Date: 10-Mar-26	Related CR: 2026064000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Pituitary crisis</i>	Approved as Requested	Proposed PT <i>Pituitary crisis</i>	10093568
			HLT primary <i>Anterior pituitary hypofunction</i>	10002701
			SOC primary <i>Endocrine disorders</i>	10014698
			HLT secondary <i>Nervous system haemorrhagic disorders</i>	10008206
			SOC secondary <i>Vascular disorders</i>	10047065
			HLT secondary <i>Central nervous system haemorrhages and cerebrovascular accidents</i>	10007948
			SOC secondary <i>Nervous system disorders</i>	10029205

MSSO Comment: The proposal to add a new term Pituitary crisis is approved as requested. Pituitary crisis is an acute, life-threatening clinical state caused by sudden failure of pituitary function most often due to pituitary apoplexy or to an acute decompensation of hypopituitarism. It is characterized by abrupt headache, visual or cranial nerve deficits, altered consciousness, and endocrine insufficiency—particularly secondary adrenal insufficiency from impaired ACTH secretion. Pituitary crisis will be added as a PT to primary HLT Anterior pituitary hypofunction, to secondary HLT Nervous system haemorrhagic disorders, and to secondary HLT Central nervous system haemorrhages and cerebrovascular accidents.

Detail Report

CR Number: 2026064001	Implementation Date: 09-Mar-26	Related CR: 2026064001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Starvation ketosis	Rejected	

MSSO Comment: The proposal to add a new term Starvation ketosis is not approved. The requested term is the physiologic state where the body produces ketone bodies for energy during prolonged fasting, carbohydrate restriction, or very low-calorie intake, while Starvation ketoacidosis is a pathological progression of starvation ketosis where ketone accumulation becomes excessive, causing metabolic acidosis. MedDRA includes LLT/PT Starvation ketoacidosis for the pathologic state.

CR Number: 2026065002	Implementation Date: 10-Mar-26	Related CR: 2026065002	MedDRA Code #
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement
Proposed PT	Gastroenteritis	Rejected	
Old Primary SOC	Infections and infestations		
New Primary SOC	Gastrointestinal disorders		

MSSO Comment: The proposal to reassign the primary SOC of PT Gastroenteritis from current SOC Infections and infestations to SOC Gastrointestinal disorders is not approved. The etiologic distribution of gastroenteritis is predominantly viral, followed by bacterial, fungal and toxin-mediated, while drug induced causes are rare, although the distribution varies by population, region, and clinical setting. For this reason and legacy data considerations, the PT is considered adequately placed.

CR Number: 2026068000	Implementation Date: 13-Mar-26	Related CR: 2026068000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Central line site hematoma	Rejected	

MSSO Comment: The proposal to add a new term Central line site hematoma is not approved. This is a multiple concept term which can be coded using more than one MedDRA term, for example, LLT Central line complication and LLT Catheter site hematoma or other depending on your specific scenario. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.

Detail Report

CR Number: 2026069000		Implementation Date: 13-Mar-26	Related CR: 2026069000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Retinotomy</i>	Approved as Requested	Proposed PT <i>Retinotomy</i>	10093569
			HLT primary <i>Retinal therapeutic procedures</i>	10038890
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Retinotomy is approved as requested. Retinotomy is a surgical procedure involving the cutting or incising of the retina, often performed during vitrectomy to treat complex retinal detachments. It is primarily used to release severe traction, relax a stiff or shortened retina, or drain subretinal fluid in cases of proliferative vitreoretinopathy. Retinotomy will be added as PT to HLT Retinal therapeutic procedures.

CR Number: 2026069001		Implementation Date: 13-Mar-26	Related CR: 2026069001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Purpuric dermatitis</i>	Rejected		

MSSO Comment: The proposal to add a new term Purpuric dermatitis is not approved. Pigmented purpuric dermatoses refer to a specific group of disorders characterized by capillary leakage in the skin that produces purpura with inflammatory changes. The proposed concept is not an established standalone term submitted as such but rather a descriptive term which can be represented by existing MedDRA terms. Please consider existing term LLT/PT Chronic pigmented purpura, which is also commonly used interchangeably with the term "pigmented purpuric dermatitis", depending on your specific case scenario.

CR Number: 2026070000		Implementation Date: 13-Mar-26	Related CR: 2026070000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>DaT scan</i>	Rejected		

MSSO Comment: The proposal to add a new term DaT scan is not approved. Within neurology or nuclear medicine: "DaT scan" is usually understood to mean dopamine transporter imaging. However, across disciplines, the abbreviation DAT itself is ambiguous. The proposed concept exists in MedDRA as LLT/PT Dopamine transporter scintigraphy including qualified normal and abnormal counterpart terms. The proposed concept is as well a trademark name.

Detail Report

CR Number: 2026072000	Implementation Date: 17-Mar-26	Related CR: 2026072000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Snuffle</i>	Rejected	

MSSO Comment: The proposal to add a new term Snuffle is not approved. The proposed term is a colloquial term and may not be used uniformly nor easily translated.

CR Number: 2026072001	Implementation Date: 15-May-26	Related CR: 2026072001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Application of a cosmetic product</i>	Rejected After Suspension	

MSSO Comment: The proposal to add a new term Application of a cosmetic product is not approved after suspension. The proposed concept is not associated with a medical diagnosis/event and thus is not within the scope of MedDRA.

CR Number: 2026072002	Implementation Date: 17-Mar-26	Related CR: 2026072002	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Subepithelial vocal fold infusion</i>	Rejected	
To PT	<i>Vocal cord augmentation</i>		

MSSO Comment: The proposal to add a new LLT Subepithelial vocal fold infusion to PT Vocal cord augmentation is not approved. The proposed term is too granular. The MSSO generally avoids adding sub-techniques which are part of a bigger surgical procedure. As well this technique may be used in several contexts.

Detail Report

CR Number: 2026072003		Implementation Date: 17-Mar-26	Related CR: 2026072003		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Drug-induced hepatitis B virus reactivation</i>	Rejected			

MSSO Comment: The proposal to add a new term Drug-induced hepatitis B virus reactivation is not approved. Although there are existing "drug induced" terms in MedDRA, additional terms that state or imply a "due to" concept are generally not added to MedDRA because causality is not inherent in the coded term but is instead determined by the reporter/investigator or by the sponsor. Please also consider LLT/PT Hepatitis B reactivation for your coding needs depending on your specific scenario.

CR Number: 2026073000		Implementation Date: 19-Mar-26	Related CR: 2026073000		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Infratemporal space infection</i>	Approved as Requested	Proposed LLT		
			<i>Infratemporal space infection</i>	10093592	Current
			To PT		
			<i>Maxillofacial space infection</i>	10089849	

MSSO Comment: The proposal to add a new LLT Infratemporal space infection is approved as requested. An infratemporal space infection is a deep facial infection involving the infratemporal fossa, a compartment posterior to the maxilla, medial to the mandibular ramus, and inferior to the temporal fossa, containing the pterygoid muscles, maxillary artery, and mandibular nerve branches. Clinically, it presents with trismus, deep facial or temporal pain, and lateral facial swelling, often originating from posterior maxillary teeth or masticator space infections. Infratemporal space infection will be added as LLT to PT Maxillofacial space infection.

CR Number: 2026073001		Implementation Date: 19-Mar-26	Related CR: 2026073001		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>External diaphragmatic pacing</i>	Approved as Requested	Proposed LLT		
			<i>External diaphragmatic pacing</i>	10093593	Current
			To PT		
			<i>Diaphragmatic pacemaker insertion</i>	10079721	

MSSO Comment: The proposal to add a new LLT External diaphragmatic pacing is approved as requested. External diaphragmatic pacing mitigates ventilation-induced diaphragmatic atrophy through targeted phrenic nerve stimulation in mechanically ventilated patients. External diaphragmatic pacing will be added as LLT to PT Diaphragmatic pacemaker insertion.

Detail Report

CR Number: 2026073002		Implementation Date: 18-Mar-26	Related CR: 2026073002		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	HEART YANG DEFICIENCY SYNDROME	Rejected			

MSSO Comment: The proposal to add a new LLT HEART YANG DEFICIENCY SYNDROME is not approved. Heart yang deficiency syndrome is a traditional Chinese medicine (TCM) diagnosis. TCM terms are not in the scope of MedDRA. Usually, the signs and symptoms of TCM diagnoses have a Western medicine equivalent, which can be used, if no equivalent diagnosis is available.

CR Number: 2026073003		Implementation Date: 19-Mar-26	Related CR: 2026073003		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Psychogenic Nonepileptic Seizures	Approved Not as Requested	Proposed LLT Psychogenic non-epileptic seizure To PT Psychogenic seizure	10093589 Current	

MSSO Comment: The proposal to add a new LLT Psychogenic Nonepileptic Seizures is approved but not as requested. Psychogenic non-epileptic seizures are episodic events that resemble epileptic seizures in appearance but are not caused by abnormal electrical activity in the brain. Instead, they arise from psychological mechanisms. The proposed term will be modified for MedDRA capitalization rules and added as LLT Psychogenic non-epileptic seizure to PT Psychogenic seizure.

CR Number: 2026073004		Implementation Date: 18-Mar-26	Related CR: 2026073004		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	APOE e3/e4 gene carrier	Approved Not as Requested	Proposed LLT Apolipoprotein E e3/e4 gene carrier To PT Apolipoprotein E e4 gene carrier	10093590 Current	

MSSO Comment: The proposal to add a new LLT APOE e3/e4 gene carrier is approved but not as requested. The APOE gene (Apolipoprotein E) has three common alleles: ε2, ε3, and ε4. APOE ε4 affects Alzheimer's risk in a dose-dependent manner. An e3/e4 genotype carrier has heterozygosity, with one neutral allele (ε3) and one higher-risk allele (ε4). Individuals heterozygous for the APOE ε4 allele (genotype e3/e4) exhibit a moderately elevated risk of developing late-onset Alzheimer's disease and may experience an earlier age of onset relative to ε3 homozygotes; however, the presence of a single ε4 allele is not fully penetrant, and a substantial proportion of carriers remain cognitively healthy throughout late adulthood. The proposed term will be modified and added in its spelled-out form as LLT Apolipoprotein E e3/e4 gene carrier to PT Apolipoprotein E e4 gene carrier.

Detail Report

CR Number: 2026073005		Implementation Date: 19-Mar-26	Related CR: 2026073005	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Cryptosporidium test</i>	Approved Not as Requested	Proposed PT <i>Cryptosporidium test</i>	10093587
			HLT primary <i>Parasite identification and serology</i>	10033903
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT *Cryptosporidium test* is approved but not as requested. Cryptosporidia are diagnosed mainly by detecting oocysts in stool using acid-fast staining or fluorescent microscopy, with antigen detection assays (EIA/IFA) providing faster and more sensitive results. PCR can confirm infection and identify species, while histology is reserved for unclear cases. *Cryptosporidium test* will be added as PT to HLT *Parasite identification and serology*.

CR Number: 2026073006		Implementation Date: 19-Mar-26	Related CR: 2026073006	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Masako maneuver</i>	Approved as Requested	Proposed LLT <i>Masako maneuver</i>	10093591 Current
			To PT <i>Dysphagia management</i>	10088622

MSSO Comment: The proposal to add a new LLT *Masako maneuver* is approved as requested. The *Masako maneuver* is a swallowing exercise in which the patient protrudes the tongue slightly between the teeth and swallows, aiming to strengthen the posterior pharyngeal wall and improve pharyngeal contraction. It is used in speech-language therapy because targeted pharyngeal strengthening enhances safe and effective swallowing in patients with dysphagia. *Masako maneuver* will be added as LLT to PT *Dysphagia management*.

CR Number: 2026073007		Implementation Date: 18-Mar-26	Related CR: 2026073007	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>VASOGENIC EDEMA</i>	Rejected		

MSSO Comment: The proposal to add a new LLT *VASOGENIC EDEMA* is not approved. The term "vasogenic edema" is mostly specific to the central nervous system because it specifically refers to fluid accumulation due to capillary permeability in the context of the blood-brain barrier breakdown. In other organs, similar mechanisms of edema exist (e.g., pulmonary edema, renal interstitial edema), but they are generally described with organ-specific terms rather than "vasogenic." The concept is represented in MedDRA as anatomically qualified LLT/PT *Vasogenic cerebral oedema*.

Detail Report

CR Number: 2026073008		Implementation Date: 19-Mar-26	Related CR: 2026073008	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	ASOMATOGNOSIA	Approved Not as Requested	Proposed PT <i>Asomatognosia</i>	10093588
			HLT primary <i>Cortical dysfunction NEC</i>	10011168
			SOC primary <i>Nervous system disorders</i>	10029205
			HLT secondary <i>Specific cognitive ability disturbances</i>	10041427
			SOC secondary <i>Psychiatric disorders</i>	10037175

MSSO
Comment: The proposal to add a new LLT ASOMATOGNOSIA is approved but not as requested. Asomatognosia is a neurological condition in which a person loses awareness of one or more parts of their own body. Classic asomatognosia usually involves only a part of the body, not the entire body. The proposed term will be modified for MedDRA capitalization conventions and added as PT Asomatognosia to primary HLT Cortical dysfunction NEC and to secondary HLT Specific cognitive ability disturbances. In a related change, PT Hemiasomatognosia will be demoted to new PT Asomatognosia.

CR Number: 2026078065		Implementation Date: 19-Mar-26	Related CR: 2026073008	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Hemiasomatognosia</i>	Approved as Requested	PT to merge <i>Hemiasomatognosia</i>	10077171 Current
To PT	<i>Asomatognosia</i>		To PT <i>Asomatognosia</i>	10093588

MSSO
Comment:

Detail Report

CR Number: 2026073009		Implementation Date: 19-Mar-26	Related CR: 2026073009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Somatoparaphrenia</i>	Approved Not as Requested	Proposed PT <i>Somatoparaphrenia</i>	10093594
			HLT primary <i>Cortical dysfunction NEC</i>	10011168
			SOC primary <i>Nervous system disorders</i>	10029205
			HLT secondary <i>Specific cognitive ability disturbances</i>	10041427
			SOC secondary <i>Psychiatric disorders</i>	10037175

MSSO Comment: The proposal to add a new LLT Somatoparaphrenia is approved but not as requested. Somatoparaphrenia is a neuropsychological disorder in which the person denies that a part of their body belongs to them, most often an arm or a leg, and may attribute it to someone else. It usually appears after brain lesions, particularly of the right hemisphere, and is often associated with hemineglect, anosognosia, and sensory or motor deficits. Somatoparaphrenia will be added as PT with a primary link to HLT Cortical dysfunction NEC and secondary link to HLT Specific cognitive ability disturbances.

CR Number: 2026073010		Implementation Date: 19-Mar-26	Related CR: 2026073010	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Paratracheal dissection</i>	Approved Not as Requested	Proposed LLT <i>Paratracheal neck dissection</i>	10093597 Current
			To PT <i>Neck dissection</i>	10081204

MSSO Comment: The proposal to add a new LLT Paratracheal dissection is approved but not as requested. Paratracheal dissection refers to the surgical dissection of tissues adjacent to the trachea, typically within the central compartment of the neck. In clinical practice, the term is most commonly used in thyroid and head-and-neck surgery to describe the exposure and removal of paratracheal lymph nodes. It is usually performed for staging or treatment of thyroid carcinoma. The proposed term will be modified for clarity and added as LLT Paratracheal neck dissection to PT Neck dissection.

Detail Report

CR Number: 2026073011		Implementation Date: 19-Mar-26	Related CR: 2026073011		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Ova and parasite (O&P) test</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Ova and parasite (O&P) test is not approved. The proposed concept exists as LLT Ova and parasite test.

CR Number: 2026073012		Implementation Date: 19-Mar-26	Related CR: 2026073012		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>POUCHOSCOPY</i>	Approved Not as Requested	Proposed LLT <i>Pouchoscopy</i>	10093596	Current
			To PT <i>Colonoscopy</i>	10010007	

MSSO Comment: The proposal to add a new LLT POUCHOSCOPY is approved but not as requested. Pouchoscopy is an endoscopy of the pouch, usually the ileal pouch–anal reservoir (J-pouch). It consists of introducing an endoscope through the anus to visualize the pouch, obtain biopsies, and assess complications such as pouchitis, cuffitis, ulcers, strictures, or neoplastic lesions. It is usually the first-line test to evaluate pouch dysfunction. The proposed term will be modified in accordance with MedDRA capitalization conventions and added as LLT Pouchoscopy to PT Colonoscopy.

CR Number: 2026073013		Implementation Date: 19-Mar-26	Related CR: 2026073013		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Rectal Culture</i>	Approved Not as Requested	Proposed LLT <i>Rectal culture</i>	10093599	Current
			To PT <i>Culture stool</i>	10011629	

MSSO Comment: The proposal to add a new LLT Rectal Culture is approved but not as requested. Rectal culture is a microbiological test in which a swab is inserted into the rectum to obtain a sample of rectal mucosa and fecal material, which is then cultured in the laboratory to detect microorganisms present in the rectal flora. The proposed term will be modified according to MedDRA capitalization conventions and added as the LLT Rectal culture to the PT Culture stool.

Detail Report

CR Number: 2026073014	Implementation Date: 19-Mar-26	Related CR: 2026073014	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	ENDOMETRIAL FLUID COLLECTION	Rejected	

MSSO Comment: The proposal to add a new LLT ENDOMETRIAL FLUID COLLECTION is not approved. Endometrial fluid collection is a descriptive imaging finding rather than a clinical diagnosis or a defined pathological entity. It simply describes the presence of fluid within the endometrial cavity, which may result from various conditions. It is generally preferable to code the underlying clinical condition when identified (e.g., hydrometra, pyometra, or hematometra) or to code a related clinical diagnosis.

CR Number: 2026073015	Implementation Date: 19-Mar-26	Related CR: 2026073015	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Rapid Rota-Adenovirus Combo Test	Approved Not as Requested	Proposed PT Rotavirus and adenovirus rapid combination test 10093595 HLT primary Virus identification and serology 10047490 SOC primary Investigations 10022891

MSSO Comment: The proposal to add a new LLT Rapid Rota-Adenovirus Combo Test is approved but not as requested. A rapid rota-adenovirus combo test is a point-of-care diagnostic assay that simultaneously detects rotavirus and adenovirus antigens in stool specimens. It is used to identify viral causes of acute gastroenteritis, especially in children, and typically provides results within minutes. The proposed term will be modified to comply with MedDRA capitalization conventions and added as PT Rotavirus and adenovirus rapid combination test to HLT Virus identification and serology.

CR Number: 2026073016	Implementation Date: 18-Mar-26	Related CR: 2026073016	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Electrolyte and Osmolality Panel, Feces	Rejected	

MSSO Comment: The proposal to add a new LLT Electrolyte and Osmolality Panel, Feces is not approved. The proposed term does not constitute a specific laboratory test but rather describes a group of laboratory tests performed on a stool sample to measure the concentration of electrolytes (usually sodium, potassium, and chloride) and to calculate or measure fecal osmolality, generally to evaluate the pathophysiology of a diarrheal condition. Such tests are not necessarily standardized across laboratories.

Detail Report

CR Number: 2026073017		Implementation Date: 19-Mar-26	Related CR: 2026073017	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Laparoscopic and endoscopic cooperative surgery (LECS)	Approved Not as Requested	Proposed LLT Laparoscopic and endoscopic cooperative surgery To PT Laparoscopic surgery	10093598 Current 10058468
MSSO Comment:	The proposal to add a new LLT Laparoscopic and endoscopic cooperative surgery (LECS) is approved but not as requested. Laparoscopic and endoscopic cooperative surgery is a hybrid minimally invasive surgical technique that combines endoscopic and laparoscopic approaches to resect lesions of the gastrointestinal tract, most commonly gastric tumors. The proposed term will be modified to remove the parenthesized acronym and added as LLT Laparoscopic and endoscopic cooperative surgery to PT Laparoscopic surgery.			

CR Number: 2026073018		Implementation Date: 19-Mar-26	Related CR: 2026073018	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Increased renal echogenicity	Rejected		
MSSO Comment:	The proposal to add a new LLT Increased renal echogenicity is not approved. The proposed term is one of the many possible descriptive findings resulting from a renal ultrasound. "Increased echogenicity" is a descriptor that could be applied to any anatomical site examined by ultrasonography, and it could be a finding representing normal variation, so to avoid overpopulation, MedDRA cannot accommodate individual terms to represent such non-specific findings of all possible anatomical sites.			

Detail Report

CR Number: 2026075000		Implementation Date: 19-Mar-26	Related CR: 2026075000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Autosomal dominant tubulointerstitial kidney disease</i>	Approved as Requested	Proposed PT <i>Autosomal dominant tubulointerstitial kidney disease</i>	10093601
			HLT primary <i>Renal disorders congenital</i>	10038431
			SOC primary <i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary <i>Nephropathies and tubular disorders NEC</i>	10029150
			SOC secondary <i>Renal and urinary disorders</i>	10038359

MSSO Comment: The proposal to add a new term Autosomal dominant tubulointerstitial kidney disease is approved as requested. Autosomal dominant tubulointerstitial kidney disease (ADTKD) is a recently defined entity that includes rare kidney diseases characterized by tubular damage and interstitial fibrosis in the absence of glomerular lesions, with inescapable progression to end-stage renal disease. ADTKD is one of the more common monogenic kidney diseases after autosomal dominant polycystic kidney disease, accounting for ~5% of monogenic disorders causing chronic kidney disease. Autosomal dominant tubulointerstitial kidney disease will be added as PT to primary HLT Renal disorders congenital and to secondary HLT Nephropathies and tubular disorders NEC.

CR Number: 2026075001		Implementation Date: 19-Mar-26	Related CR: 2026075001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Infero nasal feeder vessels</i>	Approved Not as Requested	Proposed LLT <i>Conjunctival infero-nasal feeder vessel</i>	10093602 Current
			To PT <i>Conjunctival feeder vessel</i>	10093600

MSSO Comment: The proposal to add a new term Infero nasal feeder vessels is approved but not as requested. A conjunctival infero-nasal feeder vessel is a prominent, dilated blood vessel located in the lower-nasal quadrant of the bulbar conjunctiva that supplies blood to an underlying lesion or growth. These vessels are frequently associated with ocular surface tumors, such as conjunctival nevi, squamous cell carcinoma, or other vascularized, benign, and malignant lesions. Proposed term will be qualified and added in singular form as LLT Conjunctival infero-nasal feeder vessel to new PT Conjunctival feeder vessel. In a related change, new PT Conjunctival feeder vessel will be added to primary HLT Conjunctival and corneal bleeding and vascular disorders and secondary HLT Ocular vascular disorders NEC.

Detail Report

CR Number: 2026078066		Implementation Date: 19-Mar-26	Related CR: 2026075001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Conjunctival feeder vessel</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Conjunctival and corneal bleeding and vascular disorders</i>		<i>Conjunctival feeder vessel</i>	10093600
SOC primary	<i>Eye disorders</i>		HLT primary	
			<i>Conjunctival and corneal bleeding and vascular disorders</i>	10010728
			SOC primary	
			<i>Eye disorders</i>	10015919
			HLT secondary	
			<i>Ocular vascular disorders NEC</i>	10030067
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO
Comment:

CR Number: 2026075002		Implementation Date: 18-Mar-26	Related CR: 2026075002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Biopsy of any part of the eye.</i>	Rejected		

MSSO
Comment: The proposal to add a new term Biopsy of any part of the eye is not approved. Please consider concepts within the HLT Ophthalmic histopathology and imaging procedures where various site-specific eye biopsy terms are located, e.g. LLT/PT Biopsy cornea, LLT/PT Biopsy conjunctiva, LLT/PT Biopsy eyelid, LLT/PT Biopsy retina, LLT/PT Biopsy sclera, etc. depending on your specific scenario.

CR Number: 2026075003		Implementation Date: 19-Mar-26	Related CR: 2026075003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Chronic vs acute cervical pain</i>	Rejected		

MSSO
Comment: The proposal to add a new term Chronic vs acute cervical pain is not approved. It is not possible to have a chronic/acute descriptor for every condition. MedDRA generally limits adding terms with more than one qualifier.

Detail Report

CR Number: 2026075004	Implementation Date: 19-Mar-26	Related CR: 2026075004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Punctate telangiectasia vs telangiectasia</i>	Rejected	

MSSO Comment: The proposal to add a new term Punctate telangiectasia vs telangiectasia is not approved. The proposed term is a descriptive term, but not standard terminology for a defined concept. It is rather a free way of describing very small (punctate) telangiectasias.

CR Number: 2026076000	Implementation Date: 19-Mar-26	Related CR: 2026076000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Pre-Glenn</i>	Rejected	

MSSO Comment: The proposal to add a new term Pre-Glenn is not approved. "Pre-Glenn" is not a universally standardized or formally recognized evaluation term. Its meaning is context-dependent, and in the context of the Glenn procedure there is strong international consensus on which physiological criteria must be assessed and met before surgery. However, there is no single universal protocol. Such preparatory procedures are not generally included in MedDRA.

CR Number: 2026076001	Implementation Date: 19-Mar-26	Related CR: 2026076001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Vascular ring division</i>	Approved as Requested	Proposed LLT <i>Vascular ring division</i> 10093604 Current To PT <i>Vascular ring repair</i> 10093603

MSSO Comment: The proposal to add a new term Vascular ring division is approved as requested. Vascular ring division, or vascular ring repair, is a surgical procedure used to treat a vascular ring, a congenital condition in which abnormal formations of the aortic arch and its branches encircle and compress the trachea and/or esophagus. The procedure involves cutting the constricting vascular structure to relieve pressure on the airway and esophagus, thereby improving breathing and swallowing. Vascular ring division will be added as an LLT to new PT Vascular ring repair. In a related change, PT Vascular ring repair will be added to HLT Aortic therapeutic procedures.

Detail Report

CR Number: 2026078067		Implementation Date: 19-Mar-26	Related CR: 2026076001	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Vascular ring repair</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Aortic therapeutic procedures</i>		<i>Vascular ring repair</i>	10093603
SOC primary	<i>Surgical and medical procedures</i>		HLT primary	
			<i>Aortic therapeutic procedures</i>	10002903
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613

MSSO
Comment:

CR Number: 2026076002		Implementation Date: 24-Mar-26	Related CR: 2026076002	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	<i>Sano conduit upsizing</i>	Approved as Requested	Proposed LLT	
			<i>Sano conduit upsizing</i>	10093606 Current
			To PT	
			<i>Norwood procedure</i>	10068098

MSSO
Comment: The proposal to add a new term Sano conduit upsizing is approved as requested. Sano conduit upsizing is a secondary surgical modification performed after a Norwood procedure with a Sano shunt, in which the original right ventricle-to-pulmonary artery conduit is replaced with a larger-diameter graft to increase pulmonary blood flow, pulmonary venous return to the left heart and consequently left ventricular preload in an attempt to improve left ventricular function. Sano conduit upsizing will be added as LLT to the PT Norwood procedure.

CR Number: 2026076003		Implementation Date: 19-Mar-26	Related CR: 2026076003	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Drug-laboratory test interactions</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Therapeutic agent-diagnostic test interaction</i>		<i>Drug-laboratory test interaction</i>	10093605 Current
			To PT	
			<i>Therapeutic agent-diagnostic test interaction</i>	10079427

MSSO
Comment: The proposal to add a new LLT Drug-laboratory test interactions to PT Therapeutic agent-diagnostic test interaction is approved but not as requested. Drug-laboratory test interaction represents a more specific interaction between a drug and resultant laboratory test interactions. The proposed term will be modified and added in singular form as LLT Drug-laboratory test interaction to PT Therapeutic agent-diagnostic test interaction.

Detail Report

CR Number: 2026076138	Implementation Date: 19-Mar-26	Related CR: 2026076138	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Precancerous polyp of colon</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Precancerous polyp of colon is not approved. This is a multiple concept term which can be coded using more than one MedDRA term, for example, LLT Colonic polyp and LLT Precancerous lesion of colon, depending on your specific scenario. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.

CR Number: 2026076139	Implementation Date: 19-Mar-26	Related CR: 2026076139	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Neurological ultrasound</i>	Rejected	

MSSO Comment: The proposal to add a new term Neurological ultrasound is not approved. The concept neurological ultrasound is overly broad and unspecific. Please consider existing terms like e.g. LLT/PT Peripheral nerve ultrasound or LLTs linked to PT Ultrasound head depending on your specific scenario.

CR Number: 2026076140	Implementation Date: 24-Mar-26	Related CR: 2026076140	MedDRA Code #
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement
Proposed PT	<i>Varicella zoster virus infection</i>	Approved as Requested	Proposed PT
To HLT	<i>Skin and subcutaneous tissue viral infections</i>		<i>Varicella zoster virus infection</i> 10075611
			To HLT
			<i>Skin and subcutaneous tissue viral infections</i> 10040797

MSSO Comment: The proposal to link the PT Varicella zoster virus infection to the HLT Skin and subcutaneous tissue viral infections is approved as requested. Linking PT Varicella zoster virus to HLT Skin and subcutaneous tissue viral infections improves the representation of this concept in MedDRA.

Detail Report

CR Number: 2026076141		Implementation Date: 24-Mar-26	Related CR: 2026076141	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pityriasis capitis</i>	Approved as Requested	Proposed LLT	10093627 Current
To PT	<i>Dandruff</i>		<i>Pityriasis capitis</i>	
			To PT	10011859
		<i>Dandruff</i>		
MSSO Comment:	The proposal to add a new LLT Pityriasis capitis to PT Dandruff is approved as requested. Pityriasis capitis commonly known as dandruff is one of the most common and widely seen dermatological disease.			

CR Number: 2026076142		Implementation Date: 24-Mar-26	Related CR: 2026076142	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Chronic relapsing vesiculosquamous dermatitis</i>	Approved as Requested	Proposed LLT	10093623 Current
To PT	<i>Dyshidrotic eczema</i>		<i>Chronic relapsing vesiculosquamous dermatitis</i>	
			To PT	10013913
		<i>Dyshidrotic eczema</i>		
MSSO Comment:	The proposal to add a new LLT Chronic relapsing vesiculosquamous dermatitis to PT Dyshidrotic eczema is approved as requested. Chronic relapsing vesiculosquamous dermatitis is a subtype of dyshidrotic eczema. Chronic relapsing vesiculosquamous dermatitis is characterized by recurrent vesicular eruptions and scaling, often affecting the palms and fingers.			

CR Number: 2026076143		Implementation Date: 24-Mar-26	Related CR: 2026076143	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Nummular psoriasis</i>	Approved as Requested	Proposed LLT	10093624 Current
To PT	<i>Psoriasis</i>		<i>Nummular psoriasis</i>	
			To PT	10037153
		<i>Psoriasis</i>		
MSSO Comment:	The proposal to add a new LLT Nummular psoriasis to PT Psoriasis is approved as requested. Nummular psoriasis is a form of psoriasis characterized by the appearance of small, discoid red lesions a few centimeters in diameter, whose size and shape resemble coins, hence the term "nummular", often associated with intense itching.			

Detail Report

CR Number: 2026076144		Implementation Date: 24-Mar-26	Related CR: 2026076144	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Generalised pustular psoriasis of pregnancy</i>	Approved as Requested	Proposed LLT	
To PT	<i>Impetigo herpetiformis</i>		<i>Generalised pustular psoriasis of pregnancy</i>	10093632
			To PT	
			<i>Impetigo herpetiformis</i>	10021534 Current
MSSO Comment: The proposal to add a new LLT Generalised pustular psoriasis of pregnancy to PT Impetigo herpetiformis is approved as requested. Pustular psoriasis of pregnancy, also known as impetigo herpetiformis, is a rare variant of generalised pustular psoriasis that occurs in pregnancy.				

CR Number: 2026076145		Implementation Date: 24-Mar-26	Related CR: 2026076145	
Change Requested	Swap a PT with an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Impetigo herpetiformis</i>	Approved as Requested	Proposed PT	
With LLT	<i>Generalised pustular psoriasis of pregnancy</i>		<i>Impetigo herpetiformis</i>	10021534 Current
			With LLT	
			<i>Generalised pustular psoriasis of pregnancy</i>	10093632
MSSO Comment: The proposal to swap the PT Impetigo herpetiformis with the LLT Generalised pustular psoriasis of pregnancy is approved as requested. New term Generalised pustular psoriasis of pregnancy is better represented at the PT level.				

CR Number: 2026076146		Implementation Date: 24-Mar-26	Related CR: 2026076146	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Generalized pustular psoriasis of pregnancy</i>	Approved as Requested	Proposed LLT	
To PT	<i>Impetigo herpetiformis</i>		<i>Generalized pustular psoriasis of pregnancy</i>	10093628 Current
			To PT	
			<i>Impetigo herpetiformis</i>	10021534 Current
MSSO Comment: The proposal to add a new LLT Generalized pustular psoriasis of pregnancy to PT Impetigo herpetiformis is approved as requested. Pustular psoriasis of pregnancy, also known as impetigo herpetiformis, is a rare variant of generalised pustular psoriasis that occurs in pregnancy. The requested term is the US English spelled counterpart of LLT Generalised pustular psoriasis of pregnancy, newly added in change request 2026076144.				

Detail Report

CR Number: 2026076147		Implementation Date: 24-Mar-26	Related CR: 2026076147	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Circinate psoriasis</i>	Approved as Requested	Proposed LLT	
To PT	<i>Generalised pustular psoriasis</i>		<i>Circinate psoriasis</i>	10093633 Current
			To PT	
			<i>Generalised pustular psoriasis</i>	10090621
MSSO Comment: The proposal to add a new LLT Circinate psoriasis to PT Generalised pustular psoriasis is approved as requested. Circinate (Lapière) psoriasis represents a rare variety of generalized subacute pustular psoriasis clinically characterized by rapid onset of annular circinate lesions with micro-pustules at the borders without classic plaque psoriasis manifestations.				

CR Number: 2026076148		Implementation Date: 24-Mar-26	Related CR: 2026076148	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Episodic angioedema with eosinophilia</i>	Approved as Requested	Proposed LLT	
To PT	<i>Gleich's syndrome</i>		<i>Episodic angioedema with eosinophilia</i>	10093629
			To PT	
			<i>Gleich's syndrome</i>	10066837 Current
MSSO Comment: The proposal to add a new LLT Episodic angioedema with eosinophilia to PT Gleich's syndrome is approved as requested. Episodic angioedema with eosinophilia (Gleich syndrome) is a rare disorder characterized by episodes of angioedema and eosinophilia that occur at monthly intervals and resolve spontaneously without therapy.				

CR Number: 2026076149		Implementation Date: 24-Mar-26	Related CR: 2026076149	
Change Requested	Swap a PT with an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Gleich's syndrome</i>	Approved as Requested	Proposed PT	
With LLT	<i>Episodic angioedema with eosinophilia</i>		<i>Gleich's syndrome</i>	10066837 Current
			With LLT	
			<i>Episodic angioedema with eosinophilia</i>	10093629
MSSO Comment: The proposal to swap the PT Gleich's syndrome with the LLT Episodic angioedema with eosinophilia is approved as requested. New term Episodic angioedema with eosinophilia is better represented at the PT level.				

Detail Report

CR Number: 2026076150		Implementation Date: 24-Mar-26	Related CR: 2026076150	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Cutaneous mucinosis	Approved as Requested	Proposed PT	
HLT primary	Skin and subcutaneous conditions NEC		Cutaneous mucinosis	10093621
SOC primary	Skin and subcutaneous tissue disorders		HLT primary	
			Skin and subcutaneous conditions NEC	10042356
			SOC primary	
			Skin and subcutaneous tissue disorders	10040785

MSSO Comment: The proposal to add a new PT Cutaneous mucinosis to primary HLT Skin and subcutaneous conditions NEC in SOC Skin and subcutaneous tissue disorders is approved as requested. Cutaneous mucinosis is an broad umbrella term for several specific mucinoses affecting the skin. Mucinoses are a diverse group of uncommon skin disorders, all involving accumulation in the skin of abnormal amounts of mucin. This is a jelly-like complex carbohydrate substance, called hyaluronic acid, that occurs normally as part of the connective tissue in the dermis or mid-layer of the skin.

CR Number: 2026076151		Implementation Date: 24-Mar-26	Related CR: 2026076151	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Erythrokeratoderma variabilis progressiva	Approved as Requested	Proposed LLT	
To PT	Erythrokeratoderma variabilis		Erythrokeratoderma variabilis progressiva	10093634
			To PT	
			Erythrokeratoderma variabilis	10049048 Current

MSSO Comment: The proposal to add a new LLT Erythrokeratoderma variabilis progressiva to PT Erythrokeratoderma variabilis is approved as requested. Erythrokeratoderma variabilis progressiva (EKVP) is a type of erythrokeratoderma characterized by the association of hyperkeratosis and erythema in persistent, although sometimes variable, circumscribed lesions. Progressive symmetric erythrokeratoderma and erythrokeratoderma variabilis are probably no longer two distinctive diseases but rather the two clinical manifestations of a same disease, now known as EKVP.

CR Number: 2026076152		Implementation Date: 24-Mar-26	Related CR: 2026076152	
Change Requested	Swap a PT with an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Erythrokeratoderma variabilis	Approved as Requested	Proposed PT	
With LLT	Erythrokeratoderma variabilis progressiva		Erythrokeratoderma variabilis	10049048 Current
			With LLT	
			Erythrokeratoderma variabilis progressiva	10093634

MSSO Comment: The proposal to swap the PT Erythrokeratoderma variabilis with the LLT Erythrokeratoderma variabilis progressiva is approved as requested. New term Erythrokeratoderma variabilis progressiva is better represented at the PT level.

Detail Report

CR Number: 2026076153		Implementation Date: 24-Mar-26	Related CR: 2026076153	
<u>Change Requested</u>	Link a PT to a HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Trichothiodystrophy</i>	Approved as Requested	Proposed PT	
To HLT	<i>Hair and nail disorders congenital</i>		<i>Trichothiodystrophy</i>	10044628
			To HLT	
			<i>Hair and nail disorders congenital</i>	10072983

MSSO Comment: The proposal to link the PT Trichothiodystrophy to the HLT Hair and nail disorders congenital is approved as requested. Trichothiodystrophy is a rare, genetic, syndromic hair shaft abnormality disorder characterized by short, dry, sulfur-deficient, brittle hair usually associated with highly variable neuroectodermal manifestations, such as ichthyosis, photosensitivity, and intellectual disability. In a related change, the primary SOC for PT Trichothiodystrophy will be reassigned from SOC Skin and subcutaneous tissue disorders to SOC Congenital, familial and genetic disorders.

CR Number: 2026083004		Implementation Date: 24-Mar-26	Related CR: 2026076153	
<u>Change Requested</u>	Reassign Primary SOC	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Trichothiodystrophy</i>	Approved as Requested	Proposed PT	
Old Primary SOC	<i>Skin and subcutaneous tissue disorders</i>		<i>Trichothiodystrophy</i>	10044628
New Primary SOC	<i>Congenital, familial and genetic disorders</i>		Old Primary SOC	
			<i>Skin and subcutaneous tissue disorders</i>	10040785
			New Primary SOC	
			<i>Congenital, familial and genetic disorders</i>	10010331

MSSO Comment:

Detail Report

CR Number: 2026076154		Implementation Date: 24-Mar-26	Related CR: 2026076154	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Hereditary hypotrichosis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Hair and nail disorders congenital</i>		<i>Hereditary hypotrichosis</i>	10093617
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary	
			<i>Hair and nail disorders congenital</i>	10072983
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Alopecias</i>	10001769
			SOC secondary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new PT Hereditary hypotrichosis to primary HLT Hair and nail disorders congenital in SOC Congenital, familial and genetic disorders and secondary HLT Alopecias in SOC Skin and subcutaneous tissue disorders is approved as requested. Hereditary hypotrichosis is a group of rare genetic conditions where individuals are born with sparse hair, or hair that becomes sparse early in life, due to hair follicle defects, affecting scalp, eyebrows, eyelashes, or body hair.

CR Number: 2026076155		Implementation Date: 24-Mar-26	Related CR: 2026076155	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Watson syndrome</i>	Approved as Requested	Proposed LLT	
To PT	<i>Neurofibromatosis</i>		<i>Watson syndrome</i>	10093630 Current
			To PT	
			<i>Neurofibromatosis</i>	10029268

MSSO Comment: The proposal to add a new LLT Watson syndrome to PT Neurofibromatosis is approved as requested. Watson syndrome is an autosomal dominant disorder characterized by pulmonary valvular stenosis, cafe-au-lait spots, decreased intellectual ability, and short stature. It is thought that Watson syndrome is a variant of type 1 neurofibromatosis. Most affected individuals have relative macrocephaly and Lisch nodules and about one-third of those affected have neurofibromas.

Detail Report

CR Number: 2026076156		Implementation Date: 24-Mar-26	Related CR: 2026076156	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Congenital hypomelanosis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Skin and subcutaneous tissue disorders congenital NEC</i>		<i>Congenital hypomelanosis</i>	10093612
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary	
			<i>Skin and subcutaneous tissue disorders congenital NEC</i>	10040834
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Hypopigmentation disorders</i>	10021064
			SOC secondary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new PT Congenital hypomelanosis to primary HLT Skin and subcutaneous tissue disorders congenital NEC in SOC Congenital, familial and genetic disorders and secondary HLT Hypopigmentation disorders in SOC Skin and subcutaneous tissue disorders is approved as requested. Genetic hypomelanosis includes a wide range of congenital and inherited disorders, whereby the skin changes indicate an underlying disorder or syndrome.

CR Number: 2026076157		Implementation Date: 24-Mar-26	Related CR: 2026076157	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Genital melanosis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Reproductive tract disorders NEC (excl neoplasms)</i>		<i>Genital melanosis</i>	10093620
SOC primary	<i>Reproductive system and breast disorders</i>		HLT primary	
			<i>Reproductive tract disorders NEC (excl neoplasms)</i>	10027696
			SOC primary	
			<i>Reproductive system and breast disorders</i>	10038604
			HLT secondary	
			<i>Hyperpigmentation disorders</i>	10020713
			SOC secondary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new PT Genital melanosis to primary HLT Reproductive tract disorders NEC (excl neoplasms) in SOC Reproductive system and breast disorders and secondary HLT Hyperpigmentation disorders in SOC Skin and subcutaneous tissue disorders is approved as requested to provide a broad PT for melanosis/hyperpigmentation of the genital tract. Patients with genital melanosis, and in particular those with any level of histologic atypia in the genital melanosis lesion, may require careful total body skin examinations for the possibility of melanoma in any body site.

Detail Report

CR Number: 2026076158		Implementation Date: 24-Mar-26	Related CR: 2026076158	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Trichoptilosis</i>	Approved as Requested	Proposed LLT	
To PT	<i>Trichorrhexis</i>		<i>Trichoptilosis</i>	10093631 Current
			To PT	
			<i>Trichorrhexis</i>	10044625

MSSO Comment: The proposal to add a new LLT Trichoptilosis to PT Trichorrhexis is approved as requested. Trichoptilosis is a type of split ends, where the ends of the hair branch out multiple times, resembling branches of a tree.

CR Number: 2026076159		Implementation Date: 24-Mar-26	Related CR: 2026076159	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Follicular occlusion syndrome</i>	Approved as Requested	Proposed LLT	
To PT	<i>Follicular occlusion triad</i>		<i>Follicular occlusion syndrome</i>	10093622
			To PT	
			<i>Follicular occlusion triad</i>	10091897 Current

MSSO Comment: The proposal to add a new LLT Follicular occlusion syndrome to PT Follicular occlusion triad is approved as requested. Follicular occlusion syndrome refers to a group of diseases in which hair follicles become blocked with keratin (scale) and then rupture, resulting in inflammatory skin disease. Three of these diseases are together known as the follicular occlusion triad. Hidradenitis suppurativa (acne inversa), acne conglobata (severe nodulocystic acne), dissecting cellulitis (perifolliculitis capitis abscedens et suffodiens). A fourth condition, pilonidal sinus or pilonidal disease, makes up the follicular occlusion tetrad. In a related change, this newly added broader LLT Follicular occlusion syndrome will be swapped with existing PT Follicular occlusion triad, to have the broader encompassing concept represented at the PT level.

CR Number: 2026083005		Implementation Date: 24-Mar-26	Related CR: 2026076159	
Change Requested	Swap a PT with an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Follicular occlusion triad</i>	Approved as Requested	Proposed PT	
With LLT	<i>Follicular occlusion syndrome</i>		<i>Follicular occlusion triad</i>	10091897 Current
			With LLT	
			<i>Follicular occlusion syndrome</i>	10093622

MSSO Comment:

Detail Report

CR Number: 2026076160		Implementation Date: 24-Mar-26	Related CR: 2026076160	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Follicular occlusion tetrad</i>	Approved as Requested	Proposed LLT	10093636 Current
To PT	<i>Follicular occlusion syndrome</i>		<i>Follicular occlusion tetrad</i>	
			To PT	10093622
		<i>Follicular occlusion syndrome</i>		
MSSO Comment:	The proposal to add a new LLT Follicular occlusion tetrad to PT Follicular occlusion syndrome is approved as requested. Follicular occlusion syndrome refers to a group of diseases in which hair follicles become blocked with keratin (scale) and then rupture, resulting in inflammatory skin disease. Three of these diseases are together known as the follicular occlusion triad. Hidradenitis suppurativa (acne inversa), acne conglobata (severe nodulocystic acne), dissecting cellulitis (perifolliculitis capitis abscedens et suffodiens). A fourth condition, pilonidal sinus or pilonidal disease, makes up the follicular occlusion tetrad.			

CR Number: 2026076161		Implementation Date: 24-Mar-26	Related CR: 2026076161	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Phymatous rosacea</i>	Approved as Requested	Proposed LLT	10093625
To PT	<i>Rhinophyma</i>		<i>Phymatous rosacea</i>	
			To PT	10039098 Current
		<i>Rhinophyma</i>		
MSSO Comment:	The proposal to add a new LLT Phymatous rosacea to PT Rhinophyma is approved as requested. Phymatous rosacea is a subtype of rosacea characterized by thickened skin and enlargement of the nose, often referred to as rhinophyma. Phymatous rosacea can also affect the following sites: chin (gnathophyma), ears (otophyma), eyelids (blepharophyma) and forehead (metophyma).			

CR Number: 2026076162		Implementation Date: 24-Mar-26	Related CR: 2026076162	
<u>Change Requested</u>	Swap a PT with an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Rhinophyma</i>	Approved as Requested	Proposed PT	10039098 Current
With LLT	<i>Phymatous rosacea</i>		<i>Rhinophyma</i>	
			With LLT	<i>Phymatous rosacea</i>
MSSO Comment:	The proposal to swap the PT Rhinophyma with the LLT Phymatous rosacea is approved as requested. New term Phymatous rosacea, added in change request 2026076161 is better represented at the PT level.			

Detail Report

CR Number: 2026076163		Implementation Date: 24-Mar-26	Related CR: 2026076163	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Acute oedema/cutaneous distension syndrome</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Oedema NEC</i>		<i>Acute oedema/cutaneous distension syndrome</i>	10093618
SOC primary	<i>General disorders and administration site conditions</i>		HLT primary	
			<i>Oedema NEC</i>	10030113
			SOC primary	
			<i>General disorders and administration site conditions</i>	10018065
			HLT secondary	
			<i>Skin and subcutaneous conditions NEC</i>	10042356
			SOC secondary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new PT Acute oedema/cutaneous distension syndrome to primary HLT Oedema NEC in SOC General disorders and administration site conditions and secondary HLT Skin and subcutaneous conditions NEC in SOC Skin and subcutaneous tissue disorders is approved as requested. Acute oedema/cutaneous distension syndrome is a common sequela of acute oedema, especially of the lower extremities. It is a dermatological condition characterized by rapid, severe skin swelling (edema) leading to epidermal stretching, often causing "crazy-paving" skin (eczema craquelé), striae-like marks, and sometimes blisters, typically resolving as the underlying edema subsides, seen in severe malnutrition (refeeding syndrome), heart failure, or kidney issues like nephrotic syndrome, where rapid fluid shifts stress the skin.

CR Number: 2026076164		Implementation Date: 24-Mar-26	Related CR: 2026076164	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Acute edema/cutaneous distension syndrome</i>	Approved as Requested	Proposed LLT	
To PT	<i>Acute oedema/cutaneous distension syndrome</i>		<i>Acute edema/cutaneous distension syndrome</i>	10093626 Current
			To PT	
			<i>Acute oedema/cutaneous distension syndrome</i>	10093618

MSSO Comment: The proposal to add a new LLT Acute edema/cutaneous distension syndrome to PT Acute oedema/cutaneous distension syndrome is approved as requested. LLT Acute edema/cutaneous distension syndrome is the American spelling counterpart to new PT Acute oedema/cutaneous distension syndrome added in change request 2026076163.

Detail Report

CR Number: 2026076165		Implementation Date: 24-Mar-26	Related CR: 2026076165	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Connective tissue disorder	Approved as Requested	Proposed PT	
To HLT	Connective tissue disorders		Connective tissue disorder	10061087
			To HLT	
			Connective tissue disorders	10010760

MSSO Comment: The proposal to link the PT Connective tissue disorder to the HLT Connective tissue disorders is approved as requested. Adding the link to HLT Connective tissue disorders of PT Connective tissue disorder warrants the representation as well in SOC Skin and subcutaneous tissue disorders. In a related change, PT Congenital connective tissue disorder will also be linked to HLT Connective tissue disorders.

CR Number: 2026083007		Implementation Date: 24-Mar-26	Related CR: 2026076165	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Congenital connective tissue disorder	Approved as Requested	Proposed PT	
To HLT	Connective tissue disorders		Congenital connective tissue disorder	10086687
			To HLT	
			Connective tissue disorders	10010760

MSSO Comment:

Detail Report

CR Number: 2026076166		Implementation Date: 24-Mar-26	Related CR: 2026076166		
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Familial reactive perforating collagenosis</i>	Approved as Requested	Proposed PT		
HLT primary	<i>Skin and subcutaneous tissue disorders congenital NEC</i>		<i>Familial reactive perforating collagenosis</i>	10093613	
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary		
			<i>Skin and subcutaneous tissue disorders congenital NEC</i>	10040834	
			SOC primary		
			<i>Congenital, familial and genetic disorders</i>	10010331	
			HLT secondary		
		<i>Connective tissue disorders</i>	10010760		
		SOC secondary			
			<i>Skin and subcutaneous tissue disorders</i>	10040785	

MSSO Comment: The proposal to add a new PT Familial reactive perforating collagenosis to primary HLT Skin and subcutaneous tissue disorders congenital NEC in SOC Congenital, familial and genetic disorders and secondary HLT Connective tissue disorders in SOC Skin and subcutaneous tissue disorders is approved as requested. Reactive perforating collagenosis manifests as a rare primary perforating dermatosis, showcasing distinct transepidermal elimination of modified collagen across the epidermis. This condition typically appears in 2 distinct forms: a seldom-seen hereditary variant emerging in childhood and an acquired variant, more prevalent among adults with diabetes mellitus or end-stage renal disease.

CR Number: 2026076167		Implementation Date: 24-Mar-26	Related CR: 2026076167	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Reactive perforating collagenosis</i>	Approved as Requested	PT to merge	
To PT	<i>Acquired perforating dermatosis</i>		<i>Reactive perforating collagenosis</i>	10067155 Current
			To PT	
			<i>Acquired perforating dermatosis</i>	10082163

MSSO Comment: The proposal to demote the PT Reactive perforating collagenosis under PT Acquired perforating dermatosis is approved as requested. Acquired perforating dermatosis is a broad umbrella term that encompasses several acquired perforating disorders, typically seen in adults with systemic conditions such as diabetes mellitus or chronic renal failure. Within this spectrum, reactive perforating collagenosis represents a specific pattern characterized by transepidermal elimination of collagen. In a related change, PT Acquired perforating dermatosis will be moved from HLT Dermal and epidermal conditions NEC to HLT Connective tissue disorders in SOC Skin and subcutaneous tissue disorders.

Detail Report

CR Number: 2026083006		Implementation Date: 24-Mar-26	Related CR: 2026076167	
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Acquired perforating dermatosis</i>	Approved as Requested	Proposed PT	
From HLT	<i>Dermal and epidermal conditions NEC</i>		<i>Acquired perforating dermatosis</i>	10082163
To HLT	<i>Connective tissue disorders</i>		From HLT	
			<i>Dermal and epidermal conditions NEC</i>	10012424
			To HLT	
			<i>Connective tissue disorders</i>	10010760

MSSO
Comment:

Detail Report

CR Number: 2026076168		Implementation Date: 24-Mar-26	Related CR: 2026076168	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Hereditary progressive mucinous histiocytosis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Skin and subcutaneous tissue disorders congenital NEC</i>		<i>Hereditary progressive mucinous histiocytosis</i>	10093614
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary	
			<i>Skin and subcutaneous tissue disorders congenital NEC</i>	10040834
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Histiocytoses</i>	10020118
			SOC secondary	
			<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary	
			<i>Histiocytoses</i>	10020118
			SOC secondary	
			<i>Blood and lymphatic system disorders</i>	10005329

MSSO Comment: The proposal to add a new PT Hereditary progressive mucinous histiocytosis to primary HLT Skin and subcutaneous tissue disorders congenital NEC in SOC Congenital, familial and genetic disorders and secondary HLT Histiocytoses which is biaxial to SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps) and SOC Blood and lymphatic disorders is approved as requested. Hereditary progressive mucinous histiocytosis is a rare, benign, non-Langerhans cell histiocytosis characterized by childhood or adolescence onset of multiple, small, asymptomatic, slowly progressing, skin-colored to red-brown papules with predilection for the face, dorsal hands, forearms and legs, without associated mucosal or visceral involvement. An autosomal dominant or X-linked hereditary disease described on the skin, it has been found almost exclusively in women.

Detail Report

CR Number: 2026076169		Implementation Date: 24-Mar-26	Related CR: 2026076169	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Cold panniculitis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Panniculitides</i>		<i>Cold panniculitis</i>	10093615
SOC primary	<i>Skin and subcutaneous tissue disorders</i>		HLT primary	
			<i>Panniculitides</i>	10033674
			SOC primary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785
			HLT secondary	
			<i>Conditions caused by cold</i>	10010269
			SOC secondary	
			<i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary	
			<i>Peripheral vasoconstriction, necrosis and vascular insufficiency</i>	10034640
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO Comment: The proposal to add a new PT Cold panniculitis to primary HLT Panniculitides in SOC Skin and subcutaneous tissue disorders and secondary HLT Conditions caused by cold in SOC Injury, poisoning and procedural complications is approved as requested. Cold panniculitis represents acute cold injury resulting in inflammation of the subcutaneous fat. It manifests as erythematous, indurated nodules, and plaques on exposed skin, especially the perioral areas and cheeks, appearing 24 to 72 hours after exposure to cold. PT Cold panniculitis will also be added to secondary HLT Peripheral vasoconstriction, necrosis and vascular insufficiency.

CR Number: 2026076170		Implementation Date: 24-Mar-26	Related CR: 2026076170	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Stomatitis necrotising</i>	Approved as Requested	Proposed PT	
To HLT	<i>Bacterial infections NEC</i>		<i>Stomatitis necrotising</i>	10042135
			To HLT	
			<i>Bacterial infections NEC</i>	10004047

MSSO Comment: The proposal to link the PT Stomatitis necrotising to the HLT Bacterial infections NEC is approved as requested. Necrotising stomatitis is a fulminating anaerobic polybacterial infection affecting predominantly the oral mucosa of debilitated malnourished children or immunosuppressed HIV seropositive subjects. It starts as necrotising gingivitis which progresses to necrotising periodontitis and subsequently to necrotising stomatitis.

Detail Report

CR Number: 2026076171		Implementation Date: 24-Mar-26	Related CR: 2026076171	
<u>Change Requested</u>	Reassign Primary SOC	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Stomatitis necrotising</i>	Approved as Requested	Proposed PT	
Old Primary SOC	<i>Gastrointestinal disorders</i>		<i>Stomatitis necrotising</i>	10042135
New Primary SOC	<i>Infections and infestations</i>		Old Primary SOC	
			<i>Gastrointestinal disorders</i>	10017947
			New Primary SOC	
			<i>Infections and infestations</i>	10021881
MSSO Comment: The proposal to reassign the primary SOC of PT Stomatitis necrotising from current SOC Gastrointestinal disorders to SOC Infections and infestations is approved as requested based upon MedDRA primary SOC allocation rules.				

CR Number: 2026076172		Implementation Date: 24-Mar-26	Related CR: 2026076172	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Noma neonatorum</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Bacterial infections NEC</i>		<i>Noma neonatorum</i>	10093619
SOC primary	<i>Infections and infestations</i>		HLT primary	
			<i>Bacterial infections NEC</i>	10004047
			SOC primary	
			<i>Infections and infestations</i>	10021881
			HLT secondary	
			<i>Stomatitis and ulceration</i>	10042129
			SOC secondary	
			<i>Gastrointestinal disorders</i>	10017947
			HLT secondary	
			<i>Neonatal infections (excl congenital infections)</i>	10028949
			SOC secondary	
			<i>Pregnancy, puerperium and perinatal conditions</i>	10036585

MSSO Comment: The proposal to add a new PT Noma neonatorum to primary HLT Bacterial infections NEC in SOC Infections and infestations and secondary HLT Stomatitis and ulceration in SOC Gastrointestinal disorders is approved as requested. Noma neonatorum is a rare gangrenous form of noma, which during its course, causes progressive and mutilating destruction of the soft tissues and the bone. It occurs at birth or during the neonatal period and is characterised by a gangrenous process involving mucocutaneous junctions of oral, nasal and anal areas and occasionally, the eyelids and scrotum. PT Noma neonatorum will also be added to HLT Neonatal infections (excl congenital infections).

Detail Report

CR Number: 2026076173		Implementation Date: 24-Mar-26	Related CR: 2026076173	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Athlete's nodules</i>	Approved as Requested	Proposed LLT	10093635 Current
To PT	<i>Skin mass</i>		<i>Athlete's nodules</i>	
			To PT	10067868
			<i>Skin mass</i>	

MSSO Comment: The proposal to add a new LLT Athlete's nodules to PT Skin mass is approved as requested. Prayer nodules (or athlete's nodules or surfer's knots) are hyperplastic, flesh-colored dermal masses that arise after repetitive trauma to an area, commonly occurring with prayer rituals or sports-related activities such as boxing.

CR Number: 2026076174		Implementation Date: 24-Mar-26	Related CR: 2026076174	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Occupational exposure to allergens</i>	Approved Not as Requested	Proposed PT	10093616
HLT primary	<i>Occupational exposures</i>		<i>Occupational exposure to allergen</i>	
SOC primary	<i>Injury, poisoning and procedural complications</i>		HLT primary	10073310
			<i>Occupational exposures</i>	
			SOC primary	10022117
			<i>Injury, poisoning and procedural complications</i>	

MSSO Comment: The proposal to add a new PT Occupational exposure to allergens to primary HLT Occupational exposures in SOC Injury, poisoning and procedural complications is approved but not as requested. Occupational allergens are those allergens to which exposure at work may occur. Well-known examples of occupational allergens are: natural substances such as flour dust, animal skin or rubber latex; metals such as nickel and cobalt; preserving agents in cosmetics, paints, adhesives etc. The proposed term will be added in the singular form as PT Occupational exposure to allergen to HLT Occupational exposures.

CR Number: 2026076207		Implementation Date: 24-Mar-26	Related CR: 2026076207	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>COPD worsening</i>	Rejected		

MSSO Comment: The proposal to add a new term COPD worsening is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept, as described in the MedDRA Introductory Guide section 4.10 - RULES AND CONVENTIONS ADOPTED IN THE TERMINOLOGY: AGGRAVATION OF UNDERLYING CONDITIONS.

Detail Report

CR Number: 2026076208	Implementation Date: 24-Mar-26	Related CR: 2026076208	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Emphysema worsening</i>	Rejected	

MSSO Comment: The proposal to add a new term Emphysema worsening is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept, as described in the MedDRA Introductory Guide section 4.10 - RULES AND CONVENTIONS ADOPTED IN THE TERMINOLOGY: AGGRAVATION OF UNDERLYING CONDITIONS.

CR Number: 2026077000	Implementation Date: 24-Mar-26	Related CR: 2026077000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Sternal closure</i>	Approved as Requested	Proposed PT <i>Sternal closure</i> 10093607 HLT primary <i>Chest wall and mediastinal therapeutic procedures</i> 10008494 SOC primary <i>Surgical and medical procedures</i> 10042613

MSSO Comment: The proposal to add a new term Sternal closure is approved as requested. Sternal closure is the standard step at the end of open-heart surgery where the sternum is brought back together and stabilized. Sternal closure will be added as PT to HLT Chest wall and mediastinal therapeutic procedures. In a related change, PT Sternal wiring will be demoted to new PT Sternal closure.

CR Number: 2026083003	Implementation Date: 24-Mar-26	Related CR: 2026077000	MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement
PT to merge	<i>Sternal wiring</i>	Approved as Requested	PT to merge <i>Sternal wiring</i> 10042016 Current
To PT	<i>Sternal closure</i>		To PT <i>Sternal closure</i> 10093607

MSSO Comment:

Detail Report

CR Number: 2026077001		Implementation Date: 24-Mar-26	Related CR: 2026077001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Delayed sternal closure</i>	Approved as Requested	Proposed LLT <i>Delayed sternal closure</i>	10093609 Current
			To PT <i>Sternal closure</i>	10093607
MSSO Comment: The proposal to add a new term Delayed sternal closure is approved as requested. Delayed sternal closure is a specialized technique where the chest is left open following complex cardiac surgery to manage severe myocardial edema, hemodynamic instability, or coagulopathy. Delayed sternal closure will be added as an LLT to PT Sternal closure which was added in change request 2026077000.				

CR Number: 2026077002		Implementation Date: 24-Mar-26	Related CR: 2026077002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Sano shunt</i>	Approved Not as Requested	Proposed LLT <i>Sano shunt procedure</i>	10093608 Current
			To PT <i>Norwood procedure</i>	10068098
MSSO Comment: The proposal to add a new term Sano shunt is approved but not as requested. A Sano shunt (or Sano conduit) is a surgically placed tube connecting the right ventricle to the pulmonary arteries during the Norwood procedure to provide controlled pulmonary blood flow in infants with hypoplastic left heart syndrome. The proposed term will be modified for clarity and added as LLT Sano shunt procedure to PT Norwood procedure.				

CR Number: 2026077003		Implementation Date: 24-Mar-26	Related CR: 2026077003	
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Lymphomatoid papulosis</i>	Approved as Requested	Proposed PT <i>Lymphomatoid papulosis</i>	10056670
Old Primary SOC	<i>Skin and subcutaneous tissue disorders</i>		Old Primary SOC <i>Skin and subcutaneous tissue disorders</i>	10040785
New Primary SOC	<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>		New Primary SOC <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104

MSSO Comment: The proposal to reassign the primary SOC of PT Lymphomatoid papulosis from current SOC Skin and subcutaneous tissue disorders to SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps) is approved as requested. Lymphomatoid papulosis belongs to a family of conditions called primary cutaneous CD30-positive lymphoproliferative disorders (pcCD30+LPD). Lymphomatoid papulosis is usually classified as non-malignant or may be a precursor to Cutaneous T-cell Lymphoma, as it can present in association with malignant pcCD30+LPD. In a related change, PT Lymphomatoid papulosis will be added to bi-axial HLT Lymphoproliferative disorders NEC (excl leukaemias and lymphomas). After all changes have been applied, PT Lymphomatoid papulosis will be linked to primary SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps), to secondary SOC Blood and lymphatic system disorders and to secondary SOC Skin and subcutaneous tissue disorders.

Detail Report

CR Number: 2026083002		Implementation Date: 24-Mar-26	Related CR: 2026077003	
<u>Change Requested</u>	Link a PT to a HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Lymphomatoid papulosis</i>	Approved as Requested	Proposed PT	
To HLT	<i>Lymphoproliferative disorders NEC (excl leukaemias and lymphomas)</i>		<i>Lymphomatoid papulosis</i>	10056670
			To HLT	
			<i>Lymphoproliferative disorders NEC (excl leukaemias and lymphomas)</i>	10025353

MSSO
Comment:

CR Number: 2026077004		Implementation Date: 24-Mar-26	Related CR: 2026077004	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Type 1 diabetes mellitus aggravated</i>	Rejected		
To PT	<i>Type 1 diabetes mellitus</i>			

MSSO
Comment: The proposal to add a new LLT Type 1 diabetes mellitus aggravated to PT Type 1 diabetes mellitus is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept, as described in the MedDRA Introductory Guide section 4.10 - RULES AND CONVENTIONS ADOPTED IN THE TERMINOLOGY: AGGRAVATION OF UNDERLYING CONDITIONS. Refer to 3.9 – Modification of Pre-existing Conditions of the MedDRA® TERM SELECTION: POINTS TO CONSIDER document for coding guidance.

CR Number: 2026077005		Implementation Date: 24-Mar-26	Related CR: 2026077005	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Type 2 diabetes mellitus aggravated</i>	Rejected		
To PT	<i>Type 2 diabetes mellitus</i>			

MSSO
Comment: The proposal to add a new LLT Type 2 diabetes mellitus aggravated to PT Type 2 diabetes mellitus is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept, as described in the MedDRA Introductory Guide section 4.10 - RULES AND CONVENTIONS ADOPTED IN THE TERMINOLOGY: AGGRAVATION OF UNDERLYING CONDITIONS. Refer to 3.9 – Modification of Pre-existing Conditions of the MedDRA® TERM SELECTION: POINTS TO CONSIDER document for coding guidance.

Detail Report

CR Number: 2026078000		Implementation Date: 24-Mar-26	Related CR: 2026078000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Protein contact dermatitis</i>	Approved as Requested	Proposed PT <i>Protein contact dermatitis</i>	10093610
			HLT primary <i>Dermatitis and eczema</i>	10012435
			SOC primary <i>Skin and subcutaneous tissue disorders</i>	10040785
			HLT secondary <i>Allergies to foods, food additives, drugs and other chemicals</i>	10001737
			SOC secondary <i>Immune system disorders</i>	10021428

MSSO Comment: The proposal to add a new term Protein contact dermatitis is approved as requested. Protein contact dermatitis is a chronic eczema-like condition following repeated immediate (urticarial) reactions to proteins. Protein contact dermatitis is a dermatitis induced by a type I immediate hypersensitivity reaction to high molecular weight allergens, such as animal or plant proteins in previously sensitized individuals. It predominantly occurs in food handlers, cooks, veterinarians, or farmers. Protein contact dermatitis will be added as PT to primary HLT Dermatitis and eczema and to secondary HLT Allergies to foods, food additives, drugs and other chemicals.

CR Number: 2026078001		Implementation Date: 24-Mar-26	Related CR: 2026078001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Drug induced conjunctivitis</i>	Rejected		

MSSO Comment: The proposal to add a new term Drug induced conjunctivitis is not approved. The MSSO considers this proposed term to be of a "due to" nature. Terms that state or imply a "due to" concept are generally not added to MedDRA because causality is not inherent in the coded term but is instead determined by the reporter/investigator or by the sponsor.

Detail Report

CR Number: 2026078002		Implementation Date: 26-Mar-26	Related CR: 2026078002		
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Uterine transposition procedure</i>	Rejected			
To SMQ	<i>Fertility disorders (SMQ)</i>				
Term scope	<i>Broad</i>				
MSSO Comment:	The proposal to add PT Uterine transposition procedure to Fertility disorders (SMQ) as a broad term is not approved. All PTs in this SMQ relate or directly indicate male or female infertility. Uterine transposition surgery is done to prevent infertility and is not a procedure that relates to infertility.				

CR Number: 2026078004		Implementation Date: 06-Apr-26	Related CR: 2026078004		
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Electrocardiogram epsilon wave present</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Arrhythmia related investigations, signs and symptoms (SMQ)</i>		<i>Electrocardiogram epsilon wave present</i>	10093458	
Term scope	<i>Narrow</i>		To SMQ		
			<i>Arrhythmia related investigations, signs and symptoms (SMQ)</i>	20000051	
			Term scope		
			<i>Narrow</i>		
			Term_category		
			<i>A</i>		
			Term_weight		
			<i>0</i>		

MSSO Comment: The proposal to add PT Electrocardiogram epsilon wave present to Arrhythmia related investigations, signs and symptoms (SMQ) as a Narrow term is approved as requested. The epsilon wave is a marker of delayed activation of the right ventricular free wall and outflow tract and is considered a major diagnostic criterion for arrhythmogenic right ventricular dysplasia. PT Electrocardiogram epsilon wave present will be added to the narrow scope of Arrhythmia related investigations, signs and symptoms (SMQ).

Detail Report

CR Number: 2026078005		Implementation Date: 06-Apr-26	Related CR: 2026078005	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Low-grade fibromyxoid sarcoma</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Non-haematological malignant tumours (SMQ)</i>		<i>Low-grade fibromyxoid sarcoma</i>	10093410
Term scope	<i>Narrow</i>		To SMQ	
			<i>Non-haematological malignant tumours (SMQ)</i>	20000228
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Low-grade fibromyxoid sarcoma to Non-haematological malignant tumours (SMQ) as a Narrow term is approved as requested. Low-grade fibromyxoid sarcoma is a malignant fibroblastic neoplasm characterized by alternating collagenous and myxoid areas, deceptively bland spindle cells with a whorling growth pattern, and arcades of small blood vessels. These tumours consistently have either FUS-CREB3L2 or FUS-CREB3L1 gene fusions.

CR Number: 2026078006		Implementation Date: 06-Apr-26	Related CR: 2026078006	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Iridocyclectomy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Malignancy related therapeutic and diagnostic procedures (SMQ)</i>		<i>Iridocyclectomy</i>	10093432
Term scope	<i>Narrow</i>		To SMQ	
			<i>Malignancy related therapeutic and diagnostic procedures (SMQ)</i>	20000093
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Iridocyclectomy to Malignancy related therapeutic and diagnostic procedures (SMQ) as a Narrow term is approved as requested. Iridocyclectomy is a surgical procedure that involves the removal of a portion of the iris and the ciliary body. It is typically performed primarily for managing tumors of the anterior uveal tract, such as small melanomas of the iris or ciliary body, or occasionally for other localized anterior segment lesions.

Detail Report

CR Number: 2026078007		Implementation Date: 06-Apr-26	Related CR: 2026078007		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	Acquired vascular malformation cerebral	Approved as Requested	Proposed PT		
To SMQ	Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)		Acquired vascular malformation cerebral		10093503
Term scope	Narrow		To SMQ		
			Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)		20000166
			Term scope		
			Narrow		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Acquired vascular malformation cerebral to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Narrow term is approved as requested. Cerebral arteriovenous malformations (AVMs) are rare, complex vascular lesions of uncertain pathogenesis involving dysplastic vessel formation. Originally thought to be exclusively congenital, intracerebral AVMs are now known to also arise de novo in genetically predisposed individuals and following surgery or trauma. Because of the potential risk of cerebral hemorrhage associated with such acquired lesions, Acquired vascular malformation cerebral will be added to the narrow search terms of this SMQ.

Detail Report

CR Number: 2026078008		Implementation Date: 06-Apr-26	Related CR: 2026078008	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Acquired vascular malformation cerebral	Approved as Requested	Proposed PT	
To SMQ	Central nervous system vascular disorders, not specified as haemorrhagic or ischaemic (SMQ)		Acquired vascular malformation cerebral	10093503
Term scope	Broad		To SMQ	
			Central nervous system vascular disorders, not specified as haemorrhagic or ischaemic (SMQ)	20000165
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Acquired vascular malformation cerebral to Central nervous system vascular disorders, not specified as haemorrhagic or ischaemic (SMQ) as a Broad term is approved as requested. Cerebral arteriovenous malformations (AVMs) are rare, complex vascular lesions of uncertain pathogenesis involving dysplastic vessel formation. Originally thought to be exclusively congenital, intracerebral AVMs are now known to also arise de novo in genetically predisposed individuals and following surgery or trauma. These acquired vascular lesions have the potential to produce intracerebral complications (related to either mass effect or frank hemorrhage). As not all cases of such lesions are "clearly" related to trauma (an exclusion criteria), Acquired vascular malformation cerebral will be added to the broad search terms of this SMQ.

Detail Report

CR Number: 2026078009		Implementation Date: 06-Apr-26	Related CR: 2026078009	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Juvenile parkinsonism</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Parkinson-like events (SMQ)</i>		<i>Juvenile parkinsonism</i>	10093490
Term scope	<i>Narrow</i>		To SMQ	
			<i>Parkinson-like events (SMQ)</i>	20000099
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Juvenile parkinsonism to Parkinson-like events (SMQ) as a Narrow term is approved as requested. Juvenile parkinsonism is a rare movement disorder, presenting before age 21, which is characterized by the loss of dopamine-producing nerve cells. Often genetically inherited, it causes stiff muscles, tremor, dystonia, and bradykinesia. Treatment includes medication and therapies to manage symptoms. This term, which describes a specific sub-type of Parkinsonism, will be added to the narrow search terms of this SMQ.

CR Number: 2026078010		Implementation Date: 06-Apr-26	Related CR: 2026078010	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Tau protein increased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Dementia (SMQ)</i>		<i>Tau protein increased</i>	10093434
Term scope	<i>Narrow</i>		To SMQ	
			<i>Dementia (SMQ)</i>	20000073
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Tau protein increased to Dementia (SMQ) as a Narrow term is approved as requested. Tau protein is one of the three biomarkers used for the diagnosis of Alzheimer's disease (together with A β 42). The most common practice in clinical centers is measurement in plasma or serum. Tau protein can also be visualized using neuroimaging techniques such as PET. Even normal individuals have measurable tau. Therefore, rather than the threshold-dependent meaning conferred by "positive", PT Tau protein increased will be added to the narrow search terms of this SMQ.

Detail Report

CR Number: 2026078011		Implementation Date: 06-Apr-26	Related CR: 2026078011	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>ReNU syndrome</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Convulsions (SMQ)</i>		<i>ReNU syndrome</i>	10093464
Term scope	<i>Narrow</i>		To SMQ	
			<i>Convulsions (SMQ)</i>	20000079
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT ReNU syndrome to Convulsions (SMQ) as a Narrow term is approved as requested. ReNU syndrome is characterized by hypotonia, global developmental delay, intellectual disability with poor or absent speech, delayed motor development, feeding difficulties, short stature, seizures, and dysmorphic features. Epilepsy is also present in 60% of patients with this syndrome (with focal consciousness impaired seizures being the most frequent, followed by generalized convulsions). Neuroradiological abnormalities include ventriculomegaly, hypoplasia of the corpus callosum, and decreased white matter volume. Most individuals have the same highly recurrent single base insertion (n.64_65insT) in RNU4-2. Because epilepsy is a common presenting feature of this syndrome, this disorder will be included in the narrow search terms of this SMQ.

CR Number: 2026078012		Implementation Date: 06-Apr-26	Related CR: 2026078012	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Congenital intrahepatic portosystemic shunt</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Congenital intrahepatic portosystemic shunt</i>	10093530
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Congenital intrahepatic portosystemic shunt to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026078013		Implementation Date: 06-Apr-26	Related CR: 2026078013	
Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Double aortic arch	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Double aortic arch	10093479
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Double aortic arch to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026078014		Implementation Date: 06-Apr-26	Related CR: 2026078014	
Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Ectopic liver	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Ectopic liver	10093493
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Ectopic liver to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026078015		Implementation Date: 06-Apr-26	Related CR: 2026078015	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Juvenile parkinsonism</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Juvenile parkinsonism</i>	10093490
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Juvenile parkinsonism to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026078016		Implementation Date: 06-Apr-26	Related CR: 2026078016	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>ReNU syndrome</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>ReNU syndrome</i>	10093464
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT ReNU syndrome to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026078017		Implementation Date: 06-Apr-26	Related CR: 2026078017	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Singleton-Merten syndrome	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Singleton-Merten syndrome	10093453
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Singleton-Merten syndrome to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026078018		Implementation Date: 06-Apr-26	Related CR: 2026078018	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Verheij syndrome	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Verheij syndrome	10093501
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Verheij syndrome to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026078019		Implementation Date: 06-Apr-26	Related CR: 2026078019	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Subvalvular aortic stenosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Subvalvular aortic stenosis</i>	10042431
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Subvalvular aortic stenosis to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026078020		Implementation Date: 06-Apr-26	Related CR: 2026078020	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Contracted gallbladder</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Gallbladder related disorders (SMQ)</i>		<i>Contracted gallbladder</i>	10093425
Term scope	<i>Narrow</i>		To SMQ	
			<i>Gallbladder related disorders (SMQ)</i>	20000124
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Contracted gallbladder to Gallbladder related disorders (SMQ) as a Narrow term is approved as requested. Pathologic gallbladder contraction refers to a persistently small, non-distensible gallbladder due to chronic inflammation, fibrosis, or long-standing cystic duct obstruction, often from gallstones. Unlike normal postprandial contraction, it occurs even in a fasting state and is commonly associated with chronic cholecystitis and impaired gallbladder function.

Detail Report

CR Number: 2026078021		Implementation Date: 06-Apr-26	Related CR: 2026078021	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Bulging duodenal papilla</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Biliary tract disorders (SMQ)</i>		<i>Bulging duodenal papilla</i>	10093460
Term scope	<i>Narrow</i>		To SMQ	
			<i>Biliary tract disorders (SMQ)</i>	20000125
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Bulging duodenal papilla to Biliary tract disorders (SMQ) as a Narrow term is approved as requested. The term *bulging duodenal papilla* describes an endoscopic finding characterized by prominence or swelling of the major duodenal papilla. Although it is a morphological sign rather than an etiological diagnosis, its clinical relevance lies in the fact that it often reflects an obstructive process in the distal segment of the biliary tract or in the ampullary region.

CR Number: 2026078022		Implementation Date: 06-Apr-26	Related CR: 2026078022	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Ectopic liver</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial, neonatal and genetic disorders of the liver (SMQ)</i>		<i>Ectopic liver</i>	10093493
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial, neonatal and genetic disorders of the liver (SMQ)</i>	20000014
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Ectopic liver to Congenital, familial, neonatal and genetic disorders of the liver (SMQ) as a Narrow term is approved as requested. Ectopic liver is a rare, usually asymptomatic, congenital anomaly defined as the presence of mature, autonomous hepatic tissue located outside the normal liver without any direct vascular or biliary connection to it.

Detail Report

CR Number: 2026078023		Implementation Date: 06-Apr-26	Related CR: 2026078023	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Congenital intrahepatic portosystemic shunt</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial, neonatal and genetic disorders of the liver (SMQ)</i>		<i>Congenital intrahepatic portosystemic shunt</i>	10093530
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial, neonatal and genetic disorders of the liver (SMQ)</i>	20000014
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Congenital intrahepatic portosystemic shunt to Congenital, familial, neonatal and genetic disorders of the liver (SMQ) as a Narrow term is approved as requested. A congenital intrahepatic portosystemic shunt is a rare vascular anomaly present at birth in which an abnormal connection exists between branches of the portal vein and the hepatic veins within the liver. This connection allows portal venous blood to bypass the hepatic sinusoids and flow directly into the systemic venous circulation, thus partially or completely avoiding hepatic metabolism and detoxification and leading to complications including hyperammonemia, hepatic encephalopathy, pulmonary vascular abnormalities, or other metabolic disturbances. Some cases remain asymptomatic and may close spontaneously during infancy.

CR Number: 2026078024		Implementation Date: 06-Apr-26	Related CR: 2026078024	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Invasive pulmonary aspergillosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Opportunistic infections (SMQ)</i>		<i>Invasive pulmonary aspergillosis</i>	10093496
Term scope	<i>Narrow</i>		To SMQ	
			<i>Opportunistic infections (SMQ)</i>	20000235
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Invasive pulmonary aspergillosis to Opportunistic infections (SMQ) as a Narrow term is approved as requested. Invasive pulmonary aspergillosis is a severe fungal infection of the lungs caused by *Aspergillus* species, characterized by tissue invasion and typically occurring in immunocompromised individuals. The term will be included in the narrow scope because invasive pulmonary aspergillosis typically occurs in markedly immunocompromised hosts and is a well-recognized opportunistic fungal infection.

Detail Report

CR Number: 2026078025		Implementation Date: 06-Apr-26	Related CR: 2026078025	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Urinary tract fungal bezoar	Approved as Requested	Proposed PT	
To SMQ	Opportunistic infections (SMQ)		Urinary tract fungal bezoar	10093454
Term scope	Broad		To SMQ	
			Opportunistic infections (SMQ)	20000235
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Urinary tract fungal bezoar to Opportunistic infections (SMQ) as a Broad term is approved as requested. Urinary fungal bezoar is generally considered an opportunistic infection. It is most often caused by Candida species and typically occurs in patients with predisposing factors such as diabetes mellitus, urinary catheterization, prolonged antibiotic therapy, extended hospitalization, or immunosuppression. The organism usually colonizes the urine first and then forms aggregates of hyphae, yeast, fibrin, and cellular debris that constitute the bezoar. This term will be added to the broad scope since it describes a morphological structure which includes aggregates of hyphae, yeast, fibrin, and cellular debris creating an obstructive complication of a fungal urinary tract infection, rather than an infection itself.

CR Number: 2026078026		Implementation Date: 06-Apr-26	Related CR: 2026078026	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Laryngeal tuberculosis	Approved as Requested	Proposed PT	
To SMQ	Opportunistic infections (SMQ)		Laryngeal tuberculosis	10093438
Term scope	Narrow		To SMQ	
			Opportunistic infections (SMQ)	20000235
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Laryngeal tuberculosis to Opportunistic infections (SMQ) as a Narrow term is approved as requested. Although laryngeal tuberculosis is not classically defined as an opportunistic infection, it shows increased frequency and severity in immunocompromised individuals, particularly those with advanced HIV infection, malignancy, or receiving immunosuppressive therapies, which show a higher risk of extrapulmonary and disseminated forms of tuberculosis. Therefore, even though laryngeal tuberculosis itself can occur in immunocompetent hosts, the increased susceptibility and more severe clinical expression observed in immunocompromised populations support the inclusion of this term in the narrow scope of the SMQ.

Detail Report

CR Number: 2026078027		Implementation Date: 06-Apr-26	Related CR: 2026078027	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Posterior uveitis	Approved as Requested	Proposed PT	
To SMQ	Immune-mediated/autoimmune disorders (SMQ)		Posterior uveitis	10036370
Term scope	Broad		To SMQ	
			Immune-mediated/autoimmune disorders (SMQ)	20000236
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Posterior uveitis to Immune-mediated/autoimmune disorders (SMQ) as a Broad term is approved as requested. Although posterior uveitis is more frequently caused by infectious etiologies, a substantial proportion of cases—estimated at around 30%—are associated with immune-mediated or autoimmune mechanisms, including conditions such as Behçet disease, sarcoidosis, and other systemic inflammatory disorders.

CR Number: 2026078028		Implementation Date: 06-Apr-26	Related CR: 2026078028	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Anterior uveitis	Approved as Requested	Proposed PT	
To SMQ	Immune-mediated/autoimmune disorders (SMQ)		Anterior uveitis	10002709
Term scope	Broad		To SMQ	
			Immune-mediated/autoimmune disorders (SMQ)	20000236
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Anterior uveitis to Immune-mediated/autoimmune disorders (SMQ) as a Broad term is approved as requested. Anterior uveitis is appropriate for inclusion in the SMQ because it has a well-established association with systemic immune dysregulation. It frequently occurs in the context of immune-mediated diseases, particularly the HLA-B27–associated spondyloarthropathies, as well as other conditions such as juvenile idiopathic arthritis and inflammatory bowel disease. Since anterior uveitis may also arise from other causes and therefore lacks full specificity, it will be added to the broad scope of the SMQ.

Detail Report

CR Number: 2026078029		Implementation Date: 06-Apr-26	Related CR: 2026078029	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	HMG CoA reductase antibody test positive	Approved as Requested	Proposed PT	
To SMQ	Immune-mediated/autoimmune disorders (SMQ)		HMG CoA reductase antibody test positive	10093474
Term scope	Broad		To SMQ	
			Immune-mediated/autoimmune disorders (SMQ)	20000236
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT HMG CoA reductase antibody test positive to Immune-mediated/autoimmune disorders (SMQ) as a Broad term is approved as requested. Like other laboratory test use in the diagnose of autoimmune disorders, HMG-CoA reductase antibody test positive is appropriate for inclusion in the broad scope of the SMQ. It represents a laboratory finding indicative of immune-mediated necrotizing myopathy, particularly in the context of statin exposure.

CR Number: 2026078030		Implementation Date: 06-Apr-26	Related CR: 2026078030	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Charcot foot reconstruction	Approved as Requested	Proposed PT	
To SMQ	Arthritis (SMQ)		Charcot foot reconstruction	10093478
Term scope	Broad		To SMQ	
			Arthritis (SMQ)	20000216
			Term scope	
			Broad	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Charcot foot reconstruction to Arthritis (SMQ) as a Broad term is approved as requested. PT Charcot foot reconstruction was added in version 29.1 and describes a complex orthopedic procedure used to treat severe foot deformities caused by Charcot neuroarthropathy, most commonly associated with diabetic neuropathy. PT Charcot foot reconstruction will be added with Broad scope, in line with the already included PT Neuropathic arthropathy.

Detail Report

CR Number: 2026078031		Implementation Date: 06-Apr-26	Related CR: 2026078031	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Renal vein transposition</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Renovascular disorders (SMQ)</i>		<i>Renal vein transposition</i>	10093427
Term scope	<i>Broad</i>		To SMQ	
			<i>Renovascular disorders (SMQ)</i>	20000181
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Renal vein transposition to Renovascular disorders (SMQ) as a Broad term is approved as requested. It is a surgical intervention used in nutcracker syndrome (compression of the left renal vein by aorto-mesenteric pinching), which is a renal vascular disorder and therefore falls within the concept of renovascular disorders. Like other surgical interventions that are a consequence of reno-vascular pathology, this term will be added to the broad scope of the SMQ.

CR Number: 2026078032		Implementation Date: 06-Apr-26	Related CR: 2026078032	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Synovial thickening</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Arthritis (SMQ)</i>		<i>Synovial thickening</i>	10093539
Term scope	<i>Broad</i>		To SMQ	
			<i>Arthritis (SMQ)</i>	20000216
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Synovial thickening to Arthritis (SMQ) as a Broad term is approved as requested. PT Synovial thickening was added in version 29.1 and describes is the enlargement of the thin membrane lining joints, frequently caused by inflammation and infection. PT Synovial thickening will be added with Broad scope, as it only represents a sign of joint inflammation, in line with the already included PT Joint effusion.

Detail Report

CR Number: 2026078033		Implementation Date: 06-Apr-26	Related CR: 2026078033	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Lymphocyte subset panel abnormal</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Haematopoietic leukopenia (SMQ)</i>		<i>Lymphocyte subset panel abnormal</i>	10093414
Term scope	<i>Broad</i>		To SMQ	
			<i>Haematopoietic leukopenia (SMQ)</i>	20000030
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Lymphocyte subset panel abnormal to Haematopoietic leukopenia (SMQ) as a Broad term is approved as requested. PT Lymphocyte subset panel abnormal was added in version 29.1 and represents a laboratory result indicating that one or more lymphocyte subpopulations fall outside established reference ranges (such as reduced CD4+ T cells, an inverted CD4/CD8 ratio, expanded B-cell populations, or decreased NK cells) reflecting an underlying disturbance of immune homeostasis. It may reflect chemotherapy-induced myelosuppression, a spectrum of hematological declines that does not only include lymphocytes but also neutrophils, hemoglobin levels and platelets. The PT represents a lab result with an "abnormal" qualifier and will therefore be included with broad scope.

CR Number: 2026078034		Implementation Date: 06-Apr-26	Related CR: 2026078034	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Immature reticulocyte fraction increased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Haemolytic disorders (SMQ)</i>		<i>Immature reticulocyte fraction increased</i>	10093498
Term scope	<i>Broad</i>		To SMQ	
			<i>Haemolytic disorders (SMQ)</i>	20000019
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Immature reticulocyte fraction increased to Haemolytic disorders (SMQ) as a Broad term is approved as requested. PT Immature reticulocyte fraction increased was added in version 29.1 and is a key indicator of erythropoietic activity, reflecting the bone marrow's response to red blood cell demand. PT Immature reticulocyte fraction increased will be added with broad scope, in line with the already included PT Reticulocyte count increased and PT Reticulocyte percentage increased.

Detail Report

CR Number: 2026078035		Implementation Date: 06-Apr-26	Related CR: 2026078035		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Thyroid sarcoidosis</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Hypothyroidism (SMQ)</i>		<i>Thyroid sarcoidosis</i>	10093506	
Term scope	<i>Broad</i>		To SMQ		
			<i>Hypothyroidism (SMQ)</i>	20000160	
			Term scope		
			<i>Broad</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Thyroid sarcoidosis to Hypothyroidism (SMQ) as a Broad term is approved as requested. PT Thyroid sarcoidosis was added in V29.1 and is usually associated with the involvement of other organs, primarily the lungs and lymph nodes. Typical manifestations are diffuse goiter and solitary or multiple thyroid nodules. Thyroid function remains intact in the majority of cases, but sometimes it can result in hypothyroidism or hyperthyroidism. PT Thyroid sarcoidosis will be added with broad scope, in line with already included PT Multifocal fibrosclerosis.

CR Number: 2026078036		Implementation Date: 06-Apr-26	Related CR: 2026078036		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Thyroid sarcoidosis</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Hyperthyroidism (SMQ)</i>		<i>Thyroid sarcoidosis</i>	10093506	
Term scope	<i>Broad</i>		To SMQ		
			<i>Hyperthyroidism (SMQ)</i>	20000161	
			Term scope		
			<i>Broad</i>		
			Term_category		
			<i>A</i>		
			Term_weight		
			<i>0</i>		

MSSO Comment: The proposal to add PT Thyroid sarcoidosis to Hyperthyroidism (SMQ) as a Broad term is approved as requested. PT Thyroid sarcoidosis was added in V29.1 and is usually associated with the involvement of other organs, primarily the lungs and lymph nodes. Typical manifestations are diffuse goiter and solitary or multiple thyroid nodules. Thyroid function remains intact in the majority of cases, but sometimes it can result in hypothyroidism or hyperthyroidism. PT Thyroid sarcoidosis will be added with broad scope.

Detail Report

CR Number: 2026078037		Implementation Date: 06-Apr-26	Related CR: 2026078037	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Oropharynx papilloma</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Oropharyngeal neoplasms (SMQ)</i>		<i>Oropharynx papilloma</i>	10093424
Term scope	<i>Narrow</i>		To SMQ	
			<i>Oropharyngeal neoplasms (SMQ)</i>	20000110
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Oropharynx papilloma to Oropharyngeal neoplasms (SMQ) as a Narrow term is approved as requested. PT Oropharynx papilloma was added in
Comment: V29.1 and manifests as a benign, HPV-induced wart-like neoplasm found on the soft palate, tongue, or tonsils, often treated via simple surgical excision. All PT in this SMQ are narrow in scope.

CR Number: 2026078038		Implementation Date: 06-Apr-26	Related CR: 2026078038	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Thyroid sarcoidosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Immune-mediated/autoimmune disorders (SMQ)</i>		<i>Thyroid sarcoidosis</i>	10093506
Term scope	<i>Narrow</i>		To SMQ	
			<i>Immune-mediated/autoimmune disorders (SMQ)</i>	20000236
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Thyroid sarcoidosis to Immune-mediated/autoimmune disorders (SMQ) as a Narrow term is approved as requested. Thyroid sarcoidosis is
Comment: fundamentally an immune-mediated granulomatous disease or thyroid characterized by dysregulated cellular immunity, particularly Th1-driven T-cell activation and macrophage aggregation leading to non-caseating granuloma formation. Although its precise etiology remains uncertain and it is not classified as a classical autoantibody-mediated autoimmune disease, its pathogenesis clearly reflects an abnormal immune response against presumed antigens, resulting in tissue-specific inflammatory damage.

Detail Report

CR Number: 2026078039		Implementation Date: 06-Apr-26	Related CR: 2026078039	
<u>Change Requested</u>	Update MedDRA term_scope	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Acquired arteriovenous malformation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>		<i>Acquired arteriovenous malformation</i>	10092584
Term scope	<i>Narrow</i>		To SMQ	
			<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>	20000166
			Term scope	
			<i>Narrow</i>	

MSSO Comment: The proposal to change the scope of the PT Acquired arteriovenous malformation in Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) to Narrow is approved as requested. PT Acquired arteriovenous malformation, which was added to this SMQ in Version 28.1, will be changed from broad to narrow scope to facilitate better term alignment in MedDRA.

CR Number: 2026078040		Implementation Date: 06-Apr-26	Related CR: 2026078040	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Cerebrovascular arteriovenous malformation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>		<i>Cerebrovascular arteriovenous malformation</i>	10056371
Term scope	<i>Narrow</i>		To SMQ	
			<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>	20000166
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Cerebrovascular arteriovenous malformation to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Narrow term is approved as requested. PT Cerebrovascular arteriovenous malformation will be added to the narrow terms of this SMQ to facilitate better term alignment within MedDRA.

Detail Report

CR Number: 2026078041		Implementation Date: 06-Apr-26	Related CR: 2026078041	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Congenital cerebrovascular anomaly</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>		<i>Congenital cerebrovascular anomaly</i>	10062327
Term scope	<i>Narrow</i>		To SMQ	
			<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>	20000166
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Congenital cerebrovascular anomaly to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Narrow term is approved as requested. PT Congenital cerebrovascular anomaly will be added to the narrow terms of this SMQ to facilitate better term alignment within MedDRA.

CR Number: 2026078042		Implementation Date: 06-Apr-26	Related CR: 2026078042	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Persistent spinal pain syndrome</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Arthritis (SMQ)</i>		<i>Persistent spinal pain syndrome</i>	10093520
Term scope	<i>Broad</i>		To SMQ	
			<i>Arthritis (SMQ)</i>	20000216
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Persistent spinal pain syndrome to Arthritis (SMQ) as a Broad term is approved as requested. PT Persistent spinal pain syndrome was added in version 29.1 and can be caused by spinal osteoarthritis or spondylitis. PT Persistent spinal pain syndrome will be added with broad scope, in line with the already included PT Spinal pain.

Detail Report

CR Number: 2026078043		Implementation Date: 06-Apr-26	Related CR: 2026078043	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Ejaculatory duct obstruction</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Fertility disorders (SMQ)</i>		<i>Ejaculatory duct obstruction</i>	10093430
Term scope	<i>Narrow</i>		To SMQ	
			<i>Fertility disorders (SMQ)</i>	20000210
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Ejaculatory duct obstruction to Fertility disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this
Comment: SMQ which includes terms for medical conditions reflecting fertility problems. Similar term such as PT Ejaculatory duct cyst is already included as a narrow scope term.

CR Number: 2026078044		Implementation Date: 06-Apr-26	Related CR: 2026078044	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Transurethral resection of ejaculatory duct</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Fertility disorders (SMQ)</i>		<i>Transurethral resection of ejaculatory duct</i>	10093429
Term scope	<i>Narrow</i>		To SMQ	
			<i>Fertility disorders (SMQ)</i>	20000210
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Transurethral resection of ejaculatory duct to Fertility disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion
Comment: criteria for this SMQ which includes terms for procedures related to fertility. Transurethral resection of ejaculatory duct is a surgical procedure to treat ejaculatory duct obstruction by unblocking the ducts, allowing sperm and seminal fluid to mix, improving fertility. Similar term such as PT Ejaculatory duct dilation procedure is already included as a narrow scope term.

Detail Report

CR Number: 2026078045		Implementation Date: 06-Apr-26	Related CR: 2026078045	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Foetal ascites</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Foetal disorders (SMQ)</i>		<i>Foetal ascites</i>	10093513
Term scope	<i>Narrow</i>		To SMQ	
			<i>Foetal disorders (SMQ)</i>	20000190
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Foetal ascites to Foetal disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar
Comment: terms such as PT Foetal arrhythmia and PT Foetal megacystis are already included as narrow scope terms.

CR Number: 2026078046		Implementation Date: 13-Apr-26	Related CR: 2026078046	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Lymphocyte subset panel abnormal</i>	Approved Not as Requested	Proposed PT	
To SMQ	<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>		<i>Lymphocyte subset panel abnormal</i>	10093414
Term scope	<i>Broad</i>		To SMQ	
			<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>	20000225
			Term scope	
			<i>Broad</i>	
			Term_category	
			E	
			Term_weight	
			0	

MSSO The proposal to add PT Lymphocyte subset panel abnormal to Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ) as a broad term is approved but
Comment: not as requested. This term fits the inclusion criteria for this SMQ which includes terms related to hematologic abnormalities. Similar terms such as PT Lymphocyte count abnormal and PT Lymphocyte percentage abnormal are already included as broad scope terms. PT Lymphocyte subset panel abnormal will be added to this SMQ with an updated Term Category from -1 to E.

Detail Report

CR Number: 2026078047		Implementation Date: 06-Apr-26	Related CR: 2026078047	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Cochleostomy	Approved as Requested	Proposed PT	
To SMQ	Hearing impairment (SMQ)		Cochleostomy	10093426
Term scope	Narrow		To SMQ	
			Hearing impairment (SMQ)	20000171
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Cochleostomy to Hearing impairment (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar
Comment: terms such as PT Cochlea implant and PT Hearing aid therapy are already included as narrow scope terms.

CR Number: 2026078048		Implementation Date: 06-Apr-26	Related CR: 2026078048	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Vestibular syndrome	Approved as Requested	Proposed PT	
To SMQ	Vestibular disorders (SMQ)		Vestibular syndrome	10093488
Term scope	Narrow		To SMQ	
			Vestibular disorders (SMQ)	20000172
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Vestibular syndrome to Vestibular disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ.
Comment: Similar terms such as PT Vestibular disorder and recently demoted PT Acute vestibular syndrome are already included as narrow scope terms.

Detail Report

CR Number: 2026078049		Implementation Date: 06-Apr-26	Related CR: 2026078049	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>McMahon score for renal failure</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Acute renal failure (SMQ)</i>		<i>McMahon score for renal failure</i>	10093404
Term scope	<i>Broad</i>		To SMQ	
			<i>Acute renal failure (SMQ)</i>	20000003
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT McMahon score for renal failure to Acute renal failure (SMQ) as a Broad term is approved as requested. This PT may be seen as an exception to the SMQ exclusion criteria and may be added in alignment with PT Fractional excretion of sodium in this SMQ. The McMahon score for renal failure is done when there is a high suspicion of acute kidney injury, as test score is used to predict the risk of severe acute kidney injury.

CR Number: 2026078050		Implementation Date: 06-Apr-26	Related CR: 2026078050	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Porphyrins plasma increased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Chronic kidney disease (SMQ)</i>		<i>Porphyrins plasma increased</i>	10093470
Term scope	<i>Broad</i>		To SMQ	
			<i>Chronic kidney disease (SMQ)</i>	20000213
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Porphyrins plasma increased to Chronic kidney disease (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar terms such as PT Urinary transferrin increased and PT Blood erythropoietin abnormal are already included as broad scope terms.

Detail Report

CR Number: 2026078051		Implementation Date: 06-Apr-26	Related CR: 2026078051	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Vitamin D decreased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Chronic kidney disease (SMQ)</i>		<i>Vitamin D decreased</i>	10062189
Term scope	<i>Broad</i>		To SMQ	
			<i>Chronic kidney disease (SMQ)</i>	20000213
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	
MSSO The proposal to add PT Vitamin D decreased to Chronic kidney disease (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria for this SMQ.				
Comment:	Measurement of specific forms of Vitamin D metabolites under PT Vitamin D may be used to investigate some patients with clinical evidence of vitamin D deficiency and chronic renal impairment.			

CR Number: 2026078052		Implementation Date: 06-Apr-26	Related CR: 2026078052	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Dysmorphic urinary red blood cells present</i>	Approved Not as Requested	Proposed PT	
To SMQ	<i>Systemic lupus erythematosus (SMQ)</i>		<i>Dysmorphic urinary red blood cells present</i>	10093497
Term scope	<i>Broad</i>		To SMQ	
			<i>Systemic lupus erythematosus (SMQ)</i>	20000045
			Term scope	
			<i>Broad</i>	
			Term_category	
			F	
			Term_weight	
			1	
MSSO The proposal to add PT Dysmorphic urinary red blood cells present to Systemic lupus erythematosus (SMQ) as a broad term is approved but not as requested.				
Comment:	Dysmorphic urinary red blood cells present was added as qualified counterpart of PT Dysmorphic urinary red blood cells which was previously included in this SMQ. Most patients with systemic lupus erythematosus (SLE) will have clinical evidence of kidney disease, usually an abnormal urinalysis, at some point in the course of their disease. Lupus nephritis (LN) is a severe manifestation of SLE and typically develops early in the disease course. Clinically evident kidney disease eventually occurs in up to one-half of patients with SLE, and up to 10 percent of patients with LN will develop end-stage kidney disease. The presence of lupus nephritis should be suspected in patients with known SLE who develop an active urinary sediment with persistent hematuria (five or more red blood cells, most of which are dysmorphic). PT Dysmorphic urinary red blood cells present will be added with updated values for Category from -1 to F (Renal disorder), and Term Weight from -1 to 1 to correct the values submitted.			

Detail Report

CR Number: 2026078053		Implementation Date: 06-Apr-26	Related CR: 2026078053	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Alveolar bone reconstruction</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Osteonecrosis (SMQ)</i>		<i>Alveolar bone reconstruction</i>	10093431
Term scope	<i>Broad</i>		To SMQ	
			<i>Osteonecrosis (SMQ)</i>	20000180
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Alveolar bone reconstruction to Osteonecrosis (SMQ) as a Broad term is approved as requested. Medication-related osteonecrosis of the jaw (MRONJ) is a severe adverse drug sequelae characterized by progressive bone destruction in the maxillofacial region. Surgical intervention is increasingly recognized as a highly effective approach for treating MRONJ lesions, demonstrating high success rates across all disease stages. These procedures involve removing the infected bone beyond the necrotic margins to reach healthy, bleeding tissue. Severe maxillo-mandibular defects might need to be reconstructed using free vascularized bone or using reconstruction plates. Another treatment approach combines surgical resection with guided bone regeneration (GBR) for managing MRONJ lesions, aiming primarily to minimize the need for extensive or major surgical procedures in affected patients. This approach involves debridement of necrotic bone with the application of bone augmentation materials, aiming to preserve as much healthy tissue as possible and eliminate the need for more extensive and invasive procedures. The PT Alveolar bone reconstruction will be added to the Broad scope, similar to the existing PT Alveolar bone resection.

CR Number: 2026078054		Implementation Date: 06-Apr-26	Related CR: 2026078054	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Dysmorphic urinary red blood cells present</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Chronic kidney disease (SMQ)</i>		<i>Dysmorphic urinary red blood cells present</i>	10093497
Term scope	<i>Broad</i>		To SMQ	
			<i>Chronic kidney disease (SMQ)</i>	20000213
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Dysmorphic urinary red blood cells present to Chronic kidney disease (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar terms such as PT Dysmorphic urinary red blood cells and PT Eosinophils urine present are already included as broad scope terms.

Detail Report

CR Number: 2026078055		Implementation Date: 06-Apr-26	Related CR: 2026078055		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Hip circumference decreased</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Lipodystrophy (SMQ)</i>		<i>Hip circumference decreased</i>	10093435	
Term scope	<i>Broad</i>		To SMQ		
			<i>Lipodystrophy (SMQ)</i>	20000177	
			Term scope		
			<i>Broad</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO The proposal to add PT Hip circumference decreased to Lipodystrophy (SMQ) as a Broad term is approved as requested. Lipoatrophy, or the loss of adipose tissue, can
Comment: lead to decreased hip circumference. The term will be added under Broad scope as it is not unique to the condition depending on the context.

CR Number: 2026078056		Implementation Date: 06-Apr-26	Related CR: 2026078056		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Invasive pulmonary aspergillosis</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Infective pneumonia (SMQ)</i>		<i>Invasive pulmonary aspergillosis</i>	10093496	
Term scope	<i>Narrow</i>		To SMQ		
			<i>Infective pneumonia (SMQ)</i>	20000231	
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO The proposal to add PT Invasive pulmonary aspergillosis to Infective pneumonia (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for
Comment: terms representing infective pneumonias, including those typically associated with an immunocompromised patient for a narrow scope term. Invasive pulmonary aspergillosis is a rapidly progressive and often fatal lung fungal infection in which Aspergillus invades lung tissue, often with vascular involvement, occurring mainly in immunocompromised patients.

Detail Report

CR Number: 2026078057		Implementation Date: 06-Apr-26	Related CR: 2026078057	
<u>Change Requested</u>		<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Dopamine decreased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Hypertension (SMQ)</i>		<i>Dopamine decreased</i>	10093521
Term scope	<i>Broad</i>		To SMQ	
			<i>Hypertension (SMQ)</i>	20000147
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Dopamine decreased to Hypertension (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria for a broad scope
Comment: term, for abnormal values involved in activation of the Renin-Angiotensin-System.

CR Number: 2026078058		Implementation Date: 06-Apr-26	Related CR: 2026078058	
<u>Change Requested</u>		<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Airway device insertion</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Respiratory failure (SMQ)</i>		<i>Airway device insertion</i>	10093523
Term scope	<i>Broad</i>		To SMQ	
			<i>Respiratory failure (SMQ)</i>	20000222
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Airway device insertion to Respiratory failure (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria as a broad
Comment: scope term, for relevant treatment terms for respiratory failure. Similar PT, Endotracheal intubation is already included as a broad scope term.

Detail Report

CR Number: 2026078059		Implementation Date: 06-Apr-26	Related CR: 2026078059	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Quadricepsplasty</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Tendinopathies and ligament disorders (SMQ)</i>		<i>Quadricepsplasty</i>	10093437
Term scope	<i>Narrow</i>		To SMQ	
			<i>Tendinopathies and ligament disorders (SMQ)</i>	20000223
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Quadricepsplasty to Tendinopathies and ligament disorders (SMQ) as a Narrow term is approved as requested. Quadricepsplasty is a muscle-tendon surgical procedure to release, lengthen, or repair the quadriceps muscle or tendon to improve knee function, usually in cases where there is knee stiffness or limited range of motion. This term fits the inclusion criteria for therapeutic procedures for tendon/ligament disorders, as a narrow scope term.

CR Number: 2026078060		Implementation Date: 06-Apr-26	Related CR: 2026078060	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Epicondyle release surgery</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Tendinopathies and ligament disorders (SMQ)</i>		<i>Epicondyle release surgery</i>	10093480
Term scope	<i>Narrow</i>		To SMQ	
			<i>Tendinopathies and ligament disorders (SMQ)</i>	20000223
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Epicondyle release surgery to Tendinopathies and ligament disorders (SMQ) as a Narrow term is approved as requested. Epicondyle release surgery is a surgical procedure used to treat chronic epicondylitis that has not responded to conservative management. It involves releasing and debriding the diseased tendon fibers at their attachment to the medial or lateral epicondyle of the humerus. This term fits the inclusion criteria for therapeutic procedures for tendon/ligament disorders, as a narrow scope term.

Detail Report

CR Number: 2026078061		Implementation Date: 06-Apr-26	Related CR: 2026078061	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Invasive pulmonary aspergillosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Sepsis (SMQ)</i>		<i>Invasive pulmonary aspergillosis</i>	10093496
Term scope	<i>Broad</i>		To SMQ	
			<i>Sepsis (SMQ)</i>	20000234
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Invasive pulmonary aspergillosis to Sepsis (SMQ) as a Broad term is approved as requested. This term fits the inclusion criteria for this SMQ.
Comment: Invasive pulmonary aspergillosis is defined by the fungus spreading rapidly from the lungs to blood vessels and, subsequently, to other organs. Similar term such as PT Invasive pneumococcal disease is already included as a broad scope term.

CR Number: 2026078062		Implementation Date: 06-Apr-26	Related CR: 2026078062	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Venous recanalisation procedure</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Embolic and thrombotic events, venous (SMQ)</i>		<i>Venous recanalisation procedure</i>	10093407
Term scope	<i>Narrow</i>		To SMQ	
			<i>Embolic and thrombotic events, venous (SMQ)</i>	20000084
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Venous recanalisation procedure to Embolic and thrombotic events, venous (SMQ) as a Narrow term is approved as requested. A venous recanalization procedure is a minimally invasive intervention that restores blood flow in veins that are blocked or narrowed, often due to thrombosis or chronic occlusion. It typically involves techniques such as angioplasty, stenting, or catheter-directed thrombolysis to reopen the affected vein and improve circulation. This term fits the inclusion criteria for Terms that are procedure-related. All terms in this SMQ are narrow.

Detail Report

CR Number: 2026078063		Implementation Date: 06-Apr-26	Related CR: 2026078063	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Electrocardiogram epsilon wave present</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Cardiomyopathy (SMQ)</i>		<i>Electrocardiogram epsilon wave present</i>	10093458
Term scope	<i>Broad</i>		To SMQ	
			<i>Cardiomyopathy (SMQ)</i>	20000150
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Electrocardiogram epsilon wave present to Cardiomyopathy (SMQ) as a Broad term is approved as requested. The epsilon wave, the hallmark feature of Arrhythmogenic Right Ventricular Cardiomyopathy (ARVC), is a marker of delayed activation of the RV free wall and outflow tract and is considered a major diagnostic criterion for ARVC. Electrocardiograms revealing epsilon waves are associated with many serious conditions such as arrhythmogenic right ventricular cardiomyopathy, posterior myocardial infarction, right ventricular infarction, infiltration disease, sarcoidosis, Brugada Syndrome, Tetralogy of Fallot, and hypothermia. The PT Electrocardiogram epsilon wave present will be added under Broad scope, similar to existing terms such as PT Electrocardiogram abnormal.

CR Number: 2026078068		Implementation Date: 24-Mar-26	Related CR: 2026078068	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Coronary artery bowing</i>	Rejected		

MSSO Comment: The proposal to add a new term Coronary artery bowing is not approved. The proposed term is not a formal, referenceable medical description. Without context it may be ambiguous, e.g. artery tortuosity. Please consider existing terms for the imaging procedure with the qualifier abnormal, depending on your scenario. In addition, it may be difficult to translate the term to all MedDRA languages with the appropriate interpretation.

Detail Report

CR Number: 2026078069		Implementation Date: 24-Mar-26	Related CR: 2026078069	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Coronary artery displacement	Approved Not as Requested	Proposed PT Procedural coronary artery displacement	10093611
			HLT primary Cardiac and vascular procedural complications	10007602
			SOC primary Injury, poisoning and procedural complications	10022117
			HLT secondary Coronary artery disorders NEC	10011083
			SOC secondary Cardiac disorders	10007541
MSSO Comment: The proposal to add a new LLT Coronary artery displacement is approved but not as requested. Procedural coronary artery displacement refers to the unintended shifting or repositioning of a coronary artery during an interventional cardiac procedure (such as stent placement or valve implantation), potentially compromising blood flow or causing vascular complications. Procedural coronary artery displacement will be added as PT to primary HLT Cardiac and vascular procedural complications and to secondary HLT Coronary artery disorders NEC.				

CR Number: 2026082000		Implementation Date: 30-Mar-26	Related CR: 2026082000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	90yttrium transarterial radioembolization	Approved Not as Requested	Proposed LLT Yttrium-90 transarterial radioembolization	10093638 Current
To PT	Radioembolisation		To PT Radioembolisation	10073946
MSSO Comment: The proposal to add a new LLT 90yttrium transarterial radioembolization to PT Radioembolisation is approved but not as requested. In Transarterial Radioembolization (TARE) the therapeutic effect comes from intra-arterial delivery of radioactive microspheres that lodge in tumor vasculature and emit β -radiation locally. TARE primarily uses Yttrium-90 embedded in microspheres or TheraSphere to deliver localized β -radiation to liver tumors. MedDRA limits inclusion of terms with several modifiers. The proposed term will be modified and added as LLT Yttrium-90 transarterial radioembolization to PT Radioembolisation. In a related change, the British English spelled counterpart LLT Yttrium-90 transarterial radioembolisation will also be added to PT Radioembolisation.				

Detail Report

CR Number: 2026089001		Implementation Date: 30-Mar-26	Related CR: 2026082000		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Yttrium-90 transarterial radioembolisation	Approved as Requested	Proposed LLT		
To PT	Radioembolisation		Yttrium-90 transarterial radioembolisation	10093637	Current
			To PT		
			Radioembolisation	10073946	

MSSO
Comment:

CR Number: 2026082001		Implementation Date: 30-Mar-26	Related CR: 2026082001		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Extended pelvic lymph node dissection	Approved as Requested	Proposed LLT		
To PT	Lymphadenectomy		Extended pelvic lymph node dissection	10093639	Current
			To PT		
			Lymphadenectomy	10048956	

MSSO
Comment:

The proposal to add a new LLT Extended pelvic lymph node dissection to PT Lymphadenectomy is approved as requested. Extended pelvic lymph node dissection (ePLND) is a surgical procedure that removes lymphatic tissue along the external iliac, obturator, and internal iliac vessels, typically extending proximally to the common iliac nodes up to the ureteric crossing. It is performed to achieve accurate nodal staging and improved detection of metastases in pelvic malignancies.

CR Number: 2026082002		Implementation Date: 10-Jun-26	Related CR: 2026082002		
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Expulsion of implanted drug	Suspended			
From PT	Expulsion of medication				
To PT	Device expulsion				

MSSO
Comment:

The proposal to move the LLT Expulsion of implanted drug from PT Expulsion of medication to PT Device expulsion is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.

Detail Report

CR Number: 2026082003		Implementation Date: 12-May-26	Related CR: 2026082003		
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Device difficult to remove</i>	Rejected After Suspension			
From PT	<i>Device difficult to use</i>				
To PT	<i>Complication of device removal</i>				
MSSO Comment:	The proposal to move the LLT Device difficult to remove from PT Device difficult to use to PT Complication of device removal is not approved after suspension. Device difficult to remove refers to an intrinsic propriety of the device which makes its removal difficult in any circumstance of use rather than as a complication. Medical judgement in combination with the review of the hierarchy may be needed to select the most relevant term for drug-device combination products. Additionally, this placement was the outcome of a device related term review with the help of industry volunteers to enhance the hierarchical groupings of device related terms in MedDRA in the past. Furthermore, the impact on legacy data was considered in the decision.				

CR Number: 2026082004		Implementation Date: 12-May-26	Related CR: 2026082004		
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Difficulty removing drug device</i>	Rejected After Suspension			
From PT	<i>Device difficult to use</i>				
To PT	<i>Complication of device removal</i>				
MSSO Comment:	The proposal to move the LLT Difficulty removing drug device from PT Device difficult to use to PT Complication of device removal is not approved after suspension. Difficulty removing drug device refers to an intrinsic propriety of the device which makes its removal difficult in any circumstance of use rather than as a complication. Medical judgement in combination with the review of the hierarchy may be needed to select the most relevant term for drug-device combination products. Additionally, this placement was the outcome of a device related term review with the help of industry volunteers to enhance the hierarchical groupings of device related terms in MedDRA in the past. Furthermore, the impact on legacy data was considered in the decision.				

CR Number: 2026082005		Implementation Date: 30-Mar-26	Related CR: 2026082005		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Gastrorrhexis	Approved as Requested	Proposed LLT		
			Gastrorrhexis	10093641	Current
			To PT		
			Gastric rupture	10093640	
MSSO Comment:	The proposal to add a new term Gastrorrhexis is approved as requested. Gastrorrhexis is a rare but severe condition defined as a rupture of the stomach wall. It results in leakage of gastric contents into the peritoneal cavity, often leading to peritonitis. The most common causes include abdominal trauma, acute gastric distension, iatrogenic injury, and ischemic necrosis of the gastric wall. It represents a surgical emergency requiring prompt diagnosis and management. Gastrorrhexis will be added as LLT to new PT Gastric rupture which will be added in a related change with a primary link to HLT Gastrointestinal disorders NEC and a secondary link to HLT Abdominal and gastrointestinal injuries NEC.				

Detail Report

CR Number: 2026089002		Implementation Date: 30-Mar-26	Related CR: 2026082005	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Gastric rupture</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Gastrointestinal disorders NEC</i>		<i>Gastric rupture</i>	10093640
SOC primary	<i>Gastrointestinal disorders</i>		HLT primary	
			<i>Gastrointestinal disorders NEC</i>	10017948
			SOC primary	
			<i>Gastrointestinal disorders</i>	10017947

MSSO
Comment:

CR Number: 2026089003		Implementation Date: 30-Mar-26	Related CR: 2026082005	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Gastric rupture</i>	Approved as Requested	Proposed PT	
To HLT	<i>Abdominal and gastrointestinal injuries NEC</i>		<i>Gastric rupture</i>	10093640
			To HLT	
			<i>Abdominal and gastrointestinal injuries NEC</i>	10083613

MSSO
Comment:

CR Number: 2026083000		Implementation Date: 30-Mar-26	Related CR: 2026083000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Small intestine erosion</i>	Approved as Requested	Proposed PT	
			<i>Small intestine erosion</i>	10093642
			HLT primary	
			<i>Intestinal ulcers and perforation NEC</i>	10032172
			SOC primary	
			<i>Gastrointestinal disorders</i>	10017947

MSSO
Comment: The proposal to add a new term Small intestine erosion is approved as requested. Small intestine erosion is the wearing away of the mucosal lining of the intestine, leading to shallow sores or ulcers, often causing abdominal pain, bleeding, or infection. It may occur within certain contexts (enteritis, Crohn's disease, NSAID enteropathy). Small intestine erosion will be added as PT to HLT Intestinal ulcers and perforation NEC.

Detail Report

CR Number: 2026083001		Implementation Date: 30-Mar-26	Related CR: 2026083001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Peripheral arterial thromboembolic event</i>	Approved Not as Requested	Proposed LLT <i>Peripheral arterial thromboembolism</i> To PT <i>Peripheral embolism</i>	10093643 Current 10061340
MSSO Comment:	The proposal to add a new term Peripheral arterial thromboembolic event is approved but not as requested. Arterial embolism results when a mass of tissue or a foreign substance travels through the vascular tree, ultimately lodging in a distal artery where it obstructs blood flow. This obstruction leads to ischemia, organ dysfunction and potential infarction. Thromboemboli are large clots that dislodge from the surface of atherosclerotic lesions and occlude distal arteries causing immediate ischemia. The proposed term will be modified for clarity and added as LLT Peripheral arterial thromboembolism to PT Peripheral embolism.			

CR Number: 2026084001		Implementation Date: 30-Mar-26	Related CR: 2026084001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Haemoglobin further decreased, etc..</i>	Rejected		
MSSO Comment:	The proposal to add a new term Haemoglobin further decreased, etc.. is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation unless such a term represents an internationally accepted unique medical concept, as described in the MedDRA Introductory Guide section 4.10 - RULES AND CONVENTIONS ADOPTED IN THE TERMINOLOGY: AGGRAVATION OF UNDERLYING CONDITIONS. Refer to 3.9 – Modification of Pre-existing Conditions of the MedDRA® TERM SELECTION: POINTS TO CONSIDER document for coding guidance. The same principles apply to investigation results.			

CR Number: 2026084002		Implementation Date: 30-Mar-26	Related CR: 2026084002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Umbilical tumor</i>	Rejected		
MSSO Comment:	The proposal to add a new term Umbilical tumor is not approved. Although in MedDRA terms containing the word “tumo(u)r” are considered neoplastic, the proposed expression is very ambiguous and of little clinical use as a stand-alone term. It may simply refer to any mass located at the umbilicus, which includes highly heterogeneous entities: benign lesions (umbilical granuloma, cysts, hernias), inflammatory processes, umbilical endometriosis, or primary and metastatic neoplasms. Even if the word tumor is interpreted in its neoplastic sense, “umbilical tumor” remains ambiguous because it does not indicate whether the neoplasm is primary or metastatic, what its histologic type is, or from which tissue or organ it originated, and thus it may refer to a wide range of very different neoplasms (metastatic carcinomas, primary cutaneous tumors, sarcomas, etc.). Please consider existing PT Abdominal wall neoplasm and PT Abdominal wall mass, depending on your specific coding scenario. The MedDRA Term Selection Points to Consider document provides coding guidance in section 3.7 related to body site vs. event specificity.			

Detail Report

CR Number: 2026084003		Implementation Date: 30-Mar-26	Related CR: 2026084003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Herpetic laryngopharyngitis</i>	Approved Not as Requested	Proposed LLT <i>Herpes laryngopharyngitis</i>	10093644 Current
			To PT <i>Herpes pharyngitis</i>	10066888
MSSO Comment: The proposal to add a new term Herpetic laryngopharyngitis is approved but not as requested. Herpes typically presents with mucocutaneous or genital ulcerations but can also manifest with central nervous system involvement and occasionally other visceral or mucosal sites. Laryngopharyngitis represents an extension of pharyngitis, in which the inflammatory or infectious process spreads from the pharynx to involve the larynx. Herpes laryngopharyngitis is a rare manifestation of Herpes simplex virus infection characterized by simultaneous infection of the pharyngeal and laryngeal mucosa, leading to inflammation, ulceration, and vesicular lesions. The proposed term will be modified and added as LLT Herpes laryngopharyngitis to PT Herpes pharyngitis.				

CR Number: 2026084004		Implementation Date: 01-Apr-26	Related CR: 2026084004	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Cecal cancer metastatic</i>	Approved Not as Requested	Proposed LLT <i>Cecal cancer metastatic</i>	10093645 Current
			To PT <i>Colon cancer metastatic</i>	10055114
MSSO Comment: The proposal to add a new PT Cecal cancer metastatic is approved but not as requested. The cecum is part of the colon and generally grouped with the right-sided colon with respect to clinical behavior and metastatic spread. The cecum is sometimes singled out as having slightly more aggressive and peritoneal-prone behavior, with later-stage diagnosis and higher likelihood to present with iron deficiency anemia. Cecal cancer metastatic will be added as LLT to PT Colon cancer metastatic. In a related change, the British English spelled counterpart LLT Caecal cancer metastatic will also be added to PT Colon cancer metastatic.				

CR Number: 2026091004		Implementation Date: 01-Apr-26	Related CR: 2026084004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Caecal cancer metastatic</i>	Approved as Requested	Proposed LLT <i>Caecal cancer metastatic</i>	10093650 Current
To PT	<i>Colon cancer metastatic</i>		To PT <i>Colon cancer metastatic</i>	10055114
MSSO Comment:				

Detail Report

CR Number: 2026085001		Implementation Date: 01-Apr-26	Related CR: 2026085001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Suspected drug interaction</i>	Rejected		
To PT	<i>Drug interaction</i>			
MSSO Comment:	The proposal to add a new LLT Suspected drug interaction to PT Drug interaction is not approved. The MSSO cannot add a suspected counterpart term for every type of event. Although Drug interaction differs from provisional diagnoses, the same principles apply, that at the time of coding, the provisional term is coded to the closest MedDRA term and at the time of analysis the case needs to be further analyzed. Please refer to sections 3.20 and 3.1 in the MedDRA Term Selection Points to Consider document.			

CR Number: 2026085002		Implementation Date: 01-Apr-26	Related CR: 2026085002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Peripheral retinal degeneration</i>	Approved as Requested	Proposed LLT <i>Peripheral retinal degeneration</i>	10093646 Current
			To PT <i>Retinal degeneration</i>	10038845
MSSO Comment:	The proposal to add a new term Peripheral retinal degeneration is approved as requested. Peripheral retinal degeneration is a general term that encompasses a set of structural alterations of the retina in its outermost regions (periphery), away from the macula. These are usually degenerative changes of the retinal tissue that, in many cases, are asymptomatic, but may predispose to complications such as retinal tears or retinal detachment. Peripheral retinal degeneration will be added as an LLT to PT Retinal degeneration.			

CR Number: 2026086000		Implementation Date: 01-Apr-26	Related CR: 2026086000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Iliac vein compression</i>	Rejected		
MSSO Comment:	The proposal to add a new term Iliac vein compression is not approved. This concept already exists in MedDRA as LLT/PT Extrinsic iliac vein compression.			

Detail Report

CR Number: 2026086001		Implementation Date: 01-Apr-26	Related CR: 2026086001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Spinal schwannoma	Approved as Requested	Proposed LLT Spinal schwannoma	10093647 Current
			To PT Schwannoma	10039667

MSSO
Comment: The proposal to add a new term Spinal schwannoma is approved as requested. A spinal schwannoma is a benign tumor that originates from Schwann cells of the nerve sheaths of spinal nerve roots. It is one of the most common intradural extramedullary tumors. The proposed term will be added as LLT to PT Schwannoma. In a related change, PT Schwannoma will be moved from HLT Nervous system neoplasms unspecified malignancy NEC to HLT Nervous system neoplasms benign NEC. Furthermore, PT Neurilemmoma benign will be demoted to the LLT level under PT Schwannoma.

CR Number: 2026091005		Implementation Date: 01-Apr-26	Related CR: 2026086001	
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Schwannoma	Approved as Requested	Proposed PT Schwannoma	10039667
From HLT	Nervous system neoplasms unspecified malignancy NEC		From HLT Nervous system neoplasms unspecified malignancy NEC	10029213
To HLT	Nervous system neoplasms benign NEC		To HLT Nervous system neoplasms benign NEC	10029210

MSSO
Comment:

CR Number: 2026091006		Implementation Date: 01-Apr-26	Related CR: 2026086001	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	Neurilemmoma benign	Approved as Requested	PT to merge Neurilemmoma benign	10029235 Current
To PT	Schwannoma		To PT Schwannoma	10039667

MSSO
Comment:

Detail Report

CR Number: 2026086002		Implementation Date: 01-Apr-26	Related CR: 2026086002		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Hepatic vein embolisation</i>	Approved as Requested	Proposed LLT <i>Hepatic vein embolisation</i>	10093649	Current
			To PT <i>Liver venous deprivation procedure</i>	10092861	

MSSO
Comment: The proposal to add a new term Hepatic vein embolisation is approved as requested. Hepatic vein embolization in combination with portal vein embolization, either sequentially or simultaneously, has emerged as a viable treatment paradigm to induce greater and accelerated contralateral hepatic hypertrophy ensuring adequate future liver remnant and allowing for liver resection and reduction of posthepatectomy liver failure. Hepatic vein embolisation will be added as an LLT to PT Liver venous deprivation procedure. In a related change, the American English spelled counterpart LLT Hepatic vein embolization will also be added to PT Liver venous deprivation procedure. Furthermore, PT Portal vein embolisation will be demoted to LLT level under PT Liver venous deprivation procedure, to align the components of the procedure under one PT.

CR Number: 2026091008		Implementation Date: 01-Apr-26	Related CR: 2026086002		
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #	
PT to merge	<i>Portal vein embolisation</i>	Approved as Requested	PT to merge <i>Portal vein embolisation</i>	10076941	Current
To PT	<i>Liver venous deprivation procedure</i>		To PT <i>Liver venous deprivation procedure</i>	10092861	

MSSO
Comment:

CR Number: 2026091007		Implementation Date: 01-Apr-26	Related CR: 2026086002		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Hepatic vein embolization</i>	Approved as Requested	Proposed LLT <i>Hepatic vein embolization</i>	10093648	Current
To PT	<i>Liver venous deprivation procedure</i>		To PT <i>Liver venous deprivation procedure</i>	10092861	

MSSO
Comment:

Detail Report

CR Number: 2026086003	Implementation Date: 01-Apr-26	Related CR: 2026086003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Hepatitis panel	Rejected	

MSSO Comment: The proposal to add a new term Hepatitis panel is not approved. The concept already exists in MedDRA and may be coded to the available LLT/PT Hepatitis viral test, depending on case scenario. Alternatively, the specific type(s) of hepatitis viral tests may also be selected individually as needed to represent the specific virus/antibody/antigen test, etc.

CR Number: 2026090000	Implementation Date: 07-Apr-26	Related CR: 2026090000	MedDRA Code #
Change Requested	Change Status of LLT	Final Disposition	Final Placement
LLT to change	Acetaminophen	Approved as Requested	LLT to change
Status	non-current		Acetaminophen
			10000398 Non-Current
			Status
			non-current

MSSO Comment: The proposal to change status of LLT Acetaminophen to non-current is approved as requested. Acetaminophen is a drug name which is not within the scope of MedDRA. In a related change, LLT Acetaminophen drug level will be added to PT Analgesic drug level.

CR Number: 2026097015	Implementation Date: 07-Apr-26	Related CR: 2026090000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Acetaminophen drug level	Approved as Requested	Proposed LLT
To PT	Analgesic drug level		Acetaminophen drug level
			10093651 Current
			To PT
			Analgesic drug level
			10060090

MSSO Comment:

Detail Report

CR Number: 2026090001		Implementation Date: 07-Apr-26	Related CR: 2026090001		
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #	
LLT to change	<i>Paracetamol</i>	Approved as Requested	LLT to change		
Status	<i>non-current</i>		<i>Paracetamol</i>	10033762	Non-Current
			Status		
			<i>non-current</i>		

MSSO Comment: The proposal to change status of LLT Paracetamol to non-current is approved as requested. Paracetamol is a drug name which is not within the scope of MedDRA. In a related change, LLT Paracetamol drug level will be added to PT Analgesic drug level.

CR Number: 2026097016		Implementation Date: 07-Apr-26	Related CR: 2026090001		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Paracetamol drug level</i>	Approved as Requested	Proposed LLT		
To PT	<i>Analgesic drug level</i>		<i>Paracetamol drug level</i>	10093653	Current
			To PT		
			<i>Analgesic drug level</i>	10060090	

MSSO Comment:

CR Number: 2026090002		Implementation Date: 07-Apr-26	Related CR: 2026090002		
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #	
LLT to change	<i>Salicylate</i>	Approved as Requested	LLT to change		
Status	<i>non-current</i>		<i>Salicylate</i>	10039372	Non-Current
			Status		
			<i>non-current</i>		

MSSO Comment: The proposal to change status of LLT Salicylate to non-current is approved as requested. Salicylate is a drug name which is not within the scope of MedDRA. In a related change, LLT Salicylate drug level will be added to PT Analgesic drug level.

Detail Report

CR Number: 2026097017		Implementation Date: 07-Apr-26	Related CR: 2026090002	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Salicylate drug level	Approved as Requested	Proposed LLT	10093652 Current
To PT	Analgesic drug level		Salicylate drug level	
			To PT	
			Analgesic drug level	10060090

MSSO
Comment:

CR Number: 2026090003		Implementation Date: 07-Apr-26	Related CR: 2026090003	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Lymphomatoid papulosis	Approved as Requested	Proposed PT	10056670
To SMQ	Haematological tumours of unspecified malignancy (SMQ)		Lymphomatoid papulosis	
Term scope	Narrow		To SMQ	
			Haematological tumours of unspecified malignancy (SMQ)	20000229
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO
Comment: The proposal to add PT Lymphomatoid papulosis to Haematological tumours of unspecified malignancy (SMQ) as a narrow term is approved as requested. Lymphomatoid papulosis belongs to a family of conditions called primary cutaneous CD30-positive lymphoproliferative disorders (pcCD30+LPD). Lymphomatoid papulosis is a low-grade malignant lymphoproliferative disorder with benign clinical behavior. It matches the inclusion criteria and aligns with similar terms in Haematological tumours of unspecified malignancy (SMQ), which only has narrow search terms.

Detail Report

CR Number: 2026090004	Implementation Date: 07-Apr-26	Related CR: 2026090004	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Abdominal catheter insertion</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Abdominal catheter insertion is not approved. The proposed term is clinically uninformative because it is a broad and imprecise grouping term for distinct procedures with different anatomical targets, indications, and techniques. More specific existing terms such as LLT Peritoneal catheter insertion and LLT Suprapubic catheter insertion provide a more clear and clinically meaningful description.

CR Number: 2026090005	Implementation Date: 07-Apr-26	Related CR: 2026090005	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Microtrabecular fracture</i>	Approved as Requested	Proposed LLT <i>Microtrabecular fracture</i> 10093654 Current To PT <i>Bone contusion</i> 10066251

MSSO Comment: The proposal to add a new term Microtrabecular fracture is approved as requested. Microtrabecular fracture is not a standardized diagnostic entity but rather a descriptive term referring to microscopic trabecular disruption associated with bone marrow edema. In the setting of acute trauma, this finding is best encompassed within the concept of a bone contusion, which histologically reflects trabecular microfractures, interstitial hemorrhage, and edema without a discrete cortical break. Microtrabecular fracture will be added as an LLT to PT Bone contusion.

CR Number: 2026091000	Implementation Date: 07-Apr-26	Related CR: 2026091000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Tremor distal</i>	Approved Not as Requested	Proposed LLT <i>Distal tremor</i> 10093655 Current To PT <i>Tremor</i> 10044565

MSSO Comment: The proposal to add a new term Tremor distal is approved but not as requested. After reconsideration of the rejection in change request 2025231006, Distal tremor will help capture the specific clinical observation accurately when the exact limb is not clearly stated. The proposed term will be modified to the natural language word order and added as an LLT Distal tremor to PT Tremor.

Detail Report

CR Number: 2026091001	Implementation Date: 07-Apr-26	Related CR: 2026091001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Endovascular coiling	Rejected	

MSSO Comment: The proposal to add a new term Endovascular coiling is not approved. Based on the justification and description, the proposed concept may be represented by existing LLT Coiling of intra-cerebral aneurysm, which is more anatomically specific.

CR Number: 2026091002	Implementation Date: 07-Apr-26	Related CR: 2026091002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Arterial coiling	Rejected	

MSSO Comment: The proposal to add a new term Arterial coiling is not approved. The proposed term is ambiguous as it may also mean the abnormalities in course and geometry (tortuosity, kinking, and coiling) of an artery, such as coiling deformity of the internal carotid artery, rather than a therapeutic procedure.

CR Number: 2026091003	Implementation Date: 07-Apr-26	Related CR: 2026091003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Paroxysmal neurological episode	Rejected	

MSSO Comment: The proposal to add a new term Paroxysmal neurological episode is not approved. The proposed term is overly broad and lacks specificity to define a distinct clinical entity, as it encompasses a diverse range of etiologies.

Detail Report

CR Number: 2026091009		Implementation Date: 07-Apr-26	Related CR: 2026091009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Anti-e antibody	Approved Not as Requested	Proposed LLT	
To PT	Rhesus antibodies positive		Rhesus antibodies (anti-e)	10093658 Current
			To PT	
			Rhesus antibodies	10039033
MSSO Comment: The proposal to add a new LLT Anti-e antibody to PT Rhesus antibodies positive is approved but not as requested. Within the Rh blood group system, E and e are allelic antigens encoded by the RHCE gene and differ by point mutations that alter the RhCE protein. They are mutually exclusive on each individual chromosome, meaning a single RHCE allele encodes either E or e, not both. Possible phenotypes include E+ e- (genotype EE, only E antigen present), E- e+ (genotype ee, only e antigen present), or E+ e+ (genotype Ee, both antigens present in one person). Anti-e is an antibody directed against the e antigen in the Rh blood group system. Anti-e is implicated in hemolytic transfusion reactions and hemolytic disease of the fetus and newborn. Proposed term will be modified for better alignment and added as LLT Rhesus antibodies (anti-e) to PT Rhesus antibodies.				

CR Number: 2026091010		Implementation Date: 07-Apr-26	Related CR: 2026091010	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Anti-e antibody positive	Approved Not as Requested	Proposed LLT	
			Rhesus antibodies (anti-e) positive	10093656 Current
			To PT	
			Rhesus antibodies positive	10039036
MSSO Comment: The proposal to add a new LLT Anti-e antibody positive is approved but not as requested. Within the Rh blood group system, E and e are allelic antigens encoded by the RHCE gene and differ by point mutations that alter the RhCE protein. They are mutually exclusive on each individual chromosome, meaning a single RHCE allele encodes either E or e, not both. Possible phenotypes include E+ e- (genotype EE, only E antigen present), E- e+ (genotype ee, only e antigen present), or E+ e+ (genotype Ee, both antigens present in one person). Anti-e is an antibody directed against the e antigen in the Rh blood group system. Anti-e is implicated in hemolytic transfusion reactions and hemolytic disease of the fetus and newborn. Proposed term will be modified for better alignment and added as LLT Rhesus antibodies (anti-e) positive to PT Rhesus antibodies positive.				

Detail Report

CR Number: 2026091011		Implementation Date: 07-Apr-26	Related CR: 2026091011	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Anti-e antibody negative</i>	Approved Not as Requested	Proposed LLT <i>Rhesus antibodies (anti-e) negative</i>	10093657 Current
			To PT <i>Rhesus antibodies negative</i>	10039035
MSSO Comment: The proposal to add a new LLT Anti-e antibody negative is approved but not as requested. Within the Rh blood group system, E and e are allelic antigens encoded by the RHCE gene and differ by point mutations that alter the RhCE protein. They are mutually exclusive on each individual chromosome, meaning a single RHCE allele encodes either E or e, not both. Possible phenotypes include E+ e- (genotype EE, only E antigen present), E- e+ (genotype ee, only e antigen present), or E+ e+ (genotype Ee, both antigens present in one person). Anti-e is an antibody directed against the e antigen in the Rh blood group system. Anti-e is implicated in hemolytic transfusion reactions and hemolytic disease of the fetus and newborn. Proposed term will be modified for better alignment and added as LLT Rhesus antibodies (anti-e) negative to PT Rhesus antibodies negative.				

CR Number: 2026091012		Implementation Date: 07-Apr-26	Related CR: 2026091012	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>HIV viral load detectable</i>	Approved Not as Requested	Proposed LLT <i>HIV viral load detectable</i>	10093660 Current
			To PT <i>Blood HIV RNA detectable</i>	10093659
MSSO Comment: The proposal to add a new PT HIV viral load detectable is approved but not as requested. The viral load is used to determine how well antiretroviral therapy (ART) is working. The aim of ART is to make the viral load so low, that the virus is undetected by a viral load test. Conversely, if the viral load is detectable, therapy may need to be adjusted. Proposed concept will be added as LLT HIV viral load detectable to new PT Blood HIV RNA detectable. In a related change, PT Blood HIV RNA detectable will be added to HLT Virus identification and serology.				

CR Number: 2026097018		Implementation Date: 07-Apr-26	Related CR: 2026091012	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Blood HIV RNA detectable</i>	Approved as Requested	Proposed PT <i>Blood HIV RNA detectable</i>	10093659
HLT primary	<i>Virus identification and serology</i>		HLT primary <i>Virus identification and serology</i>	10047490
SOC primary	<i>Investigations</i>		SOC primary <i>Investigations</i>	10022891
MSSO Comment:				

Detail Report

CR Number: 2026096000		Implementation Date: 10-Apr-26	Related CR: 2026096000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Right ventricular outflow tract stenting</i>	Approved Not as Requested	Proposed PT <i>Right ventricular outflow tract stent insertion</i>	10093661
			HLT primary <i>Congenital cardiovascular anomaly therapeutic procedures</i>	10010401
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Right ventricular outflow tract stenting is approved but not as requested. Right ventricular outflow tract stenting is an interventional cardiology technique used in congenital heart disease with obstructed pulmonary blood flow, consisting of stent implantation in the right ventricular outflow tract to increase pulmonary blood flow and reduce hypoxemia, functioning as a less invasive bridging strategy to surgery. The proposed term will be modified for better alignment and added as PT Right ventricular outflow tract stent insertion to HLT Congenital cardiovascular anomaly therapeutic procedures.

CR Number: 2026097000		Implementation Date: 10-Apr-26	Related CR: 2026097000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Ischemic cerebellar stroke</i>	Approved as Requested	Proposed LLT <i>Ischemic cerebellar stroke</i>	10093662 Current
			To PT <i>Cerebellar stroke</i>	10079062

MSSO Comment: The proposal to add a new term Ischemic cerebellar stroke is approved as requested. An ischemic cerebellar stroke is caused by occlusion of the arterial blood supply (usually due to thrombosis or embolism), leading to reduced or interrupted blood flow and resulting in ischemia and tissue necrosis. It accounts for the majority of strokes (≈80–85%), whereas hemorrhagic strokes represent a smaller proportion (≈15–20%). Ischemic cerebellar stroke will be added as LLT to PT Cerebellar stroke. In a related change, its counterpart British spelling will be added as the LLT Ischaemic cerebellar stroke to the PT Cerebellar stroke.

CR Number: 2026100014		Implementation Date: 10-Apr-26	Related CR: 2026097000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Ischaemic cerebellar stroke</i>	Approved as Requested	Proposed LLT <i>Ischaemic cerebellar stroke</i>	10093663 Current
To PT	<i>Cerebellar stroke</i>		To PT <i>Cerebellar stroke</i>	10079062

MSSO Comment:

Detail Report

CR Number: 2026097001		Implementation Date: 10-Apr-26	Related CR: 2026097001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Posterior vertebral scalloping</i>	Approved Not as Requested	Proposed LLT	
HLT primary	<i>Musculoskeletal and connective tissue disorders of spine congenital</i>		<i>Posterior vertebral scalloping</i>	10093665 Current
SOC primary	<i>Congenital, familial and genetic disorders</i>		To PT	
			<i>Vertebral scalloping</i>	10093664
MSSO Comment: The proposal to add a new PT Posterior vertebral scalloping to primary HLT Musculoskeletal and connective tissue disorders of spine congenital in SOC Congenital, familial and genetic disorders is approved but not as requested. Posterior vertebral scalloping is a radiological sign characterized by an abnormal, smooth concavity of the posterior cortex of the vertebral body, best seen on lateral spine imaging. It reflects chronic remodeling of bone due to long-standing pressure or expansion within the spinal canal, such as dural ectasia, slow-growing intradural lesions, or conditions like neurofibromatosis type 1. Posterior vertebral scalloping will be added as LLT to new PT Vertebral scalloping. In a related change, PT Vertebral scalloping will be added to HLT Spine and neck deformities.				

CR Number: 2026100015		Implementation Date: 10-Apr-26	Related CR: 2026097001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Vertebral scalloping</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Spine and neck deformities</i>		<i>Vertebral scalloping</i>	10093664
SOC primary	<i>Musculoskeletal and connective tissue disorders</i>		HLT primary	
			<i>Spine and neck deformities</i>	10012140
			SOC primary	
			<i>Musculoskeletal and connective tissue disorders</i>	10028395

MSSO Comment:

CR Number: 2026097002		Implementation Date: 10-Apr-26	Related CR: 2026097002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Drug-induced vasculitis</i>	Rejected		

MSSO Comment: The proposal to add a new term Drug-induced vasculitis is not approved. The MSSO considers this proposed term to be of a "due to" nature. Terms that state or imply a "due to" concept are generally not added to MedDRA because causality is not inherent in the coded term but is instead determined by the reporter/investigator or by the sponsor.

Detail Report

CR Number: 2026097003	Implementation Date: 10-Apr-26	Related CR: 2026097003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Pulmonary vein repair</i>	Rejected	

MSSO Comment: The proposal to add a new term Pulmonary vein repair is not approved. The proposed concept, pulmonary vein repair is too broad and may be a part of numerous surgical procedures such as correction of congenital defects or pulmonary vein stenosis.

CR Number: 2026097004	Implementation Date: 10-Apr-26	Related CR: 2026097004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Konno-Rastan procedure</i>	Approved as Requested	Proposed PT <i>Konno-Rastan procedure</i> 10093666 HLT primary <i>Congenital cardiovascular anomaly therapeutic procedures</i> 10010401 SOC primary <i>Surgical and medical procedures</i> 10042613

MSSO Comment: The proposal to add a new term Konno-Rastan procedure is approved as requested. An anterior aortoventriculoplasty, known as the Konno-Rastan procedure, is an open-heart surgical technique used to enlarge a small or hypoplastic aortic annulus and the left ventricular outflow tract. Originally, described for congenital aortic stenosis secondary to small annular size, it also relieves subvalvar, valvar, and supravalvar stenosis. Konno-Rastan procedure will be added as PT to HLT Congenital cardiovascular anomaly therapeutic procedures.

CR Number: 2026097014	Implementation Date: 10-Apr-26	Related CR: 2026097014	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	<i>Twisted mouth</i>	Rejected	
HLT primary	<i>Facial cranial nerve disorders</i>		
SOC primary	<i>Nervous system disorders</i>		

MSSO Comment: The proposal to add a new PT Twisted mouth to primary HLT Facial cranial nerve disorders in SOC Nervous system disorders and secondary HLT Oral soft tissue disorders NEC in SOC Gastrointestinal disorders is not approved. Twisted mouth is not a standard medical term in English. The most appropriate English expression for the described situation is facial droop, which exists as LLT Facial droop to PT Facial paralysis.

Detail Report

CR Number: 2026097019		Implementation Date: 13-Apr-26	Related CR: 2026097019	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Soluble CD25 increased</i>	Approved as Requested	Proposed LLT <i>Soluble CD25 increased</i>	10093672 Current
			To PT <i>Interleukin-2 receptor increased</i>	10066315

MSSO Comment: The proposal to add a new term Soluble CD25 increased is approved as requested. Soluble Interleukin-2 receptor α chain, also referred to as soluble CD25, is the soluble form of the α -chain of the Interleukin-2 receptor, released into circulation during T-cell activation and used as a serum biomarker of immune activation. Soluble CD25 increased will be added as LLT to PT Interleukin-2 receptor increased. In a related change, LLT Soluble IL-2 receptor alpha increased will also be added to PT Interleukin-2 receptor increased for specificity.

CR Number: 2026100016		Implementation Date: 13-Apr-26	Related CR: 2026097019	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Soluble IL-2 receptor alpha increased</i>	Approved as Requested	Proposed LLT <i>Soluble IL-2 receptor alpha increased</i>	10093673 Current
To PT	<i>Interleukin-2 receptor increased</i>		To PT <i>Interleukin-2 receptor increased</i>	10066315

MSSO Comment:

CR Number: 2026098000		Implementation Date: 10-Apr-26	Related CR: 2026098000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Accident at Hospital</i>	Rejected		
To PT	<i>Accident</i>			

MSSO Comment: The proposal to add a new LLT Accident at Hospital to PT Accident is not approved. In the institutional hospital setting, the term 'Accident at Hospital' is used in a broader context, including medication errors, drug interactions etc., which makes the term ambiguous and confounding and may be used for incidents which are not clearly physical accidents.

Detail Report

CR Number: 2026098001		Implementation Date: 10-Apr-26	Related CR: 2026098001	
<u>Change Requested</u>	Link (move) a PT to another HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	SCORTEN score	Approved as Requested	Proposed PT	
From HLT	Investigations NEC		SCORTEN score	10081753
To HLT	Skin histopathology and skin investigations NEC		From HLT	
			Investigations NEC	10022892
			To HLT	
			Skin histopathology and skin investigations NEC	10087056

MSSO Comment: The proposal to move the PT SCORTEN score from HLT Investigations NEC to HLT Skin histopathology and skin investigations NEC is approved as requested. Although the SCORTEN score is built on multiple variables that are not directly related to cutaneous involvement, its underlying purpose is to assess the severity of severe dermatologic conditions such as Stevens-Johnson syndrome and toxic epidermal necrolysis.

CR Number: 2026098002		Implementation Date: 10-Apr-26	Related CR: 2026098002	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	Therapeutic use of walking aid	Approved as Requested	Proposed LLT	
To PT	Mobility support		Therapeutic use of walking aid	10093671 Current
			To PT	
			Mobility support	10092920

MSSO Comment: The proposal to add a new LLT Therapeutic use of walking aid to PT Mobility support is approved as requested. Based upon a review, LLT Walking aid support added in MedDRA 29.0 is potentially ambiguous. LLT Therapeutic use of walking aid is proposed for addition to clarify the concept as a therapeutic/supportive intervention to improve mobility. In a related change, LLT Walking aid support will be made non-current because of ambiguity of the concept.

CR Number: 2026100017		Implementation Date: 10-Apr-26	Related CR: 2026098002	
<u>Change Requested</u>	Change Status of LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
LLT to change	Walking aid support	Approved as Requested	LLT to change	
Status	Non-current		Walking aid support	10092924 Non-Current
			Status	
			Non-current	

MSSO Comment:

Detail Report

CR Number: 2026098003		Implementation Date: 10-Apr-26	Related CR: 2026098003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Incision site granuloma</i>	Approved as Requested	Proposed PT <i>Incision site granuloma</i>	10093668
			HLT primary <i>Non-site specific procedural complications</i>	10029559
			SOC primary <i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary <i>Granulomatous and deep cutaneous inflammatory conditions</i>	10018699
			SOC secondary <i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new term Incision site granuloma is approved as requested. Based upon a review, the concept Incision site granuloma will be added as new PT to primary HLT Non-site specific procedural complications and to secondary HLT Granulomatous and deep cutaneous inflammatory conditions.

CR Number: 2026098004		Implementation Date: 10-Apr-26	Related CR: 2026098004	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Psychedelic associated experience</i>	Approved as Requested	Proposed PT <i>Psychedelic associated experience</i>	10093669
HLT primary	<i>Perception disturbances NEC</i>		HLT primary <i>Perception disturbances NEC</i>	10082213
SOC primary	<i>Psychiatric disorders</i>		SOC primary <i>Psychiatric disorders</i>	10037175

MSSO Comment: The proposal to add a new PT Psychedelic associated experience to primary HLT Perception disturbances NEC in SOC Psychiatric disorders is approved as requested. Based upon a review, new PT Psychedelic associated experience will be added for clarity of the experience associated with psychedelics.

Detail Report

CR Number: 2026098005		Implementation Date: 10-Apr-26	Related CR: 2026098005	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Spiritual experience</i>	Approved as Requested	PT to merge	
To PT	<i>Psychedelic associated experience</i>		<i>Spiritual experience</i>	10093264 Non-Current
			To PT	
			<i>Psychedelic associated experience</i>	10093669

MSSO Comment: The proposal to demote the PT Spiritual experience under PT Psychedelic associated experience is approved as requested. Based upon a review, PT Spiritual experience will be demoted under new PT Psychedelic associated experience. In a related change, LLT Spiritual experience will be made non-current, as the term is broad and ambiguous without context.

CR Number: 2026100018		Implementation Date: 10-Apr-26	Related CR: 2026098005	
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #
LLT to change	<i>Spiritual experience</i>	Approved as Requested	LLT to change	
Status	<i>Non-current</i>		<i>Spiritual experience</i>	10093264 Non-Current
			Status	
			<i>Non-current</i>	

MSSO Comment:

Detail Report

CR Number: 2026098006		Implementation Date: 10-Apr-26	Related CR: 2026098006	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Staphylococcal folliculitis</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Staphylococcal infections</i>		<i>Staphylococcal folliculitis</i>	10093670
SOC primary	<i>Infections and infestations</i>		HLT primary	
			<i>Staphylococcal infections</i>	10041925
			SOC primary	
			<i>Infections and infestations</i>	10021881
			HLT secondary	
			<i>Skin and subcutaneous tissue infections NEC</i>	10040793
			SOC secondary	
			<i>Skin and subcutaneous tissue disorders</i>	10040785
MSSO The proposal to add a new PT Staphylococcal folliculitis to primary HLT Staphylococcal infections in SOC Infections and infestations is approved as requested.				
Comment: Staphylococcus aureus is a common cause of folliculitis. PT Staphylococcal folliculitis will also be added to secondary HLT Skin and subcutaneous tissue infections NEC.				

CR Number: 2026098007		Implementation Date: 10-Apr-26	Related CR: 2026098007		
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed LLT	<i>Staphylococcus aureus folliculitis</i>	Approved as Requested	Proposed LLT		
From PT	<i>Staphylococcal skin infection</i>		<i>Staphylococcus aureus folliculitis</i>	10093359 Current	
To PT	<i>Staphylococcal folliculitis</i>		From PT		
			<i>Staphylococcal skin infection</i>	10066409	
			To PT		
			<i>Staphylococcal folliculitis</i>	10093670	
MSSO The proposal to move the LLT Staphylococcus aureus folliculitis from PT Staphylococcal skin infection to PT Staphylococcal folliculitis is approved as requested. LLT					
Comment: Staphylococcus aureus folliculitis is better placed under new PT Staphylococcal folliculitis added in change request 2026098006.					

Detail Report

CR Number: 2026098008		Implementation Date: 10-Apr-26	Related CR: 2026098008	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Viral load detectable</i>	Approved as Requested	Proposed PT <i>Viral load detectable</i>	10093667
			HLT primary <i>Virus identification and serology</i>	10047490
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new PT Viral load detectable is approved as requested. A detectable viral load means that measurable amounts of a virus are present in a patient's blood or bodily fluid, indicating active viral replication. It represents an important medical information as it helps assess disease progression, treatment effectiveness, and the risk of transmission. Viral load detectable will be added as a non-pathogen qualified PT to HLT Virus identification and serology aligned with PT Viral load undetectable.

CR Number: 2026099000		Implementation Date: 24-Apr-26	Related CR: 2026099000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Centriole count decreased</i>	Rejected After Suspension		

MSSO Comment: The proposal to add a new term Centriole count decreased is not approved after suspension. MedDRA is intended for investigation results that are clinically measurable or interpretable for clinical practice. "Centriole count decreased" does not meet these criteria. The term is not a standardized parameter, as there is no routine measurement or reference range. The concept is almost exclusively limited to basic research and is highly specific in context of cell biology.

CR Number: 2026099002		Implementation Date: 10-Apr-26	Related CR: 2026099002	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Sleep related hypermotor epilepsy</i>	Approved as Requested	Term to modify <i>Sleep related hypermotor epilepsy</i>	10086468
Replacement term	<i>Sleep-related hypermotor epilepsy</i>		Replacement term <i>Sleep-related hypermotor epilepsy</i>	10086468

MSSO Comment: The proposal to rename PT/LLT Sleep related hypermotor epilepsy to Sleep-related hypermotor epilepsy is approved as requested. For consistency of hyphen placement with other "sleep-related" terms, a hyphen will be added to the existing LLT/PT Sleep related hypermotor epilepsy.

Detail Report

CR Number: 2026099003		Implementation Date: 10-Apr-26	Related CR: 2026099003	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Transfusion related complication</i>	Approved as Requested	Term to modify	
Replacement term	<i>Transfusion-related complication</i>		<i>Transfusion related complication</i>	10071090
			Replacement term	
			<i>Transfusion-related complication</i>	10071090

MSSO Comment: The proposal to rename PT/LLT Transfusion related complication to Transfusion-related complication is approved as requested. For consistency of hyphen placement with other "transfusion-related" terms, a hyphen will be added to the existing LLT/PT Transfusion related complication.

CR Number: 2026099006		Implementation Date: 16-Apr-26	Related CR: 2026099006	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Perimenopausal disorder</i>	Rejected		

MSSO Comment: The proposal to add a new term Perimenopausal disorder is not approved. Perimenopause refers to the transitional phase leading up to menopause, characterized by hormonal fluctuations and associated symptoms. The concept is represented in MedDRA by LLT Perimenopausal symptoms.

CR Number: 2026100000		Implementation Date: 30-Apr-26	Related CR: 2026100000	
Change Requested	Unlink a PT from a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Immunodeficiency common variable</i>	Rejected After Suspension		
From HLT	<i>Primary immunodeficiency syndromes</i>			

MSSO Comment: The proposal to unlink the PT Immunodeficiency common variable from the HLT Primary immunodeficiency syndromes is not approved after suspension. Common variable immunodeficiency is clearly a primary immunodeficiency and is correctly placed. However, in a related change, LLT Acquired hypogammaglobulinaemia and LLT Acquired hypogammaglobulinemia, which are broad concepts, will both be moved from PT Immunodeficiency common variable to PT Hypogammaglobulinaemia for better alignment.

Detail Report

CR Number: 2026120002		Implementation Date: 30-Apr-26	Related CR: 2026100000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Acquired hypogammaglobulinaemia</i>	Approved as Requested	Proposed LLT	10000557 Current
From PT	<i>Immunodeficiency common variable</i>		<i>Acquired hypogammaglobulinaemia</i>	
To PT	<i>Hypogammaglobulinaemia</i>		From PT	10021449
			<i>Immunodeficiency common variable</i>	
			To PT	10020983
			<i>Hypogammaglobulinaemia</i>	

MSSO
Comment:

CR Number: 2026120003		Implementation Date: 30-Apr-26	Related CR: 2026100000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Acquired hypogammaglobulinemia</i>	Approved as Requested	Proposed LLT	10000558 Current
From PT	<i>Immunodeficiency common variable</i>		<i>Acquired hypogammaglobulinemia</i>	
To PT	<i>Hypogammaglobulinaemia</i>		From PT	10021449
			<i>Immunodeficiency common variable</i>	
			To PT	10020983
			<i>Hypogammaglobulinaemia</i>	

MSSO
Comment:

CR Number: 2026100013		Implementation Date: 16-Apr-26	Related CR: 2026100013	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Non-specific white matter change</i>	Rejected		

MSSO
Comment: The proposal to add a new term Non-specific white matter change is not approved. MedDRA refrains from adding unspecific terms, when a more precise "disorder" terms exists. If the nature of the white matter changes is not known, then the corresponding "disorder" or "disease" term can be used for coding, e.g. LLTs linked to PT White matter lesion, depending on your coding scenario.

Detail Report

CR Number: 2026101000		Implementation Date: 16-Apr-26	Related CR: 2026101000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Portal vein stent placement</i>	Approved as Requested	Proposed PT <i>Portal vein stent placement</i>	10093674
			HLT primary <i>Hepatic therapeutic procedures</i>	10019704
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Portal vein stent placement is approved as requested. A portal vein stent is a minimally invasive way to re-establish portal flow in focal obstruction, particularly useful after liver transplantation or in malignant compression, with good technical success but a need for imaging follow-up and thrombosis prevention. Portal vein stent placement will be added as PT to the HLT Hepatic therapeutic procedures. In a related change, PT Portal vein reconstruction will be linked from HLT Venous therapeutic procedures to HLT Hepatic therapeutic procedures, as the most relevant clinical context is the liver and its portal circulation, not the general venous system.

CR Number: 2026106002		Implementation Date: 16-Apr-26	Related CR: 2026101000	
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Portal vein reconstruction</i>	Approved as Requested	Proposed PT <i>Portal vein reconstruction</i>	10080966
From HLT	<i>Venous therapeutic procedures</i>		From HLT <i>Venous therapeutic procedures</i>	10047231
To HLT	<i>Hepatic therapeutic procedures</i>		To HLT <i>Hepatic therapeutic procedures</i>	10019704

MSSO Comment:

CR Number: 2026103000		Implementation Date: 30-Apr-26	Related CR: 2026103000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Aortic root fluid collection</i>	Rejected After Suspension		

MSSO Comment: The proposal to add a new term Aortic root fluid collection is not approved after suspension. The proposed concept is an unspecified aortic root fluid collection and is one of the possible descriptions of an imaging finding that indicates a build-up of fluid around the base of the aorta, where it connects to the heart. Often the causes vary and as it is not uniformly used internationally. Please use the most appropriate existing broad term, depending on your specific case scenario.

Detail Report

CR Number: 2026103001		Implementation Date: 16-Apr-26	Related CR: 2026103001	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Rhesus antibodies (anti D) negative</i>	Approved as Requested	Term to modify	
Replacement term	<i>Rhesus antibodies (anti-D) negative</i>		<i>Rhesus antibodies (anti D) negative</i>	10076519
			Replacement term	
			<i>Rhesus antibodies (anti-D) negative</i>	10076519 Current
MSSO The proposal to rename PT/LLT Rhesus antibodies (anti D) negative to Rhesus antibodies (anti-D) negative is approved as requested. For consistency with existing similar				
Comment: terms, a hyphen will be added in existing term Rhesus antibodies (anti D) negative, between "anti" and "D".				

CR Number: 2026103002		Implementation Date: 16-Apr-26	Related CR: 2026103002	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Rhesus antibodies (anti D) positive</i>	Approved as Requested	Term to modify	
Replacement term	<i>Rhesus antibodies (anti-D) positive</i>		<i>Rhesus antibodies (anti D) positive</i>	10076518
			Replacement term	
			<i>Rhesus antibodies (anti-D) positive</i>	10076518 Current
MSSO The proposal to rename PT/LLT Rhesus antibodies (anti D) positive to Rhesus antibodies (anti-D) positive is approved as requested. For consistency with existing similar				
Comment: terms, a hyphen will be added in existing term Rhesus antibodies (anti D) positive, between "anti" and "D".				

CR Number: 2026103003		Implementation Date: 16-Apr-26	Related CR: 2026103003	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Rhesus antibodies (anti E)</i>	Approved as Requested	Term to modify	
Replacement term	<i>Rhesus antibodies (anti-E)</i>		<i>Rhesus antibodies (anti E)</i>	10093527
			Replacement term	
			<i>Rhesus antibodies (anti-E)</i>	10093527 Current
MSSO The proposal to rename PT/LLT Rhesus antibodies (anti E) to Rhesus antibodies (anti-E) is approved as requested. For consistency with existing similar terms, a hyphen				
Comment: will be added in existing term Rhesus antibodies (anti E), between "anti" and "E".				

Detail Report

CR Number: 2026103004		Implementation Date: 16-Apr-26	Related CR: 2026103004	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Rhesus antibodies (anti E) negative</i>	Approved as Requested	Term to modify	
Replacement term	<i>Rhesus antibodies (anti-E) negative</i>		<i>Rhesus antibodies (anti E) negative</i>	10093528
			Replacement term	
			<i>Rhesus antibodies (anti-E) negative</i>	10093528 Current
MSSO The proposal to rename PT/LLT Rhesus antibodies (anti E) negative to Rhesus antibodies (anti-E) negative is approved as requested. For consistency with existing similar				
Comment: terms, a hyphen will be added in existing term Rhesus antibodies (anti E) negative, between "anti" and "E".				

CR Number: 2026103005		Implementation Date: 16-Apr-26	Related CR: 2026103005	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Rhesus antibodies (anti E) positive</i>	Approved as Requested	Term to modify	
Replacement term	<i>Rhesus antibodies (anti-E) positive</i>		<i>Rhesus antibodies (anti E) positive</i>	10093529
			Replacement term	
			<i>Rhesus antibodies (anti-E) positive</i>	10093529 Current
MSSO The proposal to rename PT/LLT Rhesus antibodies (anti E) positive to Rhesus antibodies (anti-E) positive is approved as requested. For consistency with existing similar				
Comment: terms, a hyphen will be added in existing term Rhesus antibodies (anti E) positive, between "anti" and "E".				

Detail Report

CR Number: 2026103006		Implementation Date: 16-Apr-26	Related CR: 2026103006		
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed LLT	<i>Inverse psoriasis</i>	Approved as Requested	Proposed LLT		
From PT	<i>Psoriasis</i>		<i>Inverse psoriasis</i>	10063160	
HLT primary	<i>Psoriatic conditions</i>		From PT		
			<i>Psoriasis</i>	10037153	
			HLT primary		
			<i>Psoriatic conditions</i>	10065874	
			SOC primary		
			<i>Skin and subcutaneous tissue disorders</i>	10040785	
HLT secondary			HLT secondary		
			<i>Immune and associated conditions NEC</i>	10027682	
			SOC secondary		
			<i>Immune system disorders</i>	10021428	

MSSO Comment: The proposal to promote the LLT Inverse psoriasis from PT Psoriasis to primary HLT Psoriatic conditions in SOC Skin and subcutaneous tissue disorders and secondary HLT Immune and associated conditions NEC in SOC Immune system disorders is approved as requested, as Inverse psoriasis is a characteristic type of clinical presentation of psoriasis. In related changes, LLT Interdigital psoriasis, LLT Flexural psoriasis, LLT Perianal psoriasis and LLT Psoriasis genital will be moved from PT Psoriasis to new PT Inverse psoriasis. Inverse psoriasis (also known as Flexural psoriasis) produces red, well-defined areas in skin folds (flexures) such as the armpits, between the buttocks and under the breasts. Scaling is minimal or absent. This type of psoriasis can be frequently irritated by rubbing and sweating due to its location in the skin folds and other tender areas. Such areas can also be prone to yeast or fungal infections, which might cause confusion in diagnosis.

CR Number: 2026106007		Implementation Date: 16-Apr-26	Related CR: 2026103006		
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed LLT	<i>Flexural psoriasis</i>	Approved as Requested	Proposed LLT		
From PT	<i>Psoriasis</i>		<i>Flexural psoriasis</i>	10016776 Current	
To PT	<i>Inverse psoriasis</i>		From PT		
			<i>Psoriasis</i>	10037153	
			To PT		
			<i>Inverse psoriasis</i>	10063160	

MSSO Comment:

Detail Report

CR Number: 2026106009		Implementation Date: 16-Apr-26	Related CR: 2026103006	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Psoriasis genital</i>	Approved as Requested	Proposed LLT	10063407 Current
From PT	<i>Psoriasis</i>		<i>Psoriasis genital</i>	
To PT	<i>Inverse psoriasis</i>		From PT	10037153
			<i>Psoriasis</i>	
			To PT	10063160
			<i>Inverse psoriasis</i>	

MSSO
Comment:

CR Number: 2026106006		Implementation Date: 16-Apr-26	Related CR: 2026103006	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Interdigital psoriasis</i>	Approved as Requested	Proposed LLT	10092928 Current
From PT	<i>Psoriasis</i>		<i>Interdigital psoriasis</i>	
To PT	<i>Inverse psoriasis</i>		From PT	10037153
			<i>Psoriasis</i>	
			To PT	10063160
			<i>Inverse psoriasis</i>	

MSSO
Comment:

Detail Report

CR Number: 2026106008		Implementation Date: 16-Apr-26	Related CR: 2026103006	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Perianal psoriasis</i>	Approved as Requested	Proposed LLT	10077411 Current
From PT	<i>Psoriasis</i>		<i>Perianal psoriasis</i>	
To PT	<i>Inverse psoriasis</i>		From PT	10037153
			<i>Psoriasis</i>	
			To PT	10063160
			<i>Inverse psoriasis</i>	

MSSO
Comment:

CR Number: 2026104000		Implementation Date: 16-Apr-26	Related CR: 2026104000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Prescription forgery</i>	Approved Not as Requested	Proposed LLT	10093676 Current
HLT primary	<i>Criminal activity</i>		<i>Prescription forgery</i>	
SOC primary	<i>Social circumstances</i>		To PT	10093675
			<i>Prescription fraud</i>	

MSSO
Comment: The proposal to add a new PT Prescription forgery to primary HLT Criminal activity in SOC Social circumstances is approved but not as requested. Prescription forgery is the act of creating or using a falsified prescription that appears to be issued by a legitimate healthcare provider without their authorization in order to obtain medication unlawfully. In legal and healthcare compliance contexts, "prescription fraud" is the charge category, while "forgery" identifies the specific method used. Prescription forgery will be added as LLT to a new PT Prescription fraud. In a related change, PT Prescription fraud will be added to HLT Criminal activity. Furthermore, closely related concepts PT Prescription form tampering and PT Illicit prescription attainment will both be demoted to new PT Prescription fraud to prevent signal dispersion.

CR Number: 2026106004		Implementation Date: 16-Apr-26	Related CR: 2026104000	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Prescription form tampering</i>	Approved as Requested	PT to merge	10067669 Current
To PT	<i>Prescription fraud</i>		<i>Prescription form tampering</i>	
			To PT	10093675
			<i>Prescription fraud</i>	

MSSO
Comment:

Detail Report

CR Number: 2026106003		Implementation Date: 16-Apr-26	Related CR: 2026104000	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Prescription fraud</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Criminal activity</i>		<i>Prescription fraud</i>	10093675
SOC primary	<i>Social circumstances</i>		HLT primary	
			<i>Criminal activity</i>	10011391
			SOC primary	
			<i>Social circumstances</i>	10041244

MSSO
Comment:

CR Number: 2026106005		Implementation Date: 16-Apr-26	Related CR: 2026104000	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Illicit prescription attainment</i>	Approved as Requested	PT to merge	
To PT	<i>Prescription fraud</i>		<i>Illicit prescription attainment</i>	10090768 Current
			To PT	
			<i>Prescription fraud</i>	10093675

MSSO
Comment:

CR Number: 2026105000		Implementation Date: 16-Apr-26	Related CR: 2026105000	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	<i>Alcohol-induced hepatomegaly</i>	Rejected		

MSSO
Comment: The proposal to add a new term Alcohol-induced hepatomegaly is not approved. Alcoholic liver disease is a term that encompasses hepatic manifestations of alcohol overconsumption, including fatty liver, alcoholic hepatitis, and chronic hepatitis with liver fibrosis or cirrhosis. MedDRA cannot accommodate a term for every potential combination of multiple concepts. The requested term, Alcohol-induced hepatomegaly can be represented by split coding with existing terms LLT Alcoholic liver disease and LLT Hepatomegaly, depending on your specific scenario.

Detail Report

CR Number: 2026106000		Implementation Date: 30-Apr-26	Related CR: 2026106000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Pituitary neuroendocrine tumour</i>	Approved After Suspension	Proposed PT <i>Pituitary neuroendocrine tumour</i>	10093691 Current
			HLT primary <i>Endocrine neoplasms benign NEC</i>	10014711
			SOC primary <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary <i>Pituitary neoplasms</i>	10035098
			SOC secondary <i>Endocrine disorders</i>	10014698
MSSO Comment: The proposal to add a new term Pituitary neuroendocrine tumour is approved as requested after suspension. Pituitary neuroendocrine tumors are part of pituitary adenomas which are mostly classified as benign, while part behave aggressively, these tumors are classified as pituitary neuroendocrine tumors. This classification was adopted by the WHO, but is under controversial discussion. As the new classification is still uncertain (particularly, whether high-grade malignancies of pituitary neuroendocrine tumors qualify as neuroendocrine carcinoma), Pituitary neuroendocrine tumour will be added as PT to primary HLT Endocrine neoplasms benign NEC and to secondary HLT Pituitary neoplasms. In a related change, the American English spelled counterpart will also be added as LLT Pituitary neuroendocrine tumor to PT Pituitary neuroendocrine tumour. Futhermore, LLT Pituitary adenoma, LLT Pituitary macroadenoma, and LLT Pituitary microadenoma will all be moved from PT Pituitary tumour benign to new PT Pituitary neuroendocrine tumour.				

CR Number: 2026120004		Implementation Date: 30-Apr-26	Related CR: 2026106000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pituitary neuroendocrine tumor</i>	Approved as Requested	Proposed LLT <i>Pituitary neuroendocrine tumor</i>	10093692 Current
To PT	<i>Pituitary neuroendocrine tumour</i>		To PT <i>Pituitary neuroendocrine tumour</i>	10093691 Current
MSSO Comment:				

Detail Report

CR Number: 2026120006		Implementation Date: 30-Apr-26	Related CR: 2026106000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pituitary macroadenoma</i>	Approved as Requested	Proposed LLT	
From PT	<i>Pituitary tumour benign</i>		<i>Pituitary macroadenoma</i>	10035094 Current
To PT	<i>Pituitary neuroendocrine tumour</i>		From PT	
			<i>Pituitary tumour benign</i>	10061538
			To PT	
			<i>Pituitary neuroendocrine tumour</i>	10093691 Current

MSSO
Comment:

CR Number: 2026120007		Implementation Date: 30-Apr-26	Related CR: 2026106000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pituitary microadenoma</i>	Approved as Requested	Proposed LLT	
From PT	<i>Pituitary tumour benign</i>		<i>Pituitary microadenoma</i>	10035095 Current
To PT	<i>Pituitary neuroendocrine tumour</i>		From PT	
			<i>Pituitary tumour benign</i>	10061538
			To PT	
			<i>Pituitary neuroendocrine tumour</i>	10093691 Current

MSSO
Comment:

Detail Report

CR Number: 2026120005		Implementation Date: 30-Apr-26	Related CR: 2026106000	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pituitary adenoma</i>	Approved as Requested	Proposed LLT	
From PT	<i>Pituitary tumour benign</i>		<i>Pituitary adenoma</i>	10035079
To PT	<i>Pituitary neuroendocrine tumour</i>		From PT	
			<i>Pituitary tumour benign</i>	10061538
			To PT	
			<i>Pituitary neuroendocrine tumour</i>	10093691 Current

MSSO
Comment:

CR Number: 2026106001		Implementation Date: 20-Apr-26	Related CR: 2026106001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Diabetic foot osteomyelitis</i>	Rejected		

MSSO
Comment: The proposal to add a new term Diabetic foot osteomyelitis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

CR Number: 2026106010		Implementation Date: 24-Apr-26	Related CR: 2026106010	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Vulvar excoriation</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Vulvovaginal injury</i>		<i>Vulvar excoriation</i>	10093680 Current
			To PT	
			<i>Vulvar erosion</i>	10062559

MSSO
Comment: The proposal to add a new LLT Vulvar excoriation to PT Vulvovaginal injury is approved but not as requested. Vulvar excoriation is a superficial lesion of the vulvar epithelium caused by scratching or friction, often associated with local pruritus and resulting in partial loss of the superficial skin layer. Although they are not fully equivalent concepts, most terms related to excoriation are subordinated to erosions terms in MedDRA, and therefore Vulvar excoriation will be added as LLT to PT Vulvar erosion.

Detail Report

CR Number: 2026106011	Implementation Date: 24-Apr-26	Related CR: 2026106011	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>B-cell lymphoproliferative disorder</i>	Rejected	

MSSO Comment: The proposal to add a new term B-cell lymphoproliferative disorder is not approved. B-cell lymphoproliferative disorders is a nonspecific concept and represents a wide variety of disorders because they may represent either a clonal process (e.g., lymphoma) or a polyclonal reactive expansion (e.g., infection-driven), with overlapping clinical and biological features. The proposed concept already exists with the broader concept LLT/PT Lymphoproliferative disorder.

CR Number: 2026106012	Implementation Date: 24-Apr-26	Related CR: 2026106012	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Non-healing ulcers</i>	Rejected	

MSSO Comment: The proposal to add a new term Non-healing ulcers is not approved. The proposed concept is represented in MedDRA in several anatomically specified terms including chronic ulcer. Please consider terms such as LLT Chronic skin ulcer, LLT Chronic peptic ulcer or LLT Chronic leg ulcer depending on your specific coding scenario.

CR Number: 2026106013	Implementation Date: 24-Apr-26	Related CR: 2026106013	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Non-healing wounds</i>	Rejected	

MSSO Comment: The proposal to add a new term Non-healing wounds is not approved. The concept is represented in MedDRA by LLTs and PTs linked to HLT Healing abnormal NEC, e.g. LLT Wound healing delayed.

Detail Report

CR Number: 2026107000		Implementation Date: 24-Apr-26	Related CR: 2026107000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Creatine test</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Metabolism tests NEC</i>		<i>Creatine test</i>	10093690
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Metabolism tests NEC</i>	10027434
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new PT Creatine test to primary HLT Metabolism tests NEC in SOC Investigations is approved as requested. Creatine test will be added as new PT without qualification of specimen type aligned with the Introductory Guide principles in which new concepts are added as non-specimen specific terms with any specimen terms at LLT level under the corresponding PT term concept. As well, the concept will be linked to HLT Metabolism tests NEC, which best represents use of this test for intermediate metabolites, organic acids, metabolic profiles, etc.

CR Number: 2026107001		Implementation Date: 24-Apr-26	Related CR: 2026107001	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Blood creatine</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine test</i>		<i>Blood creatine</i>	10005461 Current
			To PT	
			<i>Creatine test</i>	10093690

MSSO Comment: The proposal to demote the PT Blood creatine under PT Creatine test is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under a new set of non-specimen-specific PTs.

CR Number: 2026107002		Implementation Date: 24-Apr-26	Related CR: 2026107002	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Creatine urine</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine test</i>		<i>Creatine urine</i>	10011353 Current
			To PT	
			<i>Creatine test</i>	10093690

MSSO Comment: The proposal to demote the PT Creatine urine under PT Creatine test is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under a new set of non-specimen-specific PTs.

Detail Report

CR Number: 2026107003		Implementation Date: 28-Apr-26	Related CR: 2026107003	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Creatine abnormal</i>	Approved as Requested	Proposed LLT	
From PT	<i>Blood creatine abnormal</i>		<i>Creatine abnormal</i>	10011329
HLT primary	<i>Metabolism tests NEC</i>		From PT	
			<i>Blood creatine abnormal</i>	10005462 Current
			HLT primary	
			<i>Metabolism tests NEC</i>	10027434
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to promote the LLT Creatine abnormal from PT Blood creatine abnormal to primary HLT Metabolism tests NEC in SOC Investigations is approved as requested. Aligned with the Introductory Guide principles laboratory concepts are added with the non-specimen specific terms at PT level. As well, the concept will be linked to HLT Metabolism tests NEC, which best represents use of this test for intermediate metabolites, organic acids, metabolic profiles, etc.

CR Number: 2026107004		Implementation Date: 28-Apr-26	Related CR: 2026107004	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Blood creatine abnormal</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine abnormal</i>		<i>Blood creatine abnormal</i>	10005462 Current
			To PT	
			<i>Creatine abnormal</i>	10011329

MSSO Comment: The proposal to demote the PT Blood creatine abnormal under PT Creatine abnormal is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under non-specimen-specific PTs.

CR Number: 2026107005		Implementation Date: 28-Apr-26	Related CR: 2026107005	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Creatine urine abnormal</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine abnormal</i>		<i>Creatine urine abnormal</i>	10011354 Current
			To PT	
			<i>Creatine abnormal</i>	10011329

MSSO Comment: The proposal to demote the PT Creatine urine abnormal under PT Creatine abnormal is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under a non-specimen-specific PT.

Detail Report

CR Number: 2026107006		Implementation Date: 28-Apr-26	Related CR: 2026107006		
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed LLT	<i>Creatine decreased</i>	Approved as Requested	Proposed LLT		
From PT	<i>Blood creatine decreased</i>		<i>Creatine decreased</i>	10011331	
HLT primary	<i>Metabolism tests NEC</i>		From PT		
			<i>Blood creatine decreased</i>	10005463 Current	
			HLT primary		
			<i>Metabolism tests NEC</i>	10027434	
			SOC primary		
			<i>Investigations</i>	10022891	

MSSO Comment: The proposal to promote the LLT Creatine decreased from PT Blood creatine decreased to primary HLT Metabolism tests NEC in SOC Investigations is approved as requested. Aligned with the Introductory Guide principles, laboratory concepts are added with the non-specimen specific terms at PT level. As well, the concept will be linked to HLT Metabolism tests NEC, which best represents use of this test for intermediate metabolites, organic acids, metabolic profiles, etc.

CR Number: 2026107007		Implementation Date: 28-Apr-26	Related CR: 2026107007	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Blood creatine decreased</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine decreased</i>		<i>Blood creatine decreased</i>	10005463 Current
			To PT	
			<i>Creatine decreased</i>	10011331

MSSO Comment: The proposal to demote the PT Blood creatine decreased under PT Creatine decreased is approved as requested. This demotion is proposed to improve the placement of "creatinine" concepts in MedDRA under a set of non-specimen-specific PTs.

CR Number: 2026107008		Implementation Date: 28-Apr-26	Related CR: 2026107008	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Creatine urine decreased</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine decreased</i>		<i>Creatine urine decreased</i>	10011355 Current
			To PT	
			<i>Creatine decreased</i>	10011331

MSSO Comment: The proposal to demote the PT Creatine urine decreased under PT Creatine decreased is approved as requested. This demotion is proposed to improve the placement of "creatinine" concepts in MedDRA under a set of non-specimen-specific PTs.

Detail Report

CR Number: 2026107009		Implementation Date: 28-Apr-26	Related CR: 2026107009	MedDRA Code #
Change Requested	Promote an LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Creatine increased</i>	Approved as Requested	Proposed LLT	
From PT	<i>Blood creatine increased</i>		<i>Creatine increased</i>	10011333
HLT primary	<i>Metabolism tests NEC</i>		From PT	
			<i>Blood creatine increased</i>	10005464 Current
			HLT primary	
			<i>Metabolism tests NEC</i>	10027434
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to promote the LLT Creatine increased from PT Blood creatine increased to primary HLT Metabolism tests NEC in SOC Investigations is approved as requested. Aligned with the Introductory Guide principles, laboratory concepts are added with the non-specimen specific terms at PT level. As well, the concept will be linked to HLT Metabolism tests NEC, which best represents use of this test for intermediate metabolites, organic acids, metabolic profiles, etc.

CR Number: 2026107010		Implementation Date: 28-Apr-26	Related CR: 2026107010	MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement	
PT to merge	<i>Blood creatine increased</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine increased</i>		<i>Blood creatine increased</i>	10005464 Current
			To PT	
			<i>Creatine increased</i>	10011333

MSSO Comment: The proposal to demote the PT Blood creatine increased under PT Creatine increased is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under a set of non-specimen-specific PTs.

CR Number: 2026107011		Implementation Date: 28-Apr-26	Related CR: 2026107011	MedDRA Code #
Change Requested	Demote a PT	Final Disposition	Final Placement	
PT to merge	<i>Creatine urine increased</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine increased</i>		<i>Creatine urine increased</i>	10011356 Current
			To PT	
			<i>Creatine increased</i>	10011333

MSSO Comment: The proposal to demote the PT Creatine urine increased under PT Creatine increased is approved as requested. This demotion is proposed to improve the placement of "creatine" concepts in MedDRA under a set of non-specimen-specific PTs.

Detail Report

CR Number: 2026107012		Implementation Date: 28-Apr-26	Related CR: 2026107012	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Creatine normal</i>	Approved as Requested	Proposed LLT	
From PT	<i>Blood creatine normal</i>		<i>Creatine normal</i>	10011346
HLT primary	<i>Metabolism tests NEC</i>		From PT	
			<i>Blood creatine normal</i>	10005466 Current
			HLT primary	
			<i>Metabolism tests NEC</i>	10027434
			SOC primary	
			<i>Investigations</i>	10022891
MSSO The proposal to promote the LLT Creatine normal from PT Blood creatine normal to primary HLT Metabolism tests NEC in SOC Investigations is approved as requested.				
Comment: Aligned with the Introductory Guide principles, laboratory concepts are added with the non-specimen specific terms at PT level. As well, the concept will be linked to HLT Metabolism tests NEC, which best represents use of this test for intermediate metabolites, organic acids, metabolic profiles, etc.				

CR Number: 2026107013		Implementation Date: 28-Apr-26	Related CR: 2026107013	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Blood creatine normal</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine normal</i>		<i>Blood creatine normal</i>	10005466 Current
			To PT	
			<i>Creatine normal</i>	10011346
MSSO The proposal to demote the PT Blood creatine normal under PT Creatine normal is approved as requested. This demotion is proposed to improve the placement of				
Comment: "creatine" concepts in MedDRA under a set of non-specimen-specific PTs.				

CR Number: 2026107014		Implementation Date: 28-Apr-26	Related CR: 2026107014	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Creatine urine normal</i>	Approved as Requested	PT to merge	
To PT	<i>Creatine normal</i>		<i>Creatine urine normal</i>	10011357 Current
			To PT	
			<i>Creatine normal</i>	10011346
MSSO The proposal to demote the PT Creatine urine normal under PT Creatine normal is approved as requested. This demotion is proposed to improve the placement of				
Comment: "creatine" concepts in MedDRA under a set of non-specimen-specific PTs.				

Detail Report

CR Number: 2026107015		Implementation Date: 24-Apr-26	Related CR: 2026107015	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Radiosynoviorthesis</i>	Approved Not as Requested	Proposed LLT <i>Radiosynoviorthesis</i>	10093681 Current
			To PT <i>Synoviorthesis</i>	10048875
MSSO Comment: The proposal to add a new PT Radiosynoviorthesis is approved but not as requested. Synoviorthesis is the general term for using intra-articular injections to reduce inflamed synovium. Radiosynoviorthesis is a subset of this, specifically using radioactive materials instead of chemical agents. Radiosynoviorthesis will be added as sub-concept LLT to PT Synoviorthesis aligned with LLT Radiation synovectomy.				

CR Number: 2026107016		Implementation Date: 24-Apr-26	Related CR: 2026107016	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Scabies impetiginous</i>	Rejected		
MSSO Comment: The proposal to add a new PT Scabies impetiginous is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. Please consider LLT/PT Scabies, LLT/PT Impetigo, LLT/PT Superinfection bacteria and LLT Secondary impetigo depending on your case scenario.				

CR Number: 2026107017		Implementation Date: 24-Apr-26	Related CR: 2026107017	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Transplant infection</i>	Rejected		
MSSO Comment: The proposal to add a new term Transplant infection is not approved. The proposed concept exists already in LLT/PT Graft infection. Graft refers to any tissue or organ for implantation or transplantation.				

Detail Report

CR Number: 2026107018	Implementation Date: 24-Apr-26	Related CR: 2026107018	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Transplant infection	Rejected	

MSSO Comment: The proposal to add a new PT Transplant infection is not approved. Technical rejection as proposed term was already submitted in change request 2026107017.

CR Number: 2026107019	Implementation Date: 24-Apr-26	Related CR: 2026107019	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Obstructive coronary artery disease	Approved as Requested	Proposed LLT
To PT	Coronary artery disease		Obstructive coronary artery disease 10093678 Current
			To PT
			Coronary artery disease 10011078

MSSO Comment: The proposal to add a new LLT Obstructive coronary artery disease to PT Coronary artery disease is approved as requested. Obstructive coronary artery disease is when atherosclerosis occurs in the coronary arteries and is generally defined as stenosis of 50% or more of the left main coronary artery or stenosis of 70% or more of a major epicardial or branch vessel.

CR Number: 2026107020	Implementation Date: 24-Apr-26	Related CR: 2026107020	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Cervical facet joint syndrome	Approved as Requested	Proposed LLT
To PT	Facet joint syndrome		Cervical facet joint syndrome 10093679 Current
			To PT
			Facet joint syndrome 10054813

MSSO Comment: The proposal to add a new LLT Cervical facet joint syndrome to PT Facet joint syndrome is approved as requested. Cervical facet joint syndrome is a clinical condition characterized by neck pain arising from dysfunction or degeneration of the cervical facet (zygapophyseal) joints.

Detail Report

CR Number: 2026107021		Implementation Date: 24-Apr-26	Related CR: 2026107021	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Thoracic aortic pseudoaneurysm</i>	Approved as Requested	Proposed LLT	
To PT	<i>Aortic pseudoaneurysm</i>		<i>Thoracic aortic pseudoaneurysm</i>	10093677 Current
			To PT	
			<i>Aortic pseudoaneurysm</i>	10082505
MSSO Comment: The proposal to add a new LLT Thoracic aortic pseudoaneurysm to PT Aortic pseudoaneurysm is approved as requested. A thoracic aortic pseudoaneurysm is a contained rupture of the aorta where blood leaks out but is held by surrounding tissue, creating a high-risk, often fatal condition. Common causes include trauma, previous cardiac surgery, or infection.				

CR Number: 2026107022		Implementation Date: 24-Apr-26	Related CR: 2026107022	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Transplant abscess</i>	Rejected		
To PT	<i>Transplant infection</i>			
MSSO Comment: The proposal to demote the PT Transplant abscess under PT Transplant infection is not approved. A transplant abscess is a localized, walled-off collection of pus while a transplant infection is a broad term encompassing any invasion of microorganisms, which may be systemic or localized, treated primarily with antimicrobials.				

CR Number: 2026107023		Implementation Date: 24-Apr-26	Related CR: 2026107023	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pyelonephritis of renal graft</i>	Rejected		
From PT	<i>Pyelonephritis</i>			
To PT	<i>Graft infection</i>			
MSSO Comment: The proposal to move the LLT Pyelonephritis of renal graft from PT Pyelonephritis to PT Graft infection is not approved. As the diagnosis should take precedence, the placement of LLT Pyelonephritis of renal graft will remain under PT Pyelonephritis.				

Detail Report

CR Number: 2026108000		Implementation Date: 24-Apr-26	Related CR: 2026108000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Rhizomelic chondrodysplasia punctata</i>	Approved Not as Requested	Proposed LLT <i>Rhizomelic chondrodysplasia punctata</i>	10093682 Current
			To PT <i>Chondrodysplasia punctata</i>	10087150
MSSO Comment: The proposal to add a new PT Rhizomelic chondrodysplasia punctata is approved but not as requested. Chondrodysplasia punctata occurs in two forms, the autosomal dominant form also known as Conradi Hunermann syndrome and the severe autosomal recessive form known as rhizomelic chondrodysplasia punctata. Babies with the latter form commonly have rhizomelia of the limbs, both these types show X-ray features of multiple, small foci of punctate calcification at knees, humerus, spinal column and soft tissues of laryngeal cartilages and trachea. Rhizomelic chondrodysplasia punctata will be added as sub-concept LLT to PT Chondrodysplasia punctata.				

CR Number: 2026110000		Implementation Date: 24-Apr-26	Related CR: 2026110000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Inferior alveolar neuritis</i>	Approved Not as Requested	Proposed PT <i>Inferior alveolar nerve injury</i>	10093683
			HLT primary <i>Gastrointestinal and hepatobiliary procedural complications</i>	10017927
			SOC primary <i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary <i>Peripheral neuropathies NEC</i>	10034607
			SOC secondary <i>Nervous system disorders</i>	10029205
MSSO Comment: The proposal to add a new term Inferior alveolar neuritis is approved but not as requested. Injury to the inferior alveolar nerve is recognized as a potentially serious complication of a wide range of dental procedures. Treatment options include nonsurgical therapies, surgical intervention, or a combination of these techniques. Proposed term will be modified to the concept referenced most commonly in literature and added as PT Inferior alveolar nerve injury to primary HLT Gastrointestinal and hepatobiliary procedural complications and to secondary HLT Peripheral neuropathies NEC.				

Detail Report

CR Number: 2026110002		Implementation Date: 24-Apr-26	Related CR: 2026110002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cervicocerebral atherosclerosis</i>	Approved as Requested	Proposed LLT <i>Cervicocerebral atherosclerosis</i>	10093684 Current
			To PT <i>Cerebral arteriosclerosis</i>	10065559
MSSO Comment: The proposal to add a new term Cervicocerebral atherosclerosis is approved as requested. Cervicocerebral atherosclerosis refers to atherosclerotic changes affecting the cervical and cerebral arteries that supply the brain, resulting from lipid deposition, plaque formation, and thickening of the arterial walls. Cervicocerebral atherosclerosis will be added as LLT to PT Cerebral arteriosclerosis.				

CR Number: 2026110006		Implementation Date: 24-Apr-26	Related CR: 2026110006	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Cheilitis infectious</i>	Approved Not as Requested	Proposed PT <i>Infectious cheilitis</i>	10093685
			HLT primary <i>Dental and oral soft tissue infections</i>	10012326
			SOC primary <i>Infections and infestations</i>	10021881
			HLT secondary <i>Oral soft tissue infections</i>	10031017
			SOC secondary <i>Gastrointestinal disorders</i>	10017947
			HLT secondary <i>Dermal and epidermal conditions NEC</i>	10012424
			SOC secondary <i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new LLT Cheilitis infectious is approved but not as requested. Infectious cheilitis is inflammation of the lips caused by fungi, bacteria, or viruses, often resulting in red, cracked, painful, or scaling skin, particularly at the corners of the mouth. The most common type is angular cheilitis, typically caused by Candida (yeast) or Staphylococcus aureus bacteria. Proposed term will be modified into natural language word order and added as new PT Infectious cheilitis to primary HLT Dental and oral soft tissue infections and secondary HLT Oral soft tissue infections and secondary HLT Dermal and epidermal conditions NEC.

Detail Report

CR Number: 2026110007		Implementation Date: 28-Apr-26	Related CR: 2026110007	
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Angular cheilitis</i>	Rejected After Suspension		
Old Primary SOC	<i>Gastrointestinal disorders</i>			
New Primary SOC	<i>Infections and infestations</i>			
MSSO Comment:	The proposal to reassign the primary SOC of PT Angular cheilitis from current SOC Gastrointestinal disorders to SOC Infections and infestations is not approved after suspension. Since angular cheilitis mainly indicates inflammation of the corner of the mouth, and infection may occur later as superimposed, the primary SOC Gastrointestinal disorders will remain.			

CR Number: 2026111000		Implementation Date: 24-Apr-26	Related CR: 2026111000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Corneal epithelial detachment</i>	Approved as Requested	Proposed LLT <i>Corneal epithelial detachment</i> To PT <i>Corneal erosion</i>	10093686 Current 10011013
MSSO Comment:	The proposal to add a new LLT Corneal epithelial detachment is approved as requested. Corneal epithelial detachment is the separation of the cornea's outer epithelial layer from the underlying basement membrane, often due to trauma, disease, or weakened epithelial adhesion. The recurrent corneal erosion syndrome is characterized by recurrent detachments of the corneal epithelium from damaged epithelial basement membranes. Corneal epithelial detachment will be added as sub-concept LLT to PT Corneal erosion.			

CR Number: 2026111001		Implementation Date: 08-Jun-26	Related CR: 2026111001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Myocilin-associated glaucoma</i>	Approved Not as Requested After Suspension	Proposed PT <i>Myocilin gene mutation</i> HLT primary <i>Gene mutations and other alterations NEC</i> SOC primary <i>Congenital, familial and genetic disorders</i>	10093818 10083625 10010331
MSSO Comment:	The proposal to add a new term Myocilin-associated glaucoma is approved but not as requested after suspension. The Myocilin (MYOC) gene is the leading genetic cause of primary open-angle glaucoma and juvenile-open angle glaucoma. Located on chromosome 1q24.3, the MYOC gene provides instructions for producing the myocilin protein. This gene mutation is the pathogenesis and basis for several types of glaucoma and thus, the new PT Myocilin gene mutation will be added to primary HLT Gene mutations and other alterations NEC. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.			

Detail Report

CR Number: 2026112000	Implementation Date: 24-Apr-26	Related CR: 2026112000	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Intradialytic pruritus	Rejected	

MSSO Comment: The proposal to add a new LLT Intradialytic pruritus is not approved. The MSSO considers this proposed term to be of a “due to” nature. Terms that state or imply a “due to” concept are generally not added to MedDRA because causality is not inherent in the coded term, but is instead determined by the reporter/investigator or by the sponsor. Please note that the MedDRA Term Selection: Points to Consider document Section 3.5.4 provides the option for splitting and coding.

CR Number: 2026112001	Implementation Date: 28-Apr-26	Related CR: 2026112001	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Voluntary spitting of medication	Rejected	

MSSO Comment: The proposal to add a new LLT Voluntary spitting of medication is not approved. The proposed concept is situational and does not provide information about any aspect of the medication itself. The concept represents a patient behavioral outcome.

CR Number: 2026112002	Implementation Date: 28-Apr-26	Related CR: 2026112002	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Involuntary spitting of medication	Rejected	

MSSO Comment: The proposal to add a new LLT Involuntary spitting of medication is not approved. The proposed concept is situational and does not provide information about any aspect of the medication itself. Other existing terms such as LLT Gagging on medication or other terms describing the reflex itself depending on case context may be used.

Detail Report

CR Number: 2026112003		Implementation Date: 28-Apr-26	Related CR: 2026112003		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	<i>Spitting of medication</i>	Rejected			

MSSO Comment: The proposal to add a new PT Spitting of medication is not approved. The proposed concept is situational and does not provide information about any aspect of the medication itself. The concept represents a patient behavioral outcome.

CR Number: 2026113000		Implementation Date: 28-Apr-26	Related CR: 2026113000		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	<i>Polypoid pansinusitis</i>	Rejected			

MSSO Comment: The proposal to add a new term Polypoid pansinusitis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. Please consider existing terms LLT Pansinusitis and LLT/PT Chronic rhinosinusitis with nasal polyps depending on your specific case scenarios.

Detail Report

CR Number: 2026113001

Implementation Date: 28-Apr-26

Related CR: 2026113001

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT *Measles encephalitis*

Approved as Requested

Proposed PT
Measles encephalitis

10093687

HLT primary *Rubeola viral infections*

HLT primary
Rubeola viral infections

10039289

SOC primary *Infections and infestations*

SOC primary
Infections and infestations

10021881

HLT secondary
Encephalitis of viral origin

10057174

SOC secondary
Nervous system disorders

10029205

MSSO
Comment: The proposal to add a new PT Measles encephalitis to primary HLT Rubeola viral infections in SOC Infections and infestations and secondary HLT Encephalitis of viral origin in SOC Nervous system disorders is approved as requested. Measles encephalitis is a severe, often fatal, inflammation of the brain caused by the measles virus, occurring in roughly 1 in 1,000 cases, usually 1–2 weeks after the rash. In contrast, Subacute sclerosing panencephalitis is a progressive, disabling, and deadly brain disorder related to measles infection. The disease develops many years (7-10) after the measles infection. In a related change, LLT Post measles encephalitis and LLT Encephalitis post measles will be both be made non-current, as they represent a temporal relationship and may represent various forms of measles related encephalitis and imprecise diagnosis.

CR Number: 2026118005

Implementation Date: 28-Apr-26

Related CR: 2026113001

Change Requested

Change Status of LLT

Final Disposition

Final Placement

MedDRA
Code #

LLT to change *Post measles encephalitis*

Approved as Requested

LLT to change
Post measles encephalitis

10057301 Non-Current

Status *Non-current*

Status
Non-current

MSSO
Comment:

Detail Report

CR Number: 2026118006		Implementation Date: 28-Apr-26	Related CR: 2026113001		
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #	
LLT to change	<i>Encephalitis post measles</i>	Approved as Requested	LLT to change		
Status	<i>Non-current</i>		<i>Encephalitis post measles</i>	10014604	Non-Current
			Status		
			<i>Non-current</i>		

MSSO
Comment:

CR Number: 2026114000		Implementation Date: 28-Apr-26	Related CR: 2026114000		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>D2 lymphadenectomy</i>	Approved Not as Requested	Proposed LLT		
			<i>Gastrectomy with D2 lymphadenectomy</i>	10093689	Current
			To PT		
			<i>Gastrectomy</i>	10061965	

MSSO
Comment:

The proposal to add a new term D2 lymphadenectomy is approved but not as requested. There are different levels of lymphadenectomy: D1 includes only the lymph nodes immediately adjacent to the stomach (perigastric), whereas D2 is more extensive and, in addition to these, includes nodes along the major arteries of the celiac axis (such as the left gastric, common hepatic, and splenic arteries). D2 lymphadenectomy is considered the standard procedure with curative intent in patients with locally advanced gastric cancer. The proposed term will be modified for greater context clarity and anatomical precision and added as LLT Gastrectomy with D2 lymphadenectomy to PT Gastrectomy.

CR Number: 2026114001		Implementation Date: 28-Apr-26	Related CR: 2026114001		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>D1 lymphadenectomy</i>	Approved Not as Requested	Proposed LLT		
			<i>Gastrectomy with D1 lymphadenectomy</i>	10093688	Current
			To PT		
			<i>Gastrectomy</i>	10061965	

MSSO
Comment:

The proposal to add a new term D1 lymphadenectomy is approved but not as requested. D1 lymphadenectomy is a surgical procedure for gastric cancer that involves removing only the perigastric lymph nodes, that is, those located immediately adjacent to the stomach along its lesser and greater curvatures. It represents the most limited standard lymph node dissection and does not include nodes along the main arteries of the celiac axis. The proposed term will be modified for greater context clarity and anatomical precision and added as LLT Gastrectomy with D1 lymphadenectomy to PT Gastrectomy.

Detail Report

CR Number: 2026116000	Implementation Date: 28-Apr-26	Related CR: 2026116000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Solitary kidney congenital</i>	Rejected	

MSSO Comment: The proposal to add a new term Solitary kidney congenital is not approved. The proposed term is an anatomically and pathology unspecific concept, which does not provide clinically important distinction towards existing LLT Solitary kidney. Please consider capturing the congenital nature by a more specific diagnosis in LLTs linked to HLT Renal disorders congenital and combining it with a term for the functional disturbance, e.g., LLT Solitary kidney. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.

CR Number: 2026116001	Implementation Date: 28-Apr-26	Related CR: 2026116001	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Undifferentiated arthritis</i>	Rejected	

MSSO Comment: The proposal to add a new term Undifferentiated arthritis is not approved. Undifferentiated arthritis is a descriptive clinical term for inflammatory arthritis that cannot yet be confidently assigned to a defined rheumatologic disease category. Please consider existing concepts under PT Arthritis.

Detail Report

CR Number: 2026117050		Implementation Date: 05-May-26	Related CR: 2026117050	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Ectopic valve deployment</i>	Approved Not as Requested	Proposed PT	
To PT	<i>Device deployment issue</i>		<i>Ectopic valve deployment</i>	10093697
			HLT primary	
			<i>Cardiac and vascular procedural complications</i>	10007602
			SOC primary	
			<i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary	
			<i>Cardiac valve disorders NEC</i>	10046971
			SOC secondary	
			<i>Cardiac disorders</i>	10007541

MSSO Comment: The proposal to add a new LLT Ectopic valve deployment to PT Device deployment issue is approved but not as requested. Ectopic valve deployment is typically outside the aortic annulus—most often due to valve embolization or failed anchoring during implantation. Ectopic valve deployment will be added as a PT to primary HLT Cardiac and vascular procedural complications and to secondary HLT Cardiac valve disorders NEC.

CR Number: 2026118000		Implementation Date: 03-May-26	Related CR: 2026118000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Post infarction heart failure</i>	Rejected		

MSSO Comment: The proposal to add a new term Post infarction heart failure is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

Detail Report

CR Number: 2026118001		Implementation Date: 03-May-26	Related CR: 2026118001		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Ischemic dilated cardiomyopathy</i>	Rejected			

MSSO Comment: The proposal to add a new term Ischemic dilated cardiomyopathy is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. In addition, MedDRA tries to limit addition of terms with several qualifiers.

CR Number: 2026118002		Implementation Date: 05-May-26	Related CR: 2026118002		
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Vacuolar myelopathy</i>	Approved Not as Requested	Proposed PT		
			<i>HIV-associated vacuolar myelopathy</i>	10093693	
			HLT primary		
			<i>Retroviral infections</i>	10038997	
			SOC primary		
			<i>Infections and infestations</i>	10021881	
			HLT secondary		
			<i>Spinal cord and nerve root disorders NEC</i>	10041544	
			SOC secondary		
			<i>Nervous system disorders</i>	10029205	
			HLT secondary		
			<i>Acquired immunodeficiency syndromes</i>	10052740	
			SOC secondary		
			<i>Immune system disorders</i>	10021428	

MSSO Comment: The proposal to add a new PT Vacuolar myelopathy is approved but not as requested. Vacuolar myelopathy is a degenerative disorder of the spinal cord, typically seen in advanced HIV/AIDS, characterized by vacuolation of the white matter (mainly posterior and lateral columns) causing a progressive spastic paraparesis; it frequently coexists with HIV-associated dementia, reflecting parallel involvement of the spinal cord and brain by the same HIV-related neurodegenerative process. The proposed term will be modified for more specificity and added as PT HIV-associated vacuolar myelopathy to primary HLT Retroviral infections, and to secondary HLT Spinal cord and nerve root disorders NEC and to secondary HLT Acquired immunodeficiency syndromes.

Detail Report

CR Number: 2026118003		Implementation Date: 05-May-26	Related CR: 2026118003	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Vaccination failure</i>	Approved Not as Requested	Proposed LLT	
Replacement term	<i>Vaccine failure</i>		<i>Vaccine failure</i>	10093694 Current
			To PT	
			<i>Vaccination failure</i>	10046862

MSSO Comment: The proposal to rename PT/LLT Vaccination failure to Vaccine failure is approved but not as requested. Vaccine failure and vaccination failure are broadly overlapping terms. Therefore, the terms are grouped together in MedDRA under the broad PT Vaccination failure. Please use your internal coding conventions to advise use of the terms according to your regulatory use. Vaccine failure will be added as an LLT to PT Vaccination failure.

CR Number: 2026118004		Implementation Date: 03-May-26	Related CR: 2026118004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Orbital aspergillosis</i>	Approved Not as Requested	Proposed PT	
			<i>Orbital aspergillosis</i>	10093698
			HLT primary	
			<i>Aspergillus infections</i>	10003486
			SOC primary	
			<i>Infections and infestations</i>	10021881
			HLT secondary	
			<i>Orbital infections, inflammations and irritations</i>	10031049
			SOC secondary	
			<i>Eye disorders</i>	10015919

MSSO Comment: The proposal to add a new LLT Orbital aspergillosis is approved but not as requested. Orbital aspergillosis is a rare fungal infection mostly presenting as a unilateral orbital mass, which can cause eyelid swelling, proptosis, impairment of ocular motility, and/ or optic nerve compression, leading to vision loss. Orbital aspergillosis will be added as PT to primary HLT Aspergillus infections and secondary HLT Orbital infections, inflammations and irritations.

Detail Report

CR Number: 2026118007	Implementation Date: 03-May-26	Related CR: 2026118007	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Eczematous otitis externa</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Eczematous otitis externa is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities.

CR Number: 2026119000	Implementation Date: 03-May-26	Related CR: 2026119000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Clinical deterioration</i>	Rejected	

MSSO Comment: The proposal to add a new term Clinical deterioration is not approved. The MSSO refrains from adding new terms with a qualifier of aggravation / worsening / exacerbation or similar unless such a term represents an internationally accepted unique medical concept. Please consider existing terms e.g., LLT/PT General physical health deterioration depending on your specific coding scenario.

CR Number: 2026119002	Implementation Date: 05-May-26	Related CR: 2026119002	MedDRA Code #
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement
Proposed PT	<i>Single functional kidney</i>	Rejected	
Old Primary SOC	<i>Renal and urinary disorders</i>		
New Primary SOC	<i>Congenital, familial and genetic disorders</i>		

MSSO Comment: The proposal to reassign the primary SOC of PT Single functional kidney from current SOC Renal and urinary disorders to SOC Congenital, familial and genetic disorders is not approved. The term single functional kidney is used for a condition in which only one kidney provides meaningful filtration and normal renal function, whether because the other kidney is absent, nonfunctional, or severely impaired. The term does not distinguish congenital or acquired causes. LLT Kidney absent is a descriptive term, not describing an underlying cause, which can be renal agenesis, but also nephrectomies due to tumor, trauma, donation or other causes.

Detail Report

CR Number: 2026119003		Implementation Date: 03-May-26	Related CR: 2026119003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Neutrophilic Urticarial Dermatitis</i>	Approved Not as Requested	Proposed LLT <i>Neutrophilic urticarial dermatosis</i>	10093695 Current
			To PT <i>Neutrophilic dermatosis</i>	10058105
MSSO Comment: The proposal to add a new LLT Neutrophilic Urticarial Dermatitis is approved but not as requested. Neutrophilic dermatoses are autoinflammatory skin conditions characterized by dense infiltration of neutrophils in the affected tissue. Neutrophilic dermatoses include neutrophilic urticarial dermatosis which is a rare condition characterized by a transient urticarial eruption typically accompanied by systemic symptoms including fever and arthralgias. Proposed term will be modified for MedDRA capitalization conventions and added as LLT Neutrophilic urticarial dermatosis to PT Neutrophilic dermatosis.				
CR Number: 2026119004		Implementation Date: 05-May-26	Related CR: 2026119004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Blanching erythema</i>	Approved as Requested	Proposed LLT <i>Blanching erythema</i>	10093696 Current
			To PT <i>Erythema</i>	10015150
MSSO Comment: The proposal to add a new term Blanching erythema is approved as requested. Blanching erythema is a red or discolored area of skin that turns pale or white when pressure is applied, indicating that blood vessels are dilated but intact, typically signifying inflammation or early, reversible injury. Blanching erythema will be added as an LLT to PT Erythema.				
CR Number: 2026119005		Implementation Date: 03-May-26	Related CR: 2026119005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Computerised tomogram bone</i>	Approved as Requested	Proposed PT <i>Computerised tomogram bone</i>	10093699
			HLT primary <i>Musculoskeletal and soft tissue imaging procedures</i>	10028386
			SOC primary <i>Investigations</i>	10022891
MSSO Comment: The proposal to add a new term Computerised tomogram bone is approved as requested. A computerised tomogram of bone refers to a bone image produced using Computed Tomography, where multiple X-ray measurements are combined to create detailed cross-sectional views of bone structure. Computerised tomogram bone will be added as PT to HLT Musculoskeletal and soft tissue imaging procedures. In a related change, the American English spelled counterpart LLT Computerized tomogram bone will be added to PT Computerised tomogram bone.				

Detail Report

CR Number: 2026125003		Implementation Date: 05-May-26	Related CR: 2026119005		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Computerized tomogram bone	Approved as Requested	Proposed LLT		
To PT	Computerised tomogram bone		Computerized tomogram bone	10093704	Current
			To PT		
			Computerised tomogram bone	10093699	

MSSO
Comment:

CR Number: 2026119006		Implementation Date: 12-May-26	Related CR: 2026119006		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Triple pituitary stimulation test	Approved After Suspension	Proposed PT		
			Triple pituitary stimulation test	10093710	
			HLT primary		
			Pituitary analyses anterior	10035089	
			SOC primary		
			Investigations	10022891	

MSSO
Comment: The proposal to add a new term Triple pituitary stimulation test is approved as requested after suspension. The triple pituitary stimulation test is a dynamic endocrine test used to evaluate the functional reserve of the anterior pituitary gland by stimulating multiple hormonal axes simultaneously. Typical "triple" stimulation includes: Insulin (induces hypoglycemia → stimulates ACTH and GH); TRH (stimulates TSH). Sometimes GnRH is added to assess LH/FSH. The insulin-induced hypoglycemia component makes this test potentially risky. Modern endocrinology favors modular, axis-specific testing over global stimulation tests. Triple pituitary stimulation test will be added as a PT to HLT Pituitary analyses anterior.

CR Number: 2026119007		Implementation Date: 05-May-26	Related CR: 2026119007		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Perineural injection	Approved as Requested	Proposed PT		
			Perineural injection	10093702	
			HLT primary		
			Peripheral nerve therapeutic procedures	10034588	
			SOC primary		
			Surgical and medical procedures	10042613	

MSSO
Comment: The proposal to add a new term Perineural injection is approved as requested. Perineural injections can be either superficial injections or deep perineural injections that target a specific nerve, like the radial, ulnar or median nerve. Perineural injection will be added as a PT to HLT Peripheral nerve therapeutic procedures.

Detail Report

CR Number: 2026119008		Implementation Date: 05-May-26	Related CR: 2026119008	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Temple syndrome</i>	Approved as Requested	Proposed PT <i>Temple syndrome</i>	10093700
			HLT primary <i>Endocrine disorders congenital NEC</i>	10027671
			SOC primary <i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary <i>Muscle tone abnormal</i>	10028342
			SOC secondary <i>Nervous system disorders</i>	10029205
			HLT secondary <i>Hyperglycaemic conditions NEC</i>	10020638
			SOC secondary <i>Endocrine disorders</i>	10014698

MSSO Comment: The proposal to add a new term Temple syndrome is approved as requested. Temple syndrome is a rare genetic condition resulting from abnormal expression of imprinted genes on chromosome 14q32. It is classified as an imprinting disorder. It is characterized by pre- and postnatal growth failure with head sparing, hypotonia with poor feeding, precocious puberty, early-onset obesity with high fat mass and low lean mass, short stature, and characteristic facial features. Most affected individuals have developmental delay. Temple syndrome will be added as PT to primary HLT Endocrine disorders congenital NEC, to secondary HLT Muscle tone abnormal, and to secondary HLT Hyperglycaemic conditions NEC.

CR Number: 2026119009		Implementation Date: 03-May-26	Related CR: 2026119009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Solitary bone plasmacytoma</i>	Approved as Requested	Proposed LLT <i>Solitary bone plasmacytoma</i>	10093703 Current
To PT	<i>Plasmacytoma</i>		To PT <i>Plasmacytoma</i>	10035484

MSSO Comment: The proposal to add a new LLT Solitary bone plasmacytoma to PT Plasmacytoma is approved as requested. Solitary plasmacytoma of bone is a localized, intraosseous neoplasm composed of clonal plasma cells, with no evidence of other bony lesions and no evidence of plasma cell myeloma.

Detail Report

CR Number: 2026119010		Implementation Date: 03-May-26	Related CR: 2026119010		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Osseous plasmacytoma	Rejected			
To PT	Plasmacytoma				

MSSO Comment: The proposal to add a new LLT Osseous plasmacytoma to PT Plasmacytoma is not approved. The proposed term is ambiguous due to evolving diagnostic standards, overlaps with early multiple myeloma and is not recommended to be used synonymously for solitary plasmacytoma of bone based on WHO Classification of Tumors recommendations. Please consider LLT Solitary bone plasmacytoma added in change request 2026119009 for your specific coding needs.

CR Number: 2026119011		Implementation Date: 03-May-26	Related CR: 2026119011		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Renal decapsulation	Rejected			

MSSO Comment: The proposal to add a new term Renal decapsulation is not approved. The proposed concept is a historical term no longer used in modern practice or in contemporary medical and scientific references.

CR Number: 2026119012		Implementation Date: 05-May-26	Related CR: 2026119012		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Desmectomy	Approved Not as Requested	Proposed PT		
			Ligament excision	10093701	
			HLT primary		
			Joint therapeutic procedures	10023221	
			SOC primary		
			Surgical and medical procedures	10042613	

MSSO Comment: The proposal to add a new term Desmectomy is approved but not as requested. "Desmectomy" is not a standard English medical term and is not easily referenced in human medical or scientific articles. The proposed term will be modified to represent a commonly referenced synonym and added as PT Ligament excision to HLT Joint therapeutic procedures.

Detail Report

CR Number: 2026120000	Implementation Date: 08-May-26	Related CR: 2026120000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	Hard flaccid syndrome	Rejected	

MSSO Comment: The proposal to add a new term Hard flaccid syndrome is not approved. Hard-flaccid syndrome is not a well-established diagnostic entity, nor one frequently addressed in the conventional medical literature. It lacks validated diagnostic criteria, and solid evidence regarding its pathophysiology or treatment. When described, it is characterized by a semi-rigid penis in the flaccid state, often accompanied by perineal discomfort, erectile changes, urinary symptoms, and psychological distress, and is generally considered a multifactorial disorder involving pelvic floor, vascular, and psychogenic components rather than a clearly established diagnostic entity.

CR Number: 2026120001	Implementation Date: 03-May-26	Related CR: 2026120001	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Suspected off label use of device	Approved as Requested	Proposed PT
HLT primary	Off label uses		Suspected off label use of device
SOC primary	Injury, poisoning and procedural complications		10093705
			HLT primary
			Off label uses
			10076291
			SOC primary
			Injury, poisoning and procedural complications
			10022117

MSSO Comment: The proposal to add a new PT Suspected off label use of device to primary HLT Off label uses in SOC Injury, poisoning and procedural complications is approved as requested. Concept will address the suspected off label use of device and to align and complement existing PT Off label use of device.

CR Number: 2026120008	Implementation Date: 08-May-26	Related CR: 2026120008	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Chewing tobacco	Approved Not as Requested	Proposed LLT
			Chewing tobacco user
			10093706
			To PT
			Tobacco user
			10057581

MSSO Comment: The proposal to add a new LLT Chewing tobacco is approved but not as requested. There are two main types of smokeless tobacco: chewing tobacco and snuff, including snus. Proposed term will be modified to represent a user of chewing tobacco rather than simply the name of the product as submitted which is not within the scope of MedDRA. Chewing tobacco user will be added as sub-concept LLT to PT Tobacco user.

Detail Report

CR Number: 2026120009		Implementation Date: 08-May-26	Related CR: 2026120009	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Smokeless Tobacco</i>	Approved Not as Requested	Proposed LLT <i>Smokeless tobacco user</i>	10093707 Current
			To PT <i>Tobacco user</i>	10057581

MSSO Comment: The proposal to add a new LLT Smokeless Tobacco is approved but not as requested. There are two main types of smokeless tobacco: chewing tobacco and snuff, including snus. Proposed term will be modified for MedDRA capitalization conventions and to represent a user of smokeless tobacco rather than simply the name of the product as submitted which is not within the scope of MedDRA. Smokeless tobacco user will be added as sub-concept LLT to PT Tobacco user.

CR Number: 2026124000		Implementation Date: 08-May-26	Related CR: 2026124000	
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Angular cheilitis</i>	Approved Not as Requested	Proposed PT <i>Angular cheilitis</i>	10002509
Old Primary SOC	<i>Gastrointestinal disorders</i>		From HLT <i>Oral soft tissue infections</i>	10031017
New Primary SOC	<i>Infections and infestations</i>		To HLT <i>Oral soft tissue disorders NEC</i>	10031015

MSSO Comment: The proposal to reassign the primary SOC of PT Angular cheilitis from current SOC Gastrointestinal disorders to SOC Infections and infestations is approved but not as requested. Since angular cheilitis mainly indicates inflammation of the corner of the mouth, and infection may occur later as superimposed, PT Angular cheilitis will be moved from HLT Oral soft tissue infections to HLT Oral soft tissue disorders NEC.

CR Number: 2026125000		Implementation Date: 08-May-26	Related CR: 2026125000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Desire for child</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Desire for child is not approved. The term is ambiguous and could refer to multiple situations, including infertility and social situations. Please consider related existing terms in SOC Social circumstances such as LLT/PT Planning to become pregnant.

Detail Report

CR Number: 2026125001		Implementation Date: 08-May-26	Related CR: 2026125001		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Ictus reaction</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Ictus reaction is not approved. The proposed term is ambiguous and not uniformly defined. It can relate to epilepsy, stroke, blow, ictus cordis, or any other sudden medical attack. Please use existing specific terms to capture the reaction(s) reported.

CR Number: 2026125002		Implementation Date: 08-May-26	Related CR: 2026125002		
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Gangliocytoma</i>	Approved as Requested	Proposed LLT		
From PT	<i>Ganglioneuroma</i>		<i>Gangliocytoma</i>	10017700	
HLT primary	<i>Nervous system neoplasms benign NEC</i>		From PT		
			<i>Ganglioneuroma</i>	10017709	
			HLT primary		
			<i>Nervous system neoplasms benign NEC</i>	10029210	
			SOC primary		
			<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104	
			HLT secondary		
			<i>Nervous system neoplasms benign NEC</i>	10029210	
			SOC secondary		
			<i>Nervous system disorders</i>	10029205	

MSSO Comment: The proposal to promote the LLT Gangliocytoma from PT Ganglioneuroma to primary HLT Nervous system neoplasms benign NEC in SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps) and secondary HLT Nervous system neoplasms benign NEC in SOC Nervous system disorders is approved as requested. Gangliocytoma is a CNS tumor and not a peripheral nerve tumor like ganglioneuroma. Promoting LLT Gangliocytoma will improve the representation of this concept in MedDRA.

Detail Report

CR Number: 2026128000

Implementation Date: 12-May-26

Related CR: 2026128000

[Change Requested](#)

Add a New Term

[Final Disposition](#)

[Final Placement](#)

[MedDRA
Code #](#)

Proposed Term *Familial exudative vitreoretinopathy*

Approved as Requested

Proposed PT

Familial exudative vitreoretinopathy

10093708

HLT primary

Retinal disorders congenital

10038855

SOC primary

Congenital, familial and genetic disorders

10010331

HLT secondary

Retinopathies NEC

10038932

SOC secondary

Eye disorders

10015919

HLT secondary

Ocular vascular disorders NEC

10030067

SOC secondary

Vascular disorders

10047065

MSSO

Comment:

The proposal to add a new term Familial exudative vitreoretinopathy is approved as requested. Familial exudative vitreoretinopathy (FEVR) defines a group of inherited retinal diseases characterized by abnormal retinal angiogenesis leading to incomplete vascularization of the peripheral retina with subsequent retinal ischemia. It is believed the avascular retina in FEVR may lead to hypoxia and stimulation of neovascular growth into the vitreous leading to later vitreoretinal traction, subretinal exudation and hemorrhages, retinal folds, tractional retinal detachment, and foveal displacement. Clinical presentation ranges from mild, asymptomatic disease to severe vision loss. Familial exudative vitreoretinopathy will be added as PT to primary HLT Retinal disorders congenital, secondary HLT Retinopathies NEC and secondary HLT Ocular vascular disorders NEC.

Detail Report

CR Number: 2026128001		Implementation Date: 12-May-26	Related CR: 2026128001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Attenuated live vaccine derived infection</i>	Approved Not as Requested	Proposed PT <i>Attenuated live vaccine-derived infection</i>	10093709
			HLT primary <i>Infections NEC</i>	10021902
			SOC primary <i>Infections and infestations</i>	10021881
			HLT secondary <i>Vaccination related complications</i>	10068755
			SOC secondary <i>Injury, poisoning and procedural complications</i>	10022117

MSSO Comment: The proposal to add a new PT Attenuated live vaccine derived infection is approved but not as requested. Attenuated live vaccine–derived infection occurs when a live vaccine organism replicates and causes disease either because the host cannot control it (e.g., immunocompromised patients) or, rarely, because the strain mutates/reverts to a more virulent form. The most important vaccines associated with this are oral polio vaccine, BCG vaccine, Varicella vaccine, MMR vaccine, Rotavirus vaccine, Yellow fever vaccine, and Smallpox vaccine. The proposed term will be modified with a hyphen and added as PT Attenuated live vaccine-derived infection to primary HLT Infections NEC and to secondary HLT Vaccination related complications.

CR Number: 2026128002		Implementation Date: 15-May-26	Related CR: 2026128002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Neoplastic infiltration of the vena cava</i>	Rejected		

MSSO Comment: The proposal to add a new term Neoplastic infiltration of the vena cava is not approved. The proposed term is an unspecific descriptive concept with unclear clinical implications. It does not distinguish between superior and inferior vena cava involvement, and may present as an intraluminal tumour thrombus, mural invasion, extrinsic compression, partial stenosis, or complete occlusion, all of which have different clinical implications and management.

Detail Report

CR Number: 2026129000		Implementation Date: 15-May-26	Related CR: 2026129000		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Basal septal hypertrophy</i>	Approved Not as Requested	Proposed LLT <i>Basal ventricular septal hypertrophy</i>	10093711	Current
			To PT <i>Cardiac septal hypertrophy</i>	10057576	
MSSO Comment: The proposal to add a new term Basal septal hypertrophy is approved but not as requested. Basal ventricular septal hypertrophy is often an incidental imaging finding in older adults with hypertension or age-related cardiac remodeling, but it becomes clinically important when it causes left ventricular outflow tract obstruction, mimics hypertrophic cardiomyopathy, or contributes to heart failure symptoms. The proposed term will be modified for anatomical specificity and added as LLT Basal ventricular septal hypertrophy to PT Cardiac septal hypertrophy.					

CR Number: 2026131001		Implementation Date: 15-May-26	Related CR: 2026131001		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Balloon osteoplasty</i>	Approved as Requested	Proposed LLT <i>Balloon osteoplasty</i>	10093712	Current
			To PT <i>Osteoplasty</i>	10091271	
MSSO Comment: The proposal to add a new term Balloon osteoplasty is approved as requested. Balloon osteoplasty is a minimally invasive technique used to treat fractures by inflating a small, high-pressure balloon inside the bone. This process elevates depressed fragments, reduces fractures, and creates a cavity for bone cement, typically used in tibial plateau, distal radius, and calcaneus fractures to restore bone structure. Proposed term will be added as sub-concept LLT Balloon osteoplasty to PT Osteoplasty.					

CR Number: 2026131002		Implementation Date: 15-May-26	Related CR: 2026131002		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Inflatable bone tamp</i>	Rejected			

MSSO Comment: The proposal to add a new term Inflatable bone tamp is not approved. The proposed term is a device name and as such is outside the scope of MedDRA. In addition, a synonym of the concept, LLT Balloon osteoplasty has been added in change request 2026131001.

Detail Report

CR Number: 2026131003		Implementation Date: 15-May-26	Related CR: 2026131003	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Bone ablation</i>	Approved Not as Requested	Proposed PT <i>Bone lesion ablation</i>	10093713
			HLT primary <i>Bone therapeutic procedures NEC</i>	10006000
			SOC primary <i>Surgical and medical procedures</i>	10042613

MSSO Comment: The proposal to add a new term Bone ablation is approved but not as requested. Bone lesion ablation is a minimally invasive surgical technique that employs heat or cold energy to treat bone tumors without traditional surgery. The technique uses image guidance such as ultrasound, magnetic resonance imaging, or computed tomography scan to pass a needle or probe through the skin into the bone tumor to destroy the tumor cells instead of surgically removing the tumor. Proposed term will be modified for clarity and added as new PT Bone lesion ablation to HLT Bone therapeutic procedures NEC.

CR Number: 2026131004		Implementation Date: 15-May-26	Related CR: 2026131004	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Cement augmentation</i>	Rejected		

MSSO Comment: The proposal to add a new term Cement augmentation is not approved. Cement augmentation generally refers to any procedure where bone cement (most commonly Polymethyl methacrylate) is injected to improve structural stability of bone. The proposed concept exists in LLT/PT Cementoplasty or more specifically LLT Vertebral cementoplasty.

CR Number: 2026132000		Implementation Date: 15-May-26	Related CR: 2026132000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Bull's eye lesions</i>	Rejected		
To PT	<i>Erythema multiforme</i>			

MSSO Comment: The proposal to add a new LLT Bull's eye lesions to PT Erythema multiforme is not approved. The proposed concept is ambiguous as "bull's eye" appears to be a term also used in ophthalmology and ultrasound findings. Please consider existing LLT\PT Target skin lesion for your coding needs depending on your specific scenario.

Detail Report

CR Number: 2026132001		Implementation Date: 15-May-26	Related CR: 2026132001		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Traumatic vitreous hemorrhage	Rejected			

MSSO Comment: The proposal to add a new term Traumatic vitreous hemorrhage is not approved. This is a combined concept that merges a medical condition with a specific etiology (traumatic origin). The same approach could be applied to an unlimited number of medical conditions by systematically adding this etiological qualifier, which would lead to unnecessary overpopulation of MedDRA. Please consider existing terms such as PT/LLT Closed globe injury, PT/LLT Eye injury and PT/LLT Vitreous haemorrhage. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

CR Number: 2026132002		Implementation Date: 18-May-26	Related CR: 2026132002		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Vasoplegic shock	Approved as Requested	Proposed LLT Vasoplegic shock	10093726	Current
			To PT Distributive shock	10070559	

MSSO Comment: The proposal to add a new LLT Vasoplegic shock is approved as requested. Vasoplegia or vasoplegic shock, also known as vasodilatory shock, is a condition defined by profound, uncontrollable vasodilation due to persistently low systemic vascular resistance with normal or high cardiac index. It can result from a multitude of shock-inducing conditions including, but not limited to, cardiac failure, sepsis, anaphylaxis, hemorrhage, and surgery. Vasoplegic shock will be added as LLT to PT Distributive shock aligned with synonym LLT Vasodilatory shock.

CR Number: 2026132003		Implementation Date: 18-May-26	Related CR: 2026132003		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Refractory hypotension	Approved Not as Requested	Proposed PT Refractory condition	10093722	
			HLT primary General signs and symptoms NEC	10018072	
			SOC primary General disorders and administration site conditions	10018065	

MSSO Comment: The proposal to add a new LLT Refractory hypotension is approved but not as requested. Generally, it is not possible to add the refractory version for every disease condition in MedDRA, therefore new PT Refractory condition will be added to HLT General signs and symptoms NEC, to be used with combination coding.

Detail Report

CR Number: 2026132004		Implementation Date: 18-May-26	Related CR: 2026132004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	ADAN (ACKR1/DARC-associated neutropenia)	Approved Not as Requested	Proposed LLT	
To PT	Benign ethnic neutropenia		ACKR1/DARC-associated neutropenia	10093728 Current
			To PT	
			Benign ethnic neutropenia	10074832
MSSO Comment:	The proposal to add a new LLT ADAN (ACKR1/DARC-associated neutropenia) to PT Benign ethnic neutropenia is approved but not as requested. ACKR1/DARC-associated neutropenia (ADAN) is characterized by low blood neutrophil counts without increased infection risk. The condition is also known as benign ethnic neutropenia and Duffy-null associated neutrophil count (DANC). Proposed term will be modified and added without parentheses and acronym as LLT ACKR1/DARC-associated neutropenia to PT Benign ethnic neutropenia.			
CR Number: 2026132005		Implementation Date: 18-May-26	Related CR: 2026132005	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	DANC (Duffy null associated neutrophil count)	Approved Not as Requested	Proposed LLT	
To PT	Benign ethnic neutropenia		Duffy null associated neutropenia	10093723 Current
			To PT	
			Benign ethnic neutropenia	10074832
MSSO Comment:	The proposal to add a new LLT DANC (Duffy null associated neutrophil count) to PT Benign ethnic neutropenia is approved but not as requested. Duffy-null Associated Neutrophil Count, formerly known as benign ethnic neutropenia, is a normal state in which the number of neutrophils in a blood sample is lower than the usual range used by the laboratory. The proposed term will be modified by removal of the acronym and parentheses, qualified with neutropenia and added as LLT Duffy null associated neutropenia to PT Benign ethnic neutropenia.			
CR Number: 2026132006		Implementation Date: 18-May-26	Related CR: 2026132006	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Duffy null phenotype	Approved Not as Requested	Proposed LLT	
			Duffy null phenotype	10093724 Current
			To PT	
			ACKR1 gene polymorphism	10093122
MSSO Comment:	The proposal to add a new PT Duffy null phenotype is approved but not as requested. The Duffy null phenotype (also known as Fy[a-b-]) is an inherited, benign condition characterized by the absence of Duffy antigens on red blood cells due to a genetic variant in the ACKR1 gene promoter. It is highly prevalent in populations of African ancestry, providing protection against Plasmodium vivax malaria, but results in a lower baseline neutrophil count (Duffy null associated neutropenia). Individuals with the Duffy null phenotype have no increased risk of infection but are often incorrectly labeled as having neutropenia. Duffy null phenotype will be added as an LLT to PT ACKR1 gene polymorphism.			

Detail Report

CR Number: 2026132007	Implementation Date: 18-May-26	Related CR: 2026132007	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Duffy null genotype	Rejected	

MSSO Comment: The proposal to add a new LLT Duffy null genotype is not approved. Usually, in monogenic conditions, MedDRA generally does not systematically duplicate such concepts by adding parallel "genotype" terms. Please see LLT Duffy null phenotype added in change request 2026132006 for your coding needs.

CR Number: 2026132008	Implementation Date: 18-May-26	Related CR: 2026132008	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	ACKR1 null genotype	Approved as Requested	Proposed LLT ACKR1 null genotype To PT ACKR1 gene polymorphism
			10093727 Current 10093122

MSSO Comment: The proposal to add a new LLT ACKR1 null genotype is approved as requested. The ACKR1 null genotype (also known as the Duffy-null genotype) is a genetic variation where individuals lack the Duffy antigen receptor for chemokines (DARC) on their red blood cells. ACKR1 null genotype will be added as LLT to PT ACKR1 gene polymorphism.

CR Number: 2026132009	Implementation Date: 18-May-26	Related CR: 2026132009	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	DARC null genotype	Approved as Requested	Proposed LLT DARC null genotype To PT ACKR1 gene polymorphism
			10093730 Current 10093122

MSSO Comment: The proposal to add a new LLT DARC null genotype is approved as requested. The ACKR1 null genotype (also known as the Duffy-null genotype) is a genetic variation where individuals lack the Duffy antigen receptor for chemokines (DARC) on their red blood cells. DARC null genotype will be added as LLT to PT ACKR1 gene polymorphism.

Detail Report

CR Number: 2026132010		Implementation Date: 18-May-26	Related CR: 2026132010	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Duffy antigen negative</i>	Approved Not as Requested	Proposed PT <i>Duffy antigen negative</i>	10093732
			HLT primary <i>Blood grouping and cross-matching analyses</i>	10005570
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT Duffy antigen negative is approved but not as requested. Duffy antigen negative means the individuals red blood cells lack the Duffy protein (also known as DARC) on their surface. Duffy antigen negative will be added as PT to HLT Blood grouping and cross-matching analyses.

CR Number: 2026132011		Implementation Date: 18-May-26	Related CR: 2026132011	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Fy(a-b-) phenotype</i>	Approved as Requested	Proposed LLT <i>Fy(a-b-) phenotype</i>	10093725 Current
			To PT <i>ACKR1 gene polymorphism</i>	10093122

MSSO Comment: The proposal to add a new LLT Fy(a-b-) phenotype is approved as requested. The Duffy-null phenotype (also known as Fy[a-b-]) is an inherited, benign condition characterized by the absence of Duffy antigens on red blood cells due to a genetic variant in the ACKR1 gene promoter. It is highly prevalent in populations of African ancestry, providing protection against Plasmodium vivax malaria, but results in a lower baseline neutrophil count. Fy(a-b-) phenotype will be added as LLT to PT ACKR1 gene polymorphism, as a synonym of recently added LLT Duffy null phenotype.

CR Number: 2026132012		Implementation Date: 18-May-26	Related CR: 2026132012	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Distal oesophagitis</i>	Approved as Requested	Proposed LLT <i>Distal oesophagitis</i>	10093729 Current
			To PT <i>Oesophagitis</i>	10030216

MSSO Comment: The proposal to add a new LLT Distal oesophagitis is approved as requested. Distal esophagitis is inflammation of the lower part of the esophagus, frequently caused by chronic acid reflux. Distal oesophagitis will be added as LLT to PT Oesophagitis. In a related change, the American English counterpart, LLT Distal esophagitis will also be added to PT Oesophagitis.

Detail Report

CR Number: 2026138004		Implementation Date: 18-May-26	Related CR: 2026132012	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Distal esophagitis</i>	Approved as Requested	Proposed LLT	
To PT	<i>Oesophagitis</i>		<i>Distal esophagitis</i>	10093731 Current
			To PT	
			<i>Oesophagitis</i>	10030216

MSSO
Comment:

CR Number: 2026132013		Implementation Date: 18-May-26	Related CR: 2026132013	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>Bulky prostate</i>	Rejected		

MSSO
Comment: The proposal to add a new LLT Bulky prostate is not approved. Proposed term is imprecise and may be represented by existing terms such as LLT Enlarged prostate.

CR Number: 2026132014		Implementation Date: 15-May-26	Related CR: 2026132014	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement	
Proposed LLT	<i>S100 protein</i>	Approved Not as Requested	Proposed PT	
			<i>S100 protein test</i>	10093715
			HLT primary	
			<i>Protein analyses NEC</i>	10036998
			SOC primary	
			<i>Investigations</i>	10022891

MSSO
Comment: The proposal to add a new LLT S100 protein is approved but not as requested. S100 proteins are a family of low molecular-weight calcium-binding proteins that play crucial roles in various cellular processes, including growth, differentiation, and inflammation. The proposed term will be modified and added as a test term as PT S100 protein test to HLT Protein analyses NEC, to complement existing LLT/PT S100 protein increased.

Detail Report

CR Number: 2026132015		Implementation Date: 15-May-26	Related CR: 2026132015	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Tick-borne encephalitis virus test</i>	Approved as Requested	Proposed LLT <i>Tick-borne encephalitis virus test</i>	10093718 Current
			To PT <i>Flavivirus test</i>	10078302

MSSO Comment: The proposal to add a new LLT Tick-borne encephalitis virus test is approved as requested. Diagnosis of tick-borne encephalitis primarily relies on serologic testing for virus-specific antibodies, with confirmatory neutralizing antibody assays and, in select cases, molecular testing. Virus isolation or detection of viral RNA by reverse transcription-polymerase chain reaction may be needed in patients who are immunocompromised. Tick-borne encephalitis virus test will be added as unqualified counterpart LLT to PT Flavivirus test, aligned with placement of LLT Tick-borne encephalitis virus test positive to PT Flavivirus test positive.

CR Number: 2026132016		Implementation Date: 15-May-26	Related CR: 2026132016	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Ophthalmic culture</i>	Approved Not as Requested	Proposed PT <i>Ophthalmic culture</i>	10093716
			HLT primary <i>Microbiology and serology tests NEC</i>	10027530
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT Ophthalmic culture is approved but not as requested. An ophthalmic culture is a laboratory test in which a sample from the eye (such as tears, corneal scrapings, or discharge) is collected and grown on culture media to identify infectious organisms like bacteria, fungi, or viruses causing an eye infection. The proposed term will be corrected for spelling and added as PT Ophthalmic culture to HLT Microbiology and serology tests NEC. In a related change, PT Corneal culture will be demoted under broader new PT Ophthalmic culture.

CR Number: 2026135001		Implementation Date: 15-May-26	Related CR: 2026132016	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Corneal culture</i>	Approved as Requested	PT to merge <i>Corneal culture</i>	10089292 Current
To PT	<i>Ophthalmic culture</i>		To PT <i>Ophthalmic culture</i>	10093716

MSSO Comment:

Detail Report

CR Number: 2026132017		Implementation Date: 15-May-26	Related CR: 2026132017	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Nasopharyngeal culture</i>	Approved as Requested	Proposed LLT <i>Nasopharyngeal culture</i>	10093720 Current
			To PT <i>Culture throat</i>	10011632
MSSO Comment: The proposal to add a new LLT Nasopharyngeal culture is approved as requested. In nasopharyngeal culture a sample is taken from the nasopharynx, usually with a deep swab inserted through the nose. Nasopharyngeal culture will be added as LLT to PT Culture throat.				

CR Number: 2026132018		Implementation Date: 15-May-26	Related CR: 2026132018	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Hypermobile Ehlers-Danlos syndrome</i>	Approved as Requested	Proposed LLT <i>Hypermobile Ehlers-Danlos syndrome</i>	10093719 Current
			To PT <i>Ehlers-Danlos syndrome</i>	10014316
MSSO Comment: The proposal to add a new LLT Hypermobile Ehlers-Danlos syndrome is approved as requested. Hypermobile Ehlers-Danlos syndrome (hEDS) is a heritable connective tissue disorder that causes generalized joint hypermobility, joint instability, and chronic pain. hEDS is also associated with a variety of other symptoms and related conditions that affect many different areas of the body. hEDS is the most common type of EDS, accounting for about 90% of EDS cases. Hypermobile Ehlers-Danlos syndrome will be added as LLT to PT Ehlers-Danlos syndrome.				

Detail Report

CR Number: 2026132019		Implementation Date: 15-May-26	Related CR: 2026132019	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Atypical anorexia nervosa</i>	Approved Not as Requested	Proposed PT	
			<i>Atypical anorexia nervosa</i>	10093717
			HLT primary	
			<i>Eating disorders NEC</i>	10014068
			SOC primary	
			<i>Psychiatric disorders</i>	10037175
			HLT secondary	
			<i>Anorexia nervosa and bulimia</i>	10002650
			SOC secondary	
			<i>Metabolism and nutrition disorders</i>	10027433

MSSO Comment: The proposal to add a new LLT Atypical anorexia nervosa is approved but not as requested. Atypical anorexia nervosa refers to a diagnosis of anorexia nervosa, in which all criteria are met, except that despite significant weight loss, the individual's weight is within or above the normal range. Atypical anorexia nervosa will be added as PT to primary HLT Eating disorders NEC and secondary HLT Anorexia nervosa and bulimia.

CR Number: 2026132020		Implementation Date: 15-May-26	Related CR: 2026132020	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Diamine oxidase test</i>	Approved Not as Requested	Proposed PT	
			<i>Diamine oxidase test</i>	10093714
			HLT primary	
			<i>Tissue enzyme analyses NEC</i>	10043891
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT Diamine oxidase test is approved but not as requested. Diamine oxidase (also known as histaminase) is an enzyme that metabolizes histamine. If there is a deficiency of the enzyme, histamine levels will elevate causing gastrointestinal problems, migraines and other allergy-like symptoms. Diamine oxidase test will be added as PT to HLT Tissue enzyme analyses NEC.

Detail Report

CR Number: 2026133000	Implementation Date: 15-May-26	Related CR: 2026133000	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement

Proposed Term	<i>Megaloblastic anaemia due to vitamin B12 deficiency</i>	Rejected
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MSSO Comment: The proposal to add a new term Megaloblastic anaemia due to vitamin B12 deficiency is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance. When considering the addition of such combination terms, MedDRA generally attempts to limit inclusion to those terms which describe important, distinct and robust medical concepts which are internationally recognized diagnostic entities. Examples include terms such as diabetic retinopathy, hypertensive cardiomegaly and eosinophilic pneumonia.

CR Number: 2026133002	Implementation Date: 18-May-26	Related CR: 2026133002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement

Proposed Term	<i>Metastases to intrahepatic bile duct</i>	Rejected
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MSSO Comment: The proposal to add a new term Metastases to intrahepatic bile duct is not approved. The proposed concept can be represented by existing LLT/PT Metastases to biliary tract.

CR Number: 2026133003	Implementation Date: 15-May-26	Related CR: 2026133003	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement

Proposed PT	<i>Responsive neurostimulation system user</i>	Rejected
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MSSO Comment: The proposal to add a new PT Responsive neurostimulation system user is not approved. The proposed term is considered too specific for the SOC Surgical and medical procedures. This SOC generally captures broader procedural concepts and does not generally differentiate highly specialized neuromodulation systems at such a granular level. However, and for the same reason, the LLT Neurostimulator implantation will be promoted from PT Neuroprosthesis implantation to the HLT Nervous system therapeutic procedures NEC, because it represents a distinct recognized treatment across multiple therapeutic areas (e.g., deep brain stimulation, spinal cord stimulation, vagal nerve stimulation, responsive neurostimulation in epilepsy). Furthermore, the LLT Occipital nerve stimulator implantation will be moved from PT Neuroprosthesis implantation to the newly promoted PT Neurostimulator implantation.

Detail Report

CR Number: 2026135002		Implementation Date: 15-May-26	Related CR: 2026133003	
<u>Change Requested</u>	Promote an LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	Neurostimulator implantation	Approved as Requested	Proposed LLT	
From PT	Neuroprosthesis implantation		Neurostimulator implantation	10069205
HLT primary	Nervous system therapeutic procedures NEC		From PT	
			Neuroprosthesis implantation	10081968
			HLT primary	
			Nervous system therapeutic procedures NEC	10052731
			SOC primary	
			Surgical and medical procedures	10042613

MSSO
Comment:

CR Number: 2026135003		Implementation Date: 15-May-26	Related CR: 2026133003	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	Occipital nerve stimulator implantation	Approved as Requested	Proposed LLT	
From PT	Neuroprosthesis implantation		Occipital nerve stimulator implantation	10089991 Current
To PT	Neurostimulator implantation		From PT	
			Neuroprosthesis implantation	10081968
			To PT	
			Neurostimulator implantation	10069205

MSSO
Comment:

Detail Report

CR Number: 2026133004		Implementation Date: 15-May-26	Related CR: 2026133004	
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Generalised pustular psoriasis of pregnancy</i>	Approved as Requested	Proposed PT	
To HLT	<i>Immune and associated conditions NEC</i>		<i>Generalised pustular psoriasis of pregnancy</i>	10093632
			To HLT	
			<i>Immune and associated conditions NEC</i>	10027682
MSSO Comment: The proposal to link the PT Generalised pustular psoriasis of pregnancy to the HLT Immune and associated conditions NEC is approved as requested. Generalised pustular psoriasis of pregnancy (impetigo herpetiformis) is a pregnancy-associated variant within the spectrum of generalized pustular psoriasis, a well-recognised immune-mediated disorder driven by dysregulated innate and adaptive immune pathways. Therefore, linkage to the HLT Immune and associated conditions NEC is adequate to follow the pattern of all other psoriasis related terms.				

CR Number: 2026133005		Implementation Date: 15-May-26	Related CR: 2026133005	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Proinsulinoma</i>	Approved as Requested	Proposed LLT	
			<i>Proinsulinoma</i>	10093721 Current
			To PT	
			<i>Insulinoma</i>	10022498
MSSO Comment: The proposal to add a new LLT Proinsulinoma is approved as requested. A proinsulinoma is a rare variant of insulinoma that predominantly secretes proinsulin rather than mature insulin. It causes hypoglycemia similar to classic insulinoma but characteristically shows markedly elevated proinsulin levels with normal or only mildly increased insulin concentrations, which reflects the defective conversion of proinsulin to insulin within the tumor cells. Since proinsulinoma is generally considered a functional variant of insulinoma rather than a separate entity, the proposed term will be added as LLT to PT Insulinoma.				

CR Number: 2026133006		Implementation Date: 18-May-26	Related CR: 2026133006	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Progressive solitary sclerosis</i>	Approved as Requested	Proposed PT	
			<i>Progressive solitary sclerosis</i>	10093733
			HLT primary	
			<i>Multiple sclerosis acute and progressive</i>	10052785
			SOC primary	
			<i>Nervous system disorders</i>	10029205
MSSO Comment: The proposal to add a new PT Progressive solitary sclerosis is approved as requested. Progressive solitary sclerosis is a rare chronic demyelinating CNS disorder characterized by a single persistent lesion causing slowly progressive neurological impairment, typically motor, without fulfilling conventional diagnostic criteria for multiple sclerosis due to the absence of typical multifocal dissemination. Progressive solitary sclerosis will be added as PT to the HLT Multiple sclerosis acute and progressive.				

Detail Report

CR Number: 2026133007	Implementation Date: 18-May-26	Related CR: 2026133007	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Orthostatic bradycardia	Rejected	

MSSO Comment: The proposal to add a new PT Orthostatic bradycardia is not approved. Although bradycardia may rarely occur in association with orthostatic stress, it is not a well-established or consistently recognized standalone medical concept. In clinical practice and literature, such presentations are usually described within broader recognized conditions or mechanisms such as vasovagal syncope, neurocardiogenic syncope, etc.

CR Number: 2026133008	Implementation Date: 18-May-26	Related CR: 2026133008	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	Coronary angioplasty with arterial stent insertion	Rejected	

MSSO Comment: The proposal to add a new PT Coronary angioplasty with arterial stent insertion is not approved. This concept represents a combination of two distinct procedural concepts already separately represented in MedDRA: coronary angioplasty and arterial stent insertion. Although these procedures are frequently performed sequentially or during the same interventional procedure, this does not justify the creation of a combined procedural term.

Detail Report

CR Number: 2026133009		Implementation Date: 18-May-26	Related CR: 2026133009	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Cushing's triad</i>	Approved as Requested	Proposed PT <i>Cushing's triad</i>	10093734
			HLT primary <i>Increased intracranial pressure disorders</i>	10062912
			SOC primary <i>Nervous system disorders</i>	10029205
			HLT secondary <i>Vascular hypertensive disorders NEC</i>	10020774
			SOC secondary <i>Vascular disorders</i>	10047065
			HLT secondary <i>Rate and rhythm disorders NEC</i>	10037908
			SOC secondary <i>Cardiac disorders</i>	10007541

MSSO Comment: The proposal to add a new PT Cushing's triad is approved as requested. Although Cushing's triad represents a combined concept composed of already existing clinical signs (hypertension, bradycardia, and abnormal respiration), the term has deep historical recognition in neurology and neurosurgery and it is still widely used in clinical practice as an immediately recognizable indication of severe intracranial hypertension, which extends beyond the simple listing of its individual components. Cushing's triad will be added as PT to primary HLT Increased intracranial pressure disorders and to secondary HLT Vascular hypertensive disorders NEC, to secondary HLT Rate and rhythm disorders NEC and to secondary HLT Breathing abnormalities.

CR Number: 2026138005		Implementation Date: 18-May-26	Related CR: 2026133009	
<u>Change Requested</u>	Link a PT to a HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Cushing's triad</i>	Approved as Requested	Proposed PT <i>Cushing's triad</i>	10093734
To HLT	<i>Breathing abnormalities</i>		To HLT <i>Breathing abnormalities</i>	10006334

MSSO Comment:

Detail Report

CR Number: 2026133010		Implementation Date: 19-May-26	Related CR: 2026133010		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Biliary gastric bypass</i>	Rejected			
To PT	<i>Gastric bypass</i>				

MSSO Comment: The proposal to add a new LLT Biliary gastric bypass to PT Gastric bypass is not approved. The proposed term is not specific enough from a surgical and terminological perspective. It does not correspond to a single well-defined bariatric procedure and may refer to several distinct operations with different anatomical configurations and clinical implications, such as Roux-en-Y gastric bypass, biliopancreatic diversion, or biliopancreatic diversion with duodenal switch.

CR Number: 2026133011		Implementation Date: 18-May-26	Related CR: 2026133011		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Posterior cyclitis</i>	Approved as Requested	Proposed LLT		
To PT	<i>Intermediate uveitis</i>		<i>Posterior cyclitis</i>	10093737	Current
			To PT		
			<i>Intermediate uveitis</i>	10022557	

MSSO Comment: The proposal to add a new LLT Posterior cyclitis to PT Intermediate uveitis is approved as requested. Intermediate uveitis describes inflammation within the vitreous body, which includes pars planitis or posterior cyclitis. Posterior cyclitis will be added as LLT to PT Intermediate uveitis, aligned with synonym LLT Pars planitis.

CR Number: 2026133012		Implementation Date: 18-May-26	Related CR: 2026133012		MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement		
Proposed LLT	<i>Anterior cyclitis</i>	Approved Not as Requested	Proposed LLT		
From PT	<i>Cyclitis</i>		<i>Anterior cyclitis</i>	10002688	Current
To PT	<i>Intermediate uveitis</i>		From PT		
			<i>Cyclitis</i>	10011715	
			To PT		
			<i>Anterior uveitis</i>	10002709	

MSSO Comment: The proposal to move the LLT Anterior cyclitis from PT Cyclitis to PT Intermediate uveitis is approved but not as requested. Anterior uveitis accounts for 50–90% of cases of ocular inflammation. It refers to inflammation of the iris or ciliary body with the focus of inflammation centered in the anterior chamber. Previous terms for anterior uveitis include iritis, iridocyclitis or anterior cyclitis. LLT Anterior cyclitis will move from PT Cyclitis to PT Anterior uveitis for better alignment.

Detail Report

CR Number: 2026133013		Implementation Date: 19-May-26	Related CR: 2026133013	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Retinochoroiditis</i>	Approved Not as Requested	Proposed LLT <i>Retinochoroiditis</i>	10093738 Current
			To PT <i>Posterior uveitis</i>	10036370
MSSO Comment: The proposal to add a new PT Retinochoroiditis is approved but not as requested. In literature and in some medical dictionaries the two terms are used interchangeably; therefore, the proposed concept will be added as LLT Retinochoroiditis to PT Posterior uveitis.				

CR Number: 2026133014		Implementation Date: 18-May-26	Related CR: 2026133014	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Assisted dying</i>	Approved Not as Requested	Proposed LLT <i>Assisted dying</i>	10093736 Current
HLT primary	<i>Therapeutic procedures NEC</i>		To PT <i>Medical aid in dying</i>	10090248
SOC primary	<i>Surgical and medical procedures</i>			
MSSO Comment: The proposal to add a new PT Assisted dying to primary HLT Therapeutic procedures NEC in SOC Surgical and medical procedures is approved but not as requested. Assisted dying prevents unnecessary suffering at the end of life and provides dying adults with choice and control over their death. Assisted dying is also referred to as medical aid in dying, depending on regulatory zones. Assisted dying will be added as synonym LLT to PT Medical aid in dying.				

CR Number: 2026133015		Implementation Date: 19-May-26	Related CR: 2026133015	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Breast implant leakage</i>	Approved as Requested	Proposed LLT <i>Breast implant leakage</i>	10093739 Current
			To PT <i>Device leakage</i>	10012587
MSSO Comment: The proposal to add a new LLT Breast implant leakage is approved as requested. Saline and silicone breast implants have an outer silicone shell that can potentially develop a small tear or hole. Saline implants also have a valve that can fail, causing a leak. Breast implant leakage will be added as an LLT to PT Device leakage.				

Detail Report

CR Number: 2026133016

Implementation Date: 18-May-26

Related CR: 2026133016

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT *Corneal pseudomicrocysts*

Approved Not as Requested

Proposed PT

Corneal pseudomicrocysts

10093735

HLT primary *Corneal disorders NEC*

HLT primary

Corneal structural change, deposit and degeneration

10011049

SOC primary *Eye disorders*

SOC primary

Eye disorders

10015919

MSSO Comment: The proposal to add a new PT Corneal pseudomicrocysts to primary HLT Corneal disorders NEC in SOC Eye disorders is approved but not as requested. Corneal pseudomicrocysts are tiny, blister-like structures that form in the basal layer of the cornea's epithelial layer. Because they are not fluid-filled, they are classified as "pseudomicrocysts" and are commonly caused by certain targeted cancer therapies, specifically antibody-drug conjugates. Corneal pseudomicrocysts will be added as a PT to HLT Corneal structural change, deposit and degeneration.

CR Number: 2026134000

Implementation Date: 19-May-26

Related CR: 2026134000

Change Requested

Link (move) an LLT to another PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed LLT *Opticneuroretinitis*

Approved as Requested

Proposed LLT

Opticneuroretinitis

10030958 Non-Current

From PT *Optic neuritis*

From PT

Optic neuritis

10030942

To PT *Neuroretinitis*

To PT

Neuroretinitis

10060701

MSSO Comment: The proposal to move the LLT Opticneuroretinitis from PT Optic neuritis to PT Neuroretinitis is approved as requested. Although, LLT Opticneuroretinitis is non-current, it is generally seen as a synonym or historical variant of neuroretinitis rather than a subtype of isolated optic neuritis. For that reason, it is better aligned under PT Neuroretinitis. Unlike PT Optic neuritis, neuroretinitis specifically implies combined optic nerve and retinal involvement.

Detail Report

CR Number: 2026134001		Implementation Date: 19-May-26	Related CR: 2026134001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Polyendocrine metabolic ovarian syndrome</i>	Approved Not as Requested	Proposed LLT <i>Polyendocrine metabolic ovarian syndrome</i>	10093740 Current
			To PT <i>Polycystic ovarian syndrome</i>	10065161
MSSO Comment: The proposal to add a new PT Polyendocrine metabolic ovarian syndrome is approved but not as requested. Although polyendocrine metabolic ovarian syndrome has recently emerged through an international consensus initiative as a proposed updated nomenclature for the disorder traditionally known as polycystic ovary syndrome, this late historical denomination remains overwhelmingly established in current clinical practice, scientific literature, regulatory documents, and pharmacovigilance databases. Demotion of the well-recognized PT Polycystic ovarian syndrome could affect coding consistency and legacy data retrieval. Based on this Polyendocrine metabolic ovarian syndrome will be added as an LLT to PT Polycystic ovarian syndrome.				

CR Number: 2026138000		Implementation Date: 21-May-26	Related CR: 2026138000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Orgasm headache</i>	Approved Not as Requested	Proposed LLT <i>Orgasmic headache</i>	10093744 Current
To PT	<i>Primary headache associated with sexual activity</i>		To PT <i>Primary headache associated with sexual activity</i>	10079056
MSSO Comment: The proposal to add a new LLT Orgasm headache to PT Primary headache associated with sexual activity is approved but not as requested. Headache associated with sexual activity is one of the uncommon headache syndromes that occurs in the setting of a specific recurring trigger. Symptom onset may be gradual during sexual activity or sudden at the time of orgasm. The proposed term will be modified to the form more commonly found in medical literature and added as LLT Orgasmic headache to PT Primary headache associated with sexual activity.				

CR Number: 2026138001		Implementation Date: 21-May-26	Related CR: 2026138001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>MASCC score</i>	Approved Not as Requested	Proposed PT <i>MASCC risk index for febrile neutropenia</i>	10093743
			HLT primary <i>Investigations NEC</i>	10022892
			SOC primary <i>Investigations</i>	10022891
MSSO Comment: The proposal to add a new term MASCC score is approved but not as requested. The Multinational Association for Supportive Care in Cancer (MASCC) risk index is a validated tool for measuring the risk for neutropenic fever-related medical complications. Clinically, the score is specifically validated to identify low-risk patients with febrile neutropenia who may be candidates for outpatient management. The proposed term will be modified for clarity and added as PT MASCC risk index for febrile neutropenia to HLT Investigations NEC.				

Detail Report

CR Number: 2026138002	Implementation Date: 21-May-26	Related CR: 2026138002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Cutaneous lichen planus</i>	Rejected	

MSSO Comment: The proposal to add a new term Cutaneous lichen planus is not approved. Lichen planus is a chronic inflammatory condition in which the immune system attacks skin and mucous membranes, producing characteristic flat-topped, purple, itchy papules. It can also involve the oral cavity, nails, and scalp, sometimes leading to erosions, white reticulated patches, or hair loss depending on the site affected. The PT hierarchy reflects that the condition is cutaneous. It is not possible to qualify every concept in MedDRA with "cutaneous" when the term hierarchy supports the nature of the term.

CR Number: 2026138003	Implementation Date: 21-May-26	Related CR: 2026138003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Nail lichen planus</i>	Approved as Requested	Proposed PT
			<i>Nail lichen planus</i> 10093742
			HLT primary
			<i>Papulosquamous conditions</i> 10033738
			SOC primary
			<i>Skin and subcutaneous tissue disorders</i> 10040785

MSSO Comment: The proposal to add a new term Nail lichen planus is approved as requested. Lichen planus is an inflammatory condition of unknown etiology that affects the skin, mucous membranes, hair follicles, and nails. Approximately 10 to 25 percent of patients with lichen planus have nail involvement. Lichen planus of the nail matrix results in longitudinal ridging, nail plate thinning, longitudinal fissuring trachyonychia, and erythema of the lunula. Nail bed lichen planus usually occurs in conjunction with nail matrix involvement and is characterized by onycholysis, with or without subungual hyperkeratosis, and a violaceous hue of the nail bed. Nail lichen planus will be added as PT to HLT Papulosquamous conditions.

CR Number: 2026139000	Implementation Date: 22-May-26	Related CR: 2026139000	MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement
Proposed PT	<i>Extrapulmonary neuroendocrine carcinoma</i>	Approved Not as Requested	Proposed LLT
HLT primary	<i>Endocrine neoplasms malignant and unspecified NEC</i>		<i>Extrapulmonary neuroendocrine carcinoma</i> 10093748 Current
SOC primary	<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>		To PT
			<i>Neuroendocrine carcinoma</i> 10057270

MSSO Comment: The proposal to add a new PT Extrapulmonary neuroendocrine carcinoma to primary HLT Endocrine neoplasms malignant and unspecified NEC in SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps) and secondary HLT Endocrine neoplasms NEC in SOC Endocrine disorders is approved but not as requested. Although this concept is still a broad grouping term covering neuroendocrine carcinomas from different anatomical sites rather than a single clearly defined entity, the expression is increasingly used in medical literature as a collective descriptor for high-grade neuroendocrine carcinomas arising outside the lung. Extrapulmonary neuroendocrine carcinoma will be added as LLT to PT Neuroendocrine carcinoma.

Detail Report

CR Number: 2026139001		Implementation Date: 21-May-26	Related CR: 2026139001		
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Proliferative diabetic retinopathy</i>	Rejected			
From PT	<i>Diabetic retinopathy</i>				
HLT primary	<i>Retinopathies NEC</i>				
MSSO	The proposal to promote the LLT Proliferative diabetic retinopathy from PT Diabetic retinopathy to primary HLT Retinopathies NEC in SOC Eye disorders is not approved.				
Comment:	The concept is a sub-stage of diabetic retinopathy and well placed at LLT level. Analysis may be performed on LLT level.				

CR Number: 2026139002		Implementation Date: 22-May-26	Related CR: 2026139002		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Cavernous hemangioma of the orbit</i>	Approved as Requested	Proposed LLT		
			<i>Cavernous hemangioma of the orbit</i>	10093751	Current
			To PT		
			<i>Orbital cavernous venous malformation</i>	10093749	
MSSO	The proposal to add a new LLT Cavernous hemangioma of the orbit is approved as requested. Orbital cavernous venous malformation (formerly known as cavernous hemangioma) is a benign, non-infiltrative, slowly progressive vascular neoplasm surrounded by a well-delineated fibrous capsule. It is the most common benign neoplasm of the orbit in adults and is considered a congenital abnormality. Cavernous hemangioma of the orbit will be added as LLT to the more updated PT term for the concept- PT Orbital cavernous venous malformation. In a related change, Orbital cavernous venous malformation will be added as a new PT to primary HLT Vascular anomalies congenital NEC and to secondary HLT Ocular neoplasms benign and to secondary HLT Vascular malformations and acquired anomalies. Furthermore, the British English counterpart LLT Cavernous haemangioma of the orbit will also be added to new PT Orbital cavernous venous malformation.				
Comment:					

Detail Report

CR Number: 2026142103		Implementation Date: 22-May-26	Related CR: 2026139002	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Orbital cavernous venous malformation</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Vascular anomalies congenital NEC</i>		<i>Orbital cavernous venous malformation</i>	10093749
SOC primary	<i>Congenital, familial and genetic disorders</i>		HLT primary	
			<i>Vascular anomalies congenital NEC</i>	10047047
			SOC primary	
			<i>Congenital, familial and genetic disorders</i>	10010331
			HLT secondary	
			<i>Ocular neoplasms benign</i>	10030056
			SOC secondary	
			<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary	
			<i>Vascular malformations and acquired anomalies</i>	10047091
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO
Comment:

CR Number: 2026142104		Implementation Date: 22-May-26	Related CR: 2026139002	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Cavernous haemangioma of the orbit</i>	Approved as Requested	Proposed LLT	
To PT	<i>Orbital cavernous venous malformation</i>		<i>Cavernous haemangioma of the orbit</i>	10093750 Current
			To PT	
			<i>Orbital cavernous venous malformation</i>	10093749

MSSO
Comment:

Detail Report

CR Number: 2026139003		Implementation Date: 21-May-26	Related CR: 2026139003	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pulmonary Embolism Severity Index (PESI)</i>	Approved Not as Requested	Proposed PT <i>Pulmonary embolism severity index</i>	10093741
			HLT primary <i>Investigations NEC</i>	10022892
			SOC primary <i>Investigations</i>	10022891

MSSO Comment: The proposal to add a new LLT Pulmonary Embolism Severity Index (PESI) is approved but not as requested. The pulmonary embolism severity index is a clinical tool that predicts 30-day mortality risk for patients with acute pulmonary embolism. By evaluating 11 clinical variables, the index divides patients into five risk classes to guide management, such as determining if a patient is a candidate for outpatient treatment. Proposed concept will be modified for MedDRA capitalization conventions as well as removal of acronym and added as PT Pulmonary embolism severity index to HLT Investigations NEC.

CR Number: 2026140000		Implementation Date: 22-May-26	Related CR: 2026140000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Tachon syndrome</i>	Rejected		

MSSO Comment: The proposal to add a new term Tachon syndrome is not approved. Tachon syndrome is a legitimate medical eponym but has limited international penetration (mainly French literature) and is recognized mainly within certain rheumatology and corticosteroid-injection literature rather than as a universally established syndrome name. Please consider existing LLT/PT Immediate post-injection reaction for your coding needs.

CR Number: 2026140001		Implementation Date: 22-May-26	Related CR: 2026140001	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Microthrombus</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Microthrombus is not approved. MedDRA cannot accommodate terms related to different sizes of thrombi. Without anatomical location, the term is nonspecific and lacks clinical context.

Detail Report

CR Number: 2026140002	Implementation Date: 22-May-26	Related CR: 2026140002	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Platelet activation</i>	Rejected	

MSSO Comment: The proposal to add a new term Platelet activation is not approved. Platelet activation is the biological response that converts platelets into adhesive, clot-forming cells after vascular injury. As such this is a physiologic response and not in scope of MedDRA terminology.

CR Number: 2026140003	Implementation Date: 22-May-26	Related CR: 2026140003	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Stable (oncological) disease</i>	Approved Not as Requested	Proposed PT <i>Stable disease</i> 10093745 HLT primary <i>General signs and symptoms NEC</i> 10018072 SOC primary <i>General disorders and administration site conditions</i> 10018065

MSSO Comment: The proposal to add a new term Stable (oncological) disease is approved but not as requested. Stable disease is a clinical term used to describe a condition in which a disease is neither significantly improving nor worsening over a defined period of time. In oncology, it generally indicates that the disease has not shown enough improvement to qualify as a response, nor sufficient progression to qualify as disease progression according to established clinical or radiological criteria. The proposed concept will be modified by removing parentheses and added as PT Stable disease to HLT General signs and symptoms NEC.

CR Number: 2026140004	Implementation Date: 22-May-26	Related CR: 2026140004	MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement
Proposed Term	<i>Ileoascendostomy</i>	Approved as Requested	Proposed LLT <i>Ileoascendostomy</i> 10093747 Current To PT <i>Ileocolostomy</i> 10052997

MSSO Comment: The proposal to add a new term Ileoascendostomy is approved as requested. Ileoascendostomy is a surgical procedure in which the ileum is directly anastomosed to the ascending colon, typically after resection or bypass of the intervening bowel segment. Ileoascendostomy will be added as LLT to PT Ileocolostomy.

Detail Report

CR Number: 2026140005		Implementation Date: 22-May-26	Related CR: 2026140005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Ileodescendostomy</i>	Approved as Requested	Proposed LLT <i>Ileodescendostomy</i>	10093746 Current
			To PT <i>Ileocolostomy</i>	10052997
MSSO Comment: The proposal to add a new term Ileodescendostomy is approved as requested. Ileodescendostomy is a surgical anastomosis in which the ileum is directly connected to the descending colon, usually to restore intestinal continuity after resection or bypass of the intervening colon. Ileodescendostomy will be added as LLT to PT Ileocolostomy.				

CR Number: 2026140013		Implementation Date: 22-May-26	Related CR: 2026140013	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Pseudolipoma of inferior vena cava</i>	Rejected		
MSSO Comment: The proposal to add a new LLT Pseudolipoma of inferior vena cava is not approved. The proposed concept is usually found incidentally as an imaging artifact. Please consider existing LLT /PT Imaging procedure artifact for your coding needs.				

CR Number: 2026140014		Implementation Date: 26-May-26	Related CR: 2026140014	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Thulium laser enucleation</i>	Approved Not as Requested	Proposed LLT <i>Thulium laser enucleation of prostate</i>	10093752 Current
			To PT <i>Transurethral prostatectomy</i>	10044445
MSSO Comment: The proposal to add a new LLT Thulium laser enucleation is approved but not as requested. Thulium laser enucleation of the prostate is the complete transurethral removal of the transition zone (enucleation) of the prostate with the support of the Thulium laser. This procedure combines complete anatomical enucleation and maximum urodynamic efficacy with minimal side-effects. The proposed term will be modified for clarity and added as LLT Thulium laser enucleation of prostate to PT Transurethral prostatectomy.				

Detail Report

CR Number: 2026140015		Implementation Date: 22-May-26	Related CR: 2026140015		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Transurethral laser ablation	Rejected			

MSSO Comment: The proposal to add a new LLT Transurethral laser ablation is not approved. The proposed concept is broad and does not specify the anatomical target area. It may refer to prostatic ablation for benign hyperplasia, bladder carcinoma resection, urethral lesion removal, etc.

CR Number: 2026140016		Implementation Date: 03-Jun-26	Related CR: 2026140016		MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement		
Proposed LLT	Wrong drug dispensed	Approved After Suspension	Proposed LLT		
From PT	Product dispensing issue		Wrong drug dispensed	10065631	Current
To PT	Product dispensing error		From PT		
			Product dispensing issue	10081771	
			To PT		
			Product dispensing error	10080359	

MSSO Comment: The proposal to move the LLT Wrong drug dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested after suspension. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026140017		Implementation Date: 03-Jun-26	Related CR: 2026140017	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Active ingredient not added to diluent</i>	Approved After Suspension	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Active ingredient not added to diluent</i>	10081107 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Active ingredient not added to diluent from PT Product preparation issue to PT Product preparation error is approved as requested after suspension. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026140018		Implementation Date: 26-May-26	Related CR: 2026140018	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Local immune effector cell-associated toxicity syndrome</i>	Rejected		
MSSO Comment: The proposal to add a new PT Local immune effector cell-associated toxicity syndrome is not approved. The proposed term should not be added to MedDRA at this stage. Although Local immune effector cell-associated toxicity syndrome (LICATS) appears to describe a potentially real and clinically relevant phenomenon, the concept is not fully established yet. The Lancet article remains being the only one published, while the second publication provided is a correspondence rather than an independent full study. Importantly, that correspondence reinforces the need for consensus diagnostic criteria and acknowledges that the clinical features of LICATS are still uncertain, even challenging one of the defining features proposed in the original article, namely restriction to previously affected organs. Therefore, while LICATS should be monitored as an emerging safety concept in CAR T-cell therapy, it currently lacks the level of international validation, diagnostic stability, and definition required for inclusion as a MedDRA term.				

Detail Report

CR Number: 2026140019		Implementation Date: 22-May-26	Related CR: 2026140019	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Maize allergy</i>	Approved Not as Requested	Proposed LLT <i>Corn allergy</i>	10093753 Current
			To PT <i>Food allergy</i>	10016946

MSSO Comment: The proposal to add a new term Maize allergy is approved but not as requested. Corn allergy is an immunologically mediated hypersensitivity reaction triggered by exposure to proteins derived from corn (maize), resulting in manifestations that may involve the skin, gastrointestinal tract, respiratory system, or, in severe cases, systemic anaphylaxis. The proposed term will be modified to Corn allergy, which is more commonly used in clinical practice, food labeling and spontaneous safety reporting. Corn allergy will be added as LLT to PT Food allergy.

CR Number: 2026141103		Implementation Date: 09-Jun-26	Related CR: 2026141103	
Change Requested	Add a new SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed SMQ	<i>Non-lymphoid Hematologic Neoplasms</i>	Suspended		

MSSO Comment: The proposal to add a new SMQ Non-lymphoid Hematologic Neoplasms is suspended. This request will be evaluated to make an initial recommendation on whether to proceed forward with developing the proposed SMQ. If the recommendation is to proceed, then a team comprised of regulatory and industry representatives will be formed to develop and test the proposed SMQ. Inclusion / exclusion criteria and other parts of the SMQ documentation will be created and the SMQ will be given to the MSSO for maintenance. If the answer is not to proceed forward, you will receive a justification explaining why the SMQ was not approved.

CR Number: 2026141104		Implementation Date: 26-May-26	Related CR: 2026141104	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Fever malignant</i>	Approved as Requested	Proposed LLT <i>Fever malignant</i>	10016561 Non-Current
From PT	<i>Hyperthermia malignant</i>		From PT <i>Hyperthermia malignant</i>	10020844
To PT	<i>Tumour associated fever</i>		To PT <i>Tumour associated fever</i>	10052243

MSSO Comment: The proposal to move the LLT Fever malignant from PT Hyperthermia malignant to PT Tumour associated fever is approved as requested. Malignant fever is an ambiguous term, as it can refer to different scenarios. In oncology, malignant fever is frequently used to describe neoplastic or cancer-related fever. It has historically been used in medical literature to refer to dangerous or life-threatening fevers (virulent or rapidly worsening), including epidemic typhus or severe malaria. It is usually not considered a synonym of malignant hyperthermia, which is an autosomal dominantly inherited disorder characterized by skeletal muscle hypermetabolism following exposure to halogenated anesthetics. LLT Fever malignant will be moved to PT Tumour associated fever, for better alignment with the widely accepted usage of the term and made non-current because of the mentioned ambiguity of the concept. In a related change, LLT Fever malignant will be made non-current.

Detail Report

CR Number: 2026146058		Implementation Date: 26-May-26	Related CR: 2026141104		MedDRA Code #
Change Requested	Change Status of LLT	Final Disposition	Final Placement		
LLT to change	<i>Fever malignant</i>	Approved as Requested	LLT to change		
Status	<i>Non-current</i>		<i>Fever malignant</i>	10016561	Non-Current
			Status		
			<i>Non-current</i>		

MSSO
Comment:

CR Number: 2026142072		Implementation Date: 01-Jun-26	Related CR: 2026142072		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Infantile generalized epileptic spasms</i>	Approved After Suspension	Proposed LLT		
To PT	<i>Infantile spasms</i>		<i>Infantile generalized epileptic spasms</i>	10093772	Current
			To PT		
			<i>Infantile spasms</i>	10021750	

MSSO
Comment: The proposal to add a new LLT Infantile generalized epileptic spasms to PT Infantile spasms is approved as requested after suspension. In order to accommodate the occurrence of generalized epileptic spasms which are associated most commonly with infantile spasms, qualified new LLT Infantile generalized epileptic spasms and LLT Infantile generalised epileptic spasms will both be added to PT Infantile spasms. In a related change, a new PT Epileptic spasms will be added to HLT Seizures and seizure disorders NEC. Furthermore, LLT Generalised epileptic spasms and LLT Generalized epileptic spasms will be moved from PT Infantile spasms to new PT Epileptic spasms.

CR Number: 2026152031		Implementation Date: 01-Jun-26	Related CR: 2026142072		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Infantile generalised epileptic spasms</i>	Approved as Requested	Proposed LLT		
To PT	<i>Infantile spasms</i>		<i>Infantile generalised epileptic spasms</i>	10093771	Current
			To PT		
			<i>Infantile spasms</i>	10021750	

MSSO
Comment:

Detail Report

CR Number: 2026152034		Implementation Date: 01-Jun-26	Related CR: 2026142072	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Generalized epileptic spasms</i>	Approved as Requested	Proposed LLT	10093391 Current
From PT	<i>Infantile spasms</i>		<i>Generalized epileptic spasms</i>	
To PT	<i>Epileptic spasms</i>		From PT	10021750
			<i>Infantile spasms</i>	
			To PT	10093770
			<i>Epileptic spasms</i>	

MSSO
Comment:

CR Number: 2026152033		Implementation Date: 01-Jun-26	Related CR: 2026142072	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Generalised epileptic spasms</i>	Approved as Requested	Proposed LLT	10093398 Current
From PT	<i>Infantile spasms</i>		<i>Generalised epileptic spasms</i>	
To PT	<i>Epileptic spasms</i>		From PT	10021750
			<i>Infantile spasms</i>	
			To PT	10093770
			<i>Epileptic spasms</i>	

MSSO
Comment:

Detail Report

CR Number: 2026152032		Implementation Date: 01-Jun-26	Related CR: 2026142072	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Epileptic spasms</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Seizures and seizure disorders NEC</i>		<i>Epileptic spasms</i>	10093770
SOC primary	<i>Nervous system disorders</i>		HLT primary	
			<i>Seizures and seizure disorders NEC</i>	10039912
			SOC primary	
			<i>Nervous system disorders</i>	10029205

MSSO
Comment:

CR Number: 2026142073		Implementation Date: 28-May-26	Related CR: 2026142073	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Ocular caruncular nevus</i>	Approved Not as Requested	Proposed LLT	
			<i>Lacrimonasal caruncle nevus</i>	10093761 Current
			To PT	
			<i>Eye naevus</i>	10051045

MSSO
Comment: The proposal to add a new PT Ocular caruncular nevus is approved but not as requested. Conjunctival nevi are benign, heterogenous tumors that uncommonly arise from the lacrimal caruncle and plica semilunaris. The vast majority of lesions of the caruncle are benign, mainly nevi. Proposed term will be modified in the most commonly referenced form in literature and added as LLT Lacrimonasal caruncle nevus to PT Eye naevus. In a related change, the British English spelled LLT Lacrimonasal caruncle naevus will also be added to PT Eye naevus.

CR Number: 2026148014		Implementation Date: 28-May-26	Related CR: 2026142073	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Lacrimonasal caruncle naevus</i>	Approved as Requested	Proposed LLT	
To PT	<i>Eye naevus</i>		<i>Lacrimonasal caruncle naevus</i>	10093763 Current
			To PT	
			<i>Eye naevus</i>	10051045

MSSO
Comment:

Detail Report

CR Number: 2026142074		Implementation Date: 28-May-26	Related CR: 2026142074	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Rhizomelic dwarfism</i>	Approved as Requested	Proposed LLT	10093762 Current
To PT	<i>Dwarfism</i>		<i>Rhizomelic dwarfism</i>	
			To PT	10013883
		<i>Dwarfism</i>		
MSSO The proposal to add a new LLT Rhizomelic dwarfism to PT Dwarfism is approved as requested. Rhizomelic dwarfism is a type of dwarfism in which the dominant feature is				
Comment: proximal (i.e. femoral and humeral) limb shortening.				

CR Number: 2026142092		Implementation Date: 26-May-26	Related CR: 2026142092	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Bronchial inflammation</i>	Approved as Requested	Proposed PT	10093754
			<i>Bronchial inflammation</i>	
			HLT primary	10006429
			<i>Bronchial conditions NEC</i>	
			SOC primary	10038738
			<i>Respiratory, thoracic and mediastinal disorders</i>	

MSSO The proposal to add a new PT Bronchial inflammation is approved as requested. Bronchial inflammation is a general pathological process which can occur due to many causes (infection, smoke, asthma, chemical irritants, etc.). Bronchial inflammation will be added as PT to HLT Bronchial conditions NEC. In a related change, PT Noninfective bronchitis will be demoted under this new PT Bronchial inflammation to avoid redundancy at the PT level.

CR Number: 2026146059		Implementation Date: 26-May-26	Related CR: 2026142092	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Noninfective bronchitis</i>	Approved as Requested	PT to merge	10062644 Current
To PT	<i>Bronchial inflammation</i>		<i>Noninfective bronchitis</i>	
			To PT	10093754
			<i>Bronchial inflammation</i>	

MSSO
Comment:

Detail Report

CR Number: 2026142093		Implementation Date: 28-May-26	Related CR: 2026142093	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	SCN1A-related epilepsy	Approved Not as Requested	Proposed PT SCN1A gene mutation	10093764
			HLT primary Neurological disorders congenital NEC	10029300
			SOC primary Congenital, familial and genetic disorders	10010331
			HLT secondary Seizures and seizure disorders NEC	10039912
			SOC secondary Nervous system disorders	10029205

MSSO Comment: The proposal to add a new term SCN1A-related epilepsy is approved but not as requested. SCN1A-related epilepsy is a spectrum of genetic epilepsies caused by mutations in the SCN1A gene, ranging from mild febrile seizure disorders to severe epileptic encephalopathies such as Dravet syndrome. The concept is clinically important because some sodium channel-blocking antiepileptic drugs may worsen certain SCN1A-associated epilepsies. The proposed term will be modified as a narrower concept and added as PT SCN1A gene mutation to primary HLT Neurological disorders congenital NEC and to secondary HLT Seizures and seizure disorders NEC.

CR Number: 2026142094		Implementation Date: 26-May-26	Related CR: 2026142094	
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #
LLT to change	Closed capsulotomy	Approved as Requested	LLT to change Closed capsulotomy	10068151 Non-Current
Status	non-current		Status non-current	

MSSO Comment: The proposal to change status of LLT Closed capsulotomy to non-current is approved as requested. Based on a review, "Closed capsulotomy" is anatomically imprecise and misleading. A new term LLT Closed breast capsulotomy will be added (see change request 2026142095) to provide anatomical clarity for the concept.

Detail Report

CR Number: 2026142095		Implementation Date: 26-May-26	Related CR: 2026142095		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Closed breast capsulotomy	Approved as Requested	Proposed LLT		
To PT	Breast capsulotomy		Closed breast capsulotomy	10093755	Current
			To PT		
			Breast capsulotomy	10075014	
MSSO Comment: The proposal to add a new LLT Closed breast capsulotomy to PT Breast capsulotomy is approved as requested. Closed breast capsulotomy provides anatomic clarity and replaces existing LLT Closed capsulotomy which is anatomically imprecise and misleading. See change request 2026142094 to change the status of LLT Closed capsulotomy to non-current.					

CR Number: 2026142096		Implementation Date: 05-Jun-26	Related CR: 2026142096		
<u>Change Requested</u>	Change Status of LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
LLT to change	<i>Posterior capsulotomy</i>	Rejected After Suspension			
Status	<i>non-current</i>				
MSSO Comment: The proposal to change status of LLT Posterior capsulotomy to non-current is not approved after suspension. Posterior capsulotomy is known to be associated with cataract operations.					

CR Number: 2026142097		Implementation Date: 28-May-26	Related CR: 2026142097		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Posterior capsulotomy of lens</i>	Rejected After Suspension			
To PT	<i>Posterior lens capsulotomy</i>				
MSSO Comment: The proposal to add a new LLT Posterior capsulotomy of lens to PT Posterior lens capsulotomy is not approved after suspension. The existing LLT/PT Posterior lens capsulotomy captures the specificity of the anatomic location.					

Detail Report

CR Number: 2026142098		Implementation Date: 26-May-26	Related CR: 2026142098	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Hantavirus infection</i>	Approved as Requested	Proposed LLT	
To PT	<i>Hantaviral infection</i>		<i>Hantavirus infection</i>	10093760 Current
			To PT	
			<i>Hantaviral infection</i>	10061198
MSSO The proposal to add a new LLT Hantavirus infection to PT Hantaviral infection is approved as requested. This request is proposed to proactively add concepts that may be				
Comment: useful to MedDRA users for the recent outbreak of Hantavirus. LLT Hantavirus infection will be added to represent the most commonly reported concept term option.				

CR Number: 2026142099		Implementation Date: 26-May-26	Related CR: 2026142099	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Hantavirus test negative</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Virus identification and serology</i>		<i>Hantavirus test negative</i>	10093756
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Virus identification and serology</i>	10047490
			SOC primary	
			<i>Investigations</i>	10022891
MSSO The proposal to add a new PT Hantavirus test negative to primary HLT Virus identification and serology in SOC Investigations is approved as requested. This request is				
Comment: proposed to proactively add concepts that may be useful to MedDRA users for the recent outbreak of Hantavirus. PT Hantavirus test negative will represent the "negative" investigative finding counterpart to existing LLT/PT Hantavirus test positive.				

CR Number: 2026142100		Implementation Date: 26-May-26	Related CR: 2026142100	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Ebola virus test negative</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Virus identification and serology</i>		<i>Ebola virus test negative</i>	10093757
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Virus identification and serology</i>	10047490
			SOC primary	
			<i>Investigations</i>	10022891
MSSO The proposal to add a new PT Ebola virus test negative to primary HLT Virus identification and serology in SOC Investigations is approved as requested. This request is				
Comment: proposed to proactively add concepts that may be useful to MedDRA users for the recent outbreak of the Ebola virus. PT Ebola virus test negative will represent the "negative" investigative finding counterpart to existing LLT/PT Ebola virus test positive.				

Detail Report

CR Number: 2026142101		Implementation Date: 26-May-26	Related CR: 2026142101	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Bundibugyo Ebola virus</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Ebola disease</i>		<i>Bundibugyo Ebolavirus infection</i>	10093759 Current
			To PT	
			<i>Ebola disease</i>	10014071
MSSO Comment: The proposal to add a new LLT Bundibugyo Ebola virus to PT Ebola disease is approved but not as requested. This request is proposed to proactively add concepts that may be useful to MedDRA users for the recent outbreak of Ebola virus. The proposed term will be modified and added as LLT Bundibugyo Ebolavirus infection to PT Ebola disease.				

CR Number: 2026142102		Implementation Date: 26-May-26	Related CR: 2026142102	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Zaire Ebola virus infection</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Ebola disease</i>		<i>Zaire Ebolavirus infection</i>	10093758 Current
			To PT	
			<i>Ebola disease</i>	10014071
MSSO Comment: The proposal to add a new LLT Zaire Ebola virus infection to PT Ebola disease is approved but not as requested. This request is proposed to proactively add concepts that may be useful to MedDRA users for the recent outbreak of the Ebola virus. The proposed term will be modified and added as LLT Zaire Ebolavirus infection to PT Ebola disease.				

CR Number: 2026145000		Implementation Date: 26-May-26	Related CR: 2026145000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hypertriglyceridemia induced pancreatitis</i>	Rejected		
MSSO Comment: The proposal to add a new term Hypertriglyceridemia induced pancreatitis is not approved. The proposed term combines a medical concept with additional medical wording related to etiology and/or pathophysiology. MedDRA terminology is generally not intended to represent presumed or interpreted causality within a single term, as causality assessment depends on clinical judgment and case evaluation. Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.				

Detail Report

CR Number: 2026146000		Implementation Date: 28-May-26	Related CR: 2026146000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Ocular myasthenia gravis	Approved as Requested	Proposed LLT Ocular myasthenia gravis	10093765 Current
			To PT Myasthenia gravis	10028417

MSSO Comment: The proposal to add a new term Ocular myasthenia gravis is approved as requested. Ocular myasthenia gravis (OMG) refers to clinical manifestations of myasthenia gravis (MG) which are isolated to the levator palpebrae superioris, orbicularis oculi, and extraocular muscles affecting eye movement. Approximately 15 percent of all patients with MG have isolated OMG as the only manifestation of their disease. Ocular myasthenia gravis will be added as LLT to PT Myasthenia gravis.

CR Number: 2026146001		Implementation Date: 28-May-26	Related CR: 2026146001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Cornual heterotopic pregnancy	Approved as Requested	Proposed LLT Cornual heterotopic pregnancy	10093766 Current
			To PT Heterotopic pregnancy	10064228

MSSO Comment: The proposal to add a new term Cornual heterotopic pregnancy is approved as requested. Cornual heterotopic pregnancy (also called interstitial heterotopic pregnancy) is a heterotopic pregnancy in which the ectopic gestation is implanted in the interstitial/cornual portion of the fallopian tube at its junction with the uterus. It is a clinically highly relevant concept because a cornual/interstitial pregnancy may progress longer than other ectopic pregnancies and can rupture with severe haemorrhage due to its surrounding highly vascularised myometrium. Cornual heterotopic pregnancy will be added as LLT to PT Heterotopic pregnancy.

CR Number: 2026146002		Implementation Date: 28-May-26	Related CR: 2026146002	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Chest congestion	Rejected		
From PT	Respiratory tract congestion			
To PT	Lower respiratory tract congestion			

MSSO Comment: The proposal to move the LLT Chest congestion from PT Respiratory tract congestion to PT Lower respiratory tract congestion is not approved. Chest congestion is not truly a precise anatomical term. More in colloquial usage than in clinical language, "congestion" in the chest usually describes a subjective sensation of tightness, mucus accumulation, rather than a clear anatomical localization. This makes it difficult to state that it belongs to the lower respiratory tract. The colloquial and ambiguous nature of the term explains why it was made non-current.

Detail Report

CR Number: 2026146004 Implementation Date: 03-Jun-26 Related CR: 2026146004

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Drug maladministration</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product administration issue</i>		<i>Drug maladministration</i>	10013735 Current
To PT	<i>Product administration error</i>		From PT	
			<i>Product administration issue</i>	10092935
			To PT	
			<i>Product administration error</i>	10081576

MSSO Comment: The proposal to move the LLT Drug maladministration from PT Product administration issue to PT Product administration error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

CR Number: 2026146005 Implementation Date: 03-Jun-26 Related CR: 2026146005

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Drug misadministration</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product administration issue</i>		<i>Drug misadministration</i>	10013736 Current
To PT	<i>Product administration error</i>		From PT	
			<i>Product administration issue</i>	10092935
			To PT	
			<i>Product administration error</i>	10081576

MSSO Comment: The proposal to move the LLT Drug misadministration from PT Product administration issue to PT Product administration error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146006		Implementation Date: 03-Jun-26	Related CR: 2026146006	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong solution used in drug reconstitution</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong solution used in drug reconstitution</i>	10064356 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Wrong solution used in drug reconstitution from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146007		Implementation Date: 03-Jun-26	Related CR: 2026146007	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong drug strength selected</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product selection issue</i>		<i>Wrong drug strength selected</i>	10064361 Current
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542
MSSO Comment: The proposal to move the LLT Wrong drug strength selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146008		Implementation Date: 03-Jun-26	Related CR: 2026146008	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug product selected</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product selection issue</i>		<i>Wrong drug product selected</i>	10064362 Current
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542
MSSO Comment: The proposal to move the LLT Wrong drug product selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146009		Implementation Date: 03-Jun-26	Related CR: 2026146009	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong patient package insert selected during dispensing</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong patient package insert selected during dispensing</i>	10064363 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong patient package insert selected during dispensing from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by review of narrative data of cases coded to this LLT.				

Detail Report

CR Number: 2026146010		Implementation Date: 03-Jun-26	Related CR: 2026146010	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong label placed on medication during dispensing</i>	Approved as Requested	Proposed LLT	10064364 Current
From PT	<i>Product dispensing issue</i>		<i>Wrong label placed on medication during dispensing</i>	
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359

MSSO Comment: The proposal to move the LLT Wrong label placed on medication during dispensing from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

CR Number: 2026146011		Implementation Date: 03-Jun-26	Related CR: 2026146011	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug selected</i>	Approved as Requested	Proposed LLT	10064367 Current
From PT	<i>Product selection issue</i>		<i>Wrong drug selected</i>	
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542

MSSO Comment: The proposal to move the LLT Wrong drug selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146012		Implementation Date: 03-Jun-26	Related CR: 2026146012	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Expired device dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Device dispensing issue</i>		<i>Expired device dispensed</i>	10080363 Current
To PT	<i>Device dispensing error</i>		From PT	
			<i>Device dispensing issue</i>	10093288
			To PT	
			<i>Device dispensing error</i>	10080357
MSSO Comment: The proposal to move the LLT Expired device dispensed from PT Device dispensing issue to PT Device dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146013		Implementation Date: 03-Jun-26	Related CR: 2026146013	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong dosage form dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong dosage form dispensed</i>	10065632 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong dosage form dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146014		Implementation Date: 03-Jun-26	Related CR: 2026146014	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong directions typed on label</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong directions typed on label</i>	10065633 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong directions typed on label from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146015		Implementation Date: 03-Jun-26	Related CR: 2026146015	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Device implantation error</i>	Approved as Requested	Proposed LLT	
From PT	<i>Device placement error</i>		<i>Device implantation error</i>	10091628
HLT primary	<i>Medication errors, product use errors and issues NEC</i>		From PT	
			<i>Device placement error</i>	10092959
			HLT primary	
			<i>Medication errors, product use errors and issues NEC</i>	10079146
			SOC primary	
			<i>Injury, poisoning and procedural complications</i>	10022117
MSSO Comment: The proposal to promote the LLT Device implantation error from PT Device placement error to primary HLT Medication errors, product use errors and issues NEC in SOC Injury, poisoning and procedural complications is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded, that placement and implantation are two distinct concepts and should both be represented at PT level.				

Detail Report

CR Number: 2026146016		Implementation Date: 03-Jun-26	Related CR: 2026146016	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong route of administration dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong route of administration dispensed</i>	10065636 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong route of administration dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146017		Implementation Date: 03-Jun-26	Related CR: 2026146017	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug strength dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong drug strength dispensed</i>	10065637 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong drug strength dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146018		Implementation Date: 03-Jun-26	Related CR: 2026146018	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Device placement at incorrect location</i>	Approved as Requested	Proposed LLT	
From PT	<i>Device placement issue</i>		<i>Device placement at incorrect location</i>	10070084 Current
To PT	<i>Device placement error</i>		From PT	
			<i>Device placement issue</i>	10083995
			To PT	
			<i>Device placement error</i>	10092959
MSSO Comment: The proposal to move the LLT Device placement at incorrect location from PT Device placement issue to PT Device placement error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146019		Implementation Date: 03-Jun-26	Related CR: 2026146019	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Injector positioned upside down</i>	Approved as Requested	Proposed LLT	
From PT	<i>Device use issue</i>		<i>Injector positioned upside down</i>	10084372 Current
To PT	<i>Device use error</i>		From PT	
			<i>Device use issue</i>	10077812
			To PT	
			<i>Device use error</i>	10072878
MSSO Comment: The proposal to move the LLT Injector positioned upside down from PT Device use issue to PT Device use error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146020		Implementation Date: 03-Jun-26	Related CR: 2026146020	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Dosing error</i>	Approved as Requested	PT to merge	10093287 Current
To PT	<i>Wrong dose</i>		<i>Dosing error</i>	
			To PT	10080304
		<i>Wrong dose</i>		
MSSO Comment:	The proposal to demote the PT Dosing error under PT Wrong dose is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that to avoid the risk of signal dispersion, PT Dosing error should be demoted to PT Wrong drug, to have the more frequently reported and longer existing term on PT level.			

CR Number: 2026146021		Implementation Date: 03-Jun-26	Related CR: 2026146021	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong device dispensed</i>	Approved as Requested	Proposed LLT	10070469 Current
From PT	<i>Device dispensing issue</i>		<i>Wrong device dispensed</i>	
To PT	<i>Device dispensing error</i>		From PT	10093288
		<i>Device dispensing issue</i>		
		To PT	10080357	
		<i>Device dispensing error</i>		
MSSO Comment:	The proposal to move the LLT Wrong device dispensed from PT Device dispensing issue to PT Device dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.			

Detail Report

CR Number: 2026146022		Implementation Date: 03-Jun-26	Related CR: 2026146022	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug prescribed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product prescribing issue</i>		<i>Wrong drug prescribed</i>	10074911 Current
To PT	<i>Product prescribing error</i>		From PT	
			<i>Product prescribing issue</i>	10080459
			To PT	
			<i>Product prescribing error</i>	10081770
MSSO Comment: The proposal to move the LLT Wrong drug prescribed from PT Product prescribing issue to PT Product prescribing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146023		Implementation Date: 03-Jun-26	Related CR: 2026146023	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Inappropriate drug titration</i>	Approved as Requested	Proposed LLT	
From PT	<i>Drug titration issue</i>		<i>Inappropriate drug titration</i>	10074856 Current
To PT	<i>Drug titration error</i>		From PT	
			<i>Drug titration issue</i>	10093286
			To PT	
			<i>Drug titration error</i>	10074853
MSSO Comment: The proposal to move the LLT Inappropriate drug titration from PT Drug titration issue to PT Drug titration error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146024		Implementation Date: 03-Jun-26	Related CR: 2026146024	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Duplicate therapy with same active substance</i>	Approved as Requested	Proposed LLT	
From PT	<i>Duplicate therapy issue</i>		<i>Duplicate therapy with same active substance</i>	10083149 Current
To PT	<i>Duplicate therapy error</i>		From PT	
			<i>Duplicate therapy issue</i>	10093291
			To PT	
			<i>Duplicate therapy error</i>	10080422
MSSO Comment: The proposal to move the LLT Duplicate therapy with same active substance from PT Duplicate therapy issue to PT Duplicate therapy error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146025		Implementation Date: 03-Jun-26	Related CR: 2026146025	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Unintentional intravascular injection</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product administration error</i>		<i>Unintentional intravascular injection</i>	10075319 Current
To PT	<i>Incorrect route of product administration</i>		From PT	
			<i>Product administration error</i>	10081576
			To PT	
			<i>Incorrect route of product administration</i>	10081202
MSSO Comment: The proposal to move the LLT Unintentional intravascular injection from PT Product administration error to PT Incorrect route of product administration is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term is better placed under PT Incorrect route of product administration, as this PT is considered an error and provides more information on the type of error, which is lost in broader PT Product administration error.				

Detail Report

CR Number: 2026146026		Implementation Date: 03-Jun-26	Related CR: 2026146026	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong product selected</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product selection issue</i>		<i>Wrong product selected</i>	10076543 Current
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542
MSSO Comment: The proposal to move the LLT Wrong product selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146027		Implementation Date: 03-Jun-26	Related CR: 2026146027	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug concentration selected</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product selection issue</i>		<i>Wrong drug concentration selected</i>	10076545 Current
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542
MSSO Comment: The proposal to move the LLT Wrong drug concentration selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146028		Implementation Date: 03-Jun-26	Related CR: 2026146028	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong quantity of drug dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong quantity of drug dispensed</i>	10077344 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong quantity of drug dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146029		Implementation Date: 03-Jun-26	Related CR: 2026146029	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Drug stored in wrong location</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Drug stored in wrong location</i>	10077350 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843
MSSO Comment: The proposal to move the LLT Drug stored in wrong location from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146030		Implementation Date: 03-Jun-26	Related CR: 2026146030	
<u>Change Requested</u>	Demote a PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
PT to merge	<i>Wrong technique in product usage process performed in error</i>	Approved as Requested	PT to merge	
To PT	<i>Wrong technique in product usage process</i>		<i>Wrong technique in product usage process performed in error</i>	10093401 Non-Current
			To PT	
			<i>Wrong technique in product usage process</i>	10076573

MSSO Comment: The proposal to demote the PT Wrong technique in product usage process performed in error under PT Wrong technique in product usage process is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that to avoid the risk of signal dispersion, this PT should be demoted to PT Wrong technique in product usage process, which is considered to be an error term.

CR Number: 2026146031		Implementation Date: 03-Jun-26	Related CR: 2026146031	
<u>Change Requested</u>	Change Status of LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
LLT to change	<i>Wrong technique in product usage process performed in error</i>	Approved as Requested	LLT to change	
Status	<i>non-current</i>		<i>Wrong technique in product usage process performed in error</i>	10093401 Non-Current
			Status	
			<i>non-current</i>	

MSSO Comment: The proposal to change status of LLT Wrong technique in product usage process performed in error to non-current is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that to avoid confusion, this term should be made non-current as it is an unnecessary pleonasm by stating both wrong and performed in error in the term name.

CR Number: 2026146032		Implementation Date: 03-Jun-26	Related CR: 2026146032	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug concentration dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Wrong drug concentration dispensed</i>	10079519 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359

MSSO Comment: The proposal to move the LLT Wrong drug concentration dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146033		Implementation Date: 03-Jun-26	Related CR: 2026146033	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong concentration prepared</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong concentration prepared</i>	10079997 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Wrong concentration prepared from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146034		Implementation Date: 03-Jun-26	Related CR: 2026146034	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Deteriorated drug dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Deteriorated drug dispensed</i>	10065635 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Deteriorated drug dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146035		Implementation Date: 03-Jun-26	Related CR: 2026146035	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Expired drug dispensed</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Expired drug dispensed</i>	10067894 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Expired drug dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by review of narrative data of cases coded to this LLT.				

CR Number: 2026146036		Implementation Date: 03-Jun-26	Related CR: 2026146036	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Drug not dispensed in original container</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Drug not dispensed in original container</i>	10079998 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Drug not dispensed in original container from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146037		Implementation Date: 03-Jun-26	Related CR: 2026146037	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Pharmacy label placed incorrectly</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product dispensing issue</i>		<i>Pharmacy label placed incorrectly</i>	10080421 Current
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Pharmacy label placed incorrectly from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146038		Implementation Date: 03-Jun-26	Related CR: 2026146038	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Product concentrate not diluted</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Product concentrate not diluted</i>	10082542 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Product concentrate not diluted from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146039		Implementation Date: 03-Jun-26	Related CR: 2026146039	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Product used prior to being brought to recommended temperature</i>	Approved as Requested	Proposed LLT	10083148 Current
From PT	<i>Product preparation issue</i>		<i>Product used prior to being brought to recommended temperature</i>	
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Product used prior to being brought to recommended temperature from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146040		Implementation Date: 03-Jun-26	Related CR: 2026146040	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug product dispensed</i>	Approved as Requested	Proposed LLT	10080978 Current
From PT	<i>Product dispensing issue</i>		<i>Wrong drug product dispensed</i>	
To PT	<i>Product dispensing error</i>		From PT	
			<i>Product dispensing issue</i>	10081771
			To PT	
			<i>Product dispensing error</i>	10080359
MSSO Comment: The proposal to move the LLT Wrong drug product dispensed from PT Product dispensing issue to PT Product dispensing error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146041		Implementation Date: 03-Jun-26	Related CR: 2026146041	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Vaccine incorrect diluent used</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Vaccine incorrect diluent used</i>	10087263 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Vaccine incorrect diluent used from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146042		Implementation Date: 03-Jun-26	Related CR: 2026146042	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Vaccine incorrect diluent volume</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Vaccine incorrect diluent volume</i>	10087264 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Vaccine incorrect diluent volume from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146043		Implementation Date: 03-Jun-26	Related CR: 2026146043	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Diluent not added to active ingredient</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Diluent not added to active ingredient</i>	10090609 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Diluent not added to active ingredient from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146044		Implementation Date: 03-Jun-26	Related CR: 2026146044	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Incorrect dose selected</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product selection issue</i>		<i>Incorrect dose selected</i>	10080239 Current
To PT	<i>Product selection error</i>		From PT	
			<i>Product selection issue</i>	10093289
			To PT	
			<i>Product selection error</i>	10076542
MSSO Comment: The proposal to move the LLT Incorrect dose selected from PT Product selection issue to PT Product selection error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by review of narrative data of cases coded to this LLT.				

Detail Report

CR Number: 2026146045 Implementation Date: 03-Jun-26 Related CR: 2026146045

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong diluent used</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong diluent used</i>	10084335 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869

MSSO Comment: The proposal to move the LLT Wrong diluent used from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

CR Number: 2026146046 Implementation Date: 03-Jun-26 Related CR: 2026146046

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong eluent used</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong eluent used</i>	10084336 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869

MSSO Comment: The proposal to move the LLT Wrong eluent used from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word "wrong" in the term name and concluded unanimously, that "wrong" in a term name in HLG T Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146047		Implementation Date: 03-Jun-26	Related CR: 2026146047	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Incorrect storage of drug</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Incorrect storage of drug</i>	10064941 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843
MSSO Comment: The proposal to move the LLT Incorrect storage of drug from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146048		Implementation Date: 03-Jun-26	Related CR: 2026146048	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Incorrect product storage</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Incorrect product storage</i>	10069328 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843
MSSO Comment: The proposal to move the LLT Incorrect product storage from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146049		Implementation Date: 03-Jun-26	Related CR: 2026146049	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Improper storage of product in use</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Improper storage of product in use</i>	10080240 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843
MSSO Comment: The proposal to move the LLT Improper storage of product in use from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146050		Implementation Date: 03-Jun-26	Related CR: 2026146050	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Wrong drug strength prepared</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong drug strength prepared</i>	10090317 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Wrong drug strength prepared from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146051 Implementation Date: 03-Jun-26 Related CR: 2026146051

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Wrong drug strength compounded</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Wrong drug strength compounded</i>	10090608 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869

MSSO Comment: The proposal to move the LLT Wrong drug strength compounded from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of terms including the word “wrong” in the term name and concluded unanimously, that “wrong” in a term name in HLGT Medication errors and other product use errors and issues is always considered an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

CR Number: 2026146052 Implementation Date: 03-Jun-26 Related CR: 2026146052

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Improper storage of unused product</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Improper storage of unused product</i>	10080241 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843

MSSO Comment: The proposal to move the LLT Improper storage of unused product from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146053 Implementation Date: 03-Jun-26 Related CR: 2026146053

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Device stored with needle attached</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Device stored with needle attached</i>	10080242 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843

MSSO Comment: The proposal to move the LLT Device stored with needle attached from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

CR Number: 2026146054 Implementation Date: 03-Jun-26 Related CR: 2026146054

Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Reconstituted product storage time exceeded</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product storage issue</i>		<i>Reconstituted product storage time exceeded</i>	10092041 Current
To PT	<i>Product storage error</i>		From PT	
			<i>Product storage issue</i>	10093285
			To PT	
			<i>Product storage error</i>	10079843

MSSO Comment: The proposal to move the LLT Reconstituted product storage time exceeded from PT Product storage issue to PT Product storage error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026146055		Implementation Date: 03-Jun-26	Related CR: 2026146055	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Administration incorrect</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product administration issue</i>		<i>Administration incorrect</i>	10001314 Non-Current
To PT	<i>Product administration error</i>		From PT	
			<i>Product administration issue</i>	10092935
			To PT	
			<i>Product administration error</i>	10081576
MSSO Comment: The proposal to move the LLT Administration incorrect from PT Product administration issue to PT Product administration error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

CR Number: 2026146056		Implementation Date: 03-Jun-26	Related CR: 2026146056	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Frozen solvent used in drug reconstitution</i>	Approved as Requested	Proposed LLT	
From PT	<i>Product preparation issue</i>		<i>Frozen solvent used in drug reconstitution</i>	10092454 Current
To PT	<i>Product preparation error</i>		From PT	
			<i>Product preparation issue</i>	10081301
			To PT	
			<i>Product preparation error</i>	10076869
MSSO Comment: The proposal to move the LLT Frozen solvent used in drug reconstitution from PT Product preparation issue to PT Product preparation error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.				

Detail Report

CR Number: 2026146057		Implementation Date: 01-Jun-26	Related CR: 2026146057	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Metastatic uveal melanoma</i>	Approved as Requested	Proposed LLT <i>Metastatic uveal melanoma</i>	10093768 Current
			To PT <i>Uveal melanoma</i>	10081431
MSSO Comment: The proposal to add a new term Metastatic uveal melanoma is approved as requested. Uveal melanoma, sometimes called ocular melanoma, is a type of cancer that affects the uvea. Up to 50% of patients with uveal melanoma will develop metastatic disease, which can be life-threatening. Metastatic uveal melanoma will be added as sub-concept LLT to PT Uveal melanoma.				
CR Number: 2026146060		Implementation Date: 01-Jun-26	Related CR: 2026146060	
Change Requested	Change Status of LLT	Final Disposition	Final Placement	MedDRA Code #
LLT to change	<i>Congenital condition aggravated</i>	Approved as Requested	LLT to change <i>Congenital condition aggravated</i>	10093535 Non-Current
Status	<i>non-current</i>		Status <i>non-current</i>	
MSSO Comment: The proposal to change status of LLT Congenital condition aggravated to non-current is approved as requested. Congenital conditions—those present at birth—can sometimes remain asymptomatic or mild during childhood, only to be aggravated or diagnosed in adulthood due to physiological changes, wear-and-tear, or stress. Key examples include congenital heart diseases, hormonal disorders, and structural issues that worsen over time. By placing LLT Congenital condition aggravated under PT Condition aggravated, the significance of the underlying congenital hierarchical aspect is lost. Please note that Section 3.5.4 of the MedDRA Term Selection: Points to Consider document provides the option for splitting and coding verbatim terms of a 'due to' nature.				
CR Number: 2026146061		Implementation Date: 01-Jun-26	Related CR: 2026146061	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Retinal hole</i>	Approved Not as Requested	Proposed PT <i>Retinal tear</i>	10038897 Current
From PT	<i>Retinal tear</i>		With LLT <i>Retinal break</i>	10066422
MSSO Comment: The proposal to promote the LLT Retinal hole from PT Retinal tear is approved but not as requested. A break in the retina can be categorized as a tear or a hole. Retinal tears and retinal holes may differ from a pathophysiological perspective in that retinal tears are associated with vitreoretinal traction and acute mechanical disruption, whereas retinal holes are often atrophic defects arising in the setting of peripheral retinal thinning or lattice degeneration. However, the two concepts remain closely related clinically and anatomically within the broader spectrum of retinal breaks. PT Retinal tear will be swapped with LLT Retinal break to represent the broader concept at PT level.				

Detail Report

CR Number: 2026146062		Implementation Date: 01-Jun-26	Related CR: 2026146062	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Vitreous pigmented cells</i>	Approved Not as Requested	Proposed LLT <i>Vitreous pigment cells</i>	10092020
			From PT <i>Vitreous cells</i>	10066421
			HLT primary <i>Choroid and vitreous structural change, deposit and degeneration</i>	10008776
			SOC primary <i>Eye disorders</i>	10015919

MSSO Comment: The proposal to add a new PT Vitreous pigmented cells is approved but not as requested. Vitreous pigmented cells refer to pigment particles suspended in the vitreous, usually originating from the retinal pigment epithelium. Their presence may indicate vitreoretinal traction, retinal tear, or retinal detachment, and is clinically important because pigment in the vitreous can be a clue to an underlying retinal break. LLT Vitreous pigment cells will be promoted from PT Vitreous cells to PT level and placed under HLT Choroid and vitreous structural change, deposit and degeneration. In a related change, LLT Shafer's sign and LLT Vitreous tobacco dust will both be moved from PT Vitreous cells to promoted PT Vitreous pigment cells.

CR Number: 2026152013		Implementation Date: 01-Jun-26	Related CR: 2026146062	
<u>Change Requested</u>	Link (move) an LLT to another PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Shafer's sign</i>	Approved as Requested	Proposed LLT <i>Shafer's sign</i>	10086335 Current
From PT	<i>Vitreous cells</i>		From PT <i>Vitreous cells</i>	10066421
To PT	<i>Vitreous pigment cells</i>		To PT <i>Vitreous pigment cells</i>	10092020

MSSO Comment:

Detail Report

CR Number: 2026152014		Implementation Date: 01-Jun-26	Related CR: 2026146062	
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Vitreous tobacco dust</i>	Approved as Requested	Proposed LLT	
From PT	<i>Vitreous cells</i>		<i>Vitreous tobacco dust</i>	10086338 Current
To PT	<i>Vitreous pigment cells</i>		From PT	
			<i>Vitreous cells</i>	10066421
			To PT	
			<i>Vitreous pigment cells</i>	10092020

MSSO
Comment:

CR Number: 2026146063		Implementation Date: 08-Jun-26	Related CR: 2026146063	
Change Requested	Promote an LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Geographic atrophy</i>	Approved After Suspension	Proposed LLT	
From PT	<i>Dry age-related macular degeneration</i>		<i>Geographic atrophy</i>	10063947
			From PT	
			<i>Dry age-related macular degeneration</i>	10075567
			HLT primary	
			<i>Retinal structural change, deposit and degeneration</i>	10038896
			SOC primary	
			<i>Eye disorders</i>	10015919

MSSO
Comment: The proposal to promote the LLT Geographic atrophy from PT Dry age-related macular degeneration is approved as requested after suspension. The significant increase in research around therapies for geographic atrophy - which is the advanced form in the continuum of dry AMD, has made the MSSO reconsider and promote LLT Geographic atrophy to PT level linked to HLT Retinal structural change, deposit and degeneration. As well, recent advances in Optical Coherence Tomography (OCT) allow the possibility to differentiate incomplete and complete forms of Retinal Pigment Epithelium and Outer Retinal Atrophy (RORA).

Detail Report

CR Number: 2026146064		Implementation Date: 01-Jun-26	Related CR: 2026146064	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>IL-2 receptor assay</i>	Approved as Requested	PT to merge	
To PT	<i>Interleukin-2 receptor assay</i>		<i>IL-2 receptor assay</i>	10071599 Current
			To PT	
			<i>Interleukin-2 receptor assay</i>	10073452

MSSO Comment: The proposal to demote the PT IL-2 receptor assay under PT Interleukin-2 receptor assay is approved as requested. Interleukin-2 (IL-2) is a cytokine released mainly by activated T lymphocytes. Assays that measure IL-2 or soluble IL-2 receptor (sIL-2R/CD25) are used as biomarkers of immune activation.

CR Number: 2026147000		Implementation Date: 01-Jun-26	Related CR: 2026147000	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Anisakis allergy</i>	Rejected		

MSSO Comment: The proposal to add a new term Anisakis allergy is not approved. Anisakis allergy is an allergic hypersensitivity caused by proteins from Anisakis parasites, typically acquired through eating raw or undercooked infected fish or squid, and it can trigger symptoms ranging from hives and gastrointestinal distress to anaphylaxis. It is often considered a type of seafood allergy because the allergic reactions occur after consuming seafood products that contain the parasite or its allergens, even though the immune system is reacting primarily to the parasite rather than to the fish proteins themselves. To avoid the overpopulation of MedDRA, it is not feasible to accommodate terms relating to every possible protein, which can cause an allergy.

CR Number: 2026147001		Implementation Date: 01-Jun-26	Related CR: 2026147001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Umbilicus resection</i>	Approved Not as Requested	Proposed LLT	
			<i>Umbilicus excision</i>	10093767 Current
			To PT	
			<i>Omphalectomy</i>	10065257

MSSO Comment: The proposal to add a new term Umbilicus resection is approved but not as requested. Umbilicus excision is a surgical procedure involving the partial or complete removal of the umbilicus. It may be performed for medical reasons such as tumors, infection, hernias, endometriosis, congenital abnormalities, or as part of abdominal reconstructive surgery. The proposed term will be modified and added as the more frequently used form as LLT Umbilicus excision to PT Omphalectomy.

Detail Report

CR Number: 2026147002	Implementation Date: 01-Jun-26	Related CR: 2026147002	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Umbilectomy</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Umbilectomy is not approved. The concept exists in PT Omphalectomy and recently added LLT Umbilicus excision (change request 2026147001) as such, the proposed term will not be added to avoid overpopulation of MedDRA.

CR Number: 2026147005	Implementation Date: 02-Jun-26	Related CR: 2026147005	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Soluble Transferrin Receptor</i>	Approved Not as Requested	Proposed LLT <i>Soluble transferrin receptor assay</i> 10093780 Current To PT <i>Transferrin receptor assay</i> 10073450

MSSO Comment: The proposal to add a new LLT Soluble Transferrin Receptor is approved but not as requested. Soluble transferrin receptor (sTfR) is a useful clinical parameter for assessing functional iron deficiency and erythropoiesis. The sTfR level is elevated in various iron deficiency states such as iron-deficient erythropoiesis, and iron deficiency anemia (IDA). Also, there are reports suggesting that sTfR is effective in distinguishing anemia due to chronic inflammation from IDA. Proposed term will be modified for MedDRA capitalization conventions as well as for better alignment and added as LLT Soluble transferrin receptor assay to PT Transferrin receptor assay.

CR Number: 2026147006	Implementation Date: 02-Jun-26	Related CR: 2026147006	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Ferritin-index</i>	Approved Not as Requested	Proposed PT <i>Transferrin receptor-ferritin index</i> 10093776 HLT primary <i>Mineral and electrolyte analyses</i> 10027637 SOC primary <i>Investigations</i> 10022891

MSSO Comment: The proposal to add a new LLT Ferritin-index is approved but not as requested. The Transferrin Receptor-Ferritin index (TR-F index) is a blood test marker used to accurately diagnose iron deficiency anemia (IDA). It is calculated as the ratio of serum transferrin receptor (sTfR) to the log of serum ferritin. A positive finding on the TR-F index (>1.5) can accurately establish a diagnosis of IDA and may eliminate the need for bone marrow examination. Proposed concept will be modified into the form most often referenced in literature and added as PT Transferrin receptor-ferritin index to HLT Mineral and electrolyte analyses.

Detail Report

CR Number: 2026147007		Implementation Date: 02-Jun-26	Related CR: 2026147007		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Bone-specific alkaline phosphatase</i>	Rejected			

MSSO The proposal to add a new LLT Bone-specific alkaline phosphatase is not approved. The proposed concept exists as LLT Serum alkaline phosphatase - bone and LLT Plasma alkaline phosphatase - bone under PT Blood alkaline phosphatase and may be used for your coding needs.
Comment:

CR Number: 2026147008		Implementation Date: 02-Jun-26	Related CR: 2026147008		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Procollagen type 1 n-terminal propeptide</i>	Approved Not as Requested	Proposed PT		
			<i>Serum procollagen type I N-terminal propeptide</i>	10093775	
			HLT primary		
			<i>Biochemical markers of bone metabolism</i>	10067225	
			SOC primary		
			<i>Investigations</i>	10022891	

MSSO The proposal to add a new LLT Procollagen type 1 n-terminal propeptide is approved but not as requested. Proposed term will be modified for better alignment and added as PT Serum procollagen type I N-terminal propeptide to HLT Biochemical markers of bone metabolism to complement existing PT Serum procollagen type I N-terminal propeptide increased and PT Serum procollagen type I N-terminal propeptide decreased.
Comment:

CR Number: 2026147009		Implementation Date: 02-Jun-26	Related CR: 2026147009		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Psoriasis flare up</i>	Rejected			
To PT	<i>Psoriasis</i>				

MSSO The proposal to add a new LLT Psoriasis flare up to PT Psoriasis is not approved. The proposed concept exists already as LLT Psoriasis flare-up.
Comment:

Detail Report

CR Number: 2026147010		Implementation Date: 02-Jun-26	Related CR: 2026147010		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Borderline tumor</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Borderline tumor is not approved. The term 'borderline tumor' is most commonly used in reference to ovarian borderline tumors (tumors of low malignant potential), but it may also be applied to neoplasms at other anatomical sites. As the concept lacks anatomical specificity it may be interpreted inconsistently.

CR Number: 2026147011		Implementation Date: 02-Jun-26	Related CR: 2026147011		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>BANFF classification</i>	Approved Not as Requested	Proposed PT		
			<i>Banff classification of allograft pathology</i>	10093778	
			HLT primary		
			<i>Histopathology procedures NEC</i>	10020126	
			SOC primary		
			<i>Investigations</i>	10022891	

MSSO Comment: The proposal to add a new LLT BANFF classification is approved but not as requested. The Banff Classification of Allograft Pathology is an international consensus classification for the reporting of biopsies from solid organ transplants. Proposed concept will be modified for MedDRA capitalization conventions and for clarity and added as PT Banff classification of allograft pathology to HLT Histopathology procedures NEC.

CR Number: 2026147012		Implementation Date: 02-Jun-26	Related CR: 2026147012		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>BANFF classification 1</i>	Rejected			

MSSO Comment: The proposal to add a new LLT BANFF classification 1 is not approved. MedDRA generally does not create separate terms to reflect different typification categories within a classification system. The addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.□

Detail Report

CR Number: 2026147013	Implementation Date: 02-Jun-26	Related CR: 2026147013	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	BANFF classification 2	Rejected	

MSSO Comment: The proposal to add a new LLT BANFF classification 2 is not approved. MedDRA generally does not create separate terms to reflect different typification categories within a classification system. The addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.□

CR Number: 2026147014	Implementation Date: 02-Jun-26	Related CR: 2026147014	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	BANFF classification 3	Rejected	

MSSO Comment: The proposal to add a new LLT BANFF classification 3 is not approved. MedDRA generally does not create separate terms to reflect different typification categories within a classification system. The addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.□

CR Number: 2026147015	Implementation Date: 02-Jun-26	Related CR: 2026147015	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	BANFF classification 4	Rejected	

MSSO Comment: The proposal to add a new LLT BANFF classification 4 is not approved. MedDRA generally does not create separate terms to reflect different typification categories within a classification system. The addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.□

Detail Report

CR Number: 2026147016	Implementation Date: 02-Jun-26	Related CR: 2026147016	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Chronic pain syndrome</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Chronic pain syndrome is not approved. Chronic pain syndrome is not uniformly defined, and scopes vary across medical specialties and publications, ranging from persistent pain alone to a broader biopsychosocial condition involving functional and psychological impairment.

CR Number: 2026147017	Implementation Date: 02-Jun-26	Related CR: 2026147017	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Numeric pain rating scale 1</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 1 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147018	Implementation Date: 02-Jun-26	Related CR: 2026147018	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Numeric pain rating scale 2</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 2 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

Detail Report

CR Number: 2026147019	Implementation Date: 02-Jun-26	Related CR: 2026147019	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 3	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 3 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147020	Implementation Date: 02-Jun-26	Related CR: 2026147020	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 4	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 4 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147021	Implementation Date: 02-Jun-26	Related CR: 2026147021	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 5	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 5 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

Detail Report

CR Number: 2026147022	Implementation Date: 02-Jun-26	Related CR: 2026147022	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 6	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 6 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147023	Implementation Date: 02-Jun-26	Related CR: 2026147023	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 7	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 7 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147024	Implementation Date: 02-Jun-26	Related CR: 2026147024	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Numeric pain rating scale 8	Rejected	

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 8 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

Detail Report

CR Number: 2026147025		Implementation Date: 02-Jun-26	Related CR: 2026147025		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Numeric pain rating scale 9	Rejected			

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 9 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147026		Implementation Date: 02-Jun-26	Related CR: 2026147026		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Numeric pain rating scale 10	Rejected			

MSSO Comment: The proposal to add a new LLT Numeric pain rating scale 10 is not approved. It is not possible to add such granular concepts in MedDRA as well the numeric rating scale is a methodological tool used for assessment and documentation, and its granularity lies at the level of subjective quantitative scoring rather than clinical characterization.

CR Number: 2026147027		Implementation Date: 02-Jun-26	Related CR: 2026147027		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	GOLD system	Approved Not as Requested	Proposed PT GOLD system assessment	10093774	
			HLT primary Respiratory and pulmonary function diagnostic procedures	10037385	
			SOC primary Investigations	10022891	

MSSO Comment: The proposal to add a new LLT GOLD system is approved but not as requested. The GOLD system (Global Initiative for Chronic Obstructive Lung Disease) is a widely used international classification for assessing the severity of chronic obstructive pulmonary disease (COPD) and guiding treatment. Historically, GOLD classified COPD into four stages (I–IV) based exclusively on spirometric findings. Currently, in addition to the degree of airflow obstruction, symptom burden and the risk of exacerbations are also taken into consideration. The proposed term will be modified for greater clarity and added as PT GOLD system assessment to HLT Respiratory and pulmonary function diagnostic procedures.

Detail Report

CR Number: 2026147028	Implementation Date: 02-Jun-26	Related CR: 2026147028	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	GOLD 1	Rejected	

MSSO Comment: The proposal to add a new LLT GOLD 1 is not approved. MedDRA generally does not create separate terms to reflect different grades or stages of severity. GOLD stages I-IV represent varying degrees of severity of COPD. The addition of individual terms for each stage is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147029	Implementation Date: 02-Jun-26	Related CR: 2026147029	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	GOLD 2	Rejected	

MSSO Comment: The proposal to add a new LLT GOLD 2 is not approved. MedDRA generally does not create separate terms to reflect different grades or stages of severity. GOLD stages I-IV represent varying degrees of severity of COPD. The addition of individual terms for each stage is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147030	Implementation Date: 02-Jun-26	Related CR: 2026147030	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	GOLD 3	Rejected	

MSSO Comment: The proposal to add a new LLT GOLD 3 is not approved. MedDRA generally does not create separate terms to reflect different grades or stages of severity. GOLD stages I-IV represent varying degrees of severity of COPD. The addition of individual terms for each stage is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

Detail Report

CR Number: 2026147031	Implementation Date: 02-Jun-26	Related CR: 2026147031	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	GOLD 4	Rejected	

MSSO Comment: The proposal to add a new LLT GOLD 4 is not approved. MedDRA generally does not create separate terms to reflect different grades or stages of severity. GOLD stages I–IV represent varying degrees of severity of COPD. The addition of individual terms for each stage is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147032	Implementation Date: 02-Jun-26	Related CR: 2026147032	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification	Approved Not as Requested	Proposed PT Forrest classification stage 10093777 HLT primary Gastrointestinal and abdominal imaging procedures 10017963 SOC primary Investigations 10022891

MSSO Comment: The proposal to add a new LLT Forrest classification is approved but not as requested. The Forrest Classification is an endoscopic classification system used to categorize peptic ulcer bleeding according to the presence and appearance of active hemorrhage or stigmata of recent bleeding. It is used to assess the risk of rebleeding and to guide therapeutic management, with categories ranging from active bleeding lesions (Forrest I) to ulcers with a clean base and no signs of recent hemorrhage (Forrest III). The proposed term will be modified for greater clarity and added as PT Forrest classification stage to HLT Gastrointestinal and abdominal imaging procedures.

CR Number: 2026147033	Implementation Date: 02-Jun-26	Related CR: 2026147033	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 1	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 1 is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

Detail Report

CR Number: 2026147034	Implementation Date: 02-Jun-26	Related CR: 2026147034	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 2	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 2 is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147035	Implementation Date: 02-Jun-26	Related CR: 2026147035	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 3	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 3 is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147036	Implementation Date: 02-Jun-26	Related CR: 2026147036	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 1a	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 1a is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

Detail Report

CR Number: 2026147037	Implementation Date: 02-Jun-26	Related CR: 2026147037	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 1b	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 1b is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147038	Implementation Date: 02-Jun-26	Related CR: 2026147038	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 2a	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 2a is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147039	Implementation Date: 02-Jun-26	Related CR: 2026147039	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	Forrest classification 2b	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 2b is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

Detail Report

CR Number: 2026147040	Implementation Date: 02-Jun-26	Related CR: 2026147040	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Forrest classification 2c</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Forrest classification 2c is not approved. MedDRA generally does not create separate terms to reflect different risk categories within a classification system. The Forrest classification categories (Ia–III) describe varying endoscopic appearances and corresponding prognostic risk levels in peptic ulcer hemorrhage rather than distinct medical concepts. Besides, the addition of individual terms for each category is not in accordance with the usual representation of investigation concepts in MedDRA, which is generally based on the assessment of a range of results through the use of qualified terms.

CR Number: 2026147041	Implementation Date: 02-Jun-26	Related CR: 2026147041	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Trochanteric fixation nail advanced (TFNA)</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Trochanteric fixation nail advanced (TFNA) is not approved. The proposed term is a trademark device name and not in scope of MedDRA. Please consider existing LLT Intramedullary femoral nailing under PT Intramedullary rod insertion.

CR Number: 2026147042	Implementation Date: 02-Jun-26	Related CR: 2026147042	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Endotracheal anaesthesia</i>	Approved Not as Requested	Proposed LLT <i>General endotracheal anaesthesia</i> 10093782 Current To PT <i>General anaesthesia</i> 10018060

MSSO Comment: The proposal to add a new LLT Endotracheal anaesthesia is approved but not as requested. Endotracheal anaesthesia is a form of general anesthesia in which an endotracheal tube is placed in the trachea to maintain a secure airway, facilitate ventilation, and protect against aspiration. Proposed term will be modified for clarity and added as LLT General endotracheal anaesthesia to PT General anaesthesia. In a related change, the American-spelled counterpart will be added as LLT General endotracheal anesthesia to PT General anaesthesia.

Detail Report

CR Number: 2026153002		Implementation Date: 02-Jun-26	Related CR: 2026147042		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	General endotracheal anesthesia	Approved as Requested	Proposed LLT		
To PT	General anaesthesia		General endotracheal anesthesia	10093783	Current
			To PT		
			General anaesthesia	10018060	

MSSO
Comment:

CR Number: 2026147043		Implementation Date: 02-Jun-26	Related CR: 2026147043		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Scapulatransplantation	Rejected			

MSSO
Comment: The proposal to add a new LLT Scapulatransplantation is not approved. The proposed concept is ambiguous and may be interpreted with various meanings. Scapulatransplantation is not readily found in literature and sometimes refers to a scapular bone graft in maxillofacial defect repair, or scapula prosthetic replacement (total or partial).

CR Number: 2026147044		Implementation Date: 02-Jun-26	Related CR: 2026147044		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Port-a-cath	Rejected			

MSSO
Comment: The proposal to add a new LLT Port-a-cath is not approved. Port-a-cath is a trademark medical device name and as such not in the scope of the MedDRA terminology. Please consider terms for insertion or removal of ports, available as e.g., LLT Port insertion.

Detail Report

CR Number: 2026147045	Implementation Date: 02-Jun-26	Related CR: 2026147045	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Tingling feet</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Tingling feet is not approved. The proposed concept exists in Paresthesia foot as well as similar terms in PT Paraesthesia. To avoid overpopulation of MedDRA, this term in plural form will not be added.

CR Number: 2026147046	Implementation Date: 02-Jun-26	Related CR: 2026147046	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Reflux</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Reflux is not approved. As already mentioned by submitter, the term reflux is ambiguous without further qualifiers. Therefore, the term will not be added to the MedDRA terminology.

CR Number: 2026147047	Implementation Date: 02-Jun-26	Related CR: 2026147047	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Reconstruction plate</i>	Rejected	
To PT	<i>internal fixation of fracture</i>		

MSSO Comment: The proposal to add a new LLT Reconstruction plate to PT internal fixation of fracture is not approved. Reconstruction plate is the name of a medical device used for osteosynthesis. Pure medical device name terms are out of the scope of MedDRA. Please consider existing procedure terms e.g., LLT Plate osteosynthesis, LLT Internal fixation for your coding needs.

Detail Report

CR Number: 2026147048		Implementation Date: 02-Jun-26	Related CR: 2026147048	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Immunoblot</i>	Approved as Requested	Proposed LLT	
To PT	<i>immunoassay</i>		<i>Immunoblot</i>	10093781 Current
			To PT	
			<i>Immunoassay</i>	10091374
MSSO Comment: The proposal to add a new LLT Immunoblot to PT Immunoassay is approved as requested. Immunoblot is a laboratory procedure that combines electrophoretic separation of proteins with subsequent antibody-based detection. Proteins are first separated according to their molecular weight by gel electrophoresis and then transferred to a membrane. Specific antibodies are used to identify target proteins, which are visualized as distinct bands, producing a characteristic banding pattern for interpretation.				

CR Number: 2026147049		Implementation Date: 02-Jun-26	Related CR: 2026147049	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Radiratio</i>	Rejected		
To PT	<i>radiotherapy</i>			
MSSO Comment: The proposal to add a new LLT Radiratio to PT radiotherapy is not approved. Radiatio (likely meaning of proposed LLT Radiratio) is not used as a synonym for radiation in English medical terminology; it would be considered either Latin or used in specialist anatomical nomenclature but not as a stand-alone synonym for radiation. Therefore, the term will not be added to the English version of MedDRA.				

CR Number: 2026147050		Implementation Date: 02-Jun-26	Related CR: 2026147050	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Gastric protection</i>	Rejected		
MSSO Comment: The proposal to add a new LLT Gastric protection is not approved. Gastric protection describes a general therapeutic objective rather than a clear medical concept. As it may encompass a wide range of preventive measures and lacks a specific clinical definition. Please consider LLT Gastric ulcer prophylaxis and related terms for your coding needs.				

Detail Report

CR Number: 2026147051		Implementation Date: 02-Jun-26	Related CR: 2026147051		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Anaphylactic reaction 1	Rejected			
To PT	anaphylactic reaction				

MSSO Comment: The proposal to add a new LLT Anaphylactic reaction 1 to PT anaphylactic reaction is not approved. The MSSO generally refrains from adding terms including a severity grading of the reaction. Moreover, the Ring and Messmer classification, which uses stage I-IV grades for anaphylaxis is not universally adopted and mainly is a severity grading system for perioperative or drug-induced reactions, rather than a global diagnostic framework.

CR Number: 2026147052		Implementation Date: 02-Jun-26	Related CR: 2026147052		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Anaphylactic reaction 2	Rejected			
To PT	anaphylactic reaction				

MSSO Comment: The proposal to add a new LLT Anaphylactic reaction 2 to PT anaphylactic reaction is not approved. The MSSO generally refrains from adding terms including a severity grading of the reaction. Moreover, the Ring and Messmer classification, which uses stage I-IV grades for anaphylaxis is not universally adopted and mainly is a severity grading system for perioperative or drug-induced reactions, rather than a global diagnostic framework.

CR Number: 2026147053		Implementation Date: 02-Jun-26	Related CR: 2026147053		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	Anaphylactic reaction 3	Rejected			
To PT	anaphylactic reaction				

MSSO Comment: The proposal to add a new LLT Anaphylactic reaction 3 to PT anaphylactic reaction is not approved. The MSSO generally refrains from adding terms including a severity grading of the reaction. Moreover, the Ring and Messmer classification, which uses stage I-IV grades for anaphylaxis is not universally adopted and mainly is a severity grading system for perioperative or drug-induced reactions, rather than a global diagnostic framework.

Detail Report

CR Number: 2026147054		Implementation Date: 02-Jun-26	Related CR: 2026147054		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Anaphylactic reaction 4	Rejected			
To PT	anaphylactic reaction				

MSSO Comment: The proposal to add a new LLT Anaphylactic reaction 4 to PT anaphylactic reaction is not approved. The MSSO generally refrains from adding terms including a severity grading of the reaction. Moreover, the Ring and Messmer classification, which uses stage I-IV grades for anaphylaxis is not universally adopted and mainly is a severity grading system for perioperative or drug-induced reactions, rather than a global diagnostic framework.

CR Number: 2026147055		Implementation Date: 02-Jun-26	Related CR: 2026147055		MedDRA Code #
Change Requested	Add a New Term	Final Disposition	Final Placement		
Proposed Term	Own birth via C-section	Rejected			

MSSO Comment: The proposal to add a new term Own birth via C-section is not approved. The term Cesarean section does not differentiate between the mother or child and can be used for both scenarios. It will usually be interpreted depending on the context of the coded data field.

CR Number: 2026147056		Implementation Date: 02-Jun-26	Related CR: 2026147056		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Specific immunotherapy (SIT)	Rejected			
To PT	immune tolerance induction				

MSSO Comment: The proposal to add a new LLT Specific immunotherapy (SIT) to PT immune tolerance induction is not approved. Specific immunotherapy (SIT) is an older term for allergen immunotherapy and has now been largely replaced by the term allergen immunotherapy in guidelines and literature. With addition of LLT Allergen immunotherapy in change request 2026147057, this rather outdated term will not be added. Please consider newly added LLT Allergen immunotherapy or other existing terms such as LLT Allergenic desensitization procedure for your specific coding needs.

Detail Report

CR Number: 2026147057		Implementation Date: 02-Jun-26	Related CR: 2026147057	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Allergen immunotherapy (AIT)</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>immune tolerance induction</i>		<i>Allergen immunotherapy</i>	10093779 Current
			To PT	
			<i>Immune tolerance induction</i>	10070581
MSSO Comment: The proposal to add a new LLT Allergen immunotherapy (AIT) to PT immune tolerance induction is approved but not as requested. Allergen immunotherapy is the most internationally recognized and least ambiguous expression. It is the only disease-modifying treatment available for several common allergic diseases. The proposed term will be modified to remove the acronym and added as LLT Allergen immunotherapy to PT Immune tolerance induction.				

CR Number: 2026147058		Implementation Date: 02-Jun-26	Related CR: 2026147058	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Transperineal template-guided prostate mapping biopsy</i>	Approved as Requested	Proposed LLT	
			<i>Transperineal template-guided prostate mapping biopsy</i>	10093785 Current
			To PT	
			<i>Biopsy prostate</i>	10004857
MSSO Comment: The proposal to add a new LLT Transperineal template-guided prostate mapping biopsy is approved as requested. Transperineal template-guided prostate mapping biopsy is a transperineal prostate biopsy technique designed to obtain a systematic and comprehensive sampling of the entire prostate gland. Using a template grid with predefined coordinates placed over the perineum, multiple biopsy cores are taken through the grid under transrectal ultrasound guidance to create a three-dimensional map of the prostate. Transperineal template-guided prostate mapping biopsy will be added as LLT to PT Biopsy prostate.				

CR Number: 2026147059		Implementation Date: 02-Jun-26	Related CR: 2026147059	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Tricho-dento-osseous syndrome</i>	Approved as Requested	Proposed LLT	
			<i>Tricho-dento-osseous syndrome</i>	10093784 Current
			To PT	
			<i>Congenital ectodermal dysplasia</i>	10010452
MSSO Comment: The proposal to add a new LLT Tricho-dento-osseous syndrome is approved as requested. Tricho-dento-osseous syndrome is a rare inherited disorder characterized by abnormalities of the hair, teeth, and bone. Typical features include curly or kinky hair in early childhood, enamel defects consistent with amelogenesis imperfecta, tooth hypersensitivity and premature dental wear, together with increased bone density and cortical thickening. Tricho-dento-osseous syndrome will be added as LLT to PT Congenital ectodermal dysplasia. In a related change, the grouping PT Congenital ectodermal dysplasia will receive an additional secondary link to HLT Dental developmental disorders and anomalies for a more complete representation of the concept and the syndromes overarched by it.				

Detail Report

CR Number: 2026153004		Implementation Date: 02-Jun-26	Related CR: 2026147059		MedDRA Code #
Change Requested	Link a PT to a HLT	Final Disposition	Final Placement		
Proposed PT	<i>Congenital ectodermal dysplasia</i>	Approved as Requested	Proposed PT		
To HLT	<i>Dental developmental disorders and anomalies</i>		<i>Congenital ectodermal dysplasia</i>		10010452
			To HLT		
			<i>Dental developmental disorders and anomalies</i>		10012321

MSSO
Comment:

CR Number: 2026147060		Implementation Date: 01-Jun-26	Related CR: 2026147060		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	<i>Radioimmunoprecipitation</i>	Approved Not as Requested	Proposed LLT		
			<i>Radioimmunoprecipitation assay</i>		10093769 Current
			To PT		
			<i>Immunoassay</i>		10091374

MSSO
Comment: The proposal to add a new PT Radioimmunoprecipitation is approved but not as requested. Radioimmunoprecipitation is a laboratory technique that combines immunology and radioactive labeling to detect or measure specific molecules - usually proteins, antigens, or antibodies — in biological samples. The proposed term will be modified and added as LLT Radioimmunoprecipitation assay to PT Immunoassay.

CR Number: 2026147061		Implementation Date: 03-Jun-26	Related CR: 2026147061		MedDRA Code #
Change Requested	Link (move) an LLT to another PT	Final Disposition	Final Placement		
Proposed LLT	<i>Inappropriate device programming</i>	Approved as Requested	Proposed LLT		
From PT	<i>Device programming issue</i>		<i>Inappropriate device programming</i>		10067679 Current
To PT	<i>Device programming error</i>		From PT		
			<i>Device programming issue</i>		10093290
			To PT		
			<i>Device programming error</i>		10080718

MSSO
Comment: The proposal to move the LLT Inappropriate device programming from PT Device programming issue to PT Device programming error is approved as requested. A group of medication error experts from industry and major regulatory agencies from ICH regions reviewed the placement of this term and concluded that this term represents an error when reported in pharmacovigilance single case reports. This assessment was supported by a data driven understanding of the use of the term in cases coded to this LLT.

Detail Report

CR Number: 2026147062		Implementation Date: 02-Jun-26	Related CR: 2026147062		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Vaccine underdose, intentional</i>	Suspended			
To PT	<i>Underdose</i>				

MSSO Comment: The proposal to add a new LLT Vaccine underdose, intentional to PT Underdose is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.

CR Number: 2026147063		Implementation Date: 02-Jun-26	Related CR: 2026147063		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Vaccine underdose, accidental</i>	Approved Not as Requested	Proposed LLT		
To PT	<i>Underdose</i>		<i>Accidental vaccine underdose</i>	10093786	Current
			To PT		
			<i>Accidental underdose</i>	10074904	

MSSO Comment: The proposal to add a new LLT Vaccine underdose, accidental to PT Underdose is approved but not as requested. Vaccine underdose, accidental refers to the unintentional administration of a vaccine in an amount lower than the recommended or prescribed dose. This may result from preparation, handling, measurement, or administration errors, rather than a deliberate decision. The proposed term will be modified for natural language word order and added as LLT Accidental vaccine underdose to PT Accidental underdose.

CR Number: 2026147064		Implementation Date: 02-Jun-26	Related CR: 2026147064		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Vaccine overdose, intentional</i>	Suspended			
To PT	<i>Overdose</i>				

MSSO Comment: The proposal to add a new LLT Vaccine overdose, intentional to PT Overdose is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.

Detail Report

CR Number: 2026147065		Implementation Date: 02-Jun-26	Related CR: 2026147065	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Vaccine overdose, accidental</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Overdose</i>		<i>Accidental vaccine overdose</i>	10093787 Current
			To PT	
			<i>Accidental overdose</i>	10000381
MSSO Comment: The proposal to add a new LLT Vaccine overdose, accidental to PT Overdose is approved but not as requested. Vaccine overdose, accidental refers to the unintentional administration of a vaccine in an amount greater than the recommended or prescribed dose. This may result from errors in preparation, measurement, handling, or administration. The administered dose exceeds that intended according to the recommended immunization regimen. The proposed term will be modified for natural language word order and added as LLT Accidental vaccine overdose to PT Accidental overdose.				

CR Number: 2026147066		Implementation Date: 02-Jun-26	Related CR: 2026147066	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Extra vaccine dose administered</i>	Suspended		
To PT	<i>Extra dose administered</i>			
MSSO Comment: The proposal to add a new LLT Extra vaccine dose administered to PT Extra dose administered is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.				

CR Number: 2026147078		Implementation Date: 02-Jun-26	Related CR: 2026147078	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Pelotherapy</i>	Approved Not as Requested	Proposed PT	
HLT primary	<i>Skin and subcutaneous tissue therapeutic procedures NEC</i>		<i>Pelotherapy</i>	10093788
SOC primary	<i>Surgical and medical procedures</i>		HLT primary	
			<i>Therapeutic procedures NEC</i>	10027700
			SOC primary	
			<i>Surgical and medical procedures</i>	10042613
MSSO Comment: The proposal to add a new PT Pelotherapy to primary HLT Skin and subcutaneous tissue therapeutic procedures NEC in SOC Surgical and medical procedures is approved but not as requested. Pelotherapy is the therapeutic use of medicinal muds, clays, or mineral-rich peloids applied to the body for the treatment of musculoskeletal, rheumatic, or dermatological conditions. Pelotherapy will be added as PT to HLT Therapeutic procedures NEC.				

Detail Report

CR Number: 2026147079		Implementation Date: 02-Jun-26	Related CR: 2026147079	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Urine chylous fluid test	Rejected		
HLT primary	Urinalysis NEC			
SOC primary	Investigations			
MSSO Comment:	The proposal to add a new PT Urine chylous fluid test to primary HLT Urinalysis NEC in SOC Investigations is not approved. Urine chylous fluid test is an overarching concept which represents several laboratory tests used to detect the presence of chyle in urine for the diagnosis of chyluria. The proposed term is not a specific and standardized test name.			

CR Number: 2026148001		Implementation Date: 03-Jun-26	Related CR: 2026148001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Nasal cavity hemangioma	Approved as Requested	Proposed LLT Nasal cavity hemangioma	10093791 Current
			To PT Nasal cavity haemangioma	10093790
MSSO Comment:	The proposal to add a new term Nasal cavity hemangioma is approved as requested. Hemangioma, a benign vascular lesion, is rare in the nasal cavity but can cause significant symptoms such as epistaxis and nasal obstruction. Nasal cavity hemangioma will be added as LLT to a new PT Nasal cavity haemangioma. In a related change, PT Nasal cavity haemangioma will be added to primary HLT Nasal and paranasal sinus neoplasms benign, secondary HLT Nasal disorders NEC and secondary HLT Vascular neoplasms.			

Detail Report

CR Number: 2026154001

Implementation Date: 03-Jun-26

Related CR: 2026148001

Change Requested

Add a New PT

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT	<i>Nasal cavity haemangioma</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Nasal and paranasal sinus neoplasms benign</i>		<i>Nasal cavity haemangioma</i>	10093790
SOC primary	<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>		HLT primary	
			<i>Nasal and paranasal sinus neoplasms benign</i>	10028722
			SOC primary	
			<i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary	
			<i>Nasal disorders NEC</i>	10028731
			SOC secondary	
			<i>Respiratory, thoracic and mediastinal disorders</i>	10038738
			HLT secondary	
			<i>Vascular neoplasms</i>	10057189
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO

Comment:

CR Number: 2026148002

Implementation Date: 02-Jun-26

Related CR: 2026148002

Change Requested

Add a New LLT

Final Disposition

Final Placement

MedDRA
Code #

Proposed LLT	<i>Oculopharyngeal muscular dystrophy</i>	Approved as Requested	Proposed LLT	
			<i>Oculopharyngeal muscular dystrophy</i>	10093773
			To PT	
			<i>Oculopharyngeal dystrophy</i>	10052181 Current

MSSO

Comment: The proposal to add a new LLT Oculopharyngeal muscular dystrophy is approved as requested. Oculopharyngeal muscular dystrophy is a late-onset inherited muscle disorder caused by a mutation in the PABPN1 gene that leads to progressive weakness, especially of the eyelids and throat muscles. It typically presents with drooping eyelids and difficulty swallowing in mid-to-late adulthood and may later involve proximal limb weakness in a slowly progressive course. Oculopharyngeal muscular dystrophy will be added as LLT to PT Oculopharyngeal dystrophy. In a related change, LLT Oculopharyngeal muscular dystrophy will be swapped with PT Oculopharyngeal dystrophy to represent the updated nomenclature at PT level.

Detail Report

CR Number: 2026153001		Implementation Date: 02-Jun-26	Related CR: 2026148002		
Change Requested	Swap a PT with an LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Oculopharyngeal dystrophy</i>	Approved as Requested	Proposed PT		
With LLT	<i>Oculopharyngeal muscular dystrophy</i>		<i>Oculopharyngeal dystrophy</i>	10052181	Current
			With LLT		
			<i>Oculopharyngeal muscular dystrophy</i>	10093773	

MSSO
Comment:

CR Number: 2026148003		Implementation Date: 03-Jun-26	Related CR: 2026148003		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Carotid artery atherosclerosis</i>	Approved as Requested	Proposed LLT		
			<i>Carotid artery atherosclerosis</i>	10093793	Current
			To PT		
			<i>Carotid arteriosclerosis</i>	10067116	

MSSO
Comment:

The proposal to add a new term Carotid artery atherosclerosis is approved as requested. Carotid artery atherosclerosis is the narrowing or blockage of the carotid arteries in the neck due to a buildup of fatty plaque. This condition restricts blood flow to the brain and significantly increases the risk of stroke or transient ischemic attack. Carotid artery atherosclerosis will be added as LLT to PT Carotid arteriosclerosis.

CR Number: 2026148004		Implementation Date: 03-Jun-26	Related CR: 2026148004		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	<i>Secondary hypercholesterolemia</i>	Approved as Requested	Proposed LLT		
			<i>Secondary hypercholesterolemia</i>	10093792	Current
			To PT		
			<i>Hypercholesterolaemia</i>	10020603	

MSSO
Comment:

The proposal to add a new term Secondary hypercholesterolemia is approved as requested. Secondary hypercholesterolemia is caused by acquired medical conditions, including underlying diseases, drugs and unhealthy lifestyle factors. Secondary hypercholesterolemia accounts for approximately 30-40% of all dyslipidemia. Secondary hypercholesterolemia will be added as LLT to PT Hypercholesterolaemia. In a related change, the British English spelled counterpart LLT Secondary hypercholesterolaemia will also be added to PT Hypercholesterolaemia.

Detail Report

CR Number: 2026154002		Implementation Date: 03-Jun-26	Related CR: 2026148004	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	Secondary hypercholesterolaemia	Approved as Requested	Proposed LLT	
To PT	Hypercholesterolaemia		Secondary hypercholesterolaemia	10093795 Current
			To PT	
			Hypercholesterolaemia	10020603

MSSO
Comment:

CR Number: 2026148005		Implementation Date: 03-Jun-26	Related CR: 2026148005	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Incipient cortico-nuclear cataract	Rejected		

MSSO
Comment: The proposal to add a new term Incipient cortico-nuclear cataract is not approved. Although the proposed expression is a clinically descriptive ophthalmological expression, it does not represent a distinct pathological entity, since the term combines two recognized cataract morphologies (cortical and nuclear) with the severity/stage qualifier "incipient".

Detail Report

CR Number: 2026148006		Implementation Date: 03-Jun-26	Related CR: 2026148006	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Oral varicosity or oral varicose veins or varicose vein of the lip</i>	Approved Not as Requested	Proposed PT	
			<i>Oral varices</i>	10093794
			HLT primary	
			<i>Haemorrhoids and gastrointestinal varices (excl oesophageal)</i>	10018029
			SOC primary	
			<i>Gastrointestinal disorders</i>	10017947
			HLT secondary	
			<i>Gastrointestinal varicosities and haemorrhoids</i>	10018030
			SOC secondary	
			<i>Vascular disorders</i>	10047065

MSSO Comment: The proposal to add a new term Oral varicosity or oral varicose veins or varicose vein of the lip is approved but not as requested. Proposed term will be added in natural language word order and modified for better alignment and added as PT Oral varices to primary HLT Haemorrhoids and gastrointestinal varices (excl oesophageal) and secondary HLT Gastrointestinal varicosities and haemorrhoids. In a related change, PT Varicose veins sublingual will be demoted under new PT Oral varices to represent the broader concept at PT level.

CR Number: 2026154003		Implementation Date: 03-Jun-26	Related CR: 2026148006	
Change Requested	Demote a PT	Final Disposition	Final Placement	MedDRA Code #
PT to merge	<i>Varicose veins sublingual</i>	Approved as Requested	PT to merge	
To PT	<i>Oral varices</i>		<i>Varicose veins sublingual</i>	10047027 Current
			To PT	
			<i>Oral varices</i>	10093794

MSSO Comment:

Detail Report

CR Number: 2026148007		Implementation Date: 03-Jun-26	Related CR: 2026148007		
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #	
Proposed Term	Coronary artery Injury or injury to coronary artery	Rejected			

MSSO Comment: The proposal to add a new term Coronary artery Injury or injury to coronary artery is not approved. Coronary artery injury is a broad and non-specific concept that could refer to a wide variety of coronary lesions of differing causes and clinical significance. Existing, more specific terms are preferable for coding the underlying condition.

CR Number: 2026148008		Implementation Date: 05-Jun-26	Related CR: 2026148008		
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	Brain cannulation	Approved After Suspension	Proposed PT Brain cannulation	10093809	
			HLT primary Skull and brain therapeutic procedures	10040957	
			SOC primary Surgical and medical procedures	10042613	

MSSO Comment: The proposal to add a new PT Brain cannulation is approved as requested after suspension. Brain cannulation is a neurosurgical procedure used to implant a micro-tube (cannula) into specific brain regions. It provides a direct, localized pathway to deliver drugs, sample cerebrospinal fluid, or monitor neural activity. Brain cannulation will be added as a new PT to HLT Skull and brain therapeutic procedures.

CR Number: 2026148009		Implementation Date: 05-Jun-26	Related CR: 2026148009		
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	Brain decannulation	Approved After Suspension	Proposed PT Brain decannulation	10093810	
			HLT primary Skull and brain therapeutic procedures	10040957	
			SOC primary Surgical and medical procedures	10042613	

MSSO Comment: The proposal to add a new PT Brain decannulation is approved as requested after suspension. Brain decannulation is the removal of a previously implemented brain cannula. Brain decannulation will be added as a PT to HLT Skull and brain therapeutic procedures.

Detail Report

CR Number: 2026148010 **Implementation Date:** 03-Jun-26 **Related CR:** 2026148010

Change Requested		Final Disposition	Final Placement	MedDRA Code #
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Proposed LLT	<i>Accelerated phase myeloproliferative neoplasm</i>	Rejected		
From PT	<i>Myeloproliferative neoplasm</i>			

MSSO Comment: The proposal to promote the LLT Accelerated phase myeloproliferative neoplasm from PT Myeloproliferative neoplasm is not approved. MedDRA captures medically meaningful concepts at different hierarchical levels, and important clinical conditions can be found at the LLT level. Promotion of all clinically significant subtypes would lead to unnecessary terminology expansion of the PT level. Furthermore, the perceived importance of a concept may vary according to the context and is therefore partly subjective. Please consider, that safety data analysis at the LLT level may need to be performed to identify and differentiate the sub-concepts.

CR Number: 2026148011 **Implementation Date:** 03-Jun-26 **Related CR:** 2026148011

Change Requested		Final Disposition	Final Placement	MedDRA Code #
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Proposed LLT	<i>Zygapophyseal joint arthritis</i>	Approved as Requested	Proposed LLT	
From PT	<i>Spondylitis</i>		<i>Zygapophyseal joint arthritis</i>	10078760 Current
To PT	<i>Facet joint syndrome</i>		From PT	
			<i>Spondylitis</i>	10061371
			To PT	
			<i>Facet joint syndrome</i>	10054813

MSSO Comment: The proposal to move the LLT Zygapophyseal joint arthritis from PT Spondylitis to PT Facet joint syndrome is approved as requested. Facet joint also known as the zygapophyseal or apophyseal joint, is a synovial joint between the superior articular process of one vertebra and the inferior articular process of the vertebra directly above it. There are two facet joints in each spinal motion segment. The proposed move will be done for better alignment and anatomical specificity.

CR Number: 2026148012 **Implementation Date:** 03-Jun-26 **Related CR:** 2026148012

Change Requested		Final Disposition	Final Placement	MedDRA Code #
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Proposed LLT	<i>Non-snus user</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Non-tobacco user</i>		<i>Non-user of tobacco snus</i>	10093796 Current
			To PT	
			<i>Non-tobacco user</i>	10072840

MSSO Comment: The proposal to add a new LLT Non-snus user to PT Non-tobacco user is approved but not as requested. Snus is a tablet or tea bag the size of a chewing gum that is filled with tobacco and that is placed under the upper lip where the nicotine is absorbed through the blood vessels in the upper lip. Proposed term will be modified for better alignment and clarity and added as LLT Non-user of tobacco snus to PT Non-tobacco user.

Detail Report

CR Number: 2026148013		Implementation Date: 03-Jun-26	Related CR: 2026148013		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Drug-induced gingival overgrowth (DIGO)</i>	Rejected			
To PT	<i>Gingival hypertrophy</i>				

MSSO Comment: The proposal to add a new LLT Drug-induced gingival overgrowth (DIGO) to PT Gingival hypertrophy is not approved. The proposed concept is a "due to" nature term. Although there are existing "drug induced" terms in MedDRA, additional terms that state or imply a "due to" concept are generally not added to MedDRA because causality is not inherent in the coded term but is instead determined by the reporter/investigator or by the sponsor. In addition, MedDRA cannot accommodate a specific term for "drug-induced" and other etiological qualifiers, because this would result in an overpopulation of the terminology.

CR Number: 2026148015		Implementation Date: 03-Jun-26	Related CR: 2026148015		MedDRA Code #
Change Requested	Promote an LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Post-SSRI sexual dysfunction</i>	Suspended			
From PT	<i>Sexual dysfunction</i>				

MSSO Comment: The proposal to promote the LLT Post-SSRI sexual dysfunction from PT Sexual dysfunction is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.

Detail Report

CR Number: 2026148017		Implementation Date: 29-May-26	Related CR: 2026148017	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Episodic angioedema with eosinophilia</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Hypersensitivity (SMQ)</i>		<i>Episodic angioedema with eosinophilia</i>	10093629
Term scope	<i>Broad</i>		To SMQ	
			<i>Hypersensitivity (SMQ)</i>	20000214
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Episodic angioedema with eosinophilia to Hypersensitivity (SMQ) as a Broad term is approved as requested. Episodic angioedema with eosinophilia (Gleich syndrome) is a rare disorder characterized by episodes of angioedema and eosinophilia that occur at monthly intervals and resolve spontaneously without therapy. This new term was added as LLT and then swapped with PT Gleich syndrome, which previously was part of the narrow scope of the SMQ. As Episodic angioedema with eosinophilia is generally considered a primary eosinophilic disorder / immune dysregulation syndrome, rather than a classic drug or vaccine hypersensitivity reaction, the term will be added to the broad scope.

CR Number: 2026148018		Implementation Date: 29-May-26	Related CR: 2026148018	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Episodic angioedema with eosinophilia</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Angioedema (SMQ)</i>		<i>Episodic angioedema with eosinophilia</i>	10093629
Term scope	<i>Narrow</i>		To SMQ	
			<i>Angioedema (SMQ)</i>	20000024
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Episodic angioedema with eosinophilia to Angioedema (SMQ) as a Narrow term is approved as requested. Episodic angioedema with eosinophilia (Gleich syndrome) is a rare disorder characterized by episodes of angioedema and eosinophilia that occur at monthly intervals and resolve spontaneously without therapy. PT Gleich's syndrome was previously part of the narrow scope of the SMQ and was demoted to this new term.

Detail Report

CR Number: 2026148019		Implementation Date: 29-May-26	Related CR: 2026148019	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Conjunctival feeder vessel</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Conjunctival disorders (SMQ)</i>		<i>Conjunctival feeder vessel</i>	10093600
Term scope	<i>Narrow</i>		To SMQ	
			<i>Conjunctival disorders (SMQ)</i>	20000175
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Conjunctival feeder vessel to Conjunctival disorders (SMQ) as a Narrow term is approved as requested. A conjunctival infero-nasal feeder vessel is a prominent, dilated blood vessel located in the lower-nasal quadrant of the bulbar conjunctiva that supplies blood to an underlying lesion or growth. These vessels are frequently associated with ocular surface tumors, such as conjunctival nevi, squamous cell carcinoma, or other vascularized, benign, and malignant lesions. The term matches the inclusion criteria.

CR Number: 2026148020		Implementation Date: 29-May-26	Related CR: 2026148020	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Mixed squamous neuroendocrine carcinoma</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Non-haematological malignant tumours (SMQ)</i>		<i>Mixed squamous neuroendocrine carcinoma</i>	10093552
Term scope	<i>Narrow</i>		To SMQ	
			<i>Non-haematological malignant tumours (SMQ)</i>	20000228
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Mixed squamous neuroendocrine carcinoma to Non-haematological malignant tumours (SMQ) as a Narrow term is approved as requested. Mixed squamous neuroendocrine carcinoma is composed of a squamous cell carcinoma component and a neuroendocrine carcinoma component. It is usually found in sites where squamous carcinoma is common (e.g., esophagus, lung, cervix). It is also a subtype within the Mixed neuroendocrine–non-neuroendocrine neoplasms (MiNENs). The term matches the inclusion criteria.

Detail Report

CR Number: 2026148021		Implementation Date: 29-May-26	Related CR: 2026148021	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Retinotomy	Approved as Requested	Proposed PT	
To SMQ	Retinal disorders (SMQ)		Retinotomy	10093569
Term scope	Narrow		To SMQ	
			Retinal disorders (SMQ)	20000158
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Retinotomy to Retinal disorders (SMQ) as a Narrow term is approved as requested. Retinotomy is a surgical procedure involving the cutting or
Comment: incising of the retina, often performed during vitrectomy to treat complex retinal detachments. It is primarily used to release severe traction, relax a stiff or shortened retina, or drain subretinal fluid in cases of proliferative vitreoretinopathy. The term matches the inclusion criteria and will be added in line with similar procedure terms.

CR Number: 2026148022		Implementation Date: 29-May-26	Related CR: 2026148022	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Autosomal dominant tubulointerstitial kidney disease	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Autosomal dominant tubulointerstitial kidney disease	10093601
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Autosomal dominant tubulointerstitial kidney disease to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as
Comment: requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026148023		Implementation Date: 29-May-26	Related CR: 2026148023		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Congenital hypomelanosis</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Congenital hypomelanosis</i>		10093612
Term scope	<i>Narrow</i>		To SMQ		
			<i>Congenital, familial and genetic disorders (SMQ)</i>		20000077
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Congenital hypomelanosis to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026148024		Implementation Date: 29-May-26	Related CR: 2026148024		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Erythrokeratoderma variabilis progressiva</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Erythrokeratoderma variabilis progressiva</i>		10093634
Term scope	<i>Narrow</i>		To SMQ		
			<i>Congenital, familial and genetic disorders (SMQ)</i>		20000077
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Erythrokeratoderma variabilis progressiva to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026148025		Implementation Date: 29-May-26	Related CR: 2026148025	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Familial reactive perforating collagenosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Familial reactive perforating collagenosis</i>	10093613
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Familial reactive perforating collagenosis to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026148026		Implementation Date: 29-May-26	Related CR: 2026148026	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Hereditary hypotrichosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Hereditary hypotrichosis</i>	10093617
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Hereditary hypotrichosis to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026148027		Implementation Date: 29-May-26	Related CR: 2026148027	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Hereditary progressive mucinous histiocytosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Hereditary progressive mucinous histiocytosis</i>	10093614
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Hereditary progressive mucinous histiocytosis to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested.
Comment: This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026148028		Implementation Date: 29-May-26	Related CR: 2026148028	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Trichothiodystrophy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Trichothiodystrophy</i>	10044628
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Trichothiodystrophy to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion
Comment: criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026148029		Implementation Date: 29-May-26	Related CR: 2026148029	MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement	
Proposed PT	<i>Gastric rupture</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Gastrointestinal perforation (SMQ)</i>		<i>Gastric rupture</i>	10093640
Term scope	<i>Narrow</i>		To SMQ	
			<i>Gastrointestinal perforation (SMQ)</i>	20000107
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Gastric rupture to Gastrointestinal perforation (SMQ) as a Narrow term is approved as requested. The SMQ definition includes perforations of any part of the gastrointestinal tract, and gastric rupture represents a transmural gastric perforation from a clinical and pathophysiological perspective. Consistency with previously included PTs such as Oesophageal rupture and Duodenal rupture further supports inclusion.

CR Number: 2026148030		Implementation Date: 29-May-26	Related CR: 2026148030	MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement	
Proposed PT	<i>Parkinsonism hyperpyrexia syndrome</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Neuroleptic malignant syndrome (SMQ)</i>		<i>Parkinsonism hyperpyrexia syndrome</i>	10071243
Term scope	<i>Narrow</i>		To SMQ	
			<i>Neuroleptic malignant syndrome (SMQ)</i>	20000044
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Parkinsonism hyperpyrexia syndrome to Neuroleptic malignant syndrome (SMQ) as a Narrow term is approved as requested. PT Parkinsonism hyperpyrexia syndrome meets the scope of the Neuroleptic malignant syndrome (SMQ) or NMS. The original SMQ documentation states that "Cases specifically reported as NMS or the similar conditions serotonin syndrome or malignant hyperthermia (which might be easily misdiagnosed as NMS) that encode to the PTs Neuroleptic malignant syndrome, Serotonin syndrome, or Hyperthermia malignant" are included, to not miss cases of NMS. DSM-5 TM considers parkinsonian hyperthermia syndrome following abrupt discontinuation of dopamine agonists as similar syndrome, together with serotonin syndrome and malignant hyperthermia occurring during anesthesia. PT Parkinsonism hyperpyrexia syndrome will be added with narrow scope.

Detail Report

CR Number: 2026148031		Implementation Date: 29-May-26	Related CR: 2026148031	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Noma neonatorum</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Oropharyngeal infections (SMQ)</i>		<i>Noma neonatorum</i>	10093619
Term scope	<i>Narrow</i>		To SMQ	
			<i>Oropharyngeal infections (SMQ)</i>	20000111
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Noma neonatorum to Oropharyngeal infections (SMQ) as a Narrow term is approved as requested. PT Noma neonatorum was added in version 29.1 and is a rare gangrenous form of noma, which during its course, causes progressive and mutilating destruction of the soft tissues and the bone. It occurs at birth or during the neonatal period and is characterised by a gangrenous process involving mucocutaneous junctions of oral, nasal and anal areas and occasionally, the eyelids and scrotum. It fits the inclusion criterion "Terms for infections that can be observed within oral cavity, such as those caused by viruses and bacteria". All terms in this SMQ are narrow in scope.

CR Number: 2026148032		Implementation Date: 29-May-26	Related CR: 2026148032	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Generalised pustular psoriasis of pregnancy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Immune-mediated/autoimmune disorders (SMQ)</i>		<i>Generalised pustular psoriasis of pregnancy</i>	10093632
Term scope	<i>Narrow</i>		To SMQ	
			<i>Immune-mediated/autoimmune disorders (SMQ)</i>	20000236
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Generalised pustular psoriasis of pregnancy to Immune-mediated/autoimmune disorders (SMQ) as a Narrow term is approved as requested. Generalized pustular psoriasis is a well-recognized autoinflammatory/immune-mediated disease (already included in the narrow scope of the SMQ). The pregnancy-associated variant (also called impetigo herpetiformis) is currently considered part of the pustular psoriasis spectrum and fits clearly within the narrow scope of this SMQ.

Detail Report

CR Number: 2026148033		Implementation Date: 29-May-26	Related CR: 2026148033	MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement	
Proposed PT	<i>Juvenile parkinsonism</i>	Approved Not as Requested	Proposed PT	
To SMQ	<i>Neuroleptic malignant syndrome (SMQ)</i>		<i>Juvenile parkinsonism</i>	10093490
Term scope	<i>Broad</i>		To SMQ	
			<i>Neuroleptic malignant syndrome (SMQ)</i>	20000044
			Term scope	
			<i>Broad</i>	
			Term_category	
			<i>D</i>	
			Term_weight	
			<i>0</i>	

MSSO Comment: The proposal to add PT Juvenile parkinsonism to Neuroleptic malignant syndrome (SMQ) as a broad term is approved but not as requested. PT Juvenile parkinsonism was added in v29.0 and is characterised by parkinsonian symptoms and signs presenting prior to 21 years of age. Although frequently caused by genetic disease, it may also be caused by relevant medical history such as exposure to dopamine receptor blocking agents. PT Juvenile parkinsonism will be added with broad scope, in line with already included PTs representing Parkinsonism and Parkinson's disease. Juvenile parkinsonism will be added to Neuroleptic malignant syndrome (SMQ) in the Broad scope changing the value in category from -1 to D.

CR Number: 2026148034		Implementation Date: 29-May-26	Related CR: 2026148034	MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement	
Proposed PT	<i>Noma neonatorum</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Opportunistic infections (SMQ)</i>		<i>Noma neonatorum</i>	10093619
Term scope	<i>Narrow</i>		To SMQ	
			<i>Opportunistic infections (SMQ)</i>	20000235
			Term scope	
			<i>Narrow</i>	
			Term_category	
			<i>A</i>	
			Term_weight	
			<i>0</i>	

MSSO Comment: The proposal to add PT Noma neonatorum to Opportunistic infections (SMQ) as a Narrow term is approved as requested. Noma neonatorum is a fulminant necrotising infection occurring almost exclusively in severely debilitated or immunocompromised neonates, particularly in the setting of neonatal sepsis, extreme malnutrition, prematurity, or profound systemic illness. Although the causative organisms are not necessarily unusual pathogens, the aggressive tissue destruction, rapid progression, and occurrence in a highly vulnerable host are rarely encountered in immunocompetent individuals. The condition therefore aligns well with the second inclusion criterion of the SMQ regarding unusually severe infections caused by common pathogens in compromised hosts, and will be included in the narrow scope.

Detail Report

CR Number: 2026148035		Implementation Date: 29-May-26	Related CR: 2026148035		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Acute oedema/cutaneous distension syndrome</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Haemodynamic oedema, effusions and fluid overload (SMQ)</i>		<i>Acute oedema/cutaneous distension syndrome</i>		10093618
Term scope	<i>Narrow</i>		To SMQ		
			<i>Haemodynamic oedema, effusions and fluid overload (SMQ)</i>		20000145
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Acute oedema/cutaneous distension syndrome to Haemodynamic oedema, effusions and fluid overload (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar terms such as PT Oedema and PT Fluid retention are already included as narrow scope terms.

CR Number: 2026148036		Implementation Date: 29-May-26	Related CR: 2026148036		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Generalised pustular psoriasis of pregnancy</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Pregnancy, labour and delivery complications and risk factors (excl abortions and stillbirth) (SMQ)</i>		<i>Generalised pustular psoriasis of pregnancy</i>		10093632
Term scope	<i>Narrow</i>		To SMQ		
			<i>Pregnancy, labour and delivery complications and risk factors (excl abortions and stillbirth) (SMQ)</i>		20000186
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Generalised pustular psoriasis of pregnancy to Pregnancy, labour and delivery complications and risk factors (excl abortions and stillbirth) (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar term such as PT Impetigo herpetiformis which was in this SMQ is now demoted under this new PT. As well, similar PT Cholestasis of pregnancy and PT Polymorphic eruption of pregnancy are already included as narrow scope terms.

Detail Report

CR Number: 2026148037		Implementation Date: 29-May-26	Related CR: 2026148037	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Noma neonatorum</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Neonatal disorders (SMQ)</i>		<i>Noma neonatorum</i>	10093619
Term scope	<i>Narrow</i>		To SMQ	
			<i>Neonatal disorders (SMQ)</i>	20000191
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Noma neonatorum to Neonatal disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ.
Comment: Similar terms such as PT Erythema toxicum neonatorum and PT Eczema infantile are already included as narrow scope terms.

CR Number: 2026148038		Implementation Date: 29-May-26	Related CR: 2026148038	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Stomatitis necrotising</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Opportunistic infections (SMQ)</i>		<i>Stomatitis necrotising</i>	10042135
Term scope	<i>Narrow</i>		To SMQ	
			<i>Opportunistic infections (SMQ)</i>	20000235
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Stomatitis necrotising to Opportunistic infections (SMQ) as a Narrow term is approved as requested. Necrotising stomatitis is an unusually severe and destructive oral infection characterized by extensive mucosal and soft tissue necrosis. It is classically associated with profound immunosuppression, severe malnutrition, HIV infection, neutropenia, or major systemic debilitation, and is rarely encountered in immunocompetent individuals. This term will be added to the narrow scope of this SMQ.

Detail Report

CR Number: 2026148039		Implementation Date: 29-May-26	Related CR: 2026148039	
<u>Change Requested</u>		<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Konno-Rastan procedure	Approved as Requested	Proposed PT	
To SMQ	Neonatal disorders (SMQ)		Konno-Rastan procedure	10093666
Term scope	Narrow		To SMQ	
			Neonatal disorders (SMQ)	20000191
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Konno-Rastan procedure to Neonatal disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this
Comment: SMQ. Similar terms such as PT Birth defect correction, which is a procedure which may be performed in neonatal period and on occasion beyond, and PT Atrial septal defect repair are already included as narrow scope terms.

CR Number: 2026148040		Implementation Date: 29-May-26	Related CR: 2026148040	
<u>Change Requested</u>		<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Right ventricular outflow tract stent insertion	Approved as Requested	Proposed PT	
To SMQ	Neonatal disorders (SMQ)		Right ventricular outflow tract stent insertion	10093661
Term scope	Narrow		To SMQ	
			Neonatal disorders (SMQ)	20000191
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO The proposal to add PT Right ventricular outflow tract stent insertion to Neonatal disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion
Comment: criteria for this SMQ. Similar terms such as PT Lecompte procedure and PT Atrial septal stent placement are already included as narrow scope terms.

Detail Report

CR Number: 2026148041		Implementation Date: 29-May-26	Related CR: 2026148041	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Vascular ring repair</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Neonatal disorders (SMQ)</i>		<i>Vascular ring repair</i>	10093603
Term scope	<i>Narrow</i>		To SMQ	
			<i>Neonatal disorders (SMQ)</i>	20000191
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Vascular ring repair to Neonatal disorders (SMQ) as a Narrow term is approved as requested. Vascular ring repair: although vascular rings are a congenital condition, the procedure is often performed at age 6 months- 2 years. This term meets the inclusion criteria for procedures related to congenital and fetal disorders.

CR Number: 2026148042		Implementation Date: 29-May-26	Related CR: 2026148042	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Patent ductus arteriosus stent placement</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Neonatal disorders (SMQ)</i>		<i>Patent ductus arteriosus stent placement</i>	10081876
Term scope	<i>Narrow</i>		To SMQ	
			<i>Neonatal disorders (SMQ)</i>	20000191
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Patent ductus arteriosus stent placement to Neonatal disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ. Similar terms such as PT Tetralogy of Fallot repair and PT Atrial septal stent placement are already included as narrow scope terms.

Detail Report

CR Number: 2026148043		Implementation Date: 29-May-26	Related CR: 2026148043	
<u>Change Requested</u>	Change term status in a SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Creatine abnormal</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>		<i>Creatine abnormal</i>	10011329
Status	<i>I</i>		To SMQ	
			<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>	20000225
			Status	
			<i>I</i>	

MSSO Comment: The proposal to Change status of the term Creatine abnormal in Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ) to Inactive is approved as requested. An abnormal creatinine level may be a diagnostic indicator of kidney visceral involvement, which is a prominent feature that correlates with DRESS mortality whereas creatine is an energy-producing compound; therefore, PT Creatine abnormal will be made inactive in this SMQ.

CR Number: 2026148044		Implementation Date: 29-May-26	Related CR: 2026148044	
<u>Change Requested</u>	Change term status in a SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Creatine increased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>		<i>Creatine increased</i>	10011333
Status	<i>I</i>		To SMQ	
			<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>	20000225
			Status	
			<i>I</i>	

MSSO Comment: The proposal to Change status of the term Creatine increased in Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ) to Inactive is approved as requested. An elevated serum creatinine level is a major diagnostic indicator of kidney visceral involvement, which is a prominent feature that correlates with DRESS mortality whereas creatine is an energy-producing compound; therefore, PT Creatine increased will be made inactive in this SMQ.

Detail Report

CR Number: 2026148045		Implementation Date: 29-May-26	Related CR: 2026148045	
<u>Change Requested</u>	Change term status in a SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Stomatitis necrotising</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>		<i>Stomatitis necrotising</i>	10042135
Status	<i>I</i>		To SMQ	
			<i>Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ)</i>	20000225
			Status	
			<i>I</i>	

MSSO Comment: The proposal to Change status of the term Stomatitis necrotising in Drug reaction with eosinophilia and systemic symptoms syndrome (SMQ) to Inactive is approved as requested. Necrotizing stomatitis is a severe, rapidly progressive oral infection characterized by extensive tissue death (necrosis) and ulceration and as such is linked to primary SOC Infections and infestations. Based on exclusion criteria to exclude terms with infectious etiology, and because this level of mucosal involvement is not typical for DRESS; the term will be made inactive in this SMQ.

CR Number: 2026148046		Implementation Date: 29-May-26	Related CR: 2026148046	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Small intestine erosion</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Gastrointestinal nonspecific inflammation (SMQ)</i>		<i>Small intestine erosion</i>	10093642
Term scope	<i>Narrow</i>		To SMQ	
			<i>Gastrointestinal nonspecific inflammation (SMQ)</i>	20000138
			Term scope	
			<i>Narrow</i>	
			Term_category	
			<i>A</i>	
			Term_weight	
			<i>0</i>	

MSSO Comment: The proposal to add PT Small intestine erosion to Gastrointestinal nonspecific inflammation (SMQ) as a Narrow term is approved as requested. The term does not represent a distinct nosologic entity such as Crohn's disease or Ulcerative colitis, but rather a nonspecific pathological manifestation that may occur in several inflammatory or drug-induced gastrointestinal conditions, including NSAID-associated enteropathy. Like PT Colitis erosive, an already included comparable erosive inflammatory process, the term will be included in the narrow scope of the SMQ.

Detail Report

CR Number: 2026148047		Implementation Date: 29-May-26	Related CR: 2026148047	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Asomatognosia</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>		<i>Asomatognosia</i>	10093588
Term scope	<i>Broad</i>		To SMQ	
			<i>Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)</i>	20000166
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Asomatognosia to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Broad term is approved as requested. Asomatognosia is a neurological condition in which a person loses awareness of one or more parts of their own body. Classic asomatognosia usually involves only a part of the body, not the entire body. Asomatognosia can contribute to detrimental or harmful behaviors related to the neglected body part (e.g., failure to protect, groom, maintain or clothe the involved body part) and these patients are prone to injuries/preventable medical conditions such as fractures, burns or infections. Agnosia syndromes limited to a single body part are uncommon but, when present, usually result from very isolated lesions (including early tumors or small lacunar infarctions). Therefore, this term will be added as a broad term within this SMQ.

CR Number: 2026148048		Implementation Date: 29-May-26	Related CR: 2026148048	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Cold panniculitis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Accidents and injuries (SMQ)</i>		<i>Cold panniculitis</i>	10093615
Term scope	<i>Broad</i>		To SMQ	
			<i>Accidents and injuries (SMQ)</i>	20000135
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Cold panniculitis to Accidents and injuries (SMQ) as a Broad term is approved as requested. Cold panniculitis represents an acute thermal injury resulting in inflammation of the subcutaneous fat. It manifests as erythematous, indurated nodules, and plaques on exposed skin, especially the perioral areas and cheeks, appearing 24 to 72 hours after exposure to cold. This term, which describes a specific type of environmental injury, will be added to the broad search terms of this SMQ.

Detail Report

CR Number: 2026148049

Implementation Date: 29-May-26

Related CR: 2026148049

Change Requested

Add term to SMQ

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT

Pituitary crisis

Approved as Requested

Proposed PT

To SMQ

Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)

Pituitary crisis

10093568

To SMQ

Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)

20000166

Term scope

Broad

Term scope

Broad

Term_category

A

Term_weight

0

MSSO

Comment:

The proposal to add PT Pituitary crisis to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Broad term is approved as requested. Pituitary crisis is an acute, life-threatening clinical state caused by sudden failure of pituitary function most often due to pituitary apoplexy. It is characterized by abrupt headache, visual or cranial nerve deficits, altered consciousness, and endocrine insufficiency. Pituitary apoplexy can result from hemorrhagic or ischemic injuries to the gland, typically associated with underlying tumors. Because these types of vascular accident are secondary to underlying pituitary lesions, this term will be added to the broad search terms of this SMQ.

Detail Report

CR Number: 2026148050

Implementation Date: 29-May-26

Related CR: 2026148050

Change Requested

Add term to SMQ

Final Disposition

Final Placement

MedDRA
Code #

Proposed PT

Somatoparaphrenia

Approved as Requested

Proposed PT

Somatoparaphrenia

10093594

To SMQ

Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)

To SMQ

Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ)

20000166

Term scope

Narrow

Term scope

Narrow

Term_category

A

Term_weight

0

MSSO

Comment:

The proposal to add PT Somatoparaphrenia to Conditions associated with central nervous system haemorrhages and cerebrovascular accidents (SMQ) as a Narrow term is approved as requested. Somatoparaphrenia is a neuropsychological disorder in which the person denies that a part of their body belongs to them, most often an arm or a leg, and may attribute it to someone else. It usually appears after brain lesions, particularly of the right hemisphere, and is often associated with hemineglect, anosognosia, and sensory or motor deficits. Because somatoparaphrenia usually involves an entire body part or parts (i.e., typically the left face, arm and/or leg), it is usually the result of a hemorrhagic or ischemic vascular insult affecting the parietal lobe. The "hemi" nature of this condition is very specific for vascular accidents and, therefore, it will be included in the narrow search terms of this SMQ.

Detail Report

CR Number: 2026148051		Implementation Date: 29-May-26	Related CR: 2026148051	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Gastric rupture</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Accidents and injuries (SMQ)</i>		<i>Gastric rupture</i>	10093640
Term scope	<i>Narrow</i>		To SMQ	
			<i>Accidents and injuries (SMQ)</i>	20000135
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Gastric rupture to Accidents and injuries (SMQ) as a Narrow term is approved as requested. Gastric rupture is a rare, but severe, condition defined as a rupture of the stomach wall. It results in leakage of gastric contents into the peritoneal cavity, often leading to peritonitis. Common causes include abdominal trauma, acute gastric distension and ischemic necrosis of the gastric wall. Less commonly, gastric rupture may have an iatrogenic etiology. It represents a surgical emergency requiring prompt diagnosis and management. This term will be to the narrow search terms of this SMQ.

CR Number: 2026148052		Implementation Date: 29-May-26	Related CR: 2026148052	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Prescription fraud</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Drug abuse and dependence (SMQ)</i>		<i>Prescription fraud</i>	10093675
Term scope	<i>Broad</i>		To SMQ	
			<i>Drug abuse and dependence (SMQ)</i>	20000101
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Prescription fraud to Drug abuse and dependence (SMQ) as a Broad term is approved as requested. Prescription forgery is the act of creating or using a falsified prescription that appears to be issued by a legitimate healthcare provider without their authorization in order to obtain medication unlawfully. In legal and healthcare compliance contexts, "prescription fraud" is the charge category, while "forgery" identifies the specific method used. Prescription fraud is one of the methods utilized by individuals with drug dependency issues to obtain their preferred drug of abuse. This term will be added to the broad search terms of this SMQ.

Detail Report

CR Number: 2026148053		Implementation Date: 29-May-26	Related CR: 2026148053	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	Portal vein stent placement	Approved as Requested	Proposed PT	
To SMQ	Embolic and thrombotic events, venous (SMQ)		Portal vein stent placement	10093674
Term scope	Narrow		To SMQ	
			Embolic and thrombotic events, venous (SMQ)	20000084
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Portal vein stent placement to Embolic and thrombotic events, venous (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criterion for the procedure-related terms. Portal vein stent placement refers to a medical procedure in which a stent is placed in the portal vein to maintain or restore blood flow, often used to treat conditions such as portal vein thrombosis or other obstructions. All terms in this SMQ are narrow scope terms.

CR Number: 2026148055		Implementation Date: 03-Jun-26	Related CR: 2026148055	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Vascular congestion	Rejected		

MSSO Comment: The proposal to add a new term Vascular congestion is not approved. The proposed term is anatomically imprecise and vague. As an unqualified term, it may refer to numerous vessel types and anatomic locations.

Detail Report

CR Number: 2026149000		Implementation Date: 02-Jun-26	Related CR: 2026149000	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Corkscrew ureter</i>	Approved Not as Requested	Proposed LLT	
To PT	<i>Urinary tract malformation</i>		<i>Corkscrew ureter</i>	10093789 Current
			To PT	
			<i>Congenital ureteric anomaly</i>	10061078
MSSO Comment: The proposal to add a new LLT Corkscrew ureter to PT Urinary tract malformation is approved but not as requested. Corkscrew ureter or spiral ureter refers to a tortuous ureter exhibiting a helical or corkscrew-like configuration along its course, usually associated with congenital ureteral abnormalities. Corkscrew ureter will be added as LLT to PT Congenital ureteric anomaly.				
CR Number: 2026149045		Implementation Date: 03-Jun-26	Related CR: 2026149045	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Central line associated bloodstream infection</i>	Approved Not as Requested	Proposed LLT	
			<i>Central line-associated bloodstream infection</i>	10093797 Current
			To PT	
			<i>Device related bacteraemia</i>	10084453
MSSO Comment: The proposal to add a new term Central line associated bloodstream infection is approved but not as requested. Central line-associated bloodstream infection is defined as a laboratory-confirmed bloodstream infection in a patient with a central venous catheter in place for ≥2 calendar days and with no other identifiable source of infection. The proposed term will be modified for hyphen usage and added as LLT Central line-associated bloodstream infection to PT Device related bacteraemia.				
CR Number: 2026149046		Implementation Date: 03-Jun-26	Related CR: 2026149046	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>CLABSI</i>	Approved as Requested	Proposed LLT	
			<i>CLABSI</i>	10093798 Current
			To PT	
			<i>Device related bacteraemia</i>	10084453
MSSO Comment: The proposal to add a new term CLABSI is approved as requested. CLABSI is an acronym which stands for Central Line-Associated Bloodstream Infection, which has been added as a spelled-out concept in change request 2026149045. CLABSI will be added as LLT to PT Device related bacteraemia.				

Detail Report

CR Number: 2026149047		Implementation Date: 03-Jun-26	Related CR: 2026149047	
Change Requested	Reassign Primary SOC	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Paroxysmal nocturnal haemoglobinuria</i>	Rejected		
Old Primary SOC	<i>Blood and lymphatic system disorders</i>			
New Primary SOC	<i>Renal and urinary disorders</i>			
MSSO Comment:	The proposal to reassign the primary SOC of PT Paroxysmal nocturnal haemoglobinuria from current SOC Blood and lymphatic system disorders to SOC Renal and urinary disorders is not approved. Technical rejection. The proposed change was recently performed for version 29.1 in change request 2026043008. Please refer to the supplemental view to see the current placement of PT Paroxysmal nocturnal haemoglobinuria.			

CR Number: 2026149048		Implementation Date: 05-Jun-26	Related CR: 2026149048	
Change Requested	Rename PT/LLT	Final Disposition	Final Placement	MedDRA Code #
Term to modify	<i>Hereditary hypotrichosis</i>	Approved as Requested	Term to modify	
Replacement term	<i>Hereditary hypotrichosis</i>		<i>Hereditary hypotrichosis</i>	10093617
			Replacement term	
			<i>Hereditary hypotrichosis</i>	10093617
MSSO Comment:	The proposal to rename PT/LLT Hereditary hypotrichosis to Hereditary hypotrichosis is approved as requested. Term will be corrected for misspelling.			

CR Number: 2026151000		Implementation Date: 05-Jun-26	Related CR: 2026151000	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Cardiac pre-tamponade</i>	Rejected		
MSSO Comment:	The proposal to add a new PT Cardiac pre-tamponade is not approved. Cardiac pre-tamponade is a descriptive term used for a significant pericardial effusion that shows early hemodynamic effects but has not yet progressed to full cardiac tamponade with overt clinical instability. In practice, it functions more as a severity/staging descriptor than a distinct clinical condition, signaling a high risk of progression and the need for close monitoring and management. MedDRA does not generally add terms related to progression and severity of conditions.			

Detail Report

CR Number: 2026151001		Implementation Date: 05-Jun-26	Related CR: 2026151001	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Product administration for unapproved combination</i>	Suspended		
HLT primary	<i>Product administration errors and issues</i>			
SOC primary	<i>Injury, poisoning and procedural complications</i>			
MSSO Comment:	The proposal to add a new PT Product administration for unapproved combination to primary HLT Product administration errors and issues in SOC Injury, poisoning and procedural complications is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.			

CR Number: 2026151002		Implementation Date: 05-Jun-26	Related CR: 2026151002	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Product administration without indicated laboratory test performed</i>	Suspended		
HLT primary	<i>Product administration errors and issues</i>			
SOC primary	<i>Injury, poisoning and procedural complications</i>			
MSSO Comment:	The proposal to add a new PT Product administration without indicated laboratory test performed to primary HLT Product administration errors and issues in SOC Injury, poisoning and procedural complications is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.			

CR Number: 2026151003		Implementation Date: 05-Jun-26	Related CR: 2026151003	
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Product monotherapy administration</i>	Suspended		
HLT primary	<i>Product administration errors and issues</i>			
SOC primary	<i>Injury, poisoning and procedural complications</i>			
MSSO Comment:	The proposal to add a new PT Product monotherapy administration to primary HLT Product administration errors and issues in SOC Injury, poisoning and procedural complications is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.			

Detail Report

CR Number: 2026151004		Implementation Date: 05-Jun-26	Related CR: 2026151004		
Change Requested	Add a New PT	Final Disposition	Final Placement	MedDRA Code #	
Proposed PT	<i>Product administration for unapproved indication</i>	Suspended			
HLT primary	<i>Product administration errors and issues</i>				
SOC primary	<i>Injury, poisoning and procedural complications</i>				
MSSO Comment:	The proposal to add a new PT Product administration for unapproved indication to primary HLT Product administration errors and issues in SOC Injury, poisoning and procedural complications is suspended. The MSSO would like to take additional time to study this proposed change. You will be notified once a decision has been made on the disposition of your request.				

CR Number: 2026151005		Implementation Date:	05-Jun-26	Related CR:	2026151005	
Change Requested	Add a New PT		Final Disposition	Final Placement		MedDRA Code #
Proposed PT	<i>Bulge</i>		Rejected			
HLT primary	<i>General signs and symptoms NEC</i>					
SOC primary	<i>General disorders and administration site conditions</i>					
MSSO Comment:	The proposal to add a new PT Bulge to primary HLT General signs and symptoms NEC in SOC General disorders and administration site conditions is not approved. "Bulge" is an imprecise word, and such imprecise terms will generally not be added into MedDRA. Please consider existing unspecific terms e.g. LLT/PT Mass for your specific coding needs.					

Detail Report

CR Number: 2026151006		Implementation Date: 03-Jun-26	Related CR: 2026151006	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Post-traumatic pseudolipoma</i>	Approved Not as Requested	Proposed PT <i>Post-traumatic pseudolipoma</i>	10093799
			HLT primary <i>Non-site specific injuries NEC</i>	10029511
			SOC primary <i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary <i>Lipid metabolism and deposit disorders NEC</i>	10024581
			SOC secondary <i>Metabolism and nutrition disorders</i>	10027433
			HLT secondary <i>Lipodystrophies</i>	10024606
			SOC secondary <i>Skin and subcutaneous tissue disorders</i>	10040785

MSSO Comment: The proposal to add a new LLT Post-traumatic pseudolipoma is approved but not as requested. A post-traumatic pseudolipoma (also known as post-traumatic lipohypertrophy or fat fracture) is a benign adipose tissue mass that develops following acute, chronic, or repetitive injury. Unlike conventional lipomas, post-traumatic pseudolipomas lack a fibrous capsule and are thought to arise from the displacement or herniation of subcutaneous fat. Post-traumatic pseudolipoma will be added as new PT to primary HLT Non-site specific injuries NEC and secondary HLT Lipid metabolism and deposit disorders NEC and secondary HLT Lipodystrophies. .

Detail Report

CR Number: 2026152001		Implementation Date: 08-Jun-26	Related CR: 2026152001	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Frontal lobe stroke	Approved as Requested	Proposed PT Frontal lobe stroke	10093811
			HLT primary Central nervous system haemorrhages and cerebrovascular accidents	10007948
			SOC primary Nervous system disorders	10029205
			HLT secondary Cerebrovascular and spinal necrosis and vascular insufficiency	10008192
			SOC secondary Vascular disorders	10047065

MSSO Comment: The proposal to add a new term Frontal lobe stroke is approved as requested. Frontal lobe stroke is a cerebrovascular event caused by interruption of blood flow or bleeding affecting the frontal lobe of the brain. Because the frontal lobe is responsible for voluntary motor control, language production, executive functions, behavior, personality, and emotional regulation, damage to this region may result in motor deficits, speech impairments, cognitive dysfunction, and behavioral or personality changes. Frontal lobe stroke will be added as new PT to primary HLT Central nervous system haemorrhages and cerebrovascular accidents and secondary HLT Cerebrovascular and spinal necrosis and vascular insufficiency.

CR Number: 2026152002		Implementation Date: 08-Jun-26	Related CR: 2026152002	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	Left frontal lobe infarct	Rejected		

MSSO Comment: The proposal to add a new term Left frontal lobe infarct is not approved. The proposed concept is granular and generally laterality qualified terms are not added unless clinically significant.

Detail Report

CR Number: 2026152003		Implementation Date: 05-Jun-26	Related CR: 2026152003		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Post Finasteride Syndrome</i>	Approved Not as Requested	Proposed LLT		
To PT	<i>Post 5-alpha-reductase inhibitor syndrome</i>		<i>Post-finasteride syndrome</i>	10093800	Current
			To PT		
			<i>Post 5-alpha-reductase inhibitor syndrome</i>	10082430	

MSSO Comment: The proposal to add a new LLT Post Finasteride Syndrome to PT Post 5-alpha-reductase inhibitor syndrome is approved but not as requested. Post finasteride syndrome is a condition characterized by persistent sexual, physical, and neurological/psychiatric side effects that develop or continue long after stopping finasteride. Proposed term will be modified for MedDRA capitalization conventions with a hyphen and added as LLT Post-finasteride syndrome to PT Post 5-alpha-reductase inhibitor syndrome.

CR Number: 2026152004		Implementation Date: 05-Jun-26	Related CR: 2026152004		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>PFS</i>	Rejected			
To PT	<i>Post 5-alpha-reductase inhibitor syndrome</i>				

MSSO Comment: The proposal to add a new LLT PFS to PT Post 5-alpha-reductase inhibitor syndrome is not approved. The acronym PFS is ambiguous and may be interpreted for numerous concepts both medical and non-medical related such as Progression-free survival, Personal Financial Statement, Parallel File System ways, etc.

CR Number: 2026152005		Implementation Date: 05-Jun-26	Related CR: 2026152005		
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #	
Proposed LLT	<i>Post Dutasteride Syndrome</i>	Rejected			
To PT	<i>Post 5-alpha-reductase inhibitor syndrome</i>				

MSSO Comment: The proposal to add a new LLT Post Dutasteride Syndrome to PT Post 5-alpha-reductase inhibitor syndrome is not approved. It is not possible to represent every possible drug associated syndrome for drugs which are 5-alpha-reductase inhibitors in MedDRA.

Detail Report

CR Number: 2026152006	Implementation Date: 05-Jun-26	Related CR: 2026152006	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Burning SOE</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Burning SOE is not approved. The requested term is unclear as to what the "SOE" acronym represents. Existing unqualified term LLT/PT Burning sensation may be used in combination with the specific site term if a term for a specific anatomical burning sensation site does not exist.

CR Number: 2026152007	Implementation Date: 05-Jun-26	Related CR: 2026152007	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Agony or Agony sensation</i>	Rejected	

MSSO Comment: The proposal to add a new LLT Agony or Agony sensation is not approved. Agony may mean intense feelings of suffering; acute mental or physical pain. The proposed term is ambiguous and may be interpreted as a mental feeling of suffering or physical pain. Please consider existing synonyms for both interpretations, such as LLT Emotional suffering for the emotional aspect and other LLT related to physical pain such as LLT Pain, LLT Pain crisis, LLT Intractable pain or other depending on your specific case scenario. MedDRA also has several specific manifestations expressed as "agonal" (adjective of agony).

CR Number: 2026152008	Implementation Date: 05-Jun-26	Related CR: 2026152008	MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement
Proposed LLT	<i>Phonophoresis</i>	Approved Not as Requested	Proposed PT <i>Phonophoresis</i> 10093803 HLT primary <i>Therapeutic procedures NEC</i> 10027700 SOC primary <i>Surgical and medical procedures</i> 10042613

MSSO Comment: The proposal to add a new LLT Phonophoresis is approved but not as requested. Phonophoresis, also known as sonophoresis or ultrasonophoresis, is when an ultrasound is used to maximize the effects of a topical drug. The ultrasound waves push particles of a pain-relieving or anti-inflammatory drug deeper into skin tissues. Phonophoresis is commonly used to treat inflammatory issues and injuries to soft tissues. The effectiveness of phonophoresis is debated in the medical world. Phonophoresis will be added as PT to HLT Therapeutic procedures NEC.

Detail Report

CR Number: 2026152009		Implementation Date: 05-Jun-26	Related CR: 2026152009		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Pseudomenopause</i>	Rejected			

MSSO Comment: The proposal to add a new LLT Pseudomenopause is not approved. Pseudomenopause is not a widely recognized or standardized medical diagnosis but is sometimes used informally to describe a situation where someone experiences menopause-like hormonal changes and symptoms without being in natural menopause, e.g., medication-induced suppression of ovarian hormones, hormonal disorders that create low-estrogen states resembling menopause. Please consider existing LLTs, e.g. LLTs linked to PT Artificial menopause for your specific coding needs.

CR Number: 2026152010		Implementation Date: 05-Jun-26	Related CR: 2026152010		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Desmoglein I Antibody</i>	Approved Not as Requested	Proposed PT		
			<i>Desmoglein 1 antibody</i>		10093801
			HLT primary		
			<i>Autoimmunity analyses</i>		10003828
			SOC primary		
			<i>Investigations</i>		10022891

MSSO Comment: The proposal to add a new LLT Desmoglein I Antibody is approved but not as requested. Desmoglein 1 antibody is an autoantibody directed against desmoglein 1, a cell-adhesion protein found in the upper layers of the skin epidermis. These antibodies are commonly associated with the autoimmune blistering disease Pemphigus foliaceus, where they disrupt cell-to-cell adhesion and cause superficial skin blistering and erosions. The proposed term will be modified for Arabic numerals and MedDRA capitalization rules and added as PT Desmoglein 1 antibody to HLT Autoimmunity analyses.

CR Number: 2026152011		Implementation Date: 05-Jun-26	Related CR: 2026152011		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	<i>Desmoglein 3 Antibody</i>	Approved Not as Requested	Proposed PT		
			<i>Desmoglein 3 antibody</i>		10093802
			HLT primary		
			<i>Autoimmunity analyses</i>		10003828
			SOC primary		
			<i>Investigations</i>		10022891

MSSO Comment: The proposal to add a new LLT Desmoglein 3 Antibody is approved but not as requested. Desmoglein 3 antibody is an autoantibody that targets desmoglein 3, a cell-adhesion protein primarily expressed in the lower layers of the epidermis and in mucosal epithelium. These antibodies are strongly associated with Pemphigus vulgaris, where they impair cell-to-cell adhesion and lead to painful mucosal erosions and skin blistering. The proposed term will be modified for MedDRA capitalization rules and added as PT Desmoglein 3 antibody to HLT Autoimmunity analyses.

Detail Report

CR Number: 2026152012		Implementation Date: 05-Jun-26	Related CR: 2026152012	
<u>Change Requested</u>	Add a New LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed LLT	<i>Lipofibromatous Hamartoma</i>	Approved Not as Requested	Proposed PT <i>Lipofibromatous hamartoma</i>	10093804
			HLT primary <i>Nervous system neoplasms benign NEC</i>	10029210
			SOC primary <i>Neoplasms benign, malignant and unspecified (incl cysts and polyps)</i>	10029104
			HLT secondary <i>Nervous system neoplasms benign NEC</i>	10029210
			SOC secondary <i>Nervous system disorders</i>	10029205

MSSO Comment: The proposal to add a new LLT Lipofibromatous Hamartoma is approved but not as requested. Lipofibromatous hamartoma is a benign non-neoplastic nerve sheath tumor. It is a rare condition most commonly involving the median nerve in children or young adults, and at times associated with macrodactyly. These tumors involve the epineurium with fibrosis and fat, and sometimes other tissues such as muscle. Patients present with either a mass, enlarged fingers, or neurologic symptoms due to compression of the nerve. Neurologic symptoms are most likely if the distal median nerve is involved in the carpal tunnel. The proposed term will be modified for MedDRA capitalization rules and added as PT Lipofibromatous hamartoma to bi-axial HLT Nervous system neoplasms benign NEC (primary SOC Neoplasms benign, malignant and unspecified (incl cysts and polyps), secondary SOC Nervous system disorders).

CR Number: 2026152015		Implementation Date: 05-Jun-26	Related CR: 2026152015	
<u>Change Requested</u>	Reassign Primary SOC	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Menopause</i>	Rejected		
Old Primary SOC	<i>Social circumstances</i>			
New Primary SOC	<i>Reproductive system and breast disorders</i>			

MSSO Comment: The proposal to reassign the primary SOC of PT Menopause from current SOC Social circumstances to SOC Reproductive system and breast disorders is not approved. Menopause syndrome refers to the physical and mental symptoms caused by decreasing estrogen levels during the menopausal transition (perimenopause) and after periods have stopped completely. Common signs include hot flashes, night sweats, sleep disturbances, mood changes, and vaginal dryness. PT Menopause is adequately represented in SOC Social circumstances as an age-related circumstance which marks the end of the reproductive years.

Detail Report

CR Number: 2026152016 **Implementation Date:** 05-Jun-26 **Related CR:** 2026152016 [MedDRA Code #](#)

[Change Requested](#) Reassign Primary SOC [Final Disposition](#) [Final Placement](#)

Proposed PT *Postmenopause* Rejected

Old Primary SOC *Social circumstances*

New Primary SOC *Reproductive system and breast disorders*

MSSO Comment: The proposal to reassign the primary SOC of PT Postmenopause from current SOC Social circumstances to SOC Reproductive system and breast disorders is not approved. Postmenopausal syndrome refers to the physical, emotional, and genitourinary symptoms that occur due to dropping estrogen levels after a woman's periods have ceased for 12 consecutive months. Symptoms can include hot flashes, sleep disturbances, vaginal dryness, and mood changes. In contrast, postmenopause is an age-related circumstance for a woman which begins after 12 consecutive months without a menstrual period and is appropriately represented as PT Postmenopause in SOC Social circumstances.

CR Number: 2026152017 **Implementation Date:** 05-Jun-26 **Related CR:** 2026152017 [MedDRA Code #](#)

[Change Requested](#) Link (move) a PT to another HLT [Final Disposition](#) [Final Placement](#)

Proposed PT *Procollagen type I C-terminal propeptide* Approved as Requested Proposed PT *Procollagen type I C-terminal propeptide* 10066813

From HLT *Protein analyses NEC*

To HLT *Biochemical markers of bone metabolism*

From HLT *Protein analyses NEC* 10036998

To HLT *Biochemical markers of bone metabolism* 10067225

MSSO Comment: The proposal to move the PT Procollagen type I C-terminal propeptide from HLT Protein analyses NEC to HLT Biochemical markers of bone metabolism is approved as requested. Procollagen type I C-terminal propeptide is a direct marker of bone formation, released during the synthesis of type I collagen by osteoblasts and thus better represented by HLT Biochemical markers of bone metabolism.

Detail Report

CR Number: 2026152018		Implementation Date: 05-Jun-26	Related CR: 2026152018		MedDRA Code #
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement		
Proposed PT	<i>Procollagen type I C-terminal propeptide increased</i>	Approved as Requested	Proposed PT		
From HLT	<i>Protein analyses NEC</i>		<i>Procollagen type I C-terminal propeptide increased</i>		10065264
To HLT	<i>Biochemical markers of bone metabolism</i>		From HLT		
			<i>Protein analyses NEC</i>		10036998
			To HLT		
			<i>Biochemical markers of bone metabolism</i>		10067225

MSSO Comment: The proposal to move the PT Procollagen type I C-terminal propeptide increased from HLT Protein analyses NEC to HLT Biochemical markers of bone metabolism is approved as requested. Procollagen type I C-terminal propeptide is a direct marker of bone formation, released during the synthesis of type I collagen by osteoblasts and thus better represented by HLT Biochemical markers of bone metabolism.

CR Number: 2026152019		Implementation Date: 05-Jun-26	Related CR: 2026152019		MedDRA Code #
Change Requested	Link (move) a PT to another HLT	Final Disposition	Final Placement		
Proposed PT	<i>Serum procollagen type I N-terminal propeptide decreased</i>	Approved Not as Requested	PT to merge		
From HLT	<i>Protein analyses NEC</i>		<i>Serum procollagen type I N-terminal propeptide decreased</i>		10079584 Current
To HLT	<i>Biochemical markers of bone metabolism</i>		To PT		
			<i>Procollagen type I N-terminal propeptide decreased</i>		10093807

MSSO Comment: The proposal to move the PT Serum procollagen type I N-terminal propeptide decreased from HLT Protein analyses NEC to HLT Biochemical markers of bone metabolism is approved but not as requested. Serum procollagen type I N-terminal propeptide is a sensitive marker of bone formation and is widely used to assess osteoblastic activity. PT Serum procollagen type I N-terminal propeptide decreased will be demoted under new PT Procollagen type I N-terminal propeptide decreased. In a related change, Procollagen type I N-terminal propeptide decreased will be added as a PT to HLT Biochemical markers of bone metabolism. Furthermore, PT Serum procollagen type I N-terminal propeptide added in change request 2026147008 will be renamed to Procollagen type I N-terminal propeptide.

Detail Report

CR Number: 2026156002		Implementation Date: 05-Jun-26	Related CR: 2026152019	
<u>Change Requested</u>	Rename PT/LLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Term to modify	<i>Serum procollagen type I N-terminal propeptide</i>	Approved as Requested	Term to modify	
Replacement term	<i>Procollagen type I N-terminal propeptide</i>		<i>Serum procollagen type I N-terminal propeptide</i>	10093775
			Replacement term	
			<i>Procollagen type I N-terminal propeptide</i>	10093775

MSSO
Comment:

CR Number: 2026156001		Implementation Date: 05-Jun-26	Related CR: 2026152019	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Procollagen type I N-terminal propeptide decreased</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Biochemical markers of bone metabolism</i>		<i>Procollagen type I N-terminal propeptide decreased</i>	10093807
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Biochemical markers of bone metabolism</i>	10067225
			SOC primary	
			<i>Investigations</i>	10022891

MSSO
Comment:

Detail Report

CR Number: 2026152020		Implementation Date: 05-Jun-26	Related CR: 2026152020	
<u>Change Requested</u>	Link (move) a PT to another HLT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Serum procollagen type I N-terminal propeptide increased</i>	Approved Not as Requested	PT to merge	
From HLT	<i>Protein analyses NEC</i>		<i>Serum procollagen type I N-terminal propeptide increased</i>	10064802 Current
To HLT	<i>Biochemical markers of bone metabolism</i>		To PT	
			<i>Procollagen type I N-terminal propeptide increased</i>	10093808
MSSO Comment: The proposal to move the PT Serum procollagen type I N-terminal propeptide increased from HLT Protein analyses NEC to HLT Biochemical markers of bone metabolism is approved but not as requested. Serum procollagen type I N-terminal propeptide is a sensitive marker of bone formation and is widely used to assess osteoblastic activity. PT Serum procollagen type I N-terminal propeptide increased will be demoted under new PT Procollagen type I N-terminal propeptide increased. In a related change, Procollagen type I N-terminal propeptide increased will be added as a PT to HLT Biochemical markers of bone metabolism.				

CR Number: 2026156003		Implementation Date: 05-Jun-26	Related CR: 2026152020	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Procollagen type I N-terminal propeptide increased</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Biochemical markers of bone metabolism</i>		<i>Procollagen type I N-terminal propeptide increased</i>	10093808
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Biochemical markers of bone metabolism</i>	10067225
			SOC primary	
			<i>Investigations</i>	10022891

MSSO Comment:

Detail Report

CR Number: 2026152021		Implementation Date: 08-Jun-26	Related CR: 2026152021	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Hypervitaminosis e</i>	Approved Not as Requested	Proposed PT <i>Hypervitaminosis E</i>	10093814
			HLT primary <i>Hypervitaminoses NEC</i>	10020914
			SOC primary <i>Metabolism and nutrition disorders</i>	10027433

MSSO Comment: The proposal to add a new term Hypervitaminosis e is approved but not as requested. Hypervitaminosis E is a condition resulting from excessive accumulation of vitamin E in the body, usually due to prolonged intake of high-dose supplements, which may lead to adverse effects such as impaired blood coagulation and an increased risk of bleeding. The proposed term will be modified in accordance with MedDRA capitalization conventions and added as PT Hypervitaminosis E to HLT Hypervitaminoses NEC.

CR Number: 2026152022		Implementation Date: 08-Jun-26	Related CR: 2026152022	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Candida angular cheilosis</i>	Rejected		

MSSO Comment: The proposal to add a new term Candida angular cheilosis is not approved. The proposed term combines a medical concept (PT Angular cheilitis) with additional medical wording related to etiology and/or pathophysiology (such as in PT Oral candidiasis). Section 3.5 of the MedDRA Term Selection Points to Consider (MTS:PTC) document defines these as "combination terms" and provides coding guidance.

CR Number: 2026152023		Implementation Date: 08-Jun-26	Related CR: 2026152023	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Vulvar crohn's disease</i>	Approved Not as Requested	Proposed LLT <i>Vulvar Crohn's disease</i>	10093816 Current
			To PT <i>Metastatic cutaneous Crohn's disease</i>	10070949

MSSO Comment: The proposal to add a new term Vulvar crohn's disease is approved but not as requested. Vulvar Crohn disease is granulomatous inflammation of the vulva occurring as an extraintestinal manifestation of Crohn's disease, characterized by vulvar edema, fissures, ulceration, and chronic inflammatory changes. The proposed term will be modified according to MedDRA capitalization conventions and added as LLT Vulvar Crohn's disease to PT Metastatic cutaneous Crohn's disease.

Detail Report

CR Number: 2026152024		Implementation Date: 08-Jun-26	Related CR: 2026152024	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Parotid tumor excision</i>	Rejected		
MSSO Comment: The proposal to add a new term Parotid tumor excision is not approved. The proposed concept is very granular for the SOC Surgical and medical procedures which is not as granular as the disorder SOC's and may be represented by existing MedDRA terms. Please refer to Section 3.5 of the MedDRA Term Selection: Points to Consider Document regarding the coding of combination terms.				

CR Number: 2026152025		Implementation Date: 08-Jun-26	Related CR: 2026152025	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Pneumoparotid</i>	Approved as Requested	Proposed PT <i>Pneumoparotid</i>	10093813
			HLT primary <i>Salivary gland disorders NEC</i>	10039407
			SOC primary <i>Gastrointestinal disorders</i>	10017947
MSSO Comment: The proposal to add a new term Pneumoparotid is approved as requested. A condition characterized by the presence of air within the parotid gland and/or its ductal system due to retrograde insufflation of air through the parotid duct. It may present with parotid swelling, pain, crepitus, or secondary inflammation of the gland. Pneumoparotid will be added as PT to HLT Salivary gland disorders NEC.				

CR Number: 2026152026		Implementation Date: 08-Jun-26	Related CR: 2026152026	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>ROMA</i>	Approved Not as Requested	Proposed LLT <i>ROMA index</i>	10093817 Current
			To PT <i>Risk of ovarian malignancy algorithm index</i>	10093812

MSSO Comment: The proposal to add a new term ROMA is approved but not as requested. The Risk of Ovarian Malignancy Algorithm (ROMA) Index is a predictive score that combines serum HE4 and CA125 concentrations with menopausal status to estimate the likelihood that an ovarian or adnexal mass is malignant. A higher ROMA score indicates a greater risk that an ovarian mass represents an epithelial ovarian cancer. The proposed term will be modified for greater clarity and added as LLT ROMA index to new PT Risk of ovarian malignancy algorithm index, which will be added to HLT Cell marker analyses. Both the LLT and new PT will complement existing qualified counterparts added in version 28.1.

Detail Report

CR Number: 2026159001		Implementation Date: 08-Jun-26	Related CR: 2026152026	
<u>Change Requested</u>	Add a New PT	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Risk of ovarian malignancy algorithm index</i>	Approved as Requested	Proposed PT	
HLT primary	<i>Cell marker analyses</i>		<i>Risk of ovarian malignancy algorithm index</i>	10093812
SOC primary	<i>Investigations</i>		HLT primary	
			<i>Cell marker analyses</i>	10072981
			SOC primary	
			<i>Investigations</i>	10022891

MSSO
Comment:

CR Number: 2026152027		Implementation Date: 08-Jun-26	Related CR: 2026152027	
<u>Change Requested</u>	Add a New Term	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed Term	<i>Hammond mastopexy</i>	Rejected		

MSSO
Comment: The proposal to add a new term Hammond mastopexy is not approved. Hammond mastopexy is a specific surgical variation of periareolar mastopexy that uses a particular circumareolar suture technique to improve breast lifting and areolar contour. This technique can be adequately captured by the existing LLT Mastopexy. Adding individual eponymous terms of very specific surgical variants would introduce unnecessary granularity and could lead to excessive proliferation of procedure terms within the terminology.

Detail Report

CR Number: 2026152028		Implementation Date: 08-Jun-26	Related CR: 2026152028	
Change Requested	Add a New Term	Final Disposition	Final Placement	MedDRA Code #
Proposed Term	<i>Post operative peritonitis</i>	Approved Not as Requested	Proposed PT <i>Postoperative peritonitis</i>	10093815
			HLT primary <i>Abdominal and gastrointestinal infections</i>	10017967
			SOC primary <i>Infections and infestations</i>	10021881
			HLT secondary <i>Gastrointestinal and hepatobiliary procedural complications</i>	10017927
			SOC secondary <i>Injury, poisoning and procedural complications</i>	10022117
			HLT secondary <i>Peritoneal infections</i>	10034668
			SOC secondary <i>Gastrointestinal disorders</i>	10017947

MSSO Comment: The proposal to add a new term Post operative peritonitis is approved but not as requested. Postoperative peritonitis (PP) is a life-threatening hospital-acquired intra-abdominal infection with high rates of mortality. The most common cause of PP is anastomotic leakage. It is most frequent after rectal resection but it may complicate all gastrointestinal anastomosis. The proposed term will be modified for better alignment and added as new PT Postoperative peritonitis to primary HLT Abdominal and gastrointestinal infections and secondary HLT Gastrointestinal and hepatobiliary procedural complications and secondary HLT Peritoneal infections.

CR Number: 2026152029		Implementation Date: 05-Jun-26	Related CR: 2026152029	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Decreased skin sensitivity</i>	Rejected		

MSSO Comment: The proposal to add a new LLT Decreased skin sensitivity is not approved. Existing term LLT Skin hypoaesthesia is defined as a dysesthesia consisting of abnormally decreased sensitivity, particularly to touch and may be used to describe decreased skin sensitivity.

Detail Report

CR Number: 2026152030		Implementation Date: 05-Jun-26	Related CR: 2026152030		MedDRA Code #
Change Requested	Add a New LLT	Final Disposition	Final Placement		
Proposed LLT	Dark circles under the eyes	Rejected			

MSSO Comment: The proposal to add a new LLT Dark circles under the eyes is not approved. Proposed concept already exists as LLT/PT Dark circles under eyes since MedDRA version 8.1.

CR Number: 2026152035		Implementation Date: 05-Jun-26	Related CR: 2026152035		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	Ultrasound adrenal	Approved Not as Requested	Proposed PT		
HLT primary	Endocrine analyses and imaging NEC		Adrenal gland ultrasound		10093805
SOC primary	Investigations		HLT primary		
			Endocrine analyses and imaging NEC		10014693
			SOC primary		
			Investigations		10022891

MSSO Comment: The proposal to add a new PT Ultrasound adrenal to primary HLT Endocrine analyses and imaging NEC in SOC Investigations is approved but not as requested. Adrenal gland ultrasound is an imaging method that can provide valuable information regarding the size, shape, and structure of the adrenal gland. As a result, it can aid in diagnosing various adrenal gland-related pathologies. Proposed term will be modified for natural language word order and specificity and added as PT Adrenal gland ultrasound to HLT Endocrine analyses and imaging NEC.

CR Number: 2026152036		Implementation Date: 05-Jun-26	Related CR: 2026152036		MedDRA Code #
Change Requested	Add a New PT	Final Disposition	Final Placement		
Proposed PT	Female genital tract ultrasound	Rejected			
HLT primary	Reproductive organ and breast imaging procedures				
SOC primary	Investigations				

MSSO Comment: The proposal to add a new PT Female genital tract ultrasound to primary HLT Reproductive organ and breast imaging procedures in SOC Investigations is not approved. Existing term LLT/PT Ultrasound pelvis may be used to capture the requested concept term as a pelvic ultrasound is a noninvasive diagnostic exam that produces images that are used to assess organs and structures within the female pelvis. A pelvic ultrasound allows quick visualization of the female pelvic organs and structures including the uterus, cervix, vagina, fallopian tubes and ovaries. In addition, MedDRA has LLT Transvaginal ultrasound scan for those structures, which are not easily accessible via pelvic ultrasound.

Detail Report

CR Number: 2026152037		Implementation Date: 05-Jun-26	Related CR: 2026152037	
Change Requested	Add a New LLT	Final Disposition	Final Placement	MedDRA Code #
Proposed LLT	<i>Otosyphilis</i>	Approved as Requested	Proposed LLT	10093806 Current
To PT	<i>Neurosyphilis</i>		<i>Otosyphilis</i>	
			To PT	10029339
		<i>Neurosyphilis</i>		
MSSO Comment:	The proposal to add a new LLT Otosyphilis to PT Neurosyphilis is approved as requested. Otosyphilis is a complication of the sexually transmitted infection syphilis, occurring when the Treponema pallidum bacteria infects the inner ear or auditory nerve. It can cause irreversible sensorineural hearing loss, tinnitus, and vertigo, but may be fully reversible if diagnosed and treated early.			

CR Number: 2026163083		Implementation Date: 15-Jun-26	Related CR: 2026163083	
Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Pituitary neuroendocrine tumour</i>	Approved as Requested	Proposed PT	10093691 Current
With LLT	<i>Pituitary adenoma</i>		<i>Pituitary neuroendocrine tumour</i>	
			With LLT	10035079
		<i>Pituitary adenoma</i>		
MSSO Comment: The proposal to swap the PT Pituitary neuroendocrine tumour with the LLT Pituitary adenoma is approved as requested. Consistent with the WHO / International Agency for Research on Cancer (IARC) proposal to unify the approach to neuroendocrine tumours (NETs) a change in nomenclature has been suggested, and these neoplasms are now recognized as PitNETs. As a transition from the fourth edition of the WHO classification of tumours of endocrine organs, in which the term “pituitary adenoma” was used, however in the fifth edition, the term “PitNET/adenoma” is used to indicate that “pituitary adenomas” still can be used. Pituitary adenoma is a well-established and frequently used term and has been used for decades. In addition, most pituitary adenomas are benign and do not behave aggressively. The new term Pituitary neuroendocrine tumour is still a subject of debate; therefore, the existing term Pituitary adenoma will be swapped so it is on the PT level, and the new term Pituitary neuroendocrine tumour is on the LLT level.				

Detail Report

CR Number: 2026182009		Implementation Date: 05-Jul-26	Related CR: 2026182009	
Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Familial exudative vitreoretinopathy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Familial exudative vitreoretinopathy</i>	10093708
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Familial exudative vitreoretinopathy to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026182010		Implementation Date: 05-Jul-26	Related CR: 2026182010	
Change Requested		Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Myocilin gene mutation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Myocilin gene mutation</i>	10093818
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Myocilin gene mutation to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026182011		Implementation Date: 05-Jul-26	Related CR: 2026182011	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Oculopharyngeal muscular dystrophy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Oculopharyngeal muscular dystrophy</i>	10093773
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Oculopharyngeal muscular dystrophy to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026182012		Implementation Date: 05-Jul-26	Related CR: 2026182012	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Orbital cavernous venous malformation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Congenital, familial and genetic disorders (SMQ)</i>		<i>Orbital cavernous venous malformation</i>	10093749
Term scope	<i>Narrow</i>		To SMQ	
			<i>Congenital, familial and genetic disorders (SMQ)</i>	20000077
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Orbital cavernous venous malformation to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026182013		Implementation Date: 05-Jul-26	Related CR: 2026182013	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	SCN1A gene mutation	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		SCN1A gene mutation	10093764
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT SCN1A gene mutation to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

CR Number: 2026182014		Implementation Date: 05-Jul-26	Related CR: 2026182014	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	Temple syndrome	Approved as Requested	Proposed PT	
To SMQ	Congenital, familial and genetic disorders (SMQ)		Temple syndrome	10093700
Term scope	Narrow		To SMQ	
			Congenital, familial and genetic disorders (SMQ)	20000077
			Term scope	
			Narrow	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Temple syndrome to Congenital, familial and genetic disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria for this SMQ which includes all PTs in SOC Congenital, familial and genetic disorders. All terms in this SMQ are narrow in scope.

Detail Report

CR Number: 2026182015		Implementation Date: 05-Jul-26	Related CR: 2026182015	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Bronchial inflammation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Hypersensitivity (SMQ)</i>		<i>Bronchial inflammation</i>	10093754
Term scope	<i>Broad</i>		To SMQ	
			<i>Hypersensitivity (SMQ)</i>	20000214
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Bronchial inflammation to Hypersensitivity (SMQ) as a Broad term is approved as requested. Bronchial inflammation is a general pathological process which can occur due to many causes (infection, smoke, asthma, chemical irritants, etc.). The term fits the broad scope of the SMQ, as hypersensitivity reactions, particularly asthma, can lead to bronchial inflammation, although there are other causes which are more frequent depending on the population.

CR Number: 2026182016		Implementation Date: 05-Jul-26	Related CR: 2026182016	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Cushing's triad</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Bradyarrhythmia terms, nonspecific (SMQ)</i>		<i>Cushing's triad</i>	10093734
Term scope	<i>Narrow</i>		To SMQ	
			<i>Bradyarrhythmia terms, nonspecific (SMQ)</i>	20000163
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Cushing's triad to Bradyarrhythmia terms, nonspecific (SMQ) as a Narrow term is approved as requested. Bradycardia in Cushing triad is mainly a baroreceptor-mediated vagal reflex in response to intracranial pressure-induced hypertension, sometimes compounded by direct medullary compression. It is a secondary arrhythmia and fits the inclusion criterion "All types of conditions which are related to cardiac arrhythmias, including conduction disturbances and repolarization defects."

Detail Report

CR Number: 2026182017		Implementation Date: 05-Jul-26	Related CR: 2026182017	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Corneal pseudomicrocysts</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Corneal disorders (SMQ)</i>		<i>Corneal pseudomicrocysts</i>	10093735
Term scope	<i>Narrow</i>		To SMQ	
			<i>Corneal disorders (SMQ)</i>	20000156
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Corneal pseudomicrocysts to Corneal disorders (SMQ) as a Narrow term is approved as requested. Corneal pseudomicrocysts are tiny, blister-like structures that form in the basal layer of the cornea's epithelial layer. Because they are not fluid-filled, they are classified as "pseudomicrocysts" and are commonly caused by certain targeted cancer therapies, specifically antibody-drug conjugates. As a condition directly affecting the cornea, the term fits the narrow scope.

CR Number: 2026182018		Implementation Date: 05-Jul-26	Related CR: 2026182018	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Oculopharyngeal muscular dystrophy</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Ocular motility disorders (SMQ)</i>		<i>Oculopharyngeal muscular dystrophy</i>	10093773
Term scope	<i>Broad</i>		To SMQ	
			<i>Ocular motility disorders (SMQ)</i>	20000184
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Oculopharyngeal muscular dystrophy to Ocular motility disorders (SMQ) as a Broad term is approved as requested. Oculopharyngeal muscular dystrophy is a rare, adult-onset, progressive myopathy characterized by progressive eyelid ptosis, ophthalmoplegia, dysphagia, dysarthria and proximal limb weakness. A mutation in the PABPN1 gene leads to progressive weakness, especially of the eyelids and throat muscles. It typically presents with drooping eyelids and difficulty swallowing in mid-to-late adulthood and may later involve proximal limb weakness in a slowly progressive course. The term will be added to the broad scope of the query, as ophthalmoplegia may only be present at a later stage in the disease.

Detail Report

CR Number: 2026182019		Implementation Date: 05-Jul-26	Related CR: 2026182019		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Orbital aspergillosis</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Ocular infections (SMQ)</i>		<i>Orbital aspergillosis</i>	10093698	
Term scope	<i>Narrow</i>		To SMQ		
			<i>Ocular infections (SMQ)</i>	20000183	
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Orbital aspergillosis to Ocular infections (SMQ) as a Narrow term is approved as requested. Orbital aspergillosis is a rare fungal infection mostly presenting as a unilateral orbital mass, which can cause eyelid swelling, proptosis, impairment of ocular motility, and/ or optic nerve compression, leading to vision loss. The term meets the inclusion criterion as a narrow scope term.

CR Number: 2026182020		Implementation Date: 05-Jul-26	Related CR: 2026182020		
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>	
Proposed PT	<i>Bone lesion ablation</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Malignancy related therapeutic and diagnostic procedures (SMQ)</i>		<i>Bone lesion ablation</i>	10093713	
Term scope	<i>Narrow</i>		To SMQ		
			<i>Malignancy related therapeutic and diagnostic procedures (SMQ)</i>	20000093	
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Bone lesion ablation to Malignancy related therapeutic and diagnostic procedures (SMQ) as a Narrow term is approved as requested. Bone lesion ablation is a minimally invasive surgical technique that employs heat or cold energy to treat bone tumors without traditional surgery. The technique uses image guidance such as ultrasound, magnetic resonance imaging, or computed tomography scan to pass a needle or probe through the skin into the bone tumor to destroy the tumor cells instead of surgically removing the tumor.

Detail Report

CR Number: 2026182021		Implementation Date: 05-Jul-26	Related CR: 2026182021		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Pituitary adenoma</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Non-haematological tumours of unspecified malignancy (SMQ)</i>		<i>Pituitary adenoma</i>		10035079
Term scope	<i>Narrow</i>		To SMQ		
			<i>Non-haematological tumours of unspecified malignancy (SMQ)</i>		20000230
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Pituitary adenoma to Non-haematological tumours of unspecified malignancy (SMQ) as a Narrow term is approved as requested. Most adenoma and Pituitary neuroendocrine tumors are benign histologically, but some can invade locally and metastasize and can have a massive impact due to compression effects on adjacent pituitary tissue. Other pituitary adenoma terms, PT Secretory adenoma of pituitary, PT Non-secretory adenoma of pituitary are already part of the SMQ which only has narrow scope terms.

CR Number: 2026182022		Implementation Date: 05-Jul-26	Related CR: 2026182022		MedDRA Code #
Change Requested	Add term to SMQ	Final Disposition	Final Placement		
Proposed PT	<i>Oral varices</i>	Approved as Requested	Proposed PT		
To SMQ	<i>Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ)</i>		<i>Oral varices</i>		10093794
Term scope	<i>Narrow</i>		To SMQ		
			<i>Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ)</i>		20000114
			Term scope		
			<i>Narrow</i>		
			Term_category		
			A		
			Term_weight		
			0		

MSSO Comment: The proposal to add PT Oral varices to Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ) as a Narrow term is approved as requested. PT Oral varices was added in version 29.1 and matches the inclusion criterion "lesions not clearly related to specific infections, neoplasms or allergic reactions but due to the following etiology injuries, ulcers, hypoesthesias, hemorrhages, unspecified disorders". All PTs of Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ) are narrow in scope.

Detail Report

CR Number: 2026182023		Implementation Date: 05-Jul-26	Related CR: 2026182023	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Pneumoparotid</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ)</i>		<i>Pneumoparotid</i>	10093813
Term scope	<i>Narrow</i>		To SMQ	
			<i>Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ)</i>	20000114
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Pneumoparotid to Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ) as a Narrow term is approved as requested. PT Pneumoparotid was added in version 29.1 and represents a condition characterized by the presence of air within the parotid gland and/or its ductal system due to retrograde insufflation of air through the parotid duct. It may present with parotid swelling, pain, crepitus, or secondary inflammation of the gland. The PT matches the inclusion criterion "lesions not clearly related to specific infections, neoplasms or allergic reactions but due to the following etiology injuries, ulcers, hypoesthesias, hemorrhages, unspecified disorders". All PTs of Oropharyngeal conditions (excl neoplasms, infections and allergies) (SMQ) are narrow in scope.

CR Number: 2026182024		Implementation Date: 05-Jul-26	Related CR: 2026182024	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Refractory condition</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Lack of efficacy/effect (SMQ)</i>		<i>Refractory condition</i>	10093722
Term scope	<i>Narrow</i>		To SMQ	
			<i>Lack of efficacy/effect (SMQ)</i>	20000032
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Refractory condition to Lack of efficacy/effect (SMQ) as a Narrow term is approved as requested. PT Refractory condition was added in 29.1 and describes the absence of response to standard therapy and meets the SMQ definition "Lack of efficacy/effect is therefore evidence of less than the expected effect of a product". All PTs of Lack of efficacy/effect (SMQ) are narrow in scope.

Detail Report

CR Number: 2026182025		Implementation Date: 05-Jul-26	Related CR: 2026182025	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Epileptic spasms</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Convulsions (SMQ)</i>		<i>Epileptic spasms</i>	10093770
Term scope	<i>Narrow</i>		To SMQ	
			<i>Convulsions (SMQ)</i>	20000079
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Epileptic spasms to Convulsions (SMQ) as a Narrow term is approved as requested. Epileptic spasms occur most frequently, but not always, as a generalized seizure in infants with additional severe neurologic comorbidities (i.e., EEG abnormalities and developmental delay/regression). Because this term describes a specific type of seizure, it will be included in the narrow search terms of this SMQ.

CR Number: 2026182026		Implementation Date: 05-Jul-26	Related CR: 2026182026	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Frontal lobe stroke</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Ischaemic central nervous system vascular conditions (SMQ)</i>		<i>Frontal lobe stroke</i>	10093811
Term scope	<i>Narrow</i>		To SMQ	
			<i>Ischaemic central nervous system vascular conditions (SMQ)</i>	20000063
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Frontal lobe stroke to Ischaemic central nervous system vascular conditions (SMQ) as a Narrow term is approved as requested. Frontal lobe stroke is a cerebrovascular event caused by interruption of blood flow or bleeding affecting the frontal lobe of the brain. Because the frontal lobe is responsible for voluntary motor control, language production, executive functions, behavior, personality, and emotional regulation, damage to this region may result in motor deficits, speech impairments, cognitive dysfunction, and behavioral or personality changes. This term, which describes a specific type of stroke associated with an ischemic mechanism, will be added to the narrow search terms of this SMQ.

Detail Report

CR Number: 2026182027		Implementation Date: 05-Jul-26	Related CR: 2026182027	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Frontal lobe stroke</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Haemorrhagic central nervous system vascular conditions (SMQ)</i>		<i>Frontal lobe stroke</i>	10093811
Term scope	<i>Narrow</i>		To SMQ	
			<i>Haemorrhagic central nervous system vascular conditions (SMQ)</i>	20000064
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Frontal lobe stroke to Haemorrhagic central nervous system vascular conditions (SMQ) as a Narrow term is approved as requested. Frontal lobe stroke is a cerebrovascular event caused by interruption of blood flow or bleeding affecting the frontal lobe of the brain. Because the frontal lobe is responsible for voluntary motor control, language production, executive functions, behavior, personality, and emotional regulation, damage to this region may result in motor deficits, speech impairments, cognitive dysfunction, and behavioral or personality changes. This term, which describes a specific type of stroke associated with a hemorrhagic mechanism, will be added to the narrow search terms of this SMQ.

CR Number: 2026182028		Implementation Date: 05-Jul-26	Related CR: 2026182028	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Progressive solitary sclerosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Demyelination (SMQ)</i>		<i>Progressive solitary sclerosis</i>	10093733
Term scope	<i>Narrow</i>		To SMQ	
			<i>Demyelination (SMQ)</i>	20000154
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Progressive solitary sclerosis to Demyelination (SMQ) as a Narrow term is approved as requested. Progressive solitary sclerosis is a rare, chronic, demyelinating CNS disorder. It is characterized by a single persistent lesion causing slowly progressive neurological impairment, typically motor, without fulfilling conventional diagnostic criteria for multiple sclerosis (due to the absence of typical multifocal dissemination). This term, which describes a specific type of demyelinative disorder, will be included in the narrow search terms of this SMQ.

Detail Report

CR Number: 2026182029		Implementation Date: 05-Jul-26	Related CR: 2026182029	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>SCN1A gene mutation</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Convulsions (SMQ)</i>		<i>SCN1A gene mutation</i>	10093764
Term scope	<i>Broad</i>		To SMQ	
			<i>Convulsions (SMQ)</i>	20000079
			Term scope	
		<i>Broad</i>		
			Term_category	
			<i>A</i>	
			Term_weight	
			<i>0</i>	

MSSO Comment: The proposal to add PT SCN1A gene mutation to Convulsions (SMQ) as a Broad term is approved as requested. SCN1A-related epilepsy is a spectrum of genetic epilepsies caused by mutations in the SCN1A gene, ranging from mild febrile seizure disorders to severe epileptic encephalopathies such as Dravet syndrome. The concept is clinically important because some sodium channel-blocking antiepileptic drugs may worsen certain SCN1A-associated epilepsies. This term, which describes a gene mutation which may be associated with seizures, will be added to the broad search terms of this SMQ.

CR Number: 2026182030		Implementation Date: 05-Jul-26	Related CR: 2026182030	
<u>Change Requested</u>	Add term to SMQ	<u>Final Disposition</u>	<u>Final Placement</u>	<u>MedDRA Code #</u>
Proposed PT	<i>Post-traumatic pseudolipoma</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Accidents and injuries (SMQ)</i>		<i>Post-traumatic pseudolipoma</i>	10093799
Term scope	<i>Narrow</i>		To SMQ	
			<i>Accidents and injuries (SMQ)</i>	20000135
			Term scope	
		<i>Narrow</i>		
		Term_category		
		<i>A</i>		
		Term_weight		
		<i>0</i>		

MSSO Comment: The proposal to add PT Post-traumatic pseudolipoma to Accidents and injuries (SMQ) as a Narrow term is approved as requested. A post-traumatic pseudolipoma (also known as post-traumatic lipohypertrophy or fat fracture) is a benign adipose tissue mass that develops following acute, chronic, or repetitive injury. Unlike conventional lipomas, post-traumatic pseudolipomas lack a fibrous capsule and are thought to arise from the displacement or herniation of subcutaneous fat. This term, which describes a specific type of injury-related lesion, will be added to the narrow search terms of this SMQ.

Detail Report

CR Number: 2026182031		Implementation Date: 05-Jul-26	Related CR: 2026182031	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Procollagen type I N-terminal propeptide decreased</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Osteoporosis/osteopenia (SMQ)</i>		<i>Procollagen type I N-terminal propeptide decreased</i>	10093807
Term scope	<i>Broad</i>		To SMQ	
			<i>Osteoporosis/osteopenia (SMQ)</i>	20000178
			Term scope	
			<i>Broad</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Procollagen type I N-terminal propeptide decreased to Osteoporosis/osteopenia (SMQ) as a Broad term is approved as requested. Serum procollagen type I N-terminal propeptide (PINP) is a sensitive marker of bone formation and is widely used to assess osteoblastic activity. Serum procollagen type I N-terminal propeptide is designated the reference marker of bone formation in osteoporosis. PT Serum procollagen type I N-terminal propeptide decreased was demoted under new PT Procollagen type I N-terminal propeptide decreased since specimens are not generally represented at PT level. The new PT will be included under Broad scope of the SMQ, similar to the previous and now demoted PT, as terms related to a physiologic finding associated with decreased bone density and a resultant risk for osteoporosis.

CR Number: 2026182032		Implementation Date: 05-Jul-26	Related CR: 2026182032	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Postoperative peritonitis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Gastrointestinal perforation (SMQ)</i>		<i>Postoperative peritonitis</i>	10093815
Term scope	<i>Narrow</i>		To SMQ	
			<i>Gastrointestinal perforation (SMQ)</i>	20000107
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Postoperative peritonitis to Gastrointestinal perforation (SMQ) as a Narrow term is approved as requested. Postoperative peritonitis is most times due to peritoneal contamination following loss of integrity of the gastrointestinal tract or due to an infected viscera or collection. The more frequent cause is anastomotic dehiscence or leakage, especially after gastrointestinal surgery; it may also result from an iatrogenic perforation, intestinal ischemia/necrosis, biliary or pancreatic leakage, or intraoperative contamination. According to the first inclusion criterium, "terms related to perforation of any part of gastrointestinal tract" should be added to the SMQ. It is also of note that several peritonitis related PTs and the PT Gastrointestinal anastomotic leak (the most frequent cause of postoperative peritonitis) are in the the SMQ.

Detail Report

CR Number: 2026182033		Implementation Date: 05-Jul-26	Related CR: 2026182033	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Orbital aspergillosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Opportunistic infections (SMQ)</i>		<i>Orbital aspergillosis</i>	10093698
Term scope	<i>Narrow</i>		To SMQ	
			<i>Opportunistic infections (SMQ)</i>	20000235
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Orbital aspergillosis to Opportunistic infections (SMQ) as a Narrow term is approved as requested. Although it can also occur in otherwise immunocompetent hosts (e.g., older adults, patients with chronic sinus disease after local tissue damage or surgery) orbital aspergillosis is predominantly an opportunistic invasive fungal infection. Its site qualification indicates a serious and invasive infection rather than a superficial infection, aligning with the SMQ criterion that deep or disseminated infections are mostly narrow.

CR Number: 2026182034		Implementation Date: 05-Jul-26	Related CR: 2026182034	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Attenuated live vaccine-derived infection</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Opportunistic infections (SMQ)</i>		<i>Attenuated live vaccine-derived infection</i>	10093709
Term scope	<i>Narrow</i>		To SMQ	
			<i>Opportunistic infections (SMQ)</i>	20000235
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Attenuated live vaccine-derived infection to Opportunistic infections (SMQ) as a Narrow term is approved as requested. A live attenuated vaccine-derived infection may behave like an infection caused by an organism that would not normally cause clinically relevant disease in an immunocompetent host, but that may be clinically important in immunocompromised individuals. Although there is no reliable overall proportion, because it depends greatly on the specific attenuated live vaccine involved, there are certain frequent vaccines which are clearly more frequent in the immunosuppressed patient, such as disseminated BCG infection, persistent vaccine-derived rotavirus infection, disseminated vaccine-strain varicella infection or severe vaccine-strain measles.

Detail Report

CR Number: 2026182035		Implementation Date: 05-Jul-26	Related CR: 2026182035	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Progressive solitary sclerosis</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Immune-mediated/autoimmune disorders (SMQ)</i>		<i>Progressive solitary sclerosis</i>	10093733
Term scope	<i>Narrow</i>		To SMQ	
			<i>Immune-mediated/autoimmune disorders (SMQ)</i>	20000236
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Progressive solitary sclerosis to Immune-mediated/autoimmune disorders (SMQ) as a Narrow term is approved as requested. Progressive solitary sclerosis should be included as a narrow scope term because it represents a defined progressive inflammatory demyelinating disorder of the central nervous system closely related to the multiple sclerosis spectrum, and therefore with an immune-mediated pathophysiological basis or strong evidence supporting such a mechanism. This is aligned with the SMQ criterion allowing inclusion of conditions with strong evidence of an autoimmune or immune-mediated process, even if not fully confirmed.

CR Number: 2026182036		Implementation Date: 06-Jul-26	Related CR: 2026182036	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Nail lichen planus</i>	Approved Not as Requested	Proposed PT	
To SMQ	<i>Immune-mediated/autoimmune disorders (SMQ)</i>		<i>Nail lichen planus</i>	10093742
Term scope	<i>Broad</i>		To SMQ	
			<i>Immune-mediated/autoimmune disorders (SMQ)</i>	20000236
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Nail lichen planus to Immune-mediated/autoimmune disorders (SMQ) as a broad term is approved but not as requested. Nail lichen planus is an inflammatory, autoimmune condition where the immune system mistakenly attacks the nail matrix and nail bed. It should be included as a narrow scope term because it is a specific clinical entity within the lichen planus spectrum, and the term is appropriate for narrow scope inclusion. PT Nail lichen planus will be added to Immune-mediated/autoimmune disorders (SMQ) as a Narrow scope term.

Detail Report

CR Number: 2026182037		Implementation Date: 05-Jul-26	Related CR: 2026182037	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Frontal lobe stroke</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Embolic and thrombotic events, vessel type unspecified and mixed arterial and venous (SMQ)</i>		<i>Frontal lobe stroke</i>	10093811
Term scope	<i>Narrow</i>		To SMQ	
			<i>Embolic and thrombotic events, vessel type unspecified and mixed arterial and venous (SMQ)</i>	20000083
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Frontal lobe stroke to Embolic and thrombotic events, vessel type unspecified and mixed arterial and venous (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criteria of this SMQ for stroke related events. Similar terms, such as PT Occipital lobe stroke and PT Parietal lobe stroke are already included in this SMQ. All terms are narrow scope.

CR Number: 2026182038		Implementation Date: 05-Jul-26	Related CR: 2026182038	
Change Requested	Add term to SMQ	Final Disposition	Final Placement	MedDRA Code #
Proposed PT	<i>Ligament excision</i>	Approved as Requested	Proposed PT	
To SMQ	<i>Tendinopathies and ligament disorders (SMQ)</i>		<i>Ligament excision</i>	10093701
Term scope	<i>Narrow</i>		To SMQ	
			<i>Tendinopathies and ligament disorders (SMQ)</i>	20000223
			Term scope	
			<i>Narrow</i>	
			Term_category	
			A	
			Term_weight	
			0	

MSSO Comment: The proposal to add PT Ligament excision to Tendinopathies and ligament disorders (SMQ) as a Narrow term is approved as requested. This term fits the inclusion criterion for the therapeutic procedure for ligament disorders, as a narrow scope term. Similar terms such as PT Ligament operation and PT Tendon operation are already included in the narrow scope of the SMQ.